Mercy Health
Care first

McLean, Richard **2645287 (MHS)**
08-Apr-1973 - M
2 Mc Cubbin Street,
Footscray, 3011

Exported Patient Record
DO NOT MARK

Export details

FOI REQUEST

Export Type	Full
Generated	29-Apr-2021 11:32:14
Published	29-Apr-2021 11:39:54
Number of documents	247

Please note: Number of documents equals the number of documents exported from the patient record excluding this coversheet.

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1. Cover

1.1. iPM alert (FNH010150)

Alerts

Current Alerts for MHS

Type	Description	Identification date	End date
MENT	ALERT - HISTORY OF TRAUMA	20210216	
MENT	AT RISK - SUICIDE	20210216	
ADV	DRUG KNOWN ALLERGY - SEE MR / MASTERCARE	20210304	
ADV	ALLERGY - ANIMAL - SEE MR / MASTERCARE	20210304	

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1.2. Clinical Alert (FMH001000)

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Mercy Health

CLINICAL ALERT

UR No.
2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C
(if no UR)

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH001000

1st presentation complete this page
Subsequent presentations, admissions or research participation – complete over page

RED ID BAND ALERTS – (Specify medications, type of tapes or bandages, food or latex)

ALLERGY / SENSITIVITY	REACTION	START DATE	CEASED DATE if relevant	SIGNATURE Name & Designation
☐ Not applicable				
Bee Stings	localised swelling			F. Mumtaz RN
Augmentin	Vomiting	25/2		F. Mumtaz RN

OTHER ALERTS – use normal WHITE ID BAND

- ☐ **Infectious risk** (patient requires specialised management compared to other patients – MRSA, VRE, CPE)
- ☐ **Clinical** (patient requires specialised management compared to other patients, supported care)
- ☐ **Clinical Alert Complex Management Plan** (see form ALERT 0002)
- ☐ **Advance Care Directive** (see behind legal document divider)
- ☐ **Appointment of a Medical Treatment Decision Maker** (see behind legal document divider)
- ☐ **Appointment of a support person**
- ☐ **Aggression, risk of harm to others, Code Grey / Black**
- ☐ **Other alert/s** (NFR, research trial, absconding risk, self harm/suicide, mental health program)

ALERT DESCRIPTION	START DATE	CEASED DATE if relevant	SIGNATURE Name & Designation
☐ Not applicable			

BINDING MARGIN – NO WRITING

CLINICAL ALERT ALERT 0001

P2009 V4 06/18

NO WRITING

Page 1 of 2

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1.3. Clinical Alert (FMH001000)

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Mercy Health

UR No:	
Family Name:	
Given Name	
DOB:	Sex:
Address: (if no UR)	

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

CLINICAL ALERT

ALERTS ON FRONT REVIEWED AND UPDATED

REVIEW DATE	REVIEWED BY (Print name)	SIGNATURE & DESIGNATION	COMMENTS

RESEARCH PARTICIPATION

DATE COMMENCED	DATE COMPLETED	RESEARCH STUDY	DEPARTMENT	SIGNATURE

BINDING MARGIN - NO WRITING

Page 2 of 2

NO WRITING

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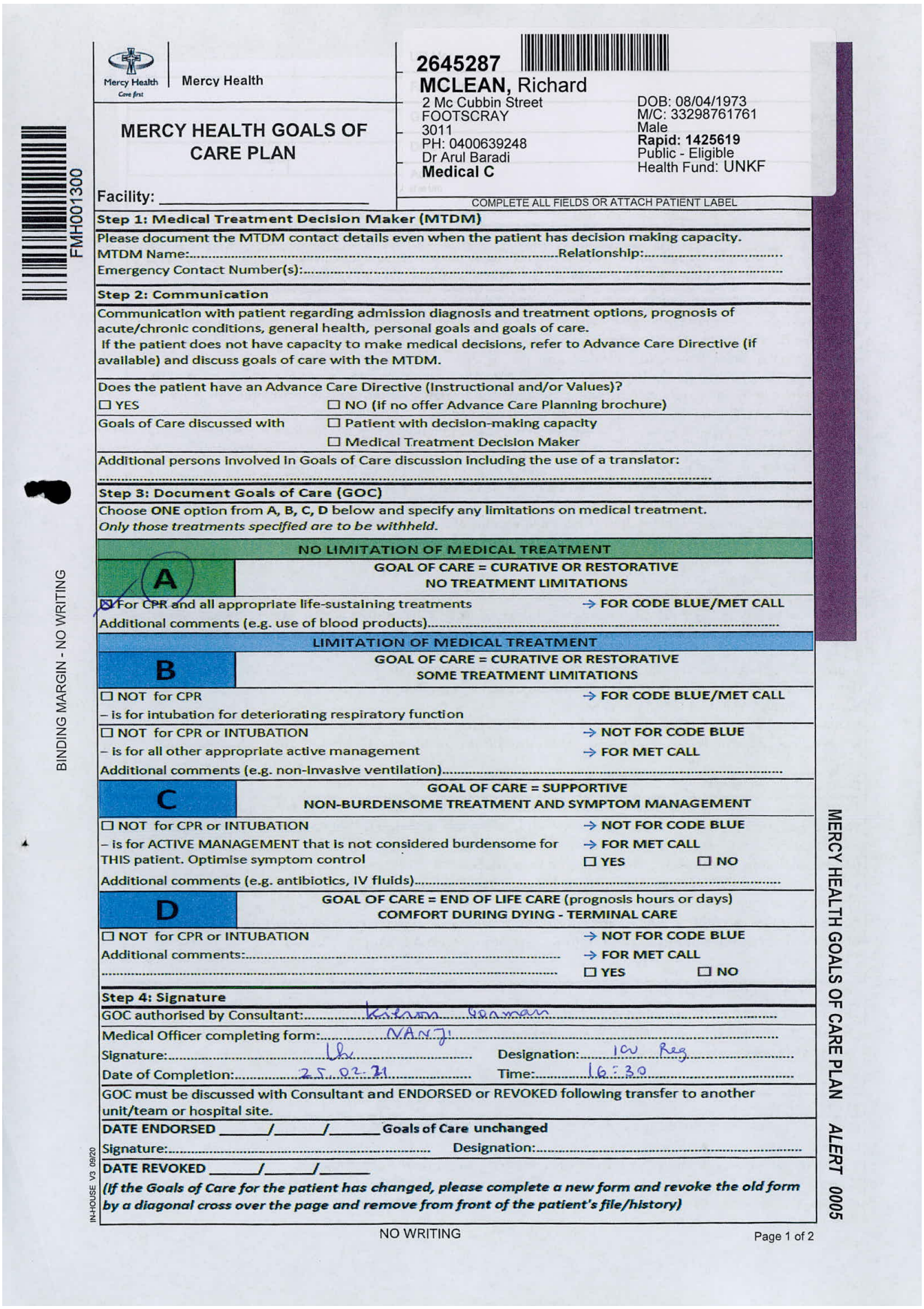
1.4. Mercy Health Goals of Care Plan (FMH001300)



Mercy Health
Care first

Mercy Health

MERCY HEALTH GOALS OF CARE PLAN

Facility: _____

2645287

MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH001300

Step 1: Medical Treatment Decision Maker (MTDM)

Please document the MTDM contact details even when the patient has decision making capacity.
MTDM Name:.....Relationship:.....
Emergency Contact Number(s):.....

Step 2: Communication

Communication with patient regarding admission diagnosis and treatment options, prognosis of acute/chronic conditions, general health, personal goals and goals of care.
If the patient does not have capacity to make medical decisions, refer to Advance Care Directive (if available) and discuss goals of care with the MTDM.

Does the patient have an Advance Care Directive (Instructional and/or Values)?
☐ YES ☐ NO (If no offer Advance Care Planning brochure)
Goals of Care discussed with ☐ Patient with decision-making capacity
☐ Medical Treatment Decision Maker
Additional persons involved in Goals of Care discussion including the use of a translator:
.....

Step 3: Document Goals of Care (GOC)

Choose **ONE** option from **A, B, C, D** below and specify any limitations on medical treatment.
Only those treatments specified are to be withheld.

NO LIMITATION OF MEDICAL TREATMENT

A — **GOAL OF CARE = CURATIVE OR RESTORATIVE**
NO TREATMENT LIMITATIONS

☒ For CPR and all appropriate life-sustaining treatments → **FOR CODE BLUE/MET CALL**
Additional comments (e.g. use of blood products).....

LIMITATION OF MEDICAL TREATMENT

B — **GOAL OF CARE = CURATIVE OR RESTORATIVE**
SOME TREATMENT LIMITATIONS

☐ NOT for CPR → **FOR CODE BLUE/MET CALL**
– is for intubation for deteriorating respiratory function
☐ NOT for CPR or INTUBATION → **NOT FOR CODE BLUE**
– is for all other appropriate active management → **FOR MET CALL**
Additional comments (e.g. non-invasive ventilation).....

C — **GOAL OF CARE = SUPPORTIVE**
NON-BURDENSOME TREATMENT AND SYMPTOM MANAGEMENT

☐ NOT for CPR or INTUBATION → **NOT FOR CODE BLUE**
– is for ACTIVE MANAGEMENT that is not considered burdensome for THIS patient. Optimise symptom control → **FOR MET CALL**
☐ YES ☐ NO
Additional comments (e.g. antibiotics, IV fluids).....

D — **GOAL OF CARE = END OF LIFE CARE** (prognosis hours or days)
COMFORT DURING DYING - TERMINAL CARE

☐ NOT for CPR or INTUBATION → **NOT FOR CODE BLUE**
Additional comments:..... → **FOR MET CALL**
..... ☐ YES ☐ NO

Step 4: Signature

GOC authorised by Consultant: Kirsten Goonan
Medical Officer completing form: NANJ
Signature: ..... Designation: ICU Reg
Date of Completion: 25.02.21 Time: 16:30

GOC must be discussed with Consultant and ENDORSED or REVOKED following transfer to another unit/team or hospital site.

DATE ENDORSED _____/_____/_____ **Goals of Care unchanged**
Signature:..... Designation:.....

DATE REVOKED _____/_____/_____
(If the Goals of Care for the patient has changed, please complete a new form and revoke the old form by a diagonal cross over the page and remove from front of the patient's file/history)

BINDING MARGIN - NO WRITING

MERCY HEALTH GOALS OF CARE PLAN ALERT 0005

IN-HOUSE V3 09/20

NO WRITING

Page 1 of 2

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1.5. Mercy Health Goals of Care Plan (FMH001300)

Mercy Health Care first | Mercy Health

MERCY HEALTH GOALS OF CARE PLAN

Facility: _____

2645287

MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

INSTRUCTIONS

This form is to be completed for all patients (adult/paediatric) within 24 hours of admission once it is clear that they meet the inclusion criteria set out in the Goals of Care Procedure.

Step 1: Medical Treatment Decision Maker (MTDM)

A patient with decision making capacity should always be involved in discussions regarding medical treatment decisions. An adult is assumed to have capacity unless there is evidence to the contrary.
Note that in the event of an acute deterioration during this admission, a currently competent patient may lose the capacity to engage in discussions about medical treatment. For this reason it is important to use the hierarchy below to establish who would become their medical treatment decision maker. Please document the MTDM name and contact details on this form.
The person with legal authority to make medical decisions for the patient is derived from this hierarchy.
1) The Appointed Medical Treatment Decision Maker
2) A guardian appointed by VCAT to make decisions about medical/dental treatment
3) The first of the following people who is in a close and continuing relationship with the person (patient)
a. The patient's spouse or domestic partner
b. The patient's primary carer, including carers in receipt of a Centrelink Carer's payment but excluding paid carers or service providers
c. An adult child of the patient
d. A parent of the patient
e. An adult sibling of the patient.
If there are two or more children, parents or siblings, start with the eldest person and if they are not reasonably willing, available and able to make decisions for the patient, move to the second eldest, and so on.
Source: Office of the Public Advocate www.publicadvocate.vic.go.au
Has the Medical Treatment Decision Maker been formally appointed?
• Ask to see the original MTDM paperwork.
• Ensure a copy of the MTDM paperwork is available in the patient's medical record.
NB: Medical Enduring Power of Attorney appointed before 12 March 2018 is recognised as Appointed MTDM.

Step 2: Communication

This must be a two way discussion where the clinician seeks to understand what is important to the patient and then explains the medical treatment options in a way that is meaningful for the patient's individual circumstances. The aim is a shared understanding of what the goals of medical care are for that patient, including what treatments would not be provided.
When a patient does not have decision-making capacity, Goals of Care should be informed by the patient's Advance Care Directive.
An Instructional Directive gives binding consent to or refusal of specific medical treatment. A Values Directive must be considered by the Medical Treatment Decision Maker (MTDM).

Step 3: Document Goals of Care

The patient must have Goals of Care Plan chosen for the form.
Choose ONE option from A, B, C, D and specify any limitations on medical treatment.
Only those treatments specified are to be withheld.

Step 4: Signature

Name of Consultant authorising Goals of Care must be documented. Approval may be in person or by phone.
The form must be signed and dated by the Medical Officer completing the form.
When a patient is transferred between participating precinct hospitals or to another unit, the receiving Medical Officer must discuss GOC with Consultant and either ENDORSE or REVOKE the current form.

IN-HOUSE V3 09/20

BINDING MARGIN - NO WRITING

NO WRITING

Page 2 of 2

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2. Diagnostic

2.1. FULL BLOOD EXAMINATION

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: FULL BLOOD EXAMINATION
Group:
Laboratory #: 21-47478099-FBE-0
Clinician: PSYCHIATRIC UNIT,WERRIBEE (0000000Y)
Copies To:
Ordering Ward: WMC
Ordering Unit: AKINA

Collected: 05-Mar-21, 09:35
Received: 05-Mar-21, 12:07
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

FULL BLOOD EXAMINATION

Coll.Date:	28/02/21		01/03/21		05/03/21			
Coll.Time:	06:35		07:45		09:35			
Lab.No:	31331326		31331519		47478099			
							Units	Ref.Range
Haemoglobin:	81	**	88	**	117	*	g/L	(130-180)
WCC:	6.5		5.1		5.6		x10 ^9 /L	(4.0-11.0)
Platelets:	113	*	149	*	349		x10 ^9 /L	(150-450)
RCC:	2.56		2.75	**	3.67	*	x10 ^12 /L	(4.50-6.50)
PCV:	0.24	*	0.26	*	0.35	*	L/L	(0.40-0.54)
MCV:	95		94		97	*	fL	(80-96)
MCH:	32		32		32		pg	(27-32)
MCHC:	335		341		331		g/L	(320-360)
Neutrophils:	3.4		2.3		3.4		x10 ^9 /L	(2.0-8.0)
Lymphocytes:	2.6		2.1		1.6		x10 ^9 /L	(1.0-4.0)
Monocytes:	0.5		0.5		0.4		x10 ^9 /L	(0.0-1.0)
Eosinophils:	0.1		0.1		0.1		x10 ^9 /L	(0.0-0.5)
Basophils:	0.0		0.1		0.1		x10 ^9 /L	(0.0-0.2)

28/02/21 31331326 Progress counts.

01/03/21 31331519 The platelet count is borderline low.

05/03/21 47478099 There is a mild anaemia.

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User Event History

User Id	Date/Time	Type	State	Text
	05-Mar-21, 12:09	Signing	Unsign	Unsigning from the translator
	05-Mar-21, 12:09	Mark for Attention	Mark	Marking for attention from the translator
	05-Mar-21, 12:08	Signing	Not For Signing	

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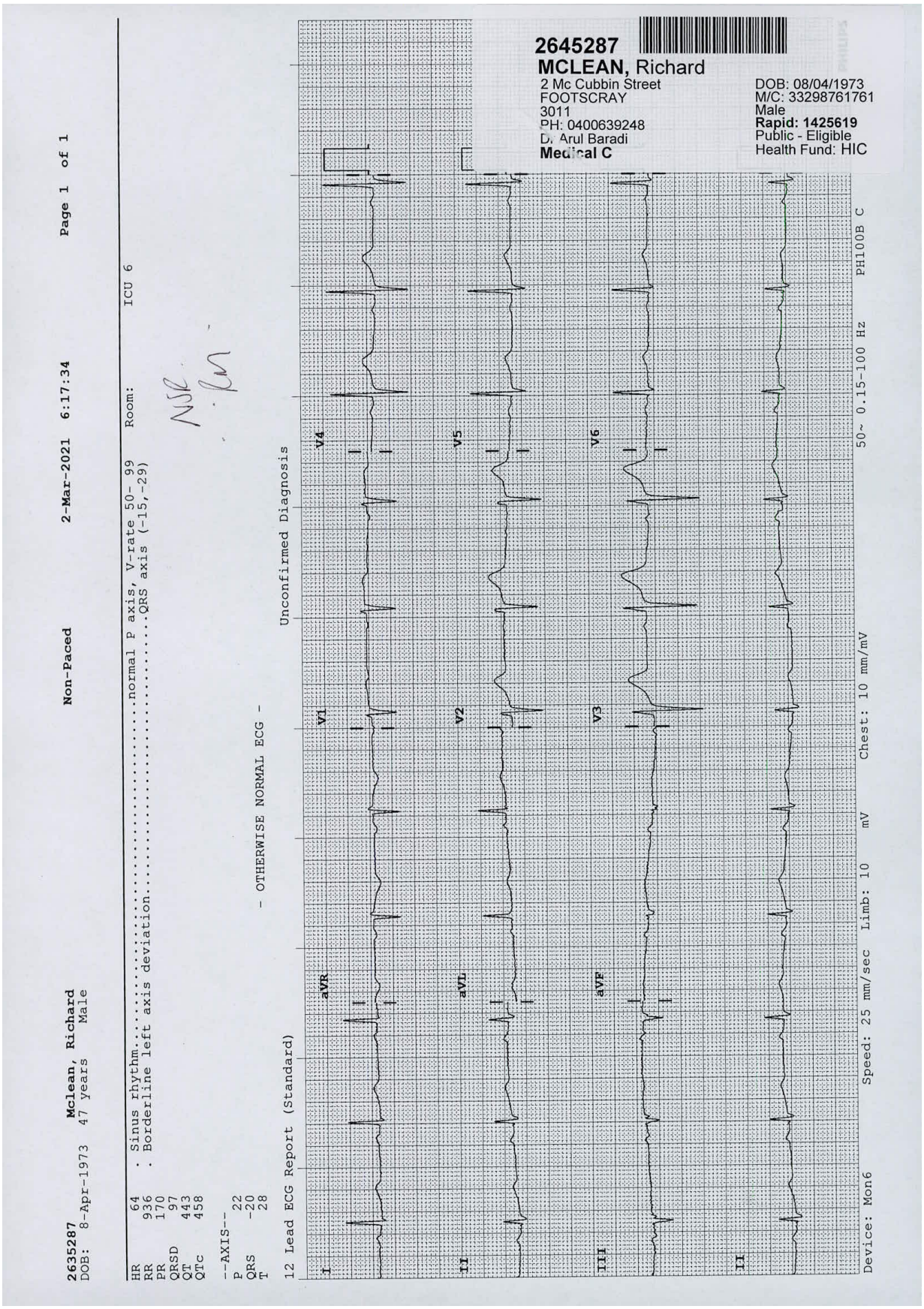
2.2. ECG Graph ECG Report (FMH300500)



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Mercy Health
Care first

2.3. GENERAL BIOCHEMISTRY

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: GENERAL BIOCHEMISTRY
Group:
Laboratory #: 21-31331519-MBI-0
Clinician: BARADI,ARUL (435647LJ)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 01-Mar-21, 07:45
Received: 01-Mar-21, 08:14
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

SERUM/PLASMA BIOCHEMISTRY

Coll.Date: 01/03/21
Coll.Time: 07:45
Lab.No: 31331519

				Units	Ref.Range
Sodium:	142	--	--	mmol/L	(135-145)
Potassium:	3.4 *	--	--	mmol/L	(3.5-5.2)
Chloride:	106	--	--	mmol/L	(95-110)
Bicarbonate:	28	--	--	mmol/L	(22-32)
Urea:	3.5	--	--	mmol/L	(2.3-7.6)
Est. GFR:	> 90	--	--	mL/min	(> 60)
Creatinine:	82	--	--	umol/L	(60-110)
Albumin:	29 *	--	--	g/L	(36-49)
Calcium:	1.96 *	--	--	mmol/L	(2.15-2.65)
Cor.Calcium:	2.18	--	--	mmol/L	(2.15-2.65)
Phosphate:	1.01	--	--	mmol/L	(0.75-1.50)
Magnesium:	0.76	--	--	mmol/L	(0.60-1.10)

Coll.Date: 01/03/21
Coll.Time: 07:45

User Event History

User Id	Date/Time	Type	State	Text
	01-Mar-21, 08:49	Signing	Unsign	Unsigning from the translator
	01-Mar-21, 08:49	Mark for Attention	Mark	Marking for attention from the translator
	01-Mar-21, 08:14	Signing	Not For Signing	

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2.4. FULL BLOOD EXAMINATION

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: FULL BLOOD EXAMINATION
Group:
Laboratory #: 21-31331519-FBE-0
Clinician: BARADI,ARUL (435647LJ)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 01-Mar-21, 07:45
Received: 01-Mar-21, 08:14
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

FULL BLOOD EXAMINATION

Coll.Date:	28/02/21		01/03/21			
Coll.Time:	06:35		07:45			
Lab.No:	31331326		31331519			
					Units	Ref.Range
Haemoglobin:	81	**	88	**	g/L	(130-180)
WCC:	6.5		5.1	--	x10 ^9 /L	(4.0-11.0)
Platelets:	113	*	149	*	x10 ^9 /L	(150-450)
RCC:	2.56		2.75	**	x10 ^12 /L	(4.50-6.50)
PCV:	0.24	*	0.26	*	L/L	(0.40-0.54)
MCV:	95		94	--	fL	(80-96)
MCH:	32		32	--	pg	(27-32)
MCHC:	335		341	--	g/L	(320-360)
Neutrophils:	3.4		2.3	--	x10 ^9 /L	(2.0-8.0)
Lymphocytes:	2.6		2.1	--	x10 ^9 /L	(1.0-4.0)
Monocytes:	0.5		0.5	--	x10 ^9 /L	(0.0-1.0)
Eosinophils:	0.1		0.1	--	x10 ^9 /L	(0.0-0.5)
Basophils:	0.0		0.1	--	x10 ^9 /L	(0.0-0.2)

28/02/21 31331326 Progress counts.

01/03/21 31331519 The platelet count is borderline low.

User Event History

User Id	Date/Time	Type	State	Text

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	01-Mar-21, 08:24	Signing	Unsign	Unsigning from the translator
	01-Mar-21, 08:24	Mark for Attention	Mark	Marking for attention from the translator
	01-Mar-21, 08:14	Signing	Not For Signing	

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Care first

2.5. BLOOD CULTURE M/C/S

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: BLOOD CULTURE M/C/S
Group:
Laboratory #: 21-31331519-BZC-0
Clinician: BARADI,ARUL (435647LJ)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 01-Mar-21, 07:45
Received: 01-Mar-21, 08:14

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			

```
BLOOD CULTURE

   1 aerobic and 1 anaerobic bottle received.
   COLLECTED:  7:45 am on 01/03/2021

   CULTURE:
      No growth at 48 hours

      Culture continues but no further report will be issued unless bottles
      become positive.
```

User Event History

User Id	Date/Time	Type	State	Text
	03-Mar-21, 14:12	Signing	Not For Signing	
	02-Mar-21, 14:11	Signing	Not For Signing	
	01-Mar-21, 08:14	Signing	Not For Signing	

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Care first

2.6. BLOOD CULTURE M/C/S

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: BLOOD CULTURE M/C/S
Group:
Laboratory #: 21-31331364-BZC-0
Clinician: BARADI,ARUL (435647LJ)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 28-Feb-21, 12:30
Received: 28-Feb-21, 12:53

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			

```
BLOOD CULTURE

   1 aerobic and 1 anaerobic bottle received.
   COLLECTED: 12:30 pm on 28/02/2021

   CULTURE:
      No growth at 48 hours

      Culture continues but no further report will be issued unless bottles
      become positive.
```

User Event History

User Id	Date/Time	Type	State	Text
	03-Mar-21, 00:10	Signing	Not For Signing	
	02-Mar-21, 00:10	Signing	Not For Signing	
	28-Feb-21, 12:53	Signing	Not For Signing	

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Mercy Health
Care first

2.7. GENERAL BIOCHEMISTRY

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: GENERAL BIOCHEMISTRY
Group:
Laboratory #: 21-31331326-MBI-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 28-Feb-21, 06:35
Received: 28-Feb-21, 07:04
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

SERUM/PLASMA BIOCHEMISTRY

Coll.Date:	27/02/21	28/02/21			
Coll.Time:	03:00	06:35			
Lab.No:	30679910	31331326			
				Units	Ref.Range
Sodium:	142	142	--	mmol/L	(135-145)
Potassium:	4.3	3.7	--	mmol/L	(3.5-5.2)
Chloride:	110	109	--	mmol/L	(95-110)
Bicarbonate:	27	27	--	mmol/L	(22-32)
Urea:	4.1	3.1	--	mmol/L	(2.3-7.6)
Est. GFR:	87	> 90	--	mL/min	(> 60)
Creatinine:	90	86	--	umol/L	(60-110)
Bilirubin:	4	5	--	umol/L	(0-20)
ALT:	--	30	--	U/L	(0-55)
AST:	31	23	--	U/L	(0-40)
ALP:	53	46	--	U/L	(30-110)
GGT:	47	46	--	U/L	(0-50)
Tot.Protein:	45 **	48 **	--	g/L	(60-80)
Albumin:	27 **	29 *	--	g/L	(36-49)
Globulin:	18 **	19 **	--	g/L	(22-40)
Calcium:	1.90 *	1.97 *	--	mmol/L	(2.15-2.65)
Cor.Calcium:	2.16	2.20	--	mmol/L	(2.15-2.65)
Phosphate:	0.63 *	1.01	--	mmol/L	(0.75-1.50)
Magnesium:	0.93	0.83	--	mmol/L	(0.60-1.10)
Coll.Date:	27/02/21	28/02/21			
Coll.Time:	03:00	06:35			

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

User Event History

User Id	Date/Time	Type	State	Text
	28-Feb-21, 07:56	Signing	Unsign	Unsigning from the translator
	28-Feb-21, 07:56	Mark for Attention	Mark	Marking for attention from the translator
	28-Feb-21, 07:06	Signing	Not For Signing	

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

Mercy Health
Care first

2.8. FULL BLOOD EXAMINATION

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: FULL BLOOD EXAMINATION
Group:
Laboratory #: 21-31331326-FBE-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 28-Feb-21, 06:35
Received: 28-Feb-21, 07:04
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

FULL BLOOD EXAMINATION

Coll.Date:	26/02/21		27/02/21		28/02/21		
Coll.Time:	17:10		03:00		06:35		
Lab.No:	30318269		30679910		31331326		
						Units	Ref.Range
Haemoglobin:	78	**	87	**	81	**	g/L (130-180)
WCC:	9.2		7.4		6.5		x10 ^9 /L (4.0-11.0)
Platelets:	99	**	103	*	113	*	x10 ^9 /L (150-450)
RCC:	2.44		2.74	**	2.56		x10 ^12 /L (4.50-6.50)
PCV:	0.23	*	0.26	*	0.24	*	L/L (0.40-0.54)
MCV:	92		93		95		fL (80-96)
MCH:	32		32		32		pg (27-32)
MCHC:	347		340		335		g/L (320-360)
Neutrophils:	7.4		5.8		3.4		x10 ^9 /L (2.0-8.0)
Lymphocytes:	1.5		1.2		2.6		x10 ^9 /L (1.0-4.0)
Monocytes:	0.4		0.4		0.5		x10 ^9 /L (0.0-1.0)
Eosinophils:	0.0		0.0		0.1		x10 ^9 /L (0.0-0.5)
Basophils:	0.0		0.0		0.0		x10 ^9 /L (0.0-0.2)

26/02/21 30318269 There is a moderate thrombocytopenia. Stated acute blood loss.

27/02/21 30679910 Progress counts.

28/02/21 31331326 Progress counts.

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

User Event History

User Id	Date/Time	Type	State	Text
	28-Feb-21, 07:09	Signing	Unsign	Unsigning from the translator
	28-Feb-21, 07:09	Mark for Attention	Mark	Marking for attention from the translator
	28-Feb-21, 07:06	Signing	Not For Signing	

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

Mercy Health
Care first

2.9. BLOOD CULTURE M/C/S

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: BLOOD CULTURE M/C/S
Group:
Laboratory #: 21-31331326-BZC-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 28-Feb-21, 06:35
Received: 28-Feb-21, 07:04

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			

```
BLOOD CULTURE

   1 aerobic and 1 anaerobic bottle received.
   COLLECTED:  6:35 am on 28/02/2021

   CULTURE:
      No growth at 48 hours

      Culture continues but no further report will be issued unless bottles
      become positive.
```

User Event History

User Id	Date/Time	Type	State	Text
	02-Mar-21, 13:40	Signing	Not For Signing	
	01-Mar-21, 13:40	Signing	Not For Signing	
	28-Feb-21, 07:06	Signing	Not For Signing	

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

Mercy Health
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2.10. GASES (BLOOD)

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: GASES (BLOOD) **Collected:** 27-Feb-21, 20:00
Group: **Received:** 28-Feb-21, 08:14
Laboratory #: 21-47361184-GAS-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

BLOOD GASES

Date:	27/02/21	27/02/21			
Time:	08:20	19:50			
Lab.No:	47361182	47361184			
				Units	Ref.Range
Spec. Type:	Venous	Venous	--		
FiO2:	21	21	--	%	
pO2:	31 ***	27 ***	--	mmHg	
pCO2:	46 *	44	--	mmHg	(35-45)
pH:	7.39	7.39	--		(7.35-7.45)
Actual HCO3:	27	26	--	mmol/L	(22-28)
Std HCO3:	26	25	--	mmol/L	(22-28)
Base Excess:	+ 2.4	+ 1.0	--	mmol/L	(-3.0-+3.0)
Oxygen Sat:	59.1 ***	44.0 ***	--	%	
Sodium:	143	143	--	mmol/L	(135-145)
Potassium:	4.1	3.9	--	mmol/L	(3.3-4.9)
Chloride:	112 *	111 *	--	mmol/L	(95-110)
Ionised Ca:	1.15	1.14 *	--	mmol/L	(1.15-1.30)
Glucose:	5.9	8.8 *	--	mmol/L	(4.0-7.8)
Lactate:	1.3	2.2	--	mmol/L	(< 2.2)
Haemoglobin:	91 **	92 **	--	g/L	(130-180)
FO2Hb:	58.2 *	43.4 *	--	%	(94.0-98.0)
FCOHb:	0.7	0.6	--	%	(< 1.5)
FmetHb:	0.9	0.7	--	%	(< 1.5)
FHHb:	40.2 *	55.3 *	--	%	(< 5.0)
Haematocrit:	0.28 *	0.28 *	--	L/L	(0.40-0.54)

PLEASE NOTE CHANGE OF UNITS: 05/12/2016 Dorevitch Pathology has standardised reporting of HAEMOGLOBIN and HAEMATOCRIT on blood gases.
- Haemoglobin will report in grams/litre e.g. Haemoglobin 127 g/L

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

Mercy Health
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```
 - Haematocrit will report in litre/litre   e.g. Hct: 0.45 L/L
COMMENTS:
          - The reference intervals for pH, PCO2 and PO2 apply for
            arterial samples only.
```

User Event History

User Id	Date/Time	Type	State	Text
	28-Feb-21, 08:49	Signing	Not For Signing	
	28-Feb-21, 08:14	Signing	Not For Signing	

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

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2.11. BLOOD CULTURE M/C/S

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: BLOOD CULTURE M/C/S
Group:
Laboratory #: 21-31331251-BZC-0
Clinician: BARADI,ARUL (435647LJ)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 27-Feb-21, 20:00
Received: 27-Feb-21, 21:18

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			

BLOOD CULTURE

1 aerobic and 1 anaerobic bottle received.
COLLECTED: 8:00 pm on 27/02/2021

CULTURE:
No growth at 48 hours

Culture continues but no further report will be issued unless bottles become positive.

User Event History

User Id	Date/Time	Type	State	Text
	02-Mar-21, 13:38	Signing	Not For Signing	
	01-Mar-21, 13:37	Signing	Not For Signing	
	27-Feb-21, 21:19	Signing	Not For Signing	

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

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2.12. GASES (BLOOD)

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: GASES (BLOOD) **Collected:** 27-Feb-21, 08:50
Group: **Received:** 28-Feb-21, 08:13
Laboratory #: 21-47361182-GAS-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

BLOOD GASES

Date:	27/02/21	27/02/21			
Time:	08:20	19:50			
Lab.No:	47361182	47361184			
				Units	Ref.Range
Spec. Type:	Venous	Venous	--		
FiO2:	21	21	--	%	
pO2:	31 ***	27 ***	--	mmHg	
pCO2:	46 *	44	--	mmHg	(35-45)
pH:	7.39	7.39	--		(7.35-7.45)
Actual HCO3:	27	26	--	mmol/L	(22-28)
Std HCO3:	26	25	--	mmol/L	(22-28)
Base Excess:	+ 2.4	+ 1.0	--	mmol/L	(-3.0-+3.0)
Oxygen Sat:	59.1 ***	44.0 ***	--	%	
Sodium:	143	143	--	mmol/L	(135-145)
Potassium:	4.1	3.9	--	mmol/L	(3.3-4.9)
Chloride:	112 *	111 *	--	mmol/L	(95-110)
Ionised Ca:	1.15	1.14 *	--	mmol/L	(1.15-1.30)
Glucose:	5.9	8.8 *	--	mmol/L	(4.0-7.8)
Lactate:	1.3	2.2	--	mmol/L	(< 2.2)
Haemoglobin:	91 **	92 **	--	g/L	(130-180)
FO2Hb:	58.2 *	43.4 *	--	%	(94.0-98.0)
FCOHb:	0.7	0.6	--	%	(< 1.5)
FmetHb:	0.9	0.7	--	%	(< 1.5)
FHHb:	40.2 *	55.3 *	--	%	(< 5.0)
Haematocrit:	0.28 *	0.28 *	--	L/L	(0.40-0.54)

PLEASE NOTE CHANGE OF UNITS: 05/12/2016 Dorevitch Pathology has standardised reporting of HAEMOGLOBIN and HAEMATOCRIT on blood gases.
- Haemoglobin will report in grams/litre e.g. Haemoglobin 127 g/L

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```
 - Haematocrit will report in litre/litre   e.g. Hct: 0.45 L/L
COMMENTS:
          - The reference intervals for pH, PCO2 and PO2 apply for
            arterial samples only.
```

User Event History

User Id	Date/Time	Type	State	Text
	28-Feb-21, 08:49	Signing	Not For Signing	
	28-Feb-21, 08:14	Signing	Not For Signing	

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2.13. GASES (BLOOD)

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: GASES (BLOOD)
Group:
Laboratory #: 21-47361180-GAS-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 27-Feb-21, 03:00
Received: 28-Feb-21, 08:13

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

BLOOD GASES

Date:	26/02/21	26/02/21	27/02/21		
Time:	15:44	15:45	03:08		
Lab.No:	47361176	47361177	47361180		
				Units	Ref.Range
Spec. Type:	Venous	Venous	Venous		
FiO2:	21	--	21	%	
pO2:	--	--	29 ***	mmHg	
pCO2:	--	--	45	mmHg	(35-45)
pH:	7.40	--	7.40		(7.35-7.45)
Actual HCO3:	--	--	27	mmol/L	(22-28)
Std HCO3:	--	--	26	mmol/L	(22-28)
Base Excess:	--	--	+ 2.7	mmol/L	(-3.0-+3.0)
Oxygen Sat:	65.8 ***	--	51.4 ***	%	
Sodium:	140	--	143	mmol/L	(135-145)
Potassium:	4.0	--	4.3	mmol/L	(3.3-4.9)
Chloride:	112 *	--	110	mmol/L	(95-110)
Ionised Ca:	1.14 *	--	1.16	mmol/L	(1.15-1.30)
Glucose:	6.7	--	7.9 *	mmol/L	(4.0-7.8)
Lactate:	1.0	--	2.0	mmol/L	(< 2.2)
Haemoglobin:	87 **	--	91 **	g/L	(130-180)
FO2Hb:	64.5 *	--	50.6 *	%	(94.0-98.0)
FCOHb:	1.3	--	0.9	%	(< 1.5)
FmetHb:	0.6	--	0.6	%	(< 1.5)
FHHb:	33.6 *	--	47.9 *	%	(< 5.0)
Haematocrit:	0.27 *	--	0.28 *	L/L	(0.40-0.54)

PLEASE NOTE CHANGE OF UNITS: 05/12/2016 Dorevitch Pathology has standardised reporting of HAEMOGLOBIN and HAEMATOCRIT on blood gases.
- Haemoglobin will report in grams/litre e.g. Haemoglobin 127 g/L

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

```
 - Haematocrit will report in litre/litre   e.g. Hct: 0.45 L/L
COMMENTS:
          - The reference intervals for pH, PCO2 and PO2 apply for
            arterial samples only.

47361177 This specimen was aborted due to sample error, a recollection
         has been  organised.
```

User Event History				
User Id	**Date/Time**	**Type**	**State**	**Text**
	28-Feb-21, 08:49	Signing	Not For Signing	
	28-Feb-21, 08:14	Signing	Not For Signing	

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2.14. GENERAL BIOCHEMISTRY

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: GENERAL BIOCHEMISTRY
Group:
Laboratory #: 21-30679910-MBI-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 27-Feb-21, 03:00
Received: 27-Feb-21, 03:41
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

SERUM/PLASMA BIOCHEMISTRY

Coll.Date: 27/02/21
Coll.Time: 03:00
Lab.No: 30679910

				Units	Ref.Range
Sodium:	142	--	--	mmol/L	(135-145)
Potassium:	4.3	--	--	mmol/L	(3.5-5.2)
Chloride:	110	--	--	mmol/L	(95-110)
Bicarbonate:	27	--	--	mmol/L	(22-32)
Urea:	4.1	--	--	mmol/L	(2.3-7.6)
Est. GFR:	87	--	--	mL/min	(> 60)
Creatinine:	90	--	--	umol/L	(60-110)
Bilirubin:	4	--	--	umol/L	(0-20)
AST:	31	--	--	U/L	(0-40)
ALP:	53	--	--	U/L	(30-110)
GGT:	47	--	--	U/L	(0-50)
Tot.Protein:	45 **	--	--	g/L	(60-80)
Albumin:	27 **	--	--	g/L	(36-49)
Globulin:	18 **	--	--	g/L	(22-40)
Calcium:	1.90 *	--	--	mmol/L	(2.15-2.65)
Cor.Calcium:	2.16	--	--	mmol/L	(2.15-2.65)
Phosphate:	0.63 *	--	--	mmol/L	(0.75-1.50)
Magnesium:	0.93	--	--	mmol/L	(0.60-1.10)

Coll.Date: 27/02/21
Coll.Time: 03:00

User Event History

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

User Id	Date/Time	Type	State	Text
	27-Feb-21, 06:33	Signing	Unsign	Unsigning from the translator
	27-Feb-21, 06:33	Mark for Attention	Mark	Marking for attention from the translator
	27-Feb-21, 03:43	Signing	Not For Signing	

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Mercy Health
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2.15. FULL BLOOD EXAMINATION

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: FULL BLOOD EXAMINATION
Group:
Laboratory #: 21-30679910-FBE-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 27-Feb-21, 03:00
Received: 27-Feb-21, 03:41
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

FULL BLOOD EXAMINATION

Coll.Date:	26/02/21		27/02/21			
Coll.Time:	17:10		03:00			
Lab.No:	30318269		30679910			
					Units	Ref.Range
Haemoglobin:	78	**	87	**	g/L	(130-180)
WCC:	9.2		7.4		x10 ^9 /L	(4.0-11.0)
Platelets:	99	**	103	*	x10 ^9 /L	(150-450)
RCC:	2.44		2.74	**	x10 ^12 /L	(4.50-6.50)
PCV:	0.23	*	0.26	*	L/L	(0.40-0.54)
MCV:	92		93		fL	(80-96)
MCH:	32		32		pg	(27-32)
MCHC:	347		340		g/L	(320-360)
Neutrophils:	7.4		5.8		x10 ^9 /L	(2.0-8.0)
Lymphocytes:	1.5		1.2		x10 ^9 /L	(1.0-4.0)
Monocytes:	0.4		0.4		x10 ^9 /L	(0.0-1.0)
Eosinophils:	0.0		0.0		x10 ^9 /L	(0.0-0.5)
Basophils:	0.0		0.0		x10 ^9 /L	(0.0-0.2)

26/02/21 30318269 There is a moderate thrombocytopenia. Stated acute blood loss.

27/02/21 30679910 Progress counts.

User Event History				

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User Id	Date/Time	Type	State	Text
	27-Feb-21, 03:51	Signing	Unsign	Unsigning from the translator
	27-Feb-21, 03:51	Mark for Attention	Mark	Marking for attention from the translator
	27-Feb-21, 03:43	Signing	Not For Signing	

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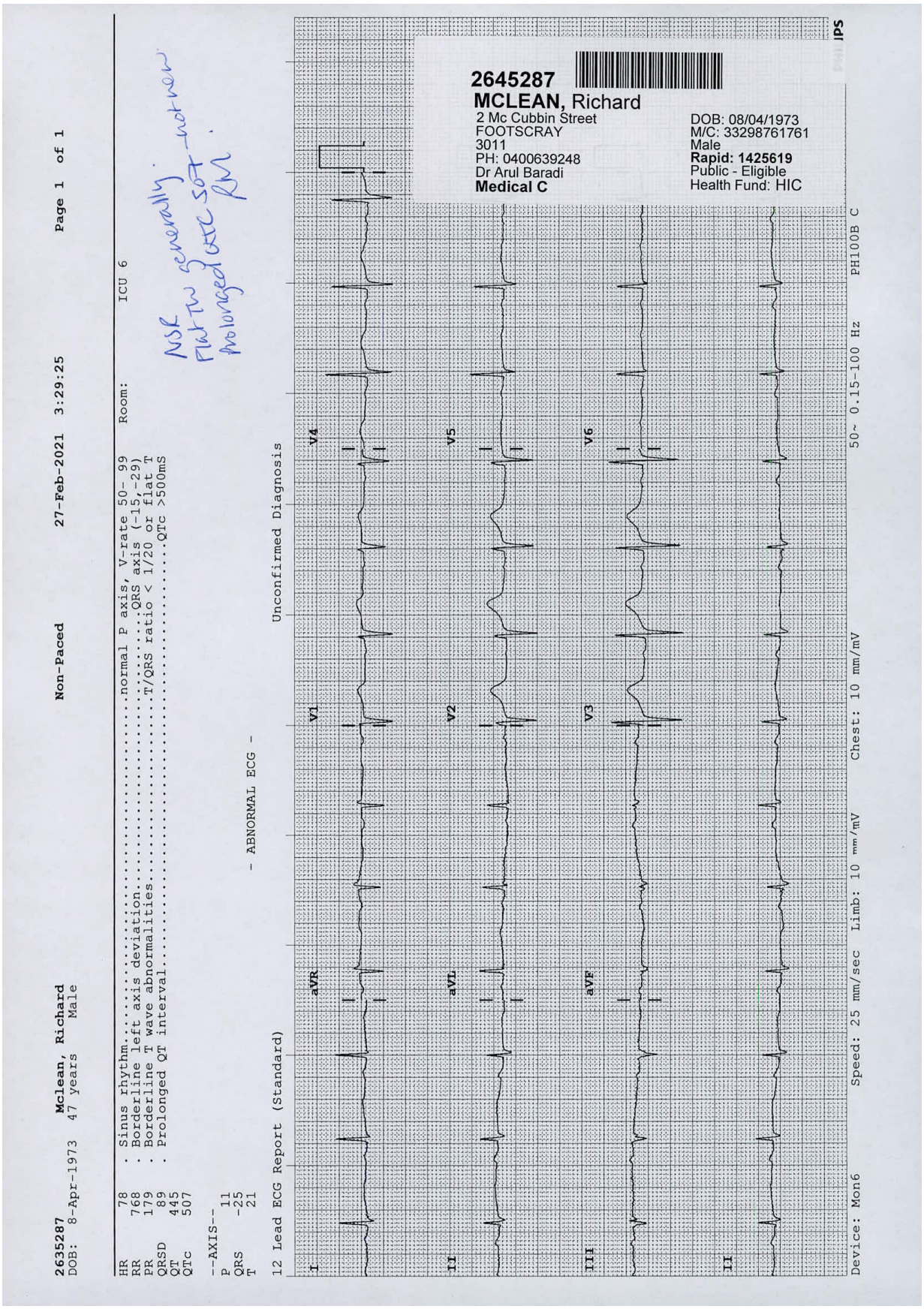
2.16. ECG Graph ECG Report (FMH300500)



CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

Mercy Health
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2.17. FULL BLOOD EXAMINATION

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: FULL BLOOD EXAMINATION
Group:
Laboratory #: 21-30318269-FBE-0
Clinician: BARADI,ARUL (435647LJ)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 26-Feb-21, 17:10
Received: 26-Feb-21, 17:20
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

FULL BLOOD EXAMINATION

Coll.Date: 26/02/21
Coll.Time: 17:10
Lab.No: 30318269

					Units	Ref.Range
Haemoglobin:	78	**	--	--	g/L	(130-180)
WCC:	9.2		--	--	x10 ^9 /L	(4.0-11.0)
Platelets:	99	**	--	--	x10 ^9 /L	(150-450)
RCC:	2.44		--	--	x10 ^12 /L	(4.50-6.50)
PCV:	0.23	*	--	--	L/L	(0.40-0.54)
MCV:	92		--	--	fL	(80-96)
MCH:	32		--	--	pg	(27-32)
MCHC:	347		--	--	g/L	(320-360)
Neutrophils:	7.4		--	--	x10 ^9 /L	(2.0-8.0)
Lymphocytes:	1.5		--	--	x10 ^9 /L	(1.0-4.0)
Monocytes:	0.4		--	--	x10 ^9 /L	(0.0-1.0)
Eosinophils:	0.0		--	--	x10 ^9 /L	(0.0-0.5)
Basophils:	0.0		--	--	x10 ^9 /L	(0.0-0.2)

26/02/21 30318269 There is a moderate thrombocytopenia. Stated acute blood loss.

User Event History

User Id	Date/Time	Type	State	Text
	26-Feb-21, 17:46	Signing	Unsign	Unsigning from the translator

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

	26-Feb-21, 17:46	Mark for Attention	Mark	Marking for attention from the translator
	26-Feb-21, 17:21	Signing	Not For Signing	

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

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2.18. HIGH SENS TROPONIN

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: HIGH SENS TROPONIN
Group:
Laboratory #: 21-30318255-STP-0
Clinician: BARADI,ARUL (435647LJ)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 26-Feb-21, 15:50
Received: 26-Feb-21, 16:23
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

```
SERUM / PLASMA HIGH SENSITIVE TROPONIN

            Trop I (Beckman) :      160  ng/L    (< 21)           *

          Time since the last Troponin collection :  10.3 hours

          CHANGE in Troponin since last specimen :  -271  ng/L

                                        Z-score :  -8.79             *


   * Normal hs-Troponin is 0-10 ng/L (Female) and 0-20 ng/L (Male)
   * In suspected Acute Coronary Syndrome consider repeat after 2-3 hrs
   * Increased Troponin may occur with non-ischaemic injury such as
     rapid AF, pulmonary embolism, heart failure or sepsis
   * Do NOT compare Troponin measured by different methods because the
     standardisation and units may differ
   * Z-score > +/- 2.0 indicates > 95% prob of a significant change
```

User Event History

User Id	Date/Time	Type	State	Text
	26-Feb-21, 16:56	Signing	Unsign	Unsigning from the translator
	26-Feb-21, 16:56	Mark for Attention	Mark	Marking for attention from the translator
	26-Feb-21, 16:23	Signing	Not For Signing	

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

Mercy Health
Care first

2.19. GENERAL BIOCHEMISTRY

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: GENERAL BIOCHEMISTRY
Group:
Laboratory #: 21-30318255-MBI-0
Clinician: BARADI,ARUL (435647LJ)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 26-Feb-21, 15:50
Received: 26-Feb-21, 16:23
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

SERUM/PLASMA BIOCHEMISTRY

Coll.Date:	25/02/21	26/02/21	26/02/21		
Coll.Time:	14:50	05:30	15:50		
Lab.No:	31331022	30318448	30318255		
				Units	Ref.Range
Sodium:	133 *	136	140	mmol/L	(135-145)
Potassium:	4.4	3.6	4.0	mmol/L	(3.5-5.2)
Chloride:	95	106	110	mmol/L	(95-110)
Bicarbonate:	15 **	25	27	mmol/L	(22-32)
Urea:	4.1	5.0	3.9	mmol/L	(2.3-7.6)
Est. GFR:	41	70	73	mL/min	(> 60)
Creatinine:	168 **	108	104	umol/L	(60-110)
Bilirubin:	11	8	8	umol/L	(0-20)
ALT:	63 *	52	48	U/L	(0-55)
AST:	47 *	44 *	36	U/L	(0-40)
ALP:	77	48	47	U/L	(30-110)
GGT:	64 *	53 *	49	U/L	(0-50)
Tot.Protein:	55 *	43 ***	44 ***	g/L	(60-80)
Albumin:	33 *	27 **	27 **	g/L	(36-49)
Globulin:	22	16 **	17 **	g/L	(22-40)
Calcium:	2.16	1.83 **	--	mmol/L	(2.15-2.65)
Cor.Calcium:	2.31	2.09 *	--	mmol/L	(2.15-2.65)
Phosphate:	2.60 **	1.17	--	mmol/L	(0.75-1.50)
Magnesium:	1.16 *	0.71	--	mmol/L	(0.60-1.10)
Coll.Date:	25/02/21	26/02/21	26/02/21		
Coll.Time:	14:50	05:30	15:50		

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User Event History

User Id	Date/Time	Type	State	Text
	26-Feb-21, 16:48	Signing	Unsign	Unsigning from the translator
	26-Feb-21, 16:48	Mark for Attention	Mark	Marking for attention from the translator
	26-Feb-21, 16:23	Signing	Not For Signing	

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Mercy Health
Care first

2.20. FULL BLOOD EXAMINATION

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: FULL BLOOD EXAMINATION
Group:
Laboratory #: 21-30318255-FBE-0
Clinician: BARADI,ARUL (435647LJ)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 26-Feb-21, 15:50
Received: 26-Feb-21, 16:23

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			

FULL BLOOD EXAMINATION

Coll.Date:	25/02/21		26/02/21		26/02/21		
Coll.Time:	14:50		05:30		15:50		
Lab.No:	31331022		30318448		30318255		
						Units	Ref.Range
Haemoglobin:	121	*	75	**	--	g/L	(130-180)
WCC:	6.8		10.8		--	x10 ^9 /L	(4.0-11.0)
Platelets:	243		153		--	x10 ^9 /L	(150-450)
RCC:	3.63	*	2.26		--	x10 ^12 /L	(4.50-6.50)
PCV:	0.36	*	0.21	*	--	L/L	(0.40-0.54)
MCV:	98	*	93		--	fL	(80-96)
MCH:	33	*	33	*	--	pg	(27-32)
MCHC:	340		357		--	g/L	(320-360)
Neutrophils:	4.4		8.0		--	x10 ^9 /L	(2.0-8.0)
Lymphocytes:	2.1		2.2		--	x10 ^9 /L	(1.0-4.0)
Monocytes:	0.3		0.6		--	x10 ^9 /L	(< 1.0)
Eosinophils:	0.0		0.0		--	x10 ^9 /L	(< 0.5)
Basophils:	0.0		0.0		--	x10 ^9 /L	(< 0.2)

25/02/21 31331022 INTERIM REPORT. A final report will follow.

26/02/21 30318448 INTERIM REPORT. A final report will follow.

The specimen was clotted, a recollection has been organised.

User Event History

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User Id	Date/Time	Type	State	Text
	26-Feb-21, 16:56	Signing	Not For Signing	
	26-Feb-21, 16:23	Signing	Not For Signing	

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2.21. GASES (BLOOD)

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: GASES (BLOOD)
Group:
Laboratory #: 21-47361177-GAS-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 26-Feb-21, 15:45
Received: 26-Feb-21, 11:04

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			

BLOOD GASES

Date:	26/02/21	26/02/21			
Time:	15:44	15:45			
Lab.No:	47361176	47361177			
				Units	Ref.Range
Spec. Type:	Venous	Venous	--		
FiO2:	21	--	--	%	
pH:	7.40	--	--		(7.35-7.45)
Oxygen Sat:	65.8 ***	--	--	%	
Sodium:	140	--	--	mmol/L	(135-145)
Potassium:	4.0	--	--	mmol/L	(3.3-4.9)
Chloride:	112 *	--	--	mmol/L	(95-110)
Ionised Ca:	1.14 *	--	--	mmol/L	(1.15-1.30)
Glucose:	6.7	--	--	mmol/L	(4.0-7.8)
Lactate:	1.0	--	--	mmol/L	(< 2.2)
Haemoglobin:	87 **	--	--	g/L	(130-180)
FO2Hb:	64.5 *	--	--	%	(94.0-98.0)
FCOHb:	1.3	--	--	%	(< 1.5)
FmetHb:	0.6	--	--	%	(< 1.5)
FHHb:	33.6 *	--	--	%	(< 5.0)
Haematocrit:	0.27 *	--	--	L/L	(0.40-0.54)

PLEASE NOTE CHANGE OF UNITS: 05/12/2016 Dorevitch Pathology has standardised reporting of HAEMOGLOBIN and HAEMATOCRIT on blood gases.
- Haemoglobin will report in grams/litre e.g. Haemoglobin 127 g/L
- Haematocrit will report in litre/litre e.g. Hct: 0.45 L/L

COMMENTS:
- The reference intervals for pH, PCO2 and PO2 apply for arterial samples only.

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47361177 This specimen was aborted due to sample error, a recollection has been organised.

User Event History				
User Id	**Date/Time**	**Type**	**State**	**Text**
	28-Feb-21, 05:43	Signing	Not For Signing	
	27-Feb-21, 11:05	Signing	Not For Signing	

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Mercy Health
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2.22. GASES (BLOOD)

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: GASES (BLOOD)
Group:
Laboratory #: 21-47361176-GAS-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 26-Feb-21, 15:15
Received: 26-Feb-21, 11:04

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			

BLOOD GASES

Date:	26/02/21	26/02/21			
Time:	15:44	15:45			
Lab.No:	47361176	47361177			
				Units	Ref.Range
Spec. Type:	Venous	Venous	--		
FiO2:	21	--	--	%	
pH:	7.40	--	--		(7.35-7.45)
Oxygen Sat:	65.8 ***	--	--	%	
Sodium:	140	--	--	mmol/L	(135-145)
Potassium:	4.0	--	--	mmol/L	(3.3-4.9)
Chloride:	112 *	--	--	mmol/L	(95-110)
Ionised Ca:	1.14 *	--	--	mmol/L	(1.15-1.30)
Glucose:	6.7	--	--	mmol/L	(4.0-7.8)
Lactate:	1.0	--	--	mmol/L	(< 2.2)
Haemoglobin:	87 **	--	--	g/L	(130-180)
FO2Hb:	64.5 *	--	--	%	(94.0-98.0)
FCOHb:	1.3	--	--	%	(< 1.5)
FmetHb:	0.6	--	--	%	(< 1.5)
FHHb:	33.6 *	--	--	%	(< 5.0)
Haematocrit:	0.27 *	--	--	L/L	(0.40-0.54)

PLEASE NOTE CHANGE OF UNITS: 05/12/2016 Dorevitch Pathology has standardised reporting of HAEMOGLOBIN and HAEMATOCRIT on blood gases.
- Haemoglobin will report in grams/litre e.g. Haemoglobin 127 g/L
- Haematocrit will report in litre/litre e.g. Hct: 0.45 L/L

COMMENTS:
- The reference intervals for pH, PCO2 and PO2 apply for arterial samples only.

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47361177 This specimen was aborted due to sample error, a recollection has been organised.

User Event History				
User Id	**Date/Time**	**Type**	**State**	**Text**
	28-Feb-21, 05:11	Signing	Not For Signing	
	27-Feb-21, 11:05	Signing	Not For Signing	

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2.23. GASES (BLOOD)

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: GASES (BLOOD) **Collected:** 26-Feb-21, 12:15
Group: **Received:** 26-Feb-21, 11:04
Laboratory #: 21-47361174-GAS-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

BLOOD GASES

Date:	26/02/21	26/02/21	26/02/21		
Time:	05:20	09:31	12:05		
Lab.No:	47361167	47361173	47361174		
				Units	Ref.Range
Spec. Type:	Arterial	Arterial	Arterial		
FiO2:	21	21	21	%	
pO2:	73 **	--	--	mmHg	(83-108)
pCO2:	38	40	38	mmHg	(35-45)
pH:	7.43	7.39	7.43		(7.35-7.45)
Actual HCO3:	25	24	25	mmol/L	(22-28)
Std HCO3:	25	24	25	mmol/L	(22-28)
Base Excess:	+ 0.8	- 0.5	+ 1.0	mmol/L	(-3.0-+3.0)
Oxygen Sat:	95.5	94.5 *	96.9	%	(95.0-100.0
Sodium:	136	138	139	mmol/L	(135-145)
Potassium:	3.6	3.9	3.9	mmol/L	(3.3-4.9)
Chloride:	108	115 *	113 *	mmol/L	(95-110)
Ionised Ca:	1.08 *	1.09 *	1.09 *	mmol/L	(1.15-1.30)
Glucose:	6.2	6.7	6.5	mmol/L	(4.0-7.8)
Lactate:	0.6	0.7	1.3	mmol/L	(< 2.2)
Haemoglobin:	77 **	80 **	78 **	g/L	(130-180)
FO2Hb:	93.6 *	92.5 *	95.0	%	(94.0-98.0)
FCOHb:	1.1	1.2	1.1	%	(< 1.5)
FmetHb:	0.9	0.9	0.9	%	(< 1.5)
FHHb:	4.4	5.4 *	3.0	%	(< 5.0)
Haematocrit:	0.24 *	0.25 *	0.24 *	L/L	(0.40-0.54)

PLEASE NOTE CHANGE OF UNITS: 05/12/2016 Dorevitch Pathology has

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standardised reporting of HAEMOGLOBIN and HAEMATOCRIT on blood gases.
- Haemoglobin will report in grams/litre e.g. Haemoglobin 127 g/L
- Haematocrit will report in litre/litre e.g. Hct: 0.45 L/L

User Event History

User Id	Date/Time	Type	State	Text
	27-Feb-21, 11:37	Signing	Not For Signing	
	27-Feb-21, 11:04	Signing	Not For Signing	

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2.24. GASES (BLOOD)

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: GASES (BLOOD)
Group:
Laboratory #: 21-47361173-GAS-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 26-Feb-21, 09:30
Received: 27-Feb-21, 11:03

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

BLOOD GASES

Date:	26/02/21	26/02/21	26/02/21		
Time:	05:20	09:31	12:05		
Lab.No:	47361167	47361173	47361174		
				Units	Ref.Range
Spec. Type:	Arterial	Arterial	Arterial		
FiO2:	21	21	21	%	
pO2:	73 **	--	--	mmHg	(83-108)
pCO2:	38	40	38	mmHg	(35-45)
pH:	7.43	7.39	7.43		(7.35-7.45)
Actual HCO3:	25	24	25	mmol/L	(22-28)
Std HCO3:	25	24	25	mmol/L	(22-28)
Base Excess:	+ 0.8	- 0.5	+ 1.0	mmol/L	(-3.0-+3.0)
Oxygen Sat:	95.5	94.5 *	96.9	%	(95.0-100.0
Sodium:	136	138	139	mmol/L	(135-145)
Potassium:	3.6	3.9	3.9	mmol/L	(3.3-4.9)
Chloride:	108	115 *	113 *	mmol/L	(95-110)
Ionised Ca:	1.08 *	1.09 *	1.09 *	mmol/L	(1.15-1.30)
Glucose:	6.2	6.7	6.5	mmol/L	(4.0-7.8)
Lactate:	0.6	0.7	1.3	mmol/L	(< 2.2)
Haemoglobin:	77 **	80 **	78 **	g/L	(130-180)
FO2Hb:	93.6 *	92.5 *	95.0	%	(94.0-98.0)
FCOHb:	1.1	1.2	1.1	%	(< 1.5)
FmetHb:	0.9	0.9	0.9	%	(< 1.5)
FHHb:	4.4	5.4 *	3.0	%	(< 5.0)
Haematocrit:	0.24 *	0.25 *	0.24 *	L/L	(0.40-0.54)

PLEASE NOTE CHANGE OF UNITS: 05/12/2016 Dorevitch Pathology has

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Mercy Health
Care first

standardised reporting of HAEMOGLOBIN and HAEMATOCRIT on blood gases.
- Haemoglobin will report in grams/litre e.g. Haemoglobin 127 g/L
- Haematocrit will report in litre/litre e.g. Hct: 0.45 L/L

User Event History

User Id	Date/Time	Type	State	Text
	27-Feb-21, 11:37	Signing	Not For Signing	
	27-Feb-21, 11:04	Signing	Not For Signing	

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2.25. GASES (BLOOD)

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: GASES (BLOOD)
Group:
Laboratory #: 21-47361167-GAS-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 26-Feb-21, 05:31
Received: 26-Feb-21, 11:02

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

BLOOD GASES

Date:	26/02/21	26/02/21	26/02/21		
Time:	05:20	09:31	12:05		
Lab.No:	47361167	47361173	47361174		
				Units	Ref.Range
Spec. Type:	Arterial	Arterial	Arterial		
FiO2:	21	21	21	%	
pO2:	73 **	--	--	mmHg	(83-108)
pCO2:	38	40	38	mmHg	(35-45)
pH:	7.43	7.39	7.43		(7.35-7.45)
Actual HCO3:	25	24	25	mmol/L	(22-28)
Std HCO3:	25	24	25	mmol/L	(22-28)
Base Excess:	+ 0.8	- 0.5	+ 1.0	mmol/L	(-3.0-+3.0)
Oxygen Sat:	95.5	94.5 *	96.9	%	(95.0-100.0
Sodium:	136	138	139	mmol/L	(135-145)
Potassium:	3.6	3.9	3.9	mmol/L	(3.3-4.9)
Chloride:	108	115 *	113 *	mmol/L	(95-110)
Ionised Ca:	1.08 *	1.09 *	1.09 *	mmol/L	(1.15-1.30)
Glucose:	6.2	6.7	6.5	mmol/L	(4.0-7.8)
Lactate:	0.6	0.7	1.3	mmol/L	(< 2.2)
Haemoglobin:	77 **	80 **	78 **	g/L	(130-180)
FO2Hb:	93.6 *	92.5 *	95.0	%	(94.0-98.0)
FCOHb:	1.1	1.2	1.1	%	(< 1.5)
FmetHb:	0.9	0.9	0.9	%	(< 1.5)
FHHb:	4.4	5.4 *	3.0	%	(< 5.0)
Haematocrit:	0.24 *	0.25 *	0.24 *	L/L	(0.40-0.54)

PLEASE NOTE CHANGE OF UNITS: 05/12/2016 Dorevitch Pathology has

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standardised reporting of HAEMOGLOBIN and HAEMATOCRIT on blood gases.
- Haemoglobin will report in grams/litre e.g. Haemoglobin 127 g/L
- Haematocrit will report in litre/litre e.g. Hct: 0.45 L/L

User Event History

User Id	Date/Time	Type	State	Text
	27-Feb-21, 11:36	Signing	Not For Signing	
	27-Feb-21, 11:03	Signing	Not For Signing	

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2.26. GENERAL URINE BIOCHEM

McLean, Richard	**2645287(MHS)**
08-Apr-1973 - **M**	
2 Mc Cubbin Street	
Footscray, 3011	

Test: GENERAL URINE BIOCHEM
Group:
Laboratory #: 21-30318462-URG-0
Clinician: BARADI,ARUL (435647LJ)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 26-Feb-21, 05:30
Received: 26-Feb-21, 07:13
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

GENERAL URINE BIOCHEMISTRY

Spot Urine

			Ref.Range	
Creatinine :	14.5	mmol/L	(3.5-15.8)	
Total Protein :	0.22	g/L	(< 0.20)	*
Protein/Creatinine Ratio :	15	mg/mmol	(0-30)	(1)

Note (1) A urine protein:creatinine ratio of 100 mg/mmol is equivalent to a daily protein excretion of approximately 1 gram.

User Event History

User Id	Date/Time	Type	State	Text
	02-Mar-21, 13:12	Signing	Unsign	Unsigning from the translator
	02-Mar-21, 13:12	Mark for Attention	Mark	Marking for attention from the translator
	26-Feb-21, 07:14	Signing	Not For Signing	

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2.27. MICROALBUMIN (URINE)

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: MICROALBUMIN (URINE)
Group:
Laboratory #: 21-30318462-UMA-0
Clinician: BARADI,ARUL (435647LJ)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 26-Feb-21, 05:30
Received: 26-Feb-21, 07:13
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

URINE MICROALBUMIN

Spot Urine

			Ref.Range	
Albumin Concentration:	122	mg/L	(0-25)	***
Creatinine Concentration:	14.5	mmol/L		
Alb/Creat Ratio (ACR):	8.4	mg/mmol creat	(< 2.5)	*

User Event History

User Id	Date/Time	Type	State	Text
	26-Feb-21, 20:39	Signing	Unsign	Unsigning from the translator
	26-Feb-21, 20:39	Mark for Attention	Mark	Marking for attention from the translator

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2.28. HIGH SENS TROPONIN

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: HIGH SENS TROPONIN
Group:
Laboratory #: 21-30318448-STP-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 26-Feb-21, 05:30
Received: 26-Feb-21, 05:58
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

SERUM / PLASMA HIGH SENSITIVE TROPONIN

Trop I (Beckman) : 431 ng/L (< 21) *

Time since the last Troponin collection : 14.7 hours

CHANGE in Troponin since last specimen : 383 ng/L

Z-score : 15.26 *

* Normal hs-Troponin is 0-10 ng/L (Female) and 0-20 ng/L (Male)
* In suspected Acute Coronary Syndrome consider repeat after 2-3 hrs
* Increased Troponin may occur with non-ischaemic injury such as rapid AF, pulmonary embolism, heart failure or sepsis
* Do NOT compare Troponin measured by different methods because the standardisation and units may differ
* Z-score > +/- 2.0 indicates > 95% prob of a significant change

User Event History

User Id	Date/Time	Type	State	Text
	26-Feb-21, 06:35	Signing	Unsign	Unsigning from the translator
	26-Feb-21, 06:35	Mark for Attention	Mark	Marking for attention from the translator
	26-Feb-21, 05:59	Signing	Not For Signing	

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2.29. GENERAL BIOCHEMISTRY

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: GENERAL BIOCHEMISTRY
Group:
Laboratory #: 21-30318448-MBI-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 26-Feb-21, 05:30
Received: 26-Feb-21, 05:58
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

SERUM/PLASMA BIOCHEMISTRY

Coll.Date:	25/02/21	26/02/21			
Coll.Time:	14:50	05:30			
Lab.No:	31331022	30318448			
				Units	Ref.Range
Sodium:	133 *	136	--	mmol/L	(135-145)
Potassium:	4.4	3.6	--	mmol/L	(3.5-5.2)
Chloride:	95	106	--	mmol/L	(95-110)
Bicarbonate:	15 **	25	--	mmol/L	(22-32)
Urea:	4.1	5.0	--	mmol/L	(2.3-7.6)
Est. GFR:	41	70	--	mL/min	(> 60)
Creatinine:	168 **	108	--	umol/L	(60-110)
Bilirubin:	11	8	--	umol/L	(0-20)
ALT:	63 *	52	--	U/L	(0-55)
AST:	47 *	44 *	--	U/L	(0-40)
ALP:	77	48	--	U/L	(30-110)
GGT:	64 *	53 *	--	U/L	(0-50)
Tot.Protein:	55 *	43 ***	--	g/L	(60-80)
Albumin:	33 *	27 **	--	g/L	(36-49)
Globulin:	22	16 **	--	g/L	(22-40)
Calcium:	2.16	1.83 **	--	mmol/L	(2.15-2.65)
Cor.Calcium:	2.31	2.09 *	--	mmol/L	(2.15-2.65)
Phosphate:	2.60 **	1.17	--	mmol/L	(0.75-1.50)
Magnesium:	1.16 *	0.71	--	mmol/L	(0.60-1.10)
Coll.Date:	25/02/21	26/02/21			
Coll.Time:	14:50	05:30			

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User Event History

User Id	Date/Time	Type	State	Text
	26-Feb-21, 06:41	Signing	Unsign	Unsigning from the translator
	26-Feb-21, 06:41	Mark for Attention	Mark	Marking for attention from the translator
	26-Feb-21, 05:59	Signing	Not For Signing	

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

Mercy Health
Care first

2.30. HBA1C (GLYCATED HB)

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: HBA1C (GLYCATED HB)
Group:
Laboratory #: 21-30318448-GHB-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 26-Feb-21, 05:30
Received: 26-Feb-21, 05:58

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			

BLOOD HAEMOGLOBIN A v1 c

Date	Lab.No.	HBA1C %	HbA1c mmol/mol
26/02/21	30318448	5.5	37

For diagnosis of diabetes mellitus the cut off is 6.5%.

For monitoring diabetic patients use the guidelines below.

Guidelines	HbA1c%	HbA1c mmol/mol
General Target	< 7.1	< 54
Adequate	< 8.1	< 65
Suboptimal	> 8.0	> 64

Note: HbA1c reporting will move to mmol/mol units over the next 2 years.

User Event History

User Id	Date/Time	Type	State	Text
	26-Feb-21, 20:36	Signing	Not For Signing	
	26-Feb-21, 12:22	Signing	Not For Signing	

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Mercy Health
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2.31. IRON STUDIES

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: IRON STUDIES
Group:
Laboratory #: 21-30318448-FES-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 26-Feb-21, 05:30
Received: 26-Feb-21, 05:58
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

SERUM IRON STUDIES

Date:26/02/21
Time:05:30
Lab.No:30318448

		Units	Ref. Range
Ferritin:	979	ug/L	(30-320)
Iron:	5	umol/L	(10-30)
Transferrin:	1.6	g/L	(2.0-3.6)
Transferrin Sat:	12	%	(13-47)

Ferritin levels may be elevated due to inflammation.
Medical professionals: Please contact a pathologist on 03 9244 0444 if required.

Please note: The Ferritin reference intervals have changed from 20/08/2019

Method: Siemens Immunoassay

User Event History

User Id	Date/Time	Type	State	Text
	26-Feb-21, 15:11	Signing	Unsign	Unsigning from the translator
	26-Feb-21, 15:11	Mark for Attention	Mark	Marking for attention from the translator
	26-Feb-21, 05:59	Signing	Not For Signing	

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2.32. FULL BLOOD EXAMINATION

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: FULL BLOOD EXAMINATION
Group:
Laboratory #: 21-30318448-FBE-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 26-Feb-21, 05:30
Received: 26-Feb-21, 05:58
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

FULL BLOOD EXAMINATION

Coll.Date:	04/01/18		25/02/21		26/02/21		
Coll.Time:	13:10		14:50		05:30		
Lab.No:	35524942		31331022		30318448		
						Units	Ref.Range
Haemoglobin:	150		121	*	75	**	g/L (130-180)
WCC:	5.1		6.8		10.8		x10 ^9 /L (4.0-11.0)
Platelets:	241		243		153		x10 ^9 /L (150-450)
RCC:	4.56		3.63	*	2.26		x10 ^12 /L (4.50-6.50)
PCV:	0.44		0.36	*	0.21	*	L/L (0.40-0.54)
MCV:	95		98	*	93		fL (80-96)
MCH:	33	*	33	*	33	*	pg (27-32)
MCHC:	345		340		357		g/L (320-360)
Neutrophils:	2.7		4.4		8.0		x10 ^9 /L (2.0-8.0)
Lymphocytes:	1.9		2.1		2.2		x10 ^9 /L (1.0-4.0)
Monocytes:	0.4		0.3		0.6		x10 ^9 /L (0.0-1.0)
Eosinophils:	0.1		0.0		0.0		x10 ^9 /L (0.0-0.5)
Basophils:	0.0		0.0		0.0		x10 ^9 /L (0.0-0.2)

04/01/18 35524942 Essentially within normal limits.

25/02/21 31331022 INTERIM REPORT. A final report will follow.

26/02/21 30318448 Red cells, White cells and Platelets show normal morphology. Stated acute blood loss.

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User Event History

User Id	Date/Time	Type	State	Text
	02-Mar-21, 07:50	Signing	Unsign	Unsigning from the translator
	02-Mar-21, 07:50	Mark for Attention	Mark	Marking for attention from the translator
	26-Feb-21, 06:19	Signing	Unsign	Unsigning from the translator
	26-Feb-21, 06:19	Mark for Attention	Mark	Marking for attention from the translator
	26-Feb-21, 05:59	Signing	Not For Signing	

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2.33. C-REACTIVE PROTEIN

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: C-REACTIVE PROTEIN
Group:
Laboratory #: 21-30318448-CRP-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 26-Feb-21, 05:30
Received: 26-Feb-21, 05:58
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

SERUM C-REACTIVE PROTEIN (CRP)

Date	Lab. No.	CRP	Ref Range (0-10)	
26/02/21	30318448	27 mg/L		**

30318448 - Elevated CRP indicates inflammation, necrosis or infection.

User Event History

User Id	Date/Time	Type	State	Text
	26-Feb-21, 06:38	Signing	Unsign	Unsigning from the translator
	26-Feb-21, 06:38	Mark for Attention	Mark	Marking for attention from the translator
	26-Feb-21, 05:59	Signing	Not For Signing	

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2.34. COAGULATION PROFILE

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: COAGULATION PROFILE
Group:
Laboratory #: 21-30318448-COA-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 26-Feb-21, 05:30
Received: 26-Feb-21, 05:58
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

Coagulation Profile (Blood)

Coll.Date:	25/02/21	25/02/21	26/02/21
Coll.Time:	14:50	17:15	05:30
Lab.No:	31331022	31331058	30318448

				Units	Ref.Range
APTT:	--	<20	22	seconds	(24-34) (Therapeutic 60-80)
INR:	--	1.1	1.2		(< 1.3)
Fibrinogen:	--	1.6	2.1	g/L	(1.5-4.0)
PLTS (EDTA):	--	--	153	x10 ^9 /L	(150-450)

3133102225/02/21 Coag Profile - ? Query sample integrity. A recollection is recommended.

User Event History

User Id	Date/Time	Type	State	Text
	26-Feb-21, 06:18	Signing	Unsign	Unsigning from the translator
	26-Feb-21, 06:18	Mark for Attention	Mark	Marking for attention from the translator
	26-Feb-21, 05:59	Signing	Not For Signing	

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2.35. B12,FOLATE,R.C.FOLATE

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: B12,FOLATE,R.C.FOLATE
Group:
Laboratory #: 21-30318448-BFO-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 26-Feb-21, 05:30
Received: 26-Feb-21, 05:58

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			

VITAMIN B12 AND FOLATE

Date:	26/02/21			
Time:	05:30			
Lab.No:	30318448			
			Units	Ref. Range
Vitamin B12	: 348		pmol/L	(150-700)
Serum Folate	: 27.8		nmol/L	(> 9.0)

Vitamin B12 stores appear adequate.

Method: Siemens Immunoassay

Vitamin B12 method: Siemens Immunoassay

Note: Serum folate concentration may be falsely increased in people taking biotin supplements. Unless medical contraindicated, supplements should be withheld at least one week before testing.

User Event History

User Id	Date/Time	Type	State	Text
	26-Feb-21, 15:08	Signing	Not For Signing	

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	26-Feb-21, 05:59	Signing	Not For Signing	

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2.36. PANCREATIC STUDIES-SERUM

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: PANCREATIC STUDIES-SERUM
Group:
Laboratory #: 21-30318448-AML-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 26-Feb-21, 05:30
Received: 26-Feb-21, 05:58
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

```
SERUM PANCREATIC STUDIES
   Coll.Date: 26/02/21
   Coll.Time: 05:30
      Lab.No: 30318448

                                                  Units   Ref.Range
     Amylase:    742 ***      --          --       U/L    (0-100)
      Lipase:     29          --          --       U/L    (0-60)

 COMMENTS:

 Amylase    - Note: The elevation of serum amylase in acute pancreatitis
          is transient, usually returning to normal levels within 48
          hours. Other causes of an elevated serum amylase, in the setting
          of an acute abdomen, include gastrointestinal perforation
          (including a ruptured peptic ulcer), intestinal obstruction,
          acute biliary disease, ruptured ectopic pregnancy and intestinal
          ischaemia or infarction. Serum amylase can also be increased
          with parotitis.
```

User Event History

User Id	Date/Time	Type	State	Text
	26-Feb-21, 06:41	Signing	Unsign	Unsigning from the translator
	26-Feb-21, 06:41	Mark for Attention	Mark	Marking for attention from the translator
	26-Feb-21, 05:59	Signing	Not For Signing	

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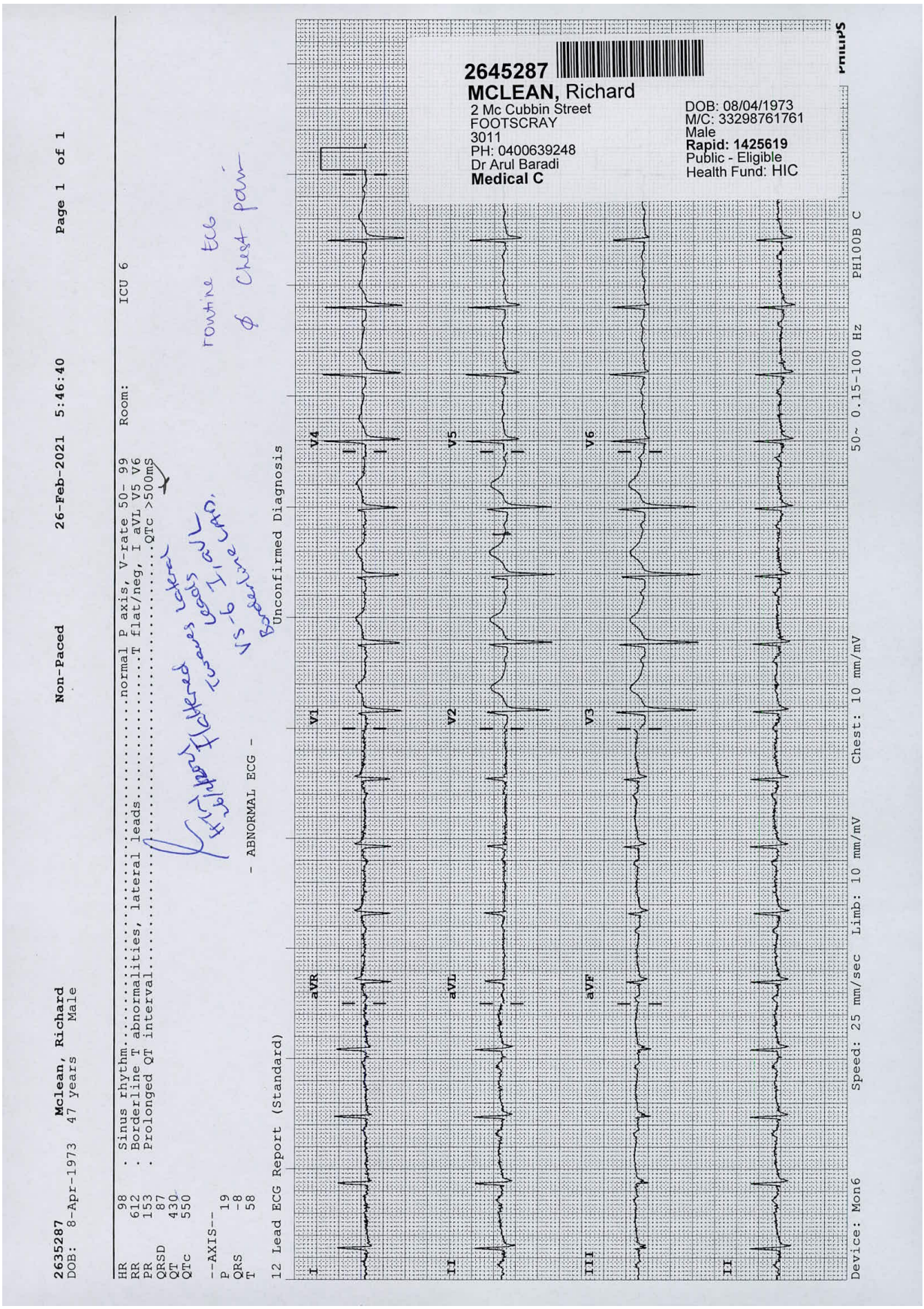
2.37. ECG Graph ECG Report (FMH300500)



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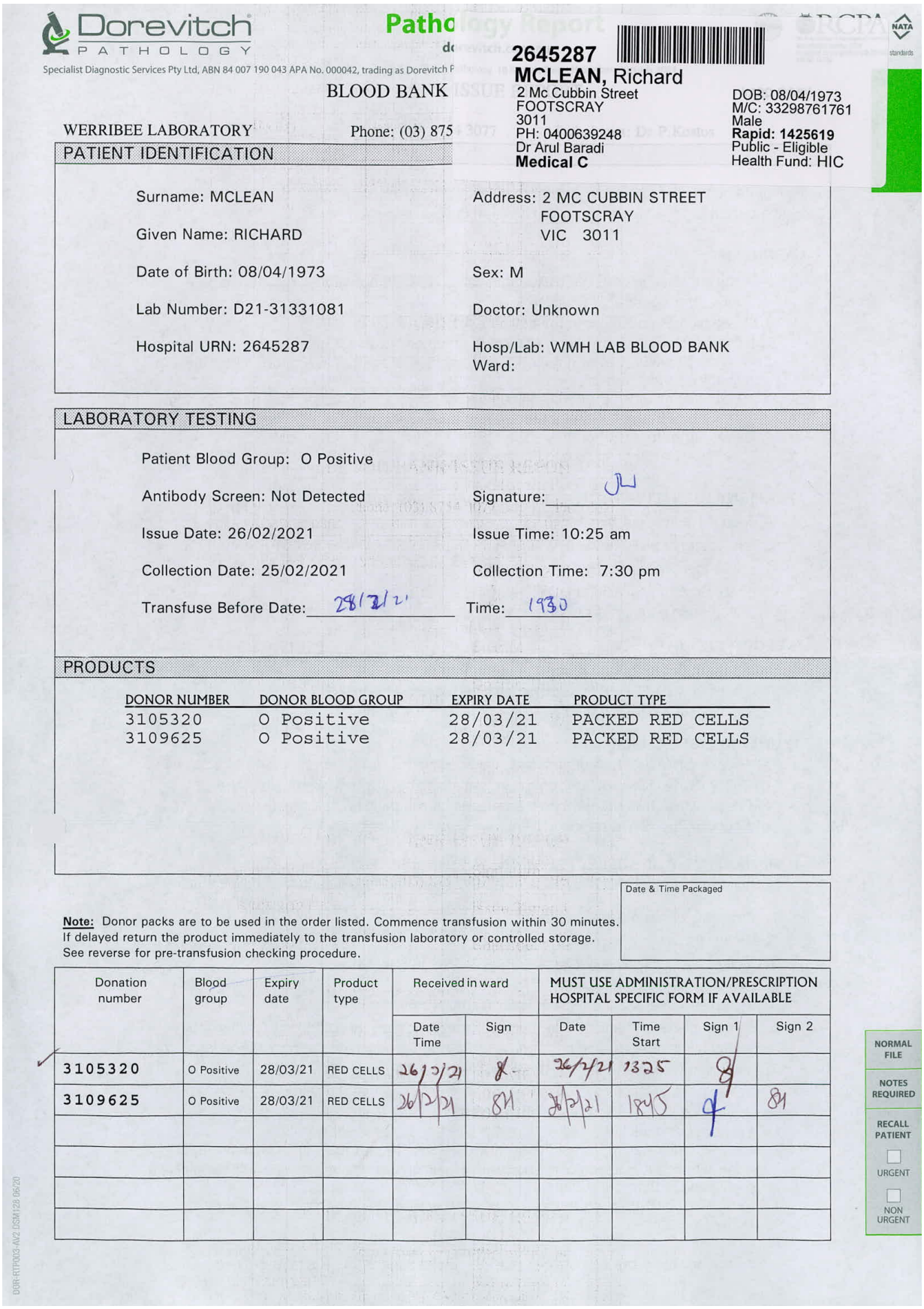
2.38. Blood Traceability Report (FMH301400)



Dorevitch PATHOLOGY

Specialist Diagnostic Services Pty Ltd, ABN 84 007 190 043 APA No. 000042, trading as Dorevitch P

BLOOD BANK

WERRIBEE LABORATORY Phone: (03) 875

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

NATA standards

PATIENT IDENTIFICATION

Surname: MCLEAN
Given Name: RICHARD
Date of Birth: 08/04/1973
Lab Number: D21-31331081
Hospital URN: 2645287

Address: 2 MC CUBBIN STREET
FOOTSCRAY
VIC 3011
Sex: M
Doctor: Unknown
Hosp/Lab: WMH LAB BLOOD BANK
Ward:

LABORATORY TESTING

Patient Blood Group: O Positive
Antibody Screen: Not Detected
Signature: JL
Issue Date: 26/02/2021
Issue Time: 10:25 am
Collection Date: 25/02/2021
Collection Time: 7:30 pm
Transfuse Before Date: 28/2/21 Time: 1930

PRODUCTS

DONOR NUMBER	DONOR BLOOD GROUP	EXPIRY DATE	PRODUCT TYPE
3105320	O Positive	28/03/21	PACKED RED CELLS
3109625	O Positive	28/03/21	PACKED RED CELLS

Date & Time Packaged

Note: Donor packs are to be used in the order listed. Commence transfusion within 30 minutes. If delayed return the product immediately to the transfusion laboratory or controlled storage. See reverse for pre-transfusion checking procedure.

Donation number	Blood group	Expiry date	Product type	Received in ward		MUST USE ADMINISTRATION/PRESCRIPTION HOSPITAL SPECIFIC FORM IF AVAILABLE			
				Date Time	Sign	Date	Time Start	Sign 1	Sign 2
3105320	O Positive	28/03/21	RED CELLS	26/2/21	[illegible]	26/2/21	1325	[illegible]	
3109625	O Positive	28/03/21	RED CELLS	26/2/21	[illegible]	26/2/21	1845	[illegible]	[illegible]

NORMAL FILE
NOTES REQUIRED
RECALL PATIENT
☐ URGENT
☐ NON URGENT

DGR-RTP003-AV2 DSM128 06/20

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2.39. Blood Traceability Report (FMH301400)

PRE-TRANSFUSION CHECKING PROCEDURE

AIMS: To accurately identify transfusion recipient by means of:

1. The patient's identity
2. The blood group
3. The donation number

The information is listed in various locations - patient's identity band, unit label and crossmatch label attached to the unit and the blood bank product issue report.

IMPORTANT

1. If any information does not correspond, or if further information is required, please contact the laboratory immediately.
2. Please file this document in the patient's records.
3. If there is a delay in starting the transfusion remember the 30-minute rule
   a) < 30 minutes, return the pack to the refrigerator. Complete return date & time in blood fridge register
   b) > 30 minutes, pack may be kept at bedside but transfusion must be completed within 4 hours from the time of removal from storage.
4. Please return to the laboratory:
   a) blood > 30 minutes and no longer required

1) PATIENT IDENTITY CHECK

a) Identify the patient by name

b) Check surname, given name(s) in full, hospital record number and date of birth on:

   a) Identity band
   b) Crossmatch label attached to unit
   c) Blood bank product issue report.

2) BLOOD GROUP CHECK

a) Check BLOOD GROUP on unit
b) Check BLOOD GROUP on crossmatch label attached to unit
c) Check BLOOD GROUP on blood bank product issue report.

3) UNIT NUMBER CHECK

a) Check DONATION NUMBER on the unit label
b) Check DONATION NUMBER on the crossmatch label attached to the unit
c) Check DONATION NUMBER on the crossmatch compatibility report
d) Check EXPIRY DATE on the UNIT.

4) MANAGEMENT OF SUSPECTED TRANSFUSION REACTION

The following is meant as a guide only. Always know and follow your local hospital policy.

1. Immediately stop transfusion.
2. Notify Doctor.
3. Follow acute transfusion reaction management instructions.
4. Notify Transfusion laboratory.
5. Complete a transfusion reaction investigation form.
6. Send transfusion reaction investigation form, specimens and all blood components including clamped spiked bags and unused blood products to the transfusion laboratory.
7. Report in Riskman, VHIMS or local incident reporting program.

Recommended investigation of transfusion reaction

1. Recheck compatibility details of blood product against patient details.
2. FBC, Antibody screen, Direct Coombs, Electrolytes, Urea, Creatinine, Haptoglobin, LDH, BIL urine sample for Haemoglobin testing. If a haemolytic reaction is suspected, coagulation profile and D-Dimer in addition.
3. Return ALL blood/blood product bags for rechecking of compatibility and culture.
4. Notify Dorevitch Pathology Blood Bank.

Further advice can be obtained from the Blood Bank or the Haematologist–on-call.

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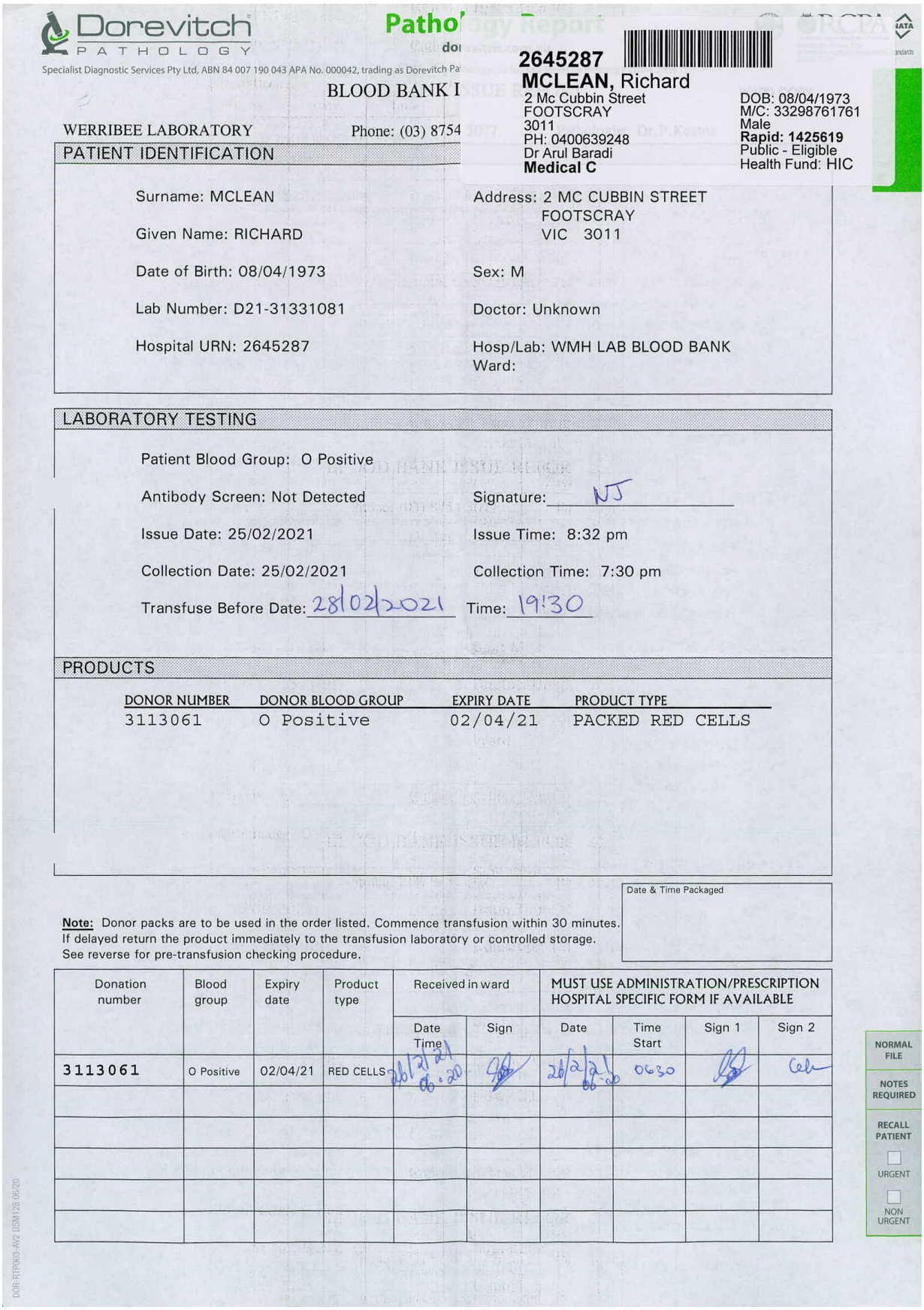
2.40. Blood Traceability Report (FMH301400)



Dorevitch PATHOLOGY

Specialist Diagnostic Services Pty Ltd, ABN 84 007 190 043 APA No. 000042, trading as Dorevitch Pa

BLOOD BANK I

WERRIBEE LABORATORY Phone: (03) 8754

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

PATIENT IDENTIFICATION

Surname: MCLEAN
Given Name: RICHARD
Date of Birth: 08/04/1973
Lab Number: D21-31331081
Hospital URN: 2645287

Address: 2 MC CUBBIN STREET
FOOTSCRAY
VIC 3011
Sex: M
Doctor: Unknown
Hosp/Lab: WMH LAB BLOOD BANK
Ward:

LABORATORY TESTING

Patient Blood Group: O Positive
Antibody Screen: Not Detected
Signature: NJ
Issue Date: 25/02/2021
Issue Time: 8:32 pm
Collection Date: 25/02/2021
Collection Time: 7:30 pm
Transfuse Before Date: 28/02/2021 Time: 19:30

PRODUCTS

DONOR NUMBER	DONOR BLOOD GROUP	EXPIRY DATE	PRODUCT TYPE
3113061	O Positive	02/04/21	PACKED RED CELLS

Date & Time Packaged

Note: Donor packs are to be used in the order listed. Commence transfusion within 30 minutes. If delayed return the product immediately to the transfusion laboratory or controlled storage. See reverse for pre-transfusion checking procedure.

Donation number	Blood group	Expiry date	Product type	Received in ward		MUST USE ADMINISTRATION/PRESCRIPTION HOSPITAL SPECIFIC FORM IF AVAILABLE			
				Date Time	Sign	Date	Time Start	Sign 1	Sign 2
3113061	O Positive	02/04/21	RED CELLS	26/2/21 06:20		26/2/21 08:30	0630		

NORMAL FILE
NOTES REQUIRED
RECALL PATIENT
URGENT
NON URGENT

DOR: RTP003-AV2 USM1 28.06/20

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2.41. Blood Traceability Report (FMH301400)

PRE-TRANSFUSION CHECKING PROCEDURE

AIMS: To accurately identify transfusion recipient by means of:

1. The patient's identity
2. The blood group
3. The donation number

The information is listed in various locations - patient's identity band, unit label and crossmatch label attached to the unit and the blood bank product issue report.

IMPORTANT

1. If any information does not correspond, or if further information is required, please contact the laboratory immediately.
2. Please file this document in the patient's records.
3. If there is a delay in starting the transfusion remember the 30-minute rule
   a) < 30 minutes, return the pack to the refrigerator. Complete return date & time in blood fridge register
   b) > 30 minutes, pack may be kept at bedside but transfusion must be completed within 4 hours from the time of removal from storage.
4. Please return to the laboratory:
   a) blood > 30 minutes and no longer required

1) PATIENT IDENTITY CHECK

a) Identify the patient by name

b) Check surname, given name(s) in full, hospital record number and date of birth on:

   a) Identity band
   b) Crossmatch label attached to unit
   c) Blood bank product issue report.

2) BLOOD GROUP CHECK

a) Check BLOOD GROUP on unit
b) Check BLOOD GROUP on crossmatch label attached to unit
c) Check BLOOD GROUP on blood bank product issue report.

3) UNIT NUMBER CHECK

a) Check DONATION NUMBER on the unit label
b) Check DONATION NUMBER on the crossmatch label attached to the unit
c) Check DONATION NUMBER on the crossmatch compatibility report
d) Check EXPIRY DATE on the UNIT.

4) MANAGEMENT OF SUSPECTED TRANSFUSION REACTION

The following is meant as a guide only. Always know and follow your local hospital policy.

1. Immediately stop transfusion.
2. Notify Doctor.
3. Follow acute transfusion reaction management instructions.
4. Notify Transfusion laboratory.
5. Complete a transfusion reaction investigation form.
6. Send transfusion reaction investigation form, specimens and all blood components including clamped spiked bags and unused blood products to the transfusion laboratory.
7. Report in Riskman, VHIMS or local incident reporting program.

Recommended investigation of transfusion reaction

1. Recheck compatibility details of blood product against patient details.
2. FBC, Antibody screen, Direct Coombs, Electrolytes, Urea, Creatinine, Haptoglobin, LDH, BIL urine sample for Haemoglobin testing. If a haemolytic reaction is suspected, coagulation profile and D-Dimer in addition.
3. Return ALL blood/blood product bags for rechecking of compatibility and culture.
4. Notify Dorevitch Pathology Blood Bank.

Further advice can be obtained from the Blood Bank or the Haematologist–on-call.

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Mercy Health
Care first

2.42. GASES (BLOOD)

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: GASES (BLOOD) **Collected:** 25-Feb-21, 23:01
Group: **Received:** 26-Feb-21, 19:33
Laboratory #: 21-47361158-GAS-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

BLOOD GASES

Date:	25/02/21	25/02/21	25/02/21		
Time:	17:08	19:03	22:49		
Lab.No:	47361150	47361152	47361158		
				Units	Ref.Range
Spec. Type:	Arterial	Arterial	Arterial		
FiO2:	0	25	21	%	
pO2:	113 *	92	78 *	mmHg	(83-108)
pCO2:	40	36	35	mmHg	(35-45)
pH:	7.33 *	7.39	7.43		(7.35-7.45)
Actual HCO3:	20 *	21 *	22	mmol/L	(22-28)
Std HCO3:	21 *	22	23	mmol/L	(22-28)
Base Excess:	- 4.5 *	- 2.6	- 1.2	mmol/L	(-3.0-+3.0)
Oxygen Sat:	98.5	97.7	96.3	%	(95.0-100.0
Sodium:	138	139	135	mmol/L	(135-145)
Potassium:	4.9	4.8	4.3	mmol/L	(3.3-4.9)
Chloride:	111 *	111 *	109	mmol/L	(95-110)
Ionised Ca:	1.05 *	1.08 *	1.08 *	mmol/L	(1.15-1.30)
Glucose:	14.1 ***	13.1 ***	8.0 *	mmol/L	(4.0-7.8)
Lactate:	3.1 *	2.8 *	2.1	mmol/L	(< 2.2)
Haemoglobin:	100 *	96 **	86 **	g/L	(130-180)
FO2Hb:	96.7	96.4	94.5	%	(94.0-98.0)
FCOHb:	0.8	0.6	1.1	%	(< 1.5)
FmetHb:	1.0	0.7	0.8	%	(< 1.5)
FHHb:	1.5	2.3	3.6	%	(< 5.0)
Haematocrit:	0.31 *	0.30 *	0.27 *	L/L	(0.40-0.54)

PLEASE NOTE CHANGE OF UNITS: 05/12/2016 Dorevitch Pathology has

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standardised reporting of HAEMOGLOBIN and HAEMATOCRIT on blood gases.
- Haemoglobin will report in grams/litre e.g. Haemoglobin 127 g/L
- Haematocrit will report in litre/litre e.g. Hct: 0.45 L/L

User Event History

User Id	Date/Time	Type	State	Text
	26-Feb-21, 19:37	Signing	Not For Signing	
	26-Feb-21, 19:34	Signing	Not For Signing	

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Mercy Health
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2.43. GROUP ANTIBODY AND HOLD

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: GROUP ANTIBODY AND HOLD
Group:
Laboratory #: 21-31331081-GHO-0
Clinician: BARADI,ARUL (435647LJ)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 25-Feb-21, 19:30
Received: 25-Feb-21, 19:45

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			

BLOOD GROUP AND ANTIBODY SCREEN

Blood Group O Rh(D) Positive

Red Cell Antibodies Not Detected

SERUM AND CELLS WILL BE SAVED FOR 72 HOURS

Pre transfusion testing and transfusion of a patient who is pregnant or who has been transfused in the past 3 months shall be completed within 72 hours of the specimen being collected.

Donation ID	Group	Expiry	Type
3113061	O Positive	02/04/21	PACKED RED CELLS
3109625	O Positive	28/03/21	PACKED RED CELLS
3105320	O Positive	28/03/21	PACKED RED CELLS

User Event History

User Id	Date/Time	Type	State	Text
	26-Feb-21, 10:26	Signing	Not For Signing	
	25-Feb-21, 20:32	Signing	Not For Signing	
	25-Feb-21, 20:31	Signing	Not For Signing	

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	25-Feb-21, 19:47	Signing	Not For Signing	

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2.44. GASES (BLOOD)

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: GASES (BLOOD) **Collected:** 25-Feb-21, 19:00
Group: **Received:** 26-Feb-21, 19:33
Laboratory #: 21-47361152-GAS-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

BLOOD GASES

Date:	25/02/21	25/02/21	25/02/21		
Time:	17:08	19:03	22:49		
Lab.No:	47361150	47361152	47361158		
				Units	Ref.Range
Spec. Type:	Arterial	Arterial	Arterial		
FiO2:	0	25	21	%	
pO2:	113 *	92	78 *	mmHg	(83-108)
pCO2:	40	36	35	mmHg	(35-45)
pH:	7.33 *	7.39	7.43		(7.35-7.45)
Actual HCO3:	20 *	21 *	22	mmol/L	(22-28)
Std HCO3:	21 *	22	23	mmol/L	(22-28)
Base Excess:	- 4.5 *	- 2.6	- 1.2	mmol/L	(-3.0-+3.0)
Oxygen Sat:	98.5	97.7	96.3	%	(95.0-100.0
Sodium:	138	139	135	mmol/L	(135-145)
Potassium:	4.9	4.8	4.3	mmol/L	(3.3-4.9)
Chloride:	111 *	111 *	109	mmol/L	(95-110)
Ionised Ca:	1.05 *	1.08 *	1.08 *	mmol/L	(1.15-1.30)
Glucose:	14.1 ***	13.1 ***	8.0 *	mmol/L	(4.0-7.8)
Lactate:	3.1 *	2.8 *	2.1	mmol/L	(< 2.2)
Haemoglobin:	100 *	96 **	86 **	g/L	(130-180)
FO2Hb:	96.7	96.4	94.5	%	(94.0-98.0)
FCOHb:	0.8	0.6	1.1	%	(< 1.5)
FmetHb:	1.0	0.7	0.8	%	(< 1.5)
FHHb:	1.5	2.3	3.6	%	(< 5.0)
Haematocrit:	0.31 *	0.30 *	0.27 *	L/L	(0.40-0.54)

PLEASE NOTE CHANGE OF UNITS: 05/12/2016 Dorevitch Pathology has

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standardised reporting of HAEMOGLOBIN and HAEMATOCRIT on blood gases.
- Haemoglobin will report in grams/litre e.g. Haemoglobin 127 g/L
- Haematocrit will report in litre/litre e.g. Hct: 0.45 L/L

User Event History

User Id	Date/Time	Type	State	Text
	26-Feb-21, 19:37	Signing	Not For Signing	
	26-Feb-21, 19:34	Signing	Not For Signing	

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2.45. URINE MICROSCOPY

McLean, Richard	**2645287(MHS)**
08-Apr-1973 - **M**	
2 Mc Cubbin Street	
Footscray, 3011	

Test: URINE MICROSCOPY
Group:
Laboratory #: 21-31331061-URC-0
Clinician: BARADI,ARUL (435647FB)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 25-Feb-21, 17:30
Received: 25-Feb-21, 18:01
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

```
URINE EXAMINATION :

PHASE CONTRAST                          CHEMISTRY
 Epith.Squames..       2 x10 ^6 /L (N <10)  pH ..... 6.5      Glucose ++
 Polymorphs.....      18 x10 ^6 /L (N <10)  Protein  ++       Ketones Neg
 Erythrocytes...       2 x10 ^6 /L (N <10)  Blood/Hb Neg
 No casts or crystals seen (unspun urine).
 Note glycosuria

   CULTURE
          No growth.
```

User Event History

User Id	Date/Time	Type	State	Text
	26-Feb-21, 17:43	Signing	Unsign	Unsigning from the translator
	26-Feb-21, 17:43	Mark for Attention	Mark	Marking for attention from the translator
	25-Feb-21, 23:54	Signing	Unsign	Unsigning from the translator
	25-Feb-21, 23:54	Mark for Attention	Mark	Marking for attention from the translator
	25-Feb-21, 18:02	Signing	Not For Signing	

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2.46. GASES (BLOOD)

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: GASES (BLOOD)
Group:
Laboratory #: 21-47361150-GAS-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 25-Feb-21, 17:20
Received: 26-Feb-21, 19:33

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

BLOOD GASES

Date:	25/02/21	25/02/21	25/02/21		
Time:	17:08	19:03	22:49		
Lab.No:	47361150	47361152	47361158		
				Units	Ref.Range
Spec. Type:	Arterial	Arterial	Arterial		
FiO2:	0	25	21	%	
pO2:	113 *	92	78 *	mmHg	(83-108)
pCO2:	40	36	35	mmHg	(35-45)
pH:	7.33 *	7.39	7.43		(7.35-7.45)
Actual HCO3:	20 *	21 *	22	mmol/L	(22-28)
Std HCO3:	21 *	22	23	mmol/L	(22-28)
Base Excess:	- 4.5 *	- 2.6	- 1.2	mmol/L	(-3.0-+3.0)
Oxygen Sat:	98.5	97.7	96.3	%	(95.0-100.0
Sodium:	138	139	135	mmol/L	(135-145)
Potassium:	4.9	4.8	4.3	mmol/L	(3.3-4.9)
Chloride:	111 *	111 *	109	mmol/L	(95-110)
Ionised Ca:	1.05 *	1.08 *	1.08 *	mmol/L	(1.15-1.30)
Glucose:	14.1 ***	13.1 ***	8.0 *	mmol/L	(4.0-7.8)
Lactate:	3.1 *	2.8 *	2.1	mmol/L	(< 2.2)
Haemoglobin:	100 *	96 **	86 **	g/L	(130-180)
FO2Hb:	96.7	96.4	94.5	%	(94.0-98.0)
FCOHb:	0.8	0.6	1.1	%	(< 1.5)
FmetHb:	1.0	0.7	0.8	%	(< 1.5)
FHHb:	1.5	2.3	3.6	%	(< 5.0)
Haematocrit:	0.31 *	0.30 *	0.27 *	L/L	(0.40-0.54)

PLEASE NOTE CHANGE OF UNITS: 05/12/2016 Dorevitch Pathology has

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

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standardised reporting of HAEMOGLOBIN and HAEMATOCRIT on blood gases.
- Haemoglobin will report in grams/litre e.g. Haemoglobin 127 g/L
- Haematocrit will report in litre/litre e.g. Hct: 0.45 L/L

User Event History

User Id	Date/Time	Type	State	Text
	26-Feb-21, 19:37	Signing	Not For Signing	
	26-Feb-21, 19:33	Signing	Not For Signing	

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2.47. BLOOD CULTURE M/C/S

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: BLOOD CULTURE M/C/S
Group:
Laboratory #: 21-31331059-BZC-0
Clinician: BARADI,ARUL (435647FB)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 25-Feb-21, 17:15
Received: 25-Feb-21, 18:02

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			

```
BLOOD CULTURE

   1 aerobic and 1 anaerobic bottle received.
   COLLECTED:  5:15 pm on 25/02/2021

   CULTURE:
      No growth at 48 hours

      Culture continues but no further report will be issued unless bottles
      become positive.
```

User Event History

User Id	Date/Time	Type	State	Text
	27-Feb-21, 23:33	Signing	Not For Signing	
	26-Feb-21, 23:34	Signing	Not For Signing	
	25-Feb-21, 18:04	Signing	Not For Signing	

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2.48. COAGULATION PROFILE

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: COAGULATION PROFILE
Group:
Laboratory #: 21-31331058-COA-0
Clinician: BARADI,ARUL (435647FB)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 25-Feb-21, 17:15
Received: 25-Feb-21, 18:02
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

Coagulation Profile (Blood)

Coll.Date:	25/02/21	25/02/21			
Coll.Time:	14:50	17:15			
Lab.No:	31331022	31331058			
				Units	Ref.Range
APTT:	--	<20	--	seconds	(24-34) (Therapeutic 60-80)
INR:	--	1.1	--		(< 1.3)
Fibrinogen:	--	1.6	--	g/L	(1.5-4.0)

3133102225/02/21 Coag Profile - ? Query sample integrity. A recollection is recommended.

User Event History

User Id	Date/Time	Type	State	Text
	25-Feb-21, 18:45	Signing	Unsign	Unsigning from the translator
	25-Feb-21, 18:45	Mark for Attention	Mark	Marking for attention from the translator
	25-Feb-21, 18:04	Signing	Not For Signing	

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2.49. BLOOD CULTURE M/C/S

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: BLOOD CULTURE M/C/S
Group:
Laboratory #: 21-31331058-BZC-0
Clinician: BARADI,ARUL (435647FB)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 25-Feb-21, 17:15
Received: 25-Feb-21, 18:02

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			

```
BLOOD CULTURE

   1 aerobic and 1 anaerobic bottle received.
   COLLECTED:  5:15 pm on 25/02/2021

   CULTURE:
      No growth at 48 hours

      Culture continues but no further report will be issued unless bottles
      become positive.
```

User Event History

User Id	Date/Time	Type	State	Text
	27-Feb-21, 23:33	Signing	Not For Signing	
	26-Feb-21, 23:34	Signing	Not For Signing	
	25-Feb-21, 18:04	Signing	Not For Signing	

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2.50. BLOOD CULTURE M/C/S

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: BLOOD CULTURE M/C/S
Group:
Laboratory #: 21-31331030-BZC-0
Clinician: BARADI,ARUL (435647LJ)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 25-Feb-21, 15:06
Received: 25-Feb-21, 15:40
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

```
BLOOD CULTURE

   1 aerobic and 1 anaerobic bottle received.
   COLLECTED:  3:06 pm on 25/02/2021

   MICROSCOPY: Gram positive cocci.
               Detected in the aerobic bottle after 1 days incubation.

   CULTURE:
     Org 1:    Staphylococcus lugdunensis

               S. lugdunensis is a coagulase negative Staphylococcus that
               may be as pathogenic as S. aureus and causes similar types
               of infections.


   SUSCEPTIBILITY:

                        Org 1
   Flucloxacillin          S
   Vancomycin              S
   Penicillin              S
```

User Event History

User Id	Date/Time	Type	State	Text
	01-Mar-21, 12:21	Signing	Unsign	Unsigning from the translator
	01-Mar-21, 12:21	Mark for Attention	Mark	Marking for attention from the translator

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	28-Feb-21, 12:26	Signing	Unsign	Unsigning from the translator
	28-Feb-21, 12:26	Mark for Attention	Mark	Marking for attention from the translator
	27-Feb-21, 16:01	Signing	Unsign	Unsigning from the translator
	27-Feb-21, 16:01	Mark for Attention	Mark	Marking for attention from the translator
	27-Feb-21, 16:01	Signing	Unsign	Unsigning from the translator
	27-Feb-21, 16:01	Mark for Attention	Mark	Marking for attention from the translator
	27-Feb-21, 00:39	Signing	Unsign	Unsigning from the translator
	27-Feb-21, 00:39	Mark for Attention	Mark	Marking for attention from the translator
	25-Feb-21, 15:41	Signing	Not For Signing	

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2.51. HIGH SENS TROPONIN

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: HIGH SENS TROPONIN
Group:
Laboratory #: 21-31331022-STP-0
Clinician: UNCODED REFERRING DO,
Copies To:
Ordering Ward: ICC
Ordering Unit: BARAA

Collected: 25-Feb-21, 14:50
Received: 25-Feb-21, 15:07
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

SERUM / PLASMA HIGH SENSITIVE TROPONIN

Trop I (Beckman) : 48 ng/L (< 21) *

* Normal hs-Troponin is 0-10 ng/L (Female) and 0-20 ng/L (Male)
* In suspected Acute Coronary Syndrome consider repeat after 2-3 hrs
* Increased Troponin may occur with non-ischaemic injury such as rapid AF, pulmonary embolism, heart failure or sepsis
* Do NOT compare Troponin measured by different methods because the standardisation and units may differ

User Event History

User Id	Date/Time	Type	State	Text
	25-Feb-21, 18:37	Signing	Unsign	Unsigning from the translator
	25-Feb-21, 18:37	Mark for Attention	Mark	Marking for attention from the translator
	25-Feb-21, 17:29	Signing	Not For Signing	

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2.52. GENERAL BIOCHEMISTRY

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: GENERAL BIOCHEMISTRY
Group:
Laboratory #: 21-31331022-MBI-0
Clinician: UNCODED REFERRING DO,
Copies To:
Ordering Ward: ICC
Ordering Unit: BARAA

Collected: 25-Feb-21, 14:50
Received: 25-Feb-21, 15:07
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

SERUM/PLASMA BIOCHEMISTRY

Coll.Date: 25/02/21
Coll.Time: 14:50
Lab.No: 31331022

				Units	Ref.Range
Sodium:	133 *	--	--	mmol/L	(135-145)
Potassium:	4.4	--	--	mmol/L	(3.5-5.2)
Chloride:	95	--	--	mmol/L	(95-110)
Bicarbonate:	15 **	--	--	mmol/L	(22-32)
Urea:	4.1	--	--	mmol/L	(2.3-7.6)
Est. GFR:	41	--	--	mL/min	(> 60)
Creatinine:	168 **	--	--	umol/L	(60-110)
Bilirubin:	11	--	--	umol/L	(0-20)
ALT:	63 *	--	--	U/L	(0-55)
AST:	47 *	--	--	U/L	(0-40)
ALP:	77	--	--	U/L	(30-110)
GGT:	64 *	--	--	U/L	(0-50)
Tot.Protein:	55 *	--	--	g/L	(60-80)
Albumin:	33 *	--	--	g/L	(36-49)
Globulin:	22	--	--	g/L	(22-40)
Calcium:	2.16	--	--	mmol/L	(2.15-2.65)
Cor.Calcium:	2.31	--	--	mmol/L	(2.15-2.65)
Phosphate:	2.60 **	--	--	mmol/L	(0.75-1.50)
Magnesium:	1.16 *	--	--	mmol/L	(0.60-1.10)

Coll.Date: 25/02/21
Coll.Time: 14:50

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User Event History

User Id	Date/Time	Type	State	Text
	25-Feb-21, 15:43	Signing	Unsign	Unsigning from the translator
	25-Feb-21, 15:43	Mark for Attention	Mark	Marking for attention from the translator
	25-Feb-21, 15:08	Signing	Not For Signing	

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2.53. FULL BLOOD EXAMINATION

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: FULL BLOOD EXAMINATION
Group:
Laboratory #: 21-31331022-FBE-0
Clinician: UNCODED REFERRING DO,
Copies To:
Ordering Ward: ICC
Ordering Unit: BARAA

Collected: 25-Feb-21, 14:50
Received: 25-Feb-21, 15:07
Unsigned
Marked

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

FULL BLOOD EXAMINATION

Coll.Date:	04/01/18		25/02/21		26/02/21			
Coll.Time:	13:10		14:50		05:30			
Lab.No:	35524942		31331022		30318448			
							Units	Ref.Range
Haemoglobin:	150		121	*	75	**	g/L	(130-180)
WCC:	5.1		6.8		10.8		x10 ^9 /L	(4.0-11.0)
Platelets:	241		243		153		x10 ^9 /L	(150-450)
RCC:	4.56		3.63	*	2.26		x10 ^12 /L	(4.50-6.50)
PCV:	0.44		0.36	*	0.21	*	L/L	(0.40-0.54)
MCV:	95		98	*	93		fL	(80-96)
MCH:	33	*	33	*	33	*	pg	(27-32)
MCHC:	345		340		357		g/L	(320-360)
Neutrophils:	2.7		4.4		8.0		x10 ^9 /L	(2.0-8.0)
Lymphocytes:	1.9		2.1		2.2		x10 ^9 /L	(1.0-4.0)
Monocytes:	0.4		0.3		0.6		x10 ^9 /L	(0.0-1.0)
Eosinophils:	0.1		0.0		0.0		x10 ^9 /L	(0.0-0.5)
Basophils:	0.0		0.0		0.0		x10 ^9 /L	(0.0-0.2)

04/01/18 35524942 Essentially within normal limits.

25/02/21 31331022 Red cells, White cells and Platelets show normal morphology. There is a mild anaemia.

26/02/21 30318448 Red cells, White cells and Platelets show normal morphology. Stated acute blood loss.

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User Event History				
User Id	**Date/Time**	**Type**	**State**	**Text**
	04-Mar-21, 16:51	Signing	Unsign	Unsigning from the translator
	04-Mar-21, 16:51	Mark for Attention	Mark	Marking for attention from the translator
	25-Feb-21, 15:36	Signing	Unsign	Unsigning from the translator
	25-Feb-21, 15:36	Mark for Attention	Mark	Marking for attention from the translator
	25-Feb-21, 15:08	Signing	Not For Signing	

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2.54. CPK (CREATINE KINASE)

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: CPK (CREATINE KINASE)
Group:
Laboratory #: 21-31331022-CPK-0
Clinician: UNCODED REFERRING DO,
Copies To:
Ordering Ward: ICC
Ordering Unit: BARAA

Collected: 25-Feb-21, 14:50
Received: 25-Feb-21, 15:07

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			

```
SERUM/PLASMA BIOCHEMISTRY

   Coll.Date: 25/02/21
   Coll.Time: 14:50
      Lab.No: 31331022
                                                      Units  Ref.Range
         CPK:     99       --       --       --       -- U/L    (0-240)

   Coll.Date: 25/02/21
   Coll.Time: 14:50
```

User Event History

User Id	Date/Time	Type	State	Text
	25-Feb-21, 17:51	Signing	Not For Signing	
	25-Feb-21, 17:29	Signing	Not For Signing	

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

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2.55. COAGULATION PROFILE

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: COAGULATION PROFILE
Group:
Laboratory #: 21-31331022-COA-0
Clinician: UNCODED REFERRING DO,
Copies To:
Ordering Ward: ICC
Ordering Unit: BARAA

Collected: 25-Feb-21, 14:50
Received: 25-Feb-21, 15:07

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			

```
Coagulation Profile (Blood)

   Coll.Date:  25/02/21
   Coll.Time:  14:50
      Lab.No:  31331022

                                                    Units   Ref.Range




   3133102225/02/21  Coag Profile - ? Query sample integrity. A
  recollection is recommended.
```

User Event History

User Id	Date/Time	Type	State	Text
	25-Feb-21, 16:53	Signing	Not For Signing	
	25-Feb-21, 15:08	Signing	Not For Signing	

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Mercy Health
Care first

2.56. GASES (BLOOD)

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: GASES (BLOOD) **Collected:** 25-Feb-21, 14:31
Group: **Received:** 26-Feb-21, 19:34
Laboratory #: 21-47361147-GAS-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

BLOOD GASES

Date:	25/02/21	25/02/21	25/02/21		
Time:	16:15	14:14	14:31		
Lab.No:	47361149	47361146	47361147		
				Units	Ref.Range
Spec. Type:	Arterial	Venous	Venous		
FiO2:	27	21	50	%	
pO2:	114 *	25 ***	12 ***	mmHg	
pCO2:	40	41	45	mmHg	(35-45)
pH:	7.33 *	7.15 **	7.12 ***		(7.35-7.45)
Actual HCO3:	20 *	14 *	14 *	mmol/L	(22-28)
Std HCO3:	21 *	12 *	12 *	mmol/L	(22-28)
Base Excess:	- 4.8 *	-14.8 *	-14.8 *	mmol/L	(-3.0-+3.0)
Oxygen Sat:	98.5	33.0 ***	7.9 ***	%	
Sodium:	140	137	134 *	mmol/L	(135-145)
Potassium:	4.7	4.1	4.4	mmol/L	(3.3-4.9)
Chloride:	111 *	105	101	mmol/L	(95-110)
Ionised Ca:	1.06 *	1.14 *	1.16	mmol/L	(1.15-1.30)
Glucose:	14.3 ***	22.5 ***	23.2 ***	mmol/L	(4.0-7.8)
Lactate:	4.1 *	13.0 ***	13.4 ***	mmol/L	(< 2.2)
Haemoglobin:	106 *	131	126 *	g/L	(130-180)
FO2Hb:	96.7	32.7 *	7.8 *	%	(94.0-98.0)
FCOHb:	0.9	0.3	0.1	%	(< 1.5)
FmetHb:	0.9	0.7	0.7	%	(< 1.5)
FHHb:	1.5	66.3 *	91.4 *	%	(< 5.0)
Haematocrit:	0.33 *	0.40	0.39 *	L/L	(0.40-0.54)

PLEASE NOTE CHANGE OF UNITS: 05/12/2016 Dorevitch Pathology has standardised reporting of HAEMOGLOBIN and HAEMATOCRIT on blood gases.
- Haemoglobin will report in grams/litre e.g. Haemoglobin 127 g/L

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Mercy Health
Care first

```
 - Haematocrit will report in litre/litre   e.g. Hct: 0.45 L/L
COMMENTS:
          - The reference intervals for pH, PCO2 and PO2 apply for
            arterial samples only.
```

User Event History				
User Id	**Date/Time**	**Type**	**State**	**Text**
	26-Feb-21, 19:37	Signing	Not For Signing	
	26-Feb-21, 19:34	Signing	Not For Signing	

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Mercy Health
Care first

2.57. GASES (BLOOD)

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: GASES (BLOOD)
Group:
Laboratory #: 21-47361146-GAS-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 25-Feb-21, 14:30
Received: 26-Feb-21, 19:34

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

BLOOD GASES

Date:	25/02/21	25/02/21	25/02/21		
Time:	16:15	14:14	14:31		
Lab.No:	47361149	47361146	47361147		
				Units	Ref.Range
Spec. Type:	Arterial	Venous	Venous		
FiO2:	27	21	50	%	
pO2:	114 *	25 ***	12 ***	mmHg	
pCO2:	40	41	45	mmHg	(35-45)
pH:	7.33 *	7.15 **	7.12 ***		(7.35-7.45)
Actual HCO3:	20 *	14 *	14 *	mmol/L	(22-28)
Std HCO3:	21 *	12 *	12 *	mmol/L	(22-28)
Base Excess:	- 4.8 *	-14.8 *	-14.8 *	mmol/L	(-3.0-+3.0)
Oxygen Sat:	98.5	33.0 ***	7.9 ***	%	
Sodium:	140	137	134 *	mmol/L	(135-145)
Potassium:	4.7	4.1	4.4	mmol/L	(3.3-4.9)
Chloride:	111 *	105	101	mmol/L	(95-110)
Ionised Ca:	1.06 *	1.14 *	1.16	mmol/L	(1.15-1.30)
Glucose:	14.3 ***	22.5 ***	23.2 ***	mmol/L	(4.0-7.8)
Lactate:	4.1 *	13.0 ***	13.4 ***	mmol/L	(< 2.2)
Haemoglobin:	106 *	131	126 *	g/L	(130-180)
FO2Hb:	96.7	32.7 *	7.8 *	%	(94.0-98.0)
FCOHb:	0.9	0.3	0.1	%	(< 1.5)
FmetHb:	0.9	0.7	0.7	%	(< 1.5)
FHHb:	1.5	66.3 *	91.4 *	%	(< 5.0)
Haematocrit:	0.33 *	0.40	0.39 *	L/L	(0.40-0.54)

PLEASE NOTE CHANGE OF UNITS: 05/12/2016 Dorevitch Pathology has standardised reporting of HAEMOGLOBIN and HAEMATOCRIT on blood gases.
- Haemoglobin will report in grams/litre e.g. Haemoglobin 127 g/L

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

Mercy Health
Care first

```
 - Haematocrit will report in litre/litre   e.g. Hct: 0.45 L/L
COMMENTS:
          - The reference intervals for pH, PCO2 and PO2 apply for
            arterial samples only.
```

User Event History

User Id	Date/Time	Type	State	Text
	26-Feb-21, 19:37	Signing	Not For Signing	
	26-Feb-21, 19:34	Signing	Not For Signing	

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Mercy Health
Care first

2.58. GASES (BLOOD)

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test: GASES (BLOOD)
Group:
Laboratory #: 21-47361149-GAS-0
Clinician: MAJUMDAR,MAINAK (290497GX)
Copies To:
Ordering Ward: WMC
Ordering Unit: BARAA

Collected: 25-Feb-21, 04:10
Received: 26-Feb-21, 19:32

Test Status: Final results
Performed By:

Test Type	Value	Units	Ref.Range	Normality
	See Below			Abnormal

BLOOD GASES

Date:	25/02/21	25/02/21	25/02/21		
Time:	16:15	14:14	14:31		
Lab.No:	47361149	47361146	47361147		
				Units	Ref.Range
Spec. Type:	Arterial	Venous	Venous		
FiO2:	27	21	50	%	
pO2:	114 *	25 ***	12 ***	mmHg	
pCO2:	40	41	45	mmHg	(35-45)
pH:	7.33 *	7.15 **	7.12 ***		(7.35-7.45)
Actual HCO3:	20 *	14 *	14 *	mmol/L	(22-28)
Std HCO3:	21 *	12 *	12 *	mmol/L	(22-28)
Base Excess:	- 4.8 *	-14.8 *	-14.8 *	mmol/L	(-3.0-+3.0)
Oxygen Sat:	98.5	33.0 ***	7.9 ***	%	
Sodium:	140	137	134 *	mmol/L	(135-145)
Potassium:	4.7	4.1	4.4	mmol/L	(3.3-4.9)
Chloride:	111 *	105	101	mmol/L	(95-110)
Ionised Ca:	1.06 *	1.14 *	1.16	mmol/L	(1.15-1.30)
Glucose:	14.3 ***	22.5 ***	23.2 ***	mmol/L	(4.0-7.8)
Lactate:	4.1 *	13.0 ***	13.4 ***	mmol/L	(< 2.2)
Haemoglobin:	106 *	131	126 *	g/L	(130-180)
FO2Hb:	96.7	32.7 *	7.8 *	%	(94.0-98.0)
FCOHb:	0.9	0.3	0.1	%	(< 1.5)
FmetHb:	0.9	0.7	0.7	%	(< 1.5)
FHHb:	1.5	66.3 *	91.4 *	%	(< 5.0)
Haematocrit:	0.33 *	0.40	0.39 *	L/L	(0.40-0.54)

PLEASE NOTE CHANGE OF UNITS: 05/12/2016 Dorevitch Pathology has standardised reporting of HAEMOGLOBIN and HAEMATOCRIT on blood gases.
- Haemoglobin will report in grams/litre e.g. Haemoglobin 127 g/L

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Mercy Health
Care first

```
 - Haematocrit will report in litre/litre   e.g. Hct: 0.45 L/L
COMMENTS:
          - The reference intervals for pH, PCO2 and PO2 apply for
            arterial samples only.
```

User Event History				
User Id	**Date/Time**	**Type**	**State**	**Text**
	26-Feb-21, 19:37	Signing	Not For Signing	
	26-Feb-21, 19:33	Signing	Not For Signing	

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2.59. Radiology

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test #: MIA-WMH-77.30320773_1
Group:
Test Type: Radiology
Clinician:
Copies To:
Ordering Ward:
Ordering Unit: BARAA

Collected: 25-02-2021, 00::0
Received: 25-Feb-21, 00::0
Unsigned
Marked

Test Status: Final results.
Performed By:

Data:

Name of Test : CT ABDOMEN AND PELVIS WITH CONTRAST, CT BRAIN W/OUT CONTRAST, CT ANGIOGRAPHY - CHEST/ABDOMEN/UPPER LIMBS

UR: 2645287

McLean, Richard
2 McCubbin Street, FOOTSCRAY VIC 3011
Birthdate: 08/04/1973 Age: Y47 Sex: M

Addendum

ADDENDUM REPORT
Left arm

The left subclavian artery, brachial, radial and ulnar arteries show normal

contrast opacification.

No pseudo aneurysm demonstrated.

There are subcutaneous pocket of gas in the region of the cubital fossa, in

keeping with the given history of laceration. There is a superficial vein coursing adjacent to this. There is soft tissue stranding, however no well-circumscribed haematoma is shown.

Addendum Report Reported by: Dr Neha Singh

Addendum electronically signed at 1:06 am Fri, 26th Feb 2021

ORIGINAL REPORT
CT of head, chest and abdomen and pelvis

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CLINICAL DATA

Hypertension? Head strike question a source of infection

TECHNIQUE

IV contrast portal venous phase CT abdomen and pelvis performed.

FINDINGS

Head

No intracranial bleed or surface fluid collection. Grey-white matter differentiation is preserved. No intracranial mass lesion or raised intracranial pressure. No hydrocephalus. No skull fracture or destructive bony lesion. No sinusitis or mastoiditis.

Chest

No hydropneumothorax or pericardial effusion. No mediastinal collection. No consolidation, lung nodule, mass lesion, or bronchiectasis. Thoracic aorta is unremarkable.

No lymphadenopathy.

No PE.

No fracture or bony lesion.

Abdomen and pelvis

IDC is in situ and bladder is empty.

Right femoral vein catheter in place.

No diverticulosis or enterocolitis.

No mechanical bowel obstruction ischaemia or perforation.

No appendicitis.

No free peritoneal gas of fluid.

No retroperitoneal or abdominal wall haematoma.

Liver bile ducts gallbladder spleen adrenal glands both kidneys renal collecting systems and pancreas are unremarkable.

Intra-abdominal vessels enhancing normally.

No lymphadenopathy.

No active GI bleed.

CONCLUSION

No acute intracranial pathology.

No intrathoracic or intra-abdominal source of infection identified.

Original Report Reported by: Dr Amirhossein Shourcheh

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Mercy Health
Care first

Electronically signed at 9:25 pm Thu, 25th Feb 2021

cc: Werribee Mercy Hospital

cc: MERCY HOSPITAL, Werribee, 300 Princes Highway WERRIBEE VIC 3030

Dictated By: cc: MERCY HOSPITAL, Werribee, 300 Princes Highway WERRIBEE VIC 3030

Approved By:

User Event History

User Id	Date/Time	Type	State	Text
	26-Feb-21, 01:09	Signing	Unsign	Unsigning from the translator
	26-Feb-21, 01:09	Mark for Attention	Mark	Marking for attention from the translator
	25-Feb-21, 21:29	Signing	Unsign	Unsigning from the translator
	25-Feb-21, 21:29	Mark for Attention	Mark	Marking for attention from the translator

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Mercy Health
Care first

2.60. Radiology

McLean, Richard **2645287(MHS)**
08-Apr-1973 - **M**
2 Mc Cubbin Street
Footscray, 3011

Test #: MIA-WMH-77.30318995_1
Group:
Test Type: Radiology
Clinician:
Copies To:
Ordering Ward:
Ordering Unit: BARAA

Collected: 25-02-2021, 00::0
Received: 25-Feb-21, 00::0
Unsigned
Marked

Test Status: Final results.
Performed By:

Data:

Name of Test : CHEST X-RAY (MOBILE)

UR: 2645287

McLean, Richard
2 McCubbin Street, FOOTSCRAY VIC 3011
Birthdate: 08/04/1973 Age: Y47 Sex: M

X-RAY CHEST - AP ERECT VIEW:

Clinical Notes: Undifferentiated shock

Findings:

The heart is not enlarged.

The superior mediastinum is within normal limits. The hilar contours are normal.

The lungs have a normal volume.

The bronchovascular markings are normal. There is no airspace opacification.

The pleural spaces are clear.

There is no significant skeletal abnormality.

Conclusion:

No significant abnormality detected.

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Mercy Health
Care first

Dr Paul MENSSINK
Electronically signed at 8:32 am Fri, 26th Feb 2021

cc: Werribee Mercy Hospital

cc: MERCY HOSPITAL, Werribee, 300 Princes Highway WERRIBEE VIC 3030

Dictated By: cc: MERCY HOSPITAL, Werribee, 300 Princes Highway WERRIBEE VIC 3030

Approved By:

User Event History

User Id	Date/Time	Type	State	Text
	26-Feb-21, 08:49	Signing	Unsign	Unsigning from the translator
	26-Feb-21, 08:49	Mark for Attention	Mark	Marking for attention from the translator

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2.61. Echocardiogram (typed) (FMH536500)

1-Mar-2021 08:07 To: Reference 1 From: Werribee Heart - 61387427788 p.1

Wyndham Private Medical Centre
1st Flr, 242 Hoppers Lane, Werribee, Vic 3030
Ph: (03) 9908 2999
Fax: (03) 8742 7788

TRANSTHORACIC ECHOCARDIOGRAM

Patient: MCLEAN, RICHARD **Date of birth:** 08/04/1973 Age: 47
Address:
Gender: M **File no:** 875-21
Height: 178 cm **Weight:** 100 kg
BSA: 2.25 m² **Exam Date:** 25/02/2021 16:13
Copies to: WMH ICU
Indications: shock of unknown origin

LV Dimension			***LV/LA Volumes***			***LA/RA Area***			***Aorta***		
IVSd:	1.22	cm	EF Biplane:		%	LA Area:	13	cm²	LVOT Diam:	2.4	cm
LVPWd:	0.9	cm	LVEDV:		ml	RA Area:		cm²	Aortic Root:	3.2	cm
LVIDd:	4.3	cm	LVESV:		ml				Aorta STJ:		cm
LVIDs:	2.7	cm	LA Vol:	31	ml				Asc Aorta:	3.8	cm
			LA VI:	13.8	ml/m²				Arch:		cm
Aortic Valve			***Mitral Valve***			***Pulmonary Vein***			***Pulm/Tricuspid Valve***		
LVOT Vmax:	0.84	m/s	E vel:	0.45	m/s	Pulm s:		m/s	PV Vmax	1.20	m/s
AV Vmax:	1.11	m/s	Decel time:	85	ms	Pulm d:		m/s	TR Vmax		m/s
AV max grad:	5	mmHg	A vel:	0.55	m/s	S/D ratio:			TR max grad:		mmHg
AV mean grad:	3	mmHg	E' sept:	0.09	m/s	Pulm A:		m/s	Est RAP:		mmHg
AVA:	3.6	cm²	E/E'	4.92		A dur:		ms	PV regurg:		
Regurg:			Regurg:						TV regurg:		

FINDINGS

Left Ventricle Normal left ventricular size and wall thickness. Normal LV ejection fraction (LVEF 60 +/- 5%) with no regional wall motion abnormalities. EA fusion diastolic filling pattern and normal estimated mean LA pressure (e' average = 9 cm/s, E/e' average = 5).

Right Ventricle Normal right ventricular size and contraction

Left Atrium Normal sized left atrium. Intact interatrial septum with no colour Doppler evidence of an interatrial shunt.

Right Atrium Normal sized right atrium. Normal IVC diameter with normal inspiratory collapse.

Aortic Valve Trileaflet aortic valve with unrestricted opening and no regurgitation. Normal sized aortic root and proximal ascending aorta. Normal flow in the descending thoracic aorta.

Mitral Valve Normal mitral valve with trivial regurgitation. Chordal SAM.

Tricuspid Valve Normal tricuspid valve with trivial regurgitation.

Pulm Art Pressure Unable to estimate pulmonary artery pressure due to insufficient TR.

Pulmonary Valve Unremarkable pulmonary valve.

Pericardium No pericardial effusion.

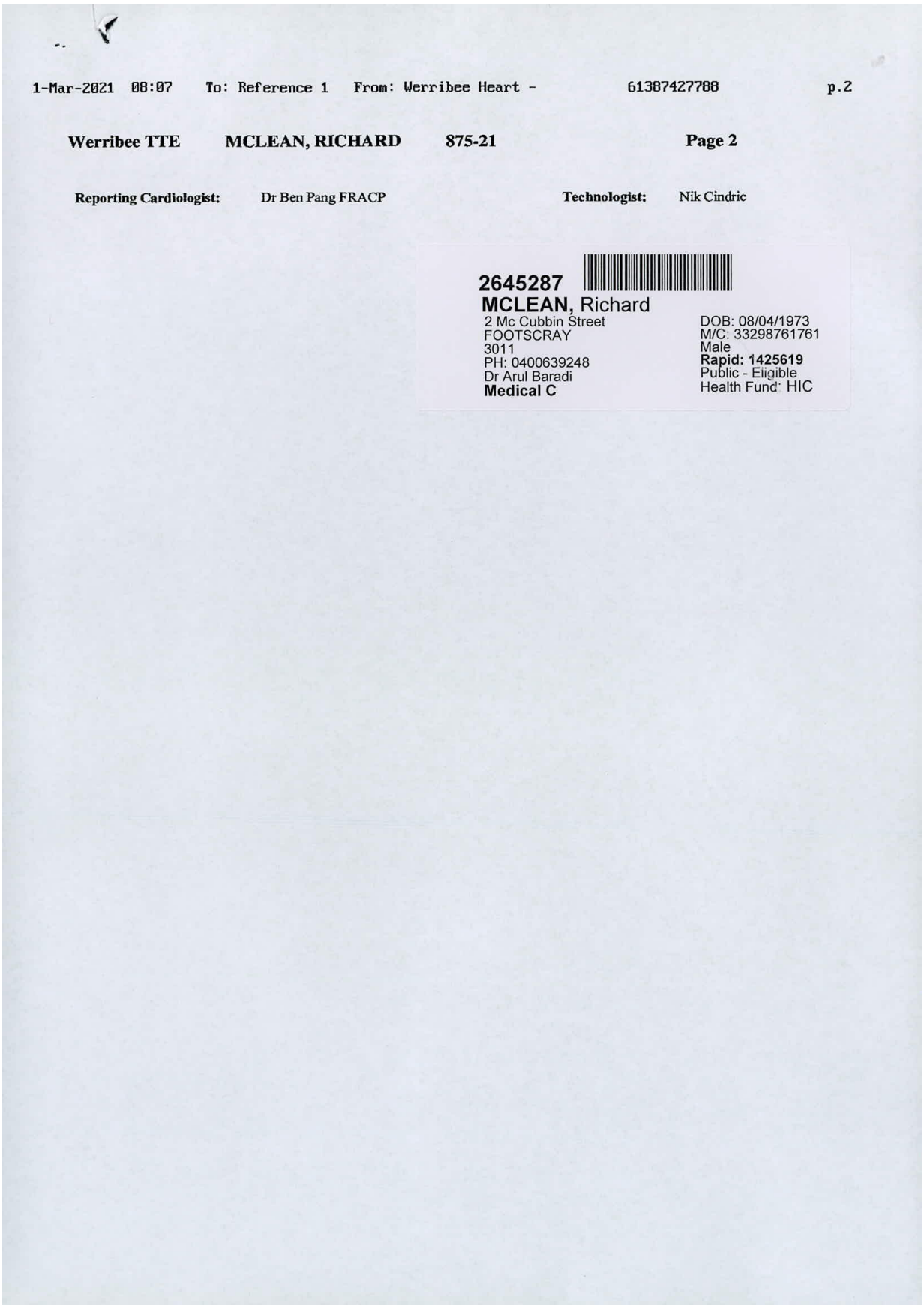
CONCLUSIONS

Normal LV size and hyperdynamic LV systolic function.
Normal atrial size.
Satisfactory valvular function.

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C
DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

2.62. Echocardiogram (typed) (FMH536500)



1-Mar-2021 08:07 To: Reference 1 From: Werribee Heart - 61387427788 p.2

Werribee TTE **MCLEAN, RICHARD** **875-21** **Page 2**

Reporting Cardiologist: Dr Ben Pang FRACP **Technologist:** Nik Cindric

2645287

MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

2.63. External Radiology (FMH300200)

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

Medical Imaging Report

Werribee Mercy Hospital

Waverley Private Hospital, 343 Blackburn Road
Werribee VIC, 3030
61394175788

REPORT PREPARED FOR:
null Werribee Mercy Hosp - ICU Reception
Ordered On:
Feb 25, 2021, 9:11 PM

PATIENT NAME: Richard MCLEAN **DOB:** 8/04/1973

MRN / UR: Visage-77.8507142

ACCESSION NUMBER(S): 77.30320773_1

REPORT:
ADDENDUM REPORT
Left arm
The left subclavian artery, brachial, radial and ulnar arteries show normal contrast opacification.
No pseudo aneurysm demonstrated.
There are subcutaneous pocket of gas in the region of the cubital fossa, in keeping with the given history of laceration. There is a superficial vein coursing adjacent to this. There is soft tissue stranding, however no well-circumscribed haematoma is shown.
Addendum Report Reported by: Dr Neha Singh

ORIGINAL REPORT
CT of head, chest and abdomen and pelvis
CLINICAL DATA
Hypertension? Head strike question a source of infection
TECHNIQUE
IV contrast portal venous phase CT abdomen and pelvis performed.
FINDINGS
Head
No intracranial bleed or surface fluid collection. Grey-white matter differentiation is preserved. No intracranial mass lesion or raised intracranial pressure. No hydrocephalus. No skull fracture or destructive bony lesion. No sinusitis or mastoiditis.
Chest
No hydropneumothorax or pericardial effusion. No mediastinal collection. No consolidation, lung nodule, mass lesion, or bronchiectasis. Thoracic aorta is unremarkable.
No lymphadenopathy.
No PE.
No fracture or bony lesion.
Abdomen and pelvis
IDC is in situ and bladder is empty.
Right femoral vein catheter in place.
No diverticulosis or enterocolitis.
No mechanical bowel obstruction ischaemia or perforation.
No appendicitis.
No free peritoneal gas of fluid.
No retroperitoneal or abdominal wall haematoma.
Liver bile ducts gallbladder spleen adrenal glands both kidneys renal collecting systems and pancreas are unremarkable.
Intra-abdominal vessels enhancing normally.
No lymphadenopathy.
No active GI bleed.
CONCLUSION
No acute intracranial pathology.
No intrathoracic or intra-abdominal source of infection identified.
Original Report Reported by: Dr Amirhossein Shourcheh

REPORT PREPARED BY:
i-TeleRAD Pty Ltd
ph: 1800 009 945

Report Printed: 25/02/2021

Page 1 of

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Mercy Health
Care first

2.64. External Radiology (FMH300200)

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

Medical Imaging Report

Werribee Mercy Hospital

Waverley Private Hospital, 343 Blackburn Road
Werribee VIC, 3030
61394175788

REPORT PREPARED FOR:
null Werribee Mercy Hosp - ICU Reception
Ordered On:
Feb 25, 2021, 9:11 PM

PATIENT NAME: Richard MCLEAN **DOB:** 8/04/1973
MRN / UR: Visage-77.8507142
ACCESSION NUMBER(S): 77.30320773_1

REPORT PREPARED BY:
I-TeleRAD Pty Ltd
ph: 1800 009 945

Report Printed: 25/02/2021

Page 2 of

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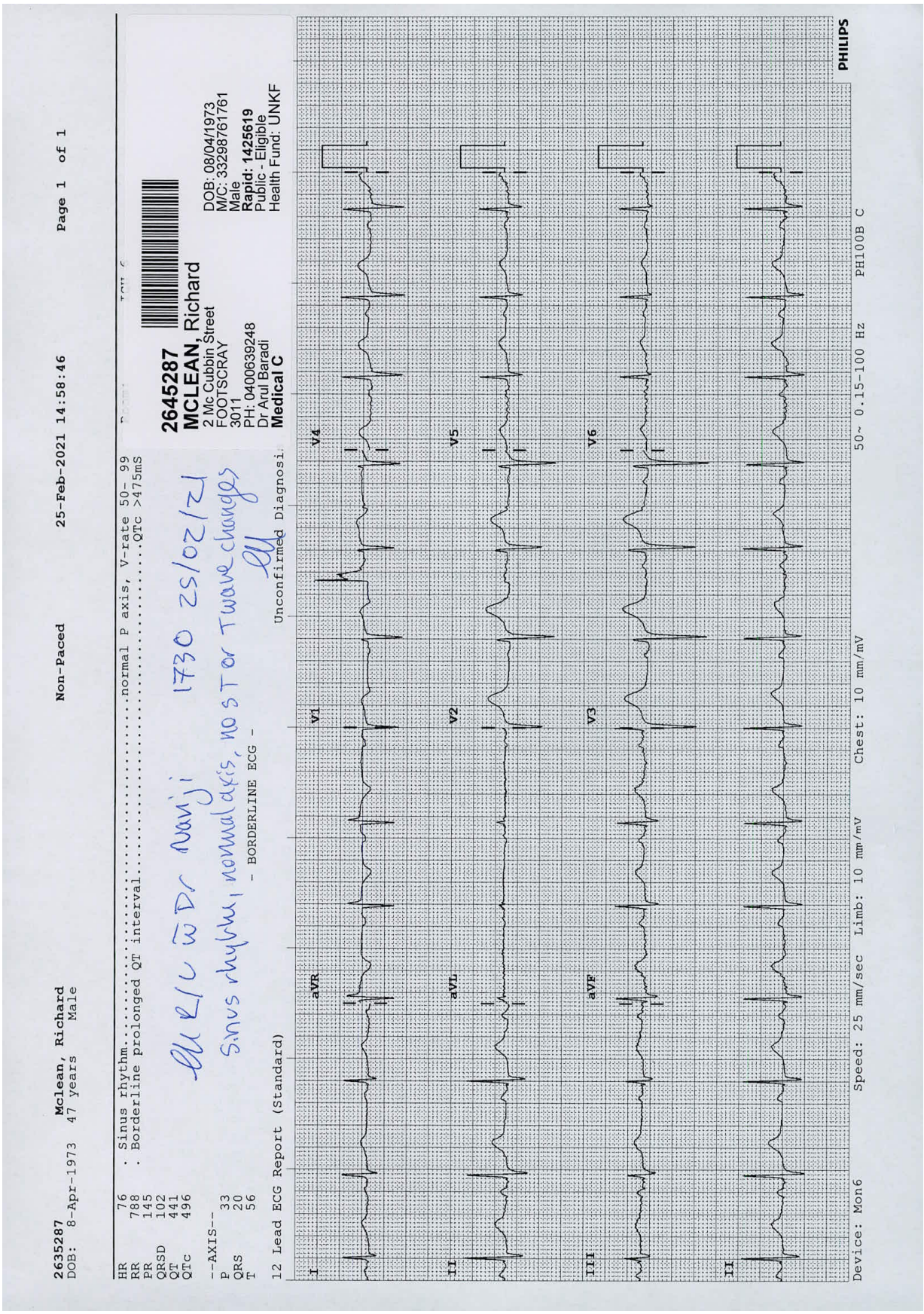
2.65. ECG Graph ECG Report (FMH300500)



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3. Correspondence

3.1. Centrelink Documents (FMH402200)

Australian Government
Department of Human Services

centrelink

Medical Certificate

Centrelink customers: did you know? You can lodge this certificate with us using Online Services or one of our Express Plus Apps.
Go to **humanservices.gov.au/submitdocumentsonline**

Patient's details

CRN 305-689-451-X

Family name: Mclean
First name: Richard
Second name:
Date of birth: 8/4/1973

Home address:
Postcode:

This person has been: My patient since 24/02/2021
A patient of this practice since 23/02/2021

Primary condition | **Secondary/Related condition(s)**

Diagnosis — Please list the main medical conditions which impact on the patient's capacity to work or study (Primary condition should be the condition with the **most** impact).

Paranoia Querulans

Date of onset (if known) / /

his condition – Tick *ONE* only
Temporary ☐ Permanent (likely to persist for 2 years or more) ☐ Temporary exacerbation of a permanent condition ☐

Prognosis — Please estimate how long the symptom(s) will affect the patient's capacity to work or study.
Less than 3 months ☐ 13–24 months ☐ Uncertain ☐
3–12 months ☐ More than 24 months ☐

Symptoms — Please list current symptoms for each condition.

Paranoia
Disordered thoughts
Persecutory delusions

Treatment — Please describe the patient's treatment regime, including past, current and planned treatment.

Past: Antidepressants
Current: Antidepressants, Anti-psychotics
anned: CBT (Psychotherapy)

Please give details of any other medical conditions which impact on the patient's capacity to work or study.

Depression

Capacity to work or study

In my opinion the patient is/has been unfit for work/study
from 23/2/21 to 25/3/21

Can the patient currently undertake their usual work or study?
Yes ☐ No ☑

Can the patient do any other work for 8 hours or more per week?
Yes ☐ No ☑

In order to prepare your patient for return to work or study, certain assistance may be offered. Please identify any factors which may impact on participation.

Is this condition – Tick *ONE* only
Temporary ☐ Permanent (likely to persist for 2 years or more) ☐ Temporary exacerbation of a permanent condition ☐

Date of onset (if known) / /

Prognosis —
Less than 3 months ☐ 13–24 months ☐ Uncertain ☐
3–12 months ☐ More than 24 months ☐

Past:
Current:
Planned:

Certification by Medical Practitioner

Doctor's name (printed): MUSTAFA ABUQLAM
Qualifications: MBBS Provider no. 5757922H
Surgery/Medical Centre/ Hospital name: Mercy Mental Health
Address: Princes Highway
Postcode:
Phone number: ( )
Signature:
Date: 05/03/2021

CLK0SU415 1507

SU415.1507

1 of 2

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3.2. Centrelink Documents (FMH402200)

Privacy and your personal information

Your personal information is protected by law, including the *Privacy Act 1988*, and is collected by the Australian Government Department of Human Services for the assessment and administration of payments and services. This information is required to process your application or claim.

Your information may be used by the department or given to other parties for the purposes of research, investigation or where you have agreed or it is required or authorised by law. Where necessary Human Services or your assessor may contact your doctor(s) and other treating providers to clarify information provided about your medical conditions.

You can get more information about the way in which the Department of Human Services will manage your personal information, including our privacy policy, at **humanservices.gov.au/privacy** or by requesting a copy from the department.

SU415.1507

2 of 2

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3.3. Correspondence generic (FMH400001)

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

Assessment

Date & Time: 23/02/2021 9:45:39 PM
Completed: 24/02/2021 12:37:00 AM
Staff Name: MachemB

Address 2 Mc Cubbin Street
3011 Footscray
VIC
Australia

Home Phone : 0400639248 **Work Phone :**

Address 2 Mc Cubbin Street
3011 Footscray
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Home Phone (AH) 0400639248 **Work Phone (BH)**

Screening Register

Screening Detail

Presenting Problem:

Introduction:
Clinician Name: Nicole Withers
Position: P3
Date: 14/2/2021
Time: 11:45

Situation
REFERRAL REASON AND / OR REASON FOR CALL:
P/C from Allanah; 0403117361
Concerned friend, reported to be a mental health therapist. Clarified is an NDIS art therapist.
States that she is worried that Richard's MH may be declining as has noticed on his website that he is escalating in distress and listing suicide dates with increased "venom" towards former GP and health care professionals.
States that he is "paranoid" about being refused services.
Making reference to ASIO
tting up $5 million fundraisers on Facebook.
...ports he has increased stress: lost job as NDIS peer worker (as misrepresenting himself allegedly as therapist) and also has had car repossessed.
States Richard to be usually "quite vocal" but concerned that he is now placing himself in danger of vulnerability and deterioration.
Spoke of anonymity in referral, however, when explaining need to disclose if asked re referrer details (as cold calls not appropriate) Allanah agreed to be named.

Relevant Background
Current legal status: VOL

CMI/Master care Search:
Nil CMI hx.
Referred to CM in Sept 2020, not accepted for CM.

Diagnosis: As per file: Schizophrenia (disputed) and ADHD
ALERTS: Nil formal listed
Current Medications: did not disclose full medication list. Stated he is on Dexamphetamine and this is primary treatment.
Relevant Medical Hx: Did not disclose.

Created By: Brian Machemedze (Nurse)
Printed By: Bryan Dimabuyo (Nurse)

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Current supports: GP and PP but states poor therapeutic relationship with same.

levant Psychosocial Factors: isolated. Hx of childhood sexual abuse, reported limited friendships, invalidated by medical professionals. Financial debt: $50K

Relevant Risk Hx : Hx of suicide attempt via overdose 3 years ago. States now has plan to kill himself (did not disclose concrete suicide plan) on his birthday (8/4) this year.

Substance abuse current use and relevant Hx: opportunistic methamphetamine and amphetamine misuse reported.

Assessment –
MENTAL STATE EXAMINATION:

Behaviour: cooperative with underlying irritability
Speech: Slight pressure, talking over writer at times. Difficult to distract.
Reported Mood: ventilated at length re: marginalization, and longstanding anxiety, grief and pain.
Thought Form: Nil clear FTD

Thought Content:
troduced self and reason for call. Willing to speak to writer.
oke at length regarding "conspiracy to silence" him and marginalisation by health professionals.
Spoke of intention to kill himself on 8/4 as he does not feel he can live in a world "like this" anymore.
Spoke of medical professionals as the "right wing of shamanism" and that Psychiatrists are the "Nazi's of them all", focusing in on the perceived power imbalance in appointments and belief that medications are designed to dull people to make them normal.
States that he is a "hedonist" and that he accepts that some of his actions are done to "self medicate" reported anxiety, grief and pain, describing long history of loss, abuse (since childhood), suicide of friend, invalidation by family, LGBTI discrimination, poor relationships etc.
Reported to be a mental health advocate for 20+ years and that he is "famous" having written a book about his experiences.
States his website: **richmclean.com.au** outlines his frustration with services to date, explaining he had been victim of medical negligence by former GP and that his attempts to pursue claim legally have been blocked by those in power with cover up of situation. States that he advised GP of plan to overdose 3 years ago and was ignored, given medication which he then used to overdose. Believes there is a large cover up/conspiracy by government, including AHPRA up to minster for health and he can no longer obtain legal supports to assist him as he is "blacklisted".
States he also is struggling to obtain any health care supports due to this interaction with GP and subsequent issues with PP Dr David Horgan. (all listed on his website publically with voice recordings of alleged inappropriate interactions).
Spoke then of former partner, who is allegedly working for ASIO and despite this is a drug dealer with extensive tax

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evasion in the past. Richard believes this to have contributed to his own debt of $50K and previous homelessness.
reported to be "self-medicating" with ice and gambling. Encouraged harm minimisation.
oke of supports which may be offered to assist to find psychiatrist and psychologist who may be more validating and establish better therapeutic relationships. Spoke of his interest in having review for diagnoses and medication discussion (suggested ADHD and Dexamphetamine to be useful in the past). Seeking support to discuss financial state and link with LGBTI supports. Agreed that he would like to meet to discuss his story in detail, and invited clinicians who will be seeing him to review his website prior to visiting. Requested clinic based visits only- has dog, but largely does not consent to visits in the home, preferring clinic reviews.
States he is "ok today" and has intention to go on Grinder and see if he can find "someone" to spend his afternoon with (encouraged safety) and invited CATT to contact him on 15/2 to arrange assessment of needs in clinic.
Upon termination of call, requested clinician full name and details of qualifications. States he "usually records these calls" but had forgotten to do so today and wanted to create a record of communication with Mercy Health.

Perceptual disturbances: Nil reported perceptual disturbances. Nil clear evidence of same during call.
Orientation: Not assessed in this call.
Judgement: Seemingly somewhat impaired given pursuit of numerous legal cases against professionals and public statements on social media and website.
Insight: Impaired. Does not see medications as useful to "help" as sees this as attributing blame onto individuals for their problems. States primary medications he is willing to explore as Dexamphetamine, however, willing to explore review and malisation of diagnosis and medication prescription.

Recommendation / Plan:

Plan: CATT CAT D referral.

H/O provided to D Spicer: RPN 4 CATT duty worker.

N Withers P3
23/2/2021 09:10
P/C from Brad: brother 0435033888
Expressing frustration and concern for brother. Attempting to escalate concern for CATT involvement as states his brother to be complex, but has alerted family to suicide plan via Endone stockpile on his birthday in April. States brother to be "unreliable" with dates and that he feels the risk is more immediate than April.
Spoke at length about presentation describing him as

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paranoid, persecuted, manipulative and impulsive. Spoke of fear that Richard is compromising his family and their ...ationships, financial stress and mismanagement (spoke of VCAT) employment and health (AOD longstanding and ....sadventure).
States that his brother has been under reporting his illness and again directed Triage to review website.
Writer redirected to welfare check via 000 in event of suicidal ideation and statements, validating difficulties in snapshot assessments and criteria for MHA. Link sent to Brad outlining MHA criteria and encouraged review, suggesting challenge is the "immediacy" of risk and "least restrictive" means.
Brad spoke of hx of Schziophrenia and "it's just so clear".

In light of deteriorating patient principles, discussed with Cons Psych for consideration of reassessment Dr Ellie Debenham. Case reviewed, suggested possible lack of clear assessment of alleged longstanding psychotic phenomena. Assessment notes indicate need for collateral from NOK to inform treatment, and requested CATT manager be alerted to review notes for re-assessment/discussion with their Consultant as notes leave medico-elgal risk. Dr Debenham not in position of being able to discuss directly with CATT Consultant, Manager to manager conversation to be facilitated (N Strensjak aware).

****

23/02/21 1600hrs
Natasha Stresnjak EMH Manager

Phone contact with CATT Manager Andrea Kavanaugh.
Discussed current situation of call received by brother today.
Review of clinical notes, including review by Psychiatrist on 16/02/21. For further review whether origional referrer/brother had been contacted at discharge from CATT.
Plan: Handover to Triage, for client to be contacted this evening for further assessment, and review of medication management (allegations made by referrer of stockpiling medications).

Claire Wennell RPN4 Triage
23/02/2021

1736hrs

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p/c to client's father Doug after message left 0416 104587
states he has called lifeline regarding concerns about his son's mental health and was redirected to call our triage
advised i am aware of his son's case and that in fact we were planning to call him again this afternoon
Doug states he last spoke directly to him yesterday but both he and the client's Mum April have had text messages from him today
he reports usually client speaks to them "as a unit, toga=ether" but lately he has been contacting them separately
Doug reports he has been asking them for help with his mental health
he has increasingly been talking about plans to kill himself on his birthday
Doug states that he has had a couple of friends who have suicided
states client has also told them he has made previous attempts - not sure re details
he has also been expressing conspiracy theories to them in line with the content on his website **www.richmclean.com.au** and his facebook page
asked how long they have had concerns for his mental health Doug says he has been deteriorating over the past 9-12/12.
Doug states that prior to that he wasn't expressing these things.
Doug reports the client has written a book about Schizophrenia and that he has a Masters Degree and a PHD and works as a mental health advocate for the NDIS but has had professional issues lately
states NDIS have told him he is not qualified to practice in the manner he has been and has been overcharging for his services, which means he has incurred a debt
he asked them recently to "leave me out of your will but give me $50K now"
Doug reports he has considerable financial issues and that today he contacted them asking for some money as he doesn't even have money for food
they sent him $100 and the client replied by text @1748hrs saying thank you via his Mum's phone but also said "I'm a drain and I'm ashamed."
Doug states that he is very educated and he thinks that his Dad is not on the same level academically so Doug thinks that is why he sends him text messages mostly rather than calling him
Doug was quite teary and says he just wants to get help for his son
he says he told him he doesn't have the expertise to help him himself
however, he hasn't told him he planned to call us
he reports that he needs different medication and that he asked for a recommendation of a psychiatrist when he recently saw another Doctor but that they told him they could not provide any recommendations for him - Doug states the client recorded this conversation
Doug states the client is becoming increasingly frustrated and feels no one is helping him
today in one text to his Mum he said that he has moved house and that "you don't know where I am"

Created By: Brian Machemedze (Nurse)
Printed By: Bryan Dimabuyo (Nurse)

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last address known to parents is 2 McCubbin Street, Footscray
Doug doesn't know of anywhere else he may go except perhaps a female friend (ex house mate) called Suzie who lives somewhere in Ocean Grove - no address known
however, he thinks it is more likely the client is still at his home address currently as he has no money (although he now has $100 they transferred to him)
Doug reports the client's car was damaged when someone tried to steal it around 10/7 ago , and he isn't sure if he has been able to get the ignition barrel replaced due to a lack of funds contrary to this a friend who called earlier today says the client's car has been repossessed
client also has a dog which he cares a lot for
advised Doug that we will refer client back to CATT for further Ax
he is happy with this plan - advised we will keep him informed

RISKS/IMPRESSION:
client has apparently been stockpiling Endone according to family
increasing expression of suicidal ideation, plan and intent
past hx of suicide attempts
considerable demographic risk factors including gender, age, recent issues with work and debt, loss of reputation
also appears to be psychotic
client has also spoken about his risk of homelessness due to a lack of funds
may be a flight risk - has told parents he has left address - ?Dad thinks this is not likely today

PLAN:
case further d/w Psych AHC due to potential flight risk
agrees referral to CATT as CAT C without speaking to client is warranted based on collateral obtained from client's family about client being possible flight risk
seems he would benefit from admission for more longitudanal Ax of MSE & risk

OUTCOME:
referred to CATT as Cat C via EMR and verbal h/o to duty worker
see previous safe h/v tool

****

Date: 23/02/2021

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Time: 2130

nician: Brian Machemedze (RPN5)

Point of service: AHC

Summary of AHC activities

- EMH consulted AHC on the case and due to unacceptable levels of risk I have advised referral to CATT as CAT C. I have considered historical information/medical record, friends and family reports and posts online
- CATT duty consulted with AHC and I strongly advised testing MHA 2014 IAO on the basis of risk and presence of a mental illness. Client has multiple risk indicators and is demonstrating poor decision making and judgement. Further assessment by a Consultant Psychiatrist is indicated. Risks will be challenging to mitigate in the community. Please note client has means and access (stockpiling medications)
- I note this client is recording all communications via video

se Consult with On-call Consultant Psychiatrist – Dr Rasha Rahman

P/C to Dr Rahman for case consult reasons

Summary

- Comprehensive handover provided
- Risk picture explain
- MHA 2014 IAO notification completed
- Agreed to remove all devices that can record others without consent and would breach privacy of other patients
- Target bed is CMB Pod 3 at this stage – direct admit.
-

Screening Description:

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Triage Scale: G - Advice or information only OR more information is ne
Previous Triage Scale: B - Crisis mental health response
Service Response: AMHS response
Outcome: Refer to crisis assessment service
Outcome Comments:

Follow-up Campus: Mercy Health
Follow-up Subcentre: CMB Inpatient Team
Acknowledged: Brian Machemedze
Review Date: 24/02/2021 12:36:54 AM

Screening Date & Time: 23/02/2021 9:45:39 PM
Duration (minutes): 55
Subcentre / Team: AHC/ELN Team
Clinician: Brian Machemedze

Program
ervice Location: Emergency department
Service Recipient: Acute Health
Medium: Direct
Length Of Stay:

Length Of Stay >24 hours:
No. Receiving Service: 1
No. Providing Service: 1

Referred From

Referral
Date / Time Referred: 14/02/2021
Name: Allanah: 0403117361
Contact Information:
Perception of the Problem:
Referral Source: Family, friend or neighbour

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Printed By: Bryan Dimabuyo (Nurse)

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Priority: Normal

ient

ClientMHA: South West
Indigenous Status: Not Specified
Preferred Language: English
Interpreter Required: Interpreter service not required

Additional Contacts:

Duration	Campus	Subcentre	Clinician	Service Location	Service Date

Carer:

D/N Contact	Surname	First Name	Relationship	Contact Type	Phone BH
☐	McLean	Jodes	Sister	Contact 2	
☐	McLean	Doug	Father	Contact 1 (NOK)	
☐	McLean	Brad	Brother	Contact 2	
☐	McLean	Doug	Father	Contact 1 (NOK)	

Created By: Brian Machemedze (Nurse)
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Patient Progress

Date & Time: 24/02/2021 12:02:53 PM

DRAFT - last modified: 24/02/2021 3:13:00 PM

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Home Phone (AH) 0400639248 **Work Phone (BH)**

Statewide UR: 1425619
Date of Entry: 24/02/2021
Time of Entry: 11:32
Clinician: Mustafa Abuqlam
Designation: Registrar
Team: Clare Moore Building Inpatient Team

Time/Date: 24/02/21 1000

Psych CWR – Akinbiyi, Abuqlam

Seen with contact nurse – Salini

37M

Single, no children

Lives in private rental house with pet dog

Quit job as NDIS art therapist 1 month ago

Explained AO status to patient.

States he realises his mistake of saying "some silly things online".

States he was lying about having medication piled up.

States he said what he said online to raise awareness for human rights.

Richard's blog is available publically online at "www.richmclean.com.au". Excerpt:

"This is a public cry for assistance compassion and due

Title: Psychiatrist Review **Page No.** 1
Created By: Mustafa Abuqlam (Registrar)
Printed By: Bryan Dimabuyo (Nurse)

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diligence to legal processes to assist and advocate for me - a gay 'mentally ill' Dr of Philosophy who is a suicide survivor & can defiantly prove a heinous conspiracy to maim and kill him systemically in institutions such as the police politics and politicians institutions and organisations with the veneer of helping people involving literally hundreds of people.

my demands ARE to get help for & be acknowledged with legal representation in a court of law.

This is in order to gain traction for the injustices thrust maliciously upon me through the vehicles of stigma shame ostracisation neglect and scapegoating, which is a national conspiracy to pervert the course of justice and rob me of prosperity and agency.

failing this I will 'POSSIBLY' die by suicide on 8th April 2021, or very shortly after in a time of my choice."

States he may have said what he said due to unresolved emotions from being sexually assaulted in the past. Believes the court case which lasted 3.5 years was rigged. States the magistrate sent him a letter saying it's a lost cause.

States he's articulate and has just finished a PhD.

States he was a therapist for NDIS until just recently (22 Jan). Stopped as he became triggered while working with a victim of child sexual assault. Applied for income protection but was rejected. States went from earning $100,000 to nothing.

States his former boyfriend works for ASIO and wants to ruin his life. He states his former boyfriend (Steve Iasondis) was accomplice to a murder and has committed tax fraud. He states he got away with it because ASIO covered it

Title: Psychiatrist Review **Page No.** 2
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Health Fund: HIC

Patient Progress

Date & Time: 24/02/2021 12:02:53 PM
DRAFT - last modified: 24/02/2021 3:13:00 PM

3011 Footscray
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up. Requested that we get the ex-boyfriend's tax records from the police for definite proof.

Richard reported that he had voiced suicidal ideation to a particular GP 6 years ago and this doctor proceeded to prescribe him tablets which he could overdose on. He recorded this conversation and has since made a complaint against that doctor with AHPRA and has attempted to sue him for compensation. He states that the GP's lawyer was very good and got the case thrown out as the recording was not admissible in court. He appealed the decision to many other legal bodies but he says they all said the same thing. He attributes this to the conspiracy against him by his ex-boyfriend.

We informed him that to the best of our belief, recordings made without consent are indeed not admissible in court. He stated that we are not lawyers and don't know that.

Further history from admission notes: Richard reported that he at times has 'other reality' experiences and that in the past he experienced visual hallucinations but he does not agree with the pathologising of these experiences as 'psychosis'. "I refused to tell you if I have hallucinations as you look at me through a DSM framework".

Wants to go home to feed his dog and catch up with his brother today who he hasn't seen since Christmas.

We informed him that we will place him under the mental health act (ITTO) to observe him for the next few days. Explained his rights to legal aide, an appeal of the order, etc.

He questioned the decision to be placed under the mental health act as he "has a PhD". Stated that this is "part of the conspiracy" against him by his former boyfriend.

Past psychiatric history:

Documented history of schizophrenia, ADHD, depression.

CMI contacts with triage

Nil admissions

Engaged with PP Dr Horgan for past few years and supported by GP

Title: Psychiatrist Review **Page No.** 3
Created By: Mustafa Abuqlam (Registrar)
Printed By: Bryan Dimabuyo (Nurse)

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Mercy Health
Care first

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

Patient Progress

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Psychotherapy: Richard reported that he has seen various psychologists in the past but found them to be helpful as they fostered 'co-dependence'.

Reg Meds:

Amisulpride 400mg nocte

Paroxetine 20mg (reported sexual dysfunction from this)

Diazepam 2.5mg nocte (for sleep)

? Pre-exposure prophylactic medication for HIV

? Dexamphetamine 4-8mg per day (Richard reported utilizing variable dosing)

?allergies

AOD:

1-2 ciders daily (documented in previous assessment).

States he has used methamphetamine, ecstasy and THC in the past.

States he has used modafinil in the past (from a friend) while he was doing his Masters Degree but he later got a prescription for a while. Believes he had narcolepsy.

Social history:

Gambling debt of 50K

Richard reported that he was 'squatting' in his current accommodation.

Family history:

Title: Psychiatrist Review **Page No.** 4
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Richard is the middle of three siblings

He reported that there was a history of bipolar disorder in the family.

Personal

Richard reported that his mother is a 'narcissist' and that his father is 'distant'.

Richard identifies as homosexual and reported growing up in a 'homophobic family'. He is not in a stable relationship at the moment but advised "I am an sexually active gay man"

History of childhood sexual abuse (from previous documentation, this was not explored at this review).

Richard reported he has completed a PHD and has published an autobiography.

He reported having friends.

He reported being spiritual and having consulted a Sharman in the past.

Forensic history:

Nil reported

MSE:

Not engaging well with interviewer

Intense stare

Psychomotor agitation

Denies suicidal/homicidal ideation currently.

Paranoid persecutory ideation regarding AHPRA, ex-boyfriend and ASIO

Title: Psychiatrist Review **Page No.** 5
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Risk Ax: Moderate risk of self-harm/suicide

Impression: Paranoid and Persecutory Delusional Ideation

Plan:

1. Place on ITTO
2. Grant hospital ground to smoke, accompanied with staff
3. Observe mental state
4. Collateral history from private psychiatrist Dr Hogan
5. Clarify NOK + GP details
6. Get GP Summary
7. Clarify allergy status and regular medications
8. Increase amisulpride to 600mg nocte
9. Chase bloods/ECG/UDS
10. Nurse to kindly provide Richard with information for involuntary patients including legal aide number
11. As per Dr Akinbiyi – No mobile phone use due to risk of videography (mentioned in notes). Phone may only be used with constant supervision from staff. Richard may use the ward phone to make calls.

ADD-ON 24/02/21 1505

Psych Reg – M. Abuqlam

Title: Psychiatrist Review **Page No.** 6
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Obtained consent from Richard to gather collateral history and update the following people:

Private Psychiatrist – Dr Hogan

NOK – father, Doug McLean

GP – Dr Richard Moore (Northside Clinc, Fitzroy)

Clarified reg meds + allergy status (bees, penicillin)

Richard asked me to listen to the recording of himself and his previous GP, available on his blog.

States that the psychiatrist he met this morning may be part of the conspiracy against him.

Plan:

12. Charted reg meds
13. Obtain collateral histories
14. Obtain GP Health Summary

AbuqlaM

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3.5. Correspondence generic (FMH400001)

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DRAFT - last modified: 24/02/2021 6:30:00 AM

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Statewide UR: 1425619
Date of Entry: 24/02/2021
Time of Entry: 00:56
Clinician: Angeline Harindar
Designation: Medical Officer
Team: Clare Moore Building Inpatient Team

Patient seen in his room with Nurse Andres

Overview
47-year-old single Caucasian gentleman gentleman brought in by CATT team after a concerned friend report decline in Richard's mental state , living in a private rental. Richard reported that he had lost his job as an art therapist with NDIS recently.

Mode of referral:

Referred by concerned friend called the CATT team about his organized plan to commit suicide.

Richard's friend reported that she is worried that Richard's mental health may be declining as has noticed on his website that he is escalating in distress and listing suicide dates with increased "venom" towards former GP and health care professionals.

Presenting symptoms from interview/recent documentation:

-Richard has been admitted under an Assessment order. He repeatedly advises " You are admitting me against my will"
- Advises that his suicidal plan was to " bring attention and traction to the social and political injustice towards him " Advises his legal issues with his GPs and how all of them are " conspiring against him along with his "malicious, toxic and narcissistic ex-boyfriend" . He advises that his Ex-boyfiend is a "millionaire" with a lot of influence. Advises that his ex-boyfriend works for the ASIO (Australian security intelligence organization) and he believes they are spying on him. and that his ex-boyfirend is conspirig with everyone " he swore to destroy and ruin me"

Title: Admission Notes **Page No.** 1
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- Richard advises that he was 'sexually abused by a neighbour and his family never supported him when he came out about it. he is unaware of when it happened
- He also described that his family was 'toxic' and that he had had a friend who had suicided in August 2018.
-Richard reported that he had voiced suicidal ideation to a particular GP 6 years ago and this doctor proceeded to prescribe him tablets which he could overdose on. He recorded this conversation and has since made a complaint against that doctor with AHPRA and has attempted to sue him for compensation.
-Richard has sought support from various agencies and ministers including Greg Hunt (Health minister) and the President of Black Dog institute (Gordon Parker) " They have all turned their back to me"
- Richard reported that a recent application for income protection had been denied.
- Advises that he is not suicidal at present
He reported that a licences/allowance to be recognised as a therapist was declined by the APS.
-Richard expressed frustration over his experiences with 'the system' and that he has not been helped adequately.

-He did not report feeling depressed or a disturbance in sleep or appetite.

Richard reported that he at times has 'other reality' experiences and that in the past he experienced visual hallucinations but he does not agree with the pathologising of these experiences as 'psychosis'. " I refused to tell you if I have hallucinations as you look at me through a DSM framework"

-It is reported in previous interviews and also the written submission that Richard provided that Richard's former partner's boss worked for ASIO which he feels may have negatively impacted on his plight. Richard does not currently feel persecuted by ASIO or any specific individual.

He reported his goal as wanting to be 'valued'.

He reported spending time with his dog and friends.

<u>Past psychiatric history:</u>

Documented history of schizophrenia, ADHD, depression.
Richard reported that he disagreed with the formulation of 'psychosis' and that he takes the medication

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'out of fear'.
CMI contacts with triage
Nil admissions
Engaged with PP Dr Horgan for past few years and supported by GP
Psychotherapy: Richard reported that he has seen various psychologists in the past but found them to be helpful as they fostered 'co-dependence'.
Coping strategies:
Referred to AMHS CM in 2019 but Richard declined engaging with medical reviews and was not accepted for CM. He was referred to local supports such as LGBT supports and encouraged to follow up with PP.
Has NDIS support.

Current medication:
Dexamphetamine 4-8mg per day (Richard reported utilizing variable dosing).
Amisulpiride 200-400mg nocte
Paroxetine – Richard reported sexual dysfunction from this.
Pre-exposure prophylactic medication for HIV

AOD:

1-2 ciders daily (documented in previous assessment). Richard refused to discuss his DA use at this interview. His written submission refers to GHB use.
He advises that he has used to use marijuana and ectasy but refuses to provide more details

Social history:

Gambling debt of 50K

Richard reported that he was 'squatting' in his current accommodation.

Family history:

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Richard is the middle of three siblings

He reported that there was a history of bipolar disorder in the family.

Personal

Richard reported that his mother is a 'narcissist' and that his father is 'distant'.

Richard identifies as homosexual and reported growing up in a 'homophobic family'. He is not in a stable relationship at the moment but advised "I am an sexually active gay man"

History of childhood sexual abuse (from previous documentation, this was not explored at this review).

Richard reported he has completed a PHD and has published an autobiography.

He reported having friends.

He reported being spiritual and having consulted a Sharman in the past.

Forensic history:

Nil reported

Mental Status Examination

Appearance: Middle aged Caucasian male of medium build wearing glasses and mask, beard visible under mask, arm tattoos visible wearing short sleeved shirt. Richard appeared to be well groomed and appropriately dressed for the weather.
Behaviour: Richard was calm but rapport seemed superficial. confrontational at times, good eye contact.
Activity level: Remained calm throughout the interview. good posture
Speech : normal rate, volume and tone
Mood: advises that he has been feeling very "low" and "lonely" lately. "Frustrated"
Affect: Reactive

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Thought: normal form. occasional derailment but easily distracted and steered away. He expressed themes of feeling let down and persecuted by the health system.
Content: delusion of persecution?
Perception: refuses to answer if has visual or auditory hallucinations
Cognition: normal
Orientation: well oriented to time and place
Attention: Need for redirection occasionally
Insight: poor insight
Judgment: normal

He voiced feeling let down by the system and expressed that responses did not adequately address his needs. Richard refused to answer certain questions and answered yet others somewhat obliquely.

Risk:
Has an organised suicide plan:He published on his website a plan to kill himself on his birthday (8/4) this year.. documented on his website
Stockpiled medications
History of suicide attempt via overdose 3 years ago.
Richard reported recreational drug use and this may destabilize his mental state. His current level of drug use in unknown.
Risk of mental state deterioration with stimulant use/overuse
Richard denied forensic history of history of violence and denied any homicidal ideation toward any individual.

ROUTINE RISK ASSESSMENT					
Mark with an "X" if applicable					
Risk Categories	**PHx**	**L**	**M**	**H**	**Rationale / Comments (mandatory)**
Suicidality / Self Harm (Inpatient staff to consider removal of ligature risks if M or H and document in clinical file)		x			

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Risk of self -neglect /accidental self-harm (homeless, personal hygiene, food/fluid intake)			x		**moderate risk of losing accommodation**
Violence/ harm to others		x			
Risk of violence, harm or neglect to infant/child (consider CPS notification if M or H)		x			
Risk of Vulnerability/Harm from others (physical, sexual, emotional or financial)		x			
Risk of absconding/ non - adherence with treatment		x			
Risk of Impulsivity/ level of distress			x		**Risk of suicidal gestures if distressed or perceived needs not met**
Current substance use, intoxication or withdrawal - Yes or No			x		**reports recreational drug use**
Physical health issues or adverse effects from treatment		x			
Other (falls risk, use of restrictive interventions, PRN medication)		x			

Delete inappropriate answer (Yes or No)

Factors to consider		**Rationale / Comments (mandatory)**
Is the patient's level of risk highly changeable?	Yes / No	
Is the reliability of this risk assessment low? (Poor engagement, conflicting info, etc.)	Yes / No	
Are tipping points present? (personal stress, recent death, end of relationship)	Yes / No	**loss of realtionhsip with psychiatrist**
Are there significant protective/ resilience factors present?	Yes / No	**help seeking**

If high changeability = Yes or low reliability = Yes, then the overall risk rating should increase by one level (i.e. medium to high)

OVERALL RISK RATING (LOW/ MEDIUM/ HIGH):	

RISK MANAGEMENT PLAN TO BE DOCUMENTED BELOW

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Impression: Paranoid schizophrenia?

Richard has a history of childhood and adult trauma. This includes a mother that may have been unavailable for him in his early years, the experience of being gay in an unaccepting environment, an invalidating adult romantic relationship and the suicide of a friend. His earlier experiences may have contributed to attachment difficulties leading to vulnerable personality traits. These include a sense that he is being exploited by others, persistently bearing a grudge, a perceived attack on his character and high expectations of others to meet his standards.

Richard has a history of schizophrenia, ADHD and depression. He was not depressed or psychotic at review.

Plan (include strategies to manage risk):

Bloods FBE, U&E, LFT. TFT, Urine drug screen, fasting lipid profile, fasting glucose
Consultant review mane
next of kin details to be obtained
to obtain more infor about his regular medication

HarindA

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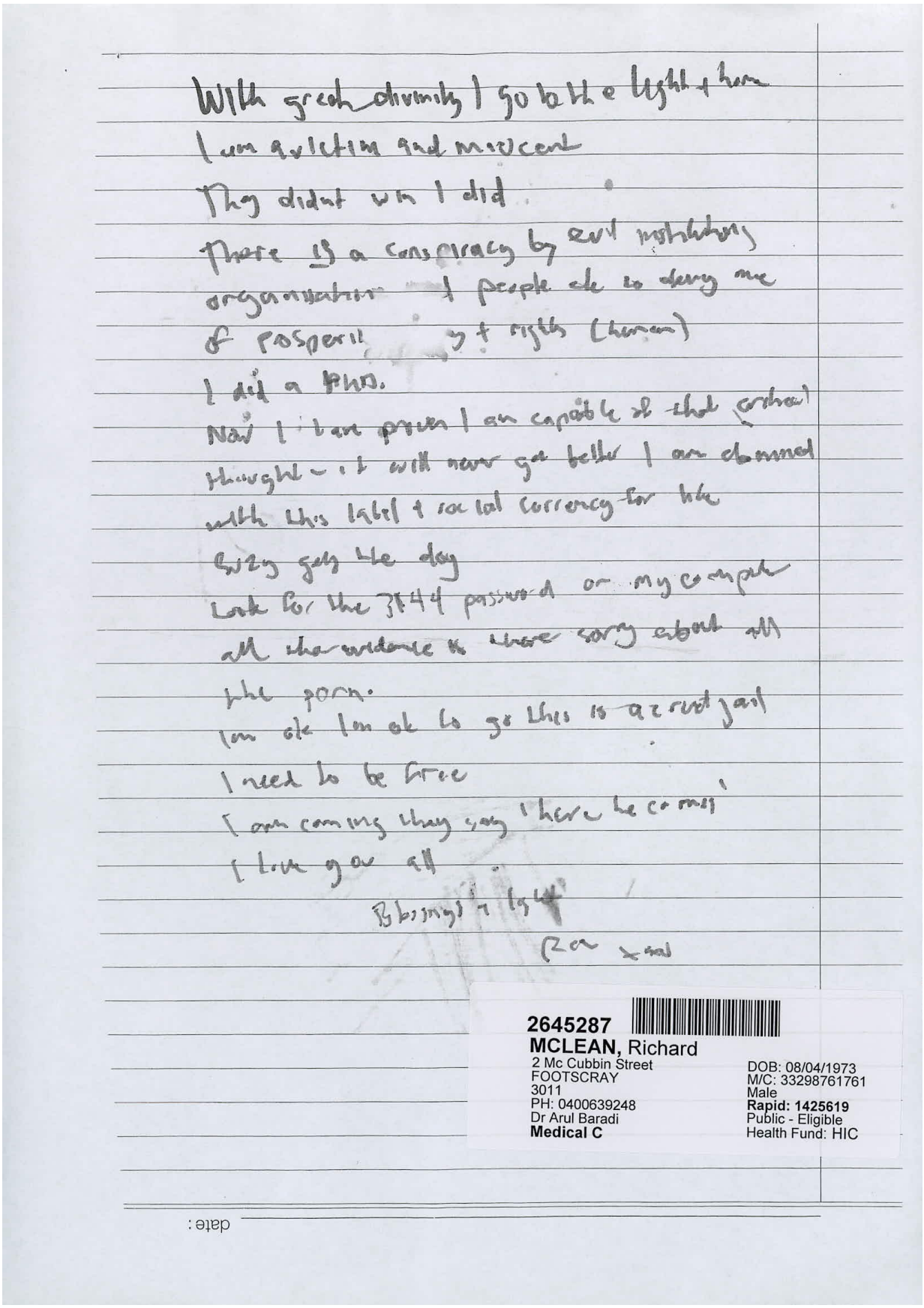
3.6. Correspondence generic (FMH400001)



With great divinity I go to the light + then
I am a victim and innocent
They didnt win I did
There is a conspiracy by evil institutions
organisations + people etc to deny me
of prosperity + rights (human)
I did a PhD.
Now I have proven I am capable of that critical
thought - it will never get better I am damned
with this label + no tax currency for life
Busy day the day
Look for the 3144 password on my computer
all the evidence is there sorry about all
the porn.
I am ok I am ok to go this is a corrupt jail
I need to be free
I am coming they say 'here he comes'
I love you all
Blessings + light
Ra + Isis

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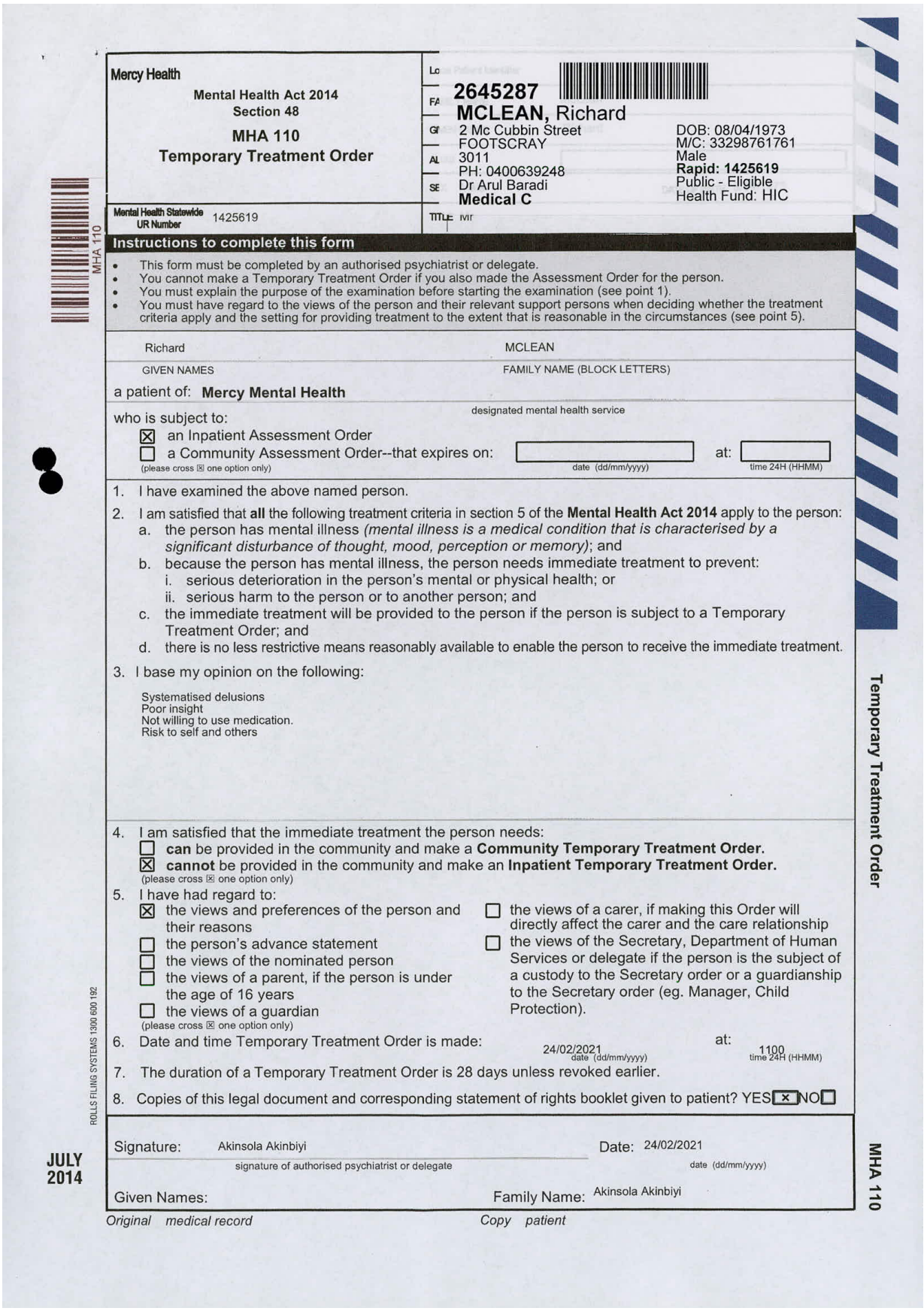

Mercy Health

Mental Health Act 2014
Section 48

MHA 110
Temporary Treatment Order

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Mental Health Statewide UR Number 1425619

MHA 110

Instructions to complete this form

- This form must be completed by an authorised psychiatrist or delegate.
- You cannot make a Temporary Treatment Order if you also made the Assessment Order for the person.
- You must explain the purpose of the examination before starting the examination (see point 1).
- You must have regard to the views of the person and their relevant support persons when deciding whether the treatment criteria apply and the setting for providing treatment to the extent that is reasonable in the circumstances (see point 5).

Richard (GIVEN NAMES) MCLEAN (FAMILY NAME (BLOCK LETTERS))

a patient of: **Mercy Mental Health** (designated mental health service)

who is subject to:
- ☒ an Inpatient Assessment Order
- ☐ a Community Assessment Order--that expires on: _____ (date (dd/mm/yyyy)) at: _____ (time 24H (HHMM))

(please cross ☒ one option only)

1. I have examined the above named person.
2. I am satisfied that **all** the following treatment criteria in section 5 of the **Mental Health Act 2014** apply to the person:
   a. the person has mental illness *(mental illness is a medical condition that is characterised by a significant disturbance of thought, mood, perception or memory)*; and
   b. because the person has mental illness, the person needs immediate treatment to prevent:
      i. serious deterioration in the person's mental or physical health; or
      ii. serious harm to the person or to another person; and
   c. the immediate treatment will be provided to the person if the person is subject to a Temporary Treatment Order; and
   d. there is no less restrictive means reasonably available to enable the person to receive the immediate treatment.
3. I base my opinion on the following:

   Systematised delusions
   Poor insight
   Not willing to use medication.
   Risk to self and others

4. I am satisfied that the immediate treatment the person needs:
   - ☐ **can** be provided in the community and make a **Community Temporary Treatment Order.**
   - ☒ **cannot** be provided in the community and make an **Inpatient Temporary Treatment Order.**

   (please cross ☒ one option only)
5. I have had regard to:
   - ☒ the views and preferences of the person and their reasons
   - ☐ the person's advance statement
   - ☐ the views of the nominated person
   - ☐ the views of a parent, if the person is under the age of 16 years
   - ☐ the views of a guardian
   - ☐ the views of a carer, if making this Order will directly affect the carer and the care relationship
   - ☐ the views of the Secretary, Department of Human Services or delegate if the person is the subject of a custody to the Secretary order or a guardianship to the Secretary order (eg. Manager, Child Protection).

   (please cross ☒ one option only)
6. Date and time Temporary Treatment Order is made: 24/02/2021 (date (dd/mm/yyyy)) at: 1100 (time 24H (HHMM))
7. The duration of a Temporary Treatment Order is 28 days unless revoked earlier.
8. Copies of this legal document and corresponding statement of rights booklet given to patient? YES ☒ NO ☐

Signature: Akinsola Akinbiyi (signature of authorised psychiatrist or delegate) Date: 24/02/2021 (date (dd/mm/yyyy))

Given Names: Family Name: Akinsola Akinbiyi

Original medical record *Copy patient*

ROLLS FILING SYSTEMS 1300 600 192

JULY 2014

Temporary Treatment Order

MHA 110

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

3.8. Correspondence generic (FMH400001)

Statewide UR: 1425619

Date of Entry: 24/02/2021

Time of Entry: 05:56

Clinician: Catherine Russell

Designation: RN Psych Div1

Team: Clare Moore Building Inpatient Team

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

NURSING ENTRY ANUM:

On arrival the AHC, Machemedze, clearly advised the client in the c/o writer and his contact nurse, Panti, that he is unable to have access to his mobile phone or any laptop devices during his admission to the ward due to historical recording and videoing of staff and police and posting these on his social media and websites, which puts both clients and staff on the unit at risk. The client is aware that staff do not consent to being recorded during any interactions whilst they are at work.

This plan has has been devised and approved by both the AHC, Machemedze, and the on call consultant Dr Rahman.

The client has not yet identified a NOK, therefore a compulsory notification is yet to be completed. The client has asked for access to his mobile phone upon waking this mane to obtain NOK details.

The client has been advised that this can be co-ordinated during his consultant RV today and that any access to his phone during this time will require it being returned immediately post. He has not been advised at any time that he will definitely be able to access his mobile today.

Impression: HIGH risk of posting video and recorded material during admission onto social media and his websites, placing consumers and staff at risk

Plan (include strategies to manage risk):

- not to have mobile phone or laptop during admission

- devise plan around obtaining access to NOK details prior to and in D/W consultant today and complete compulsory notification under the MHA

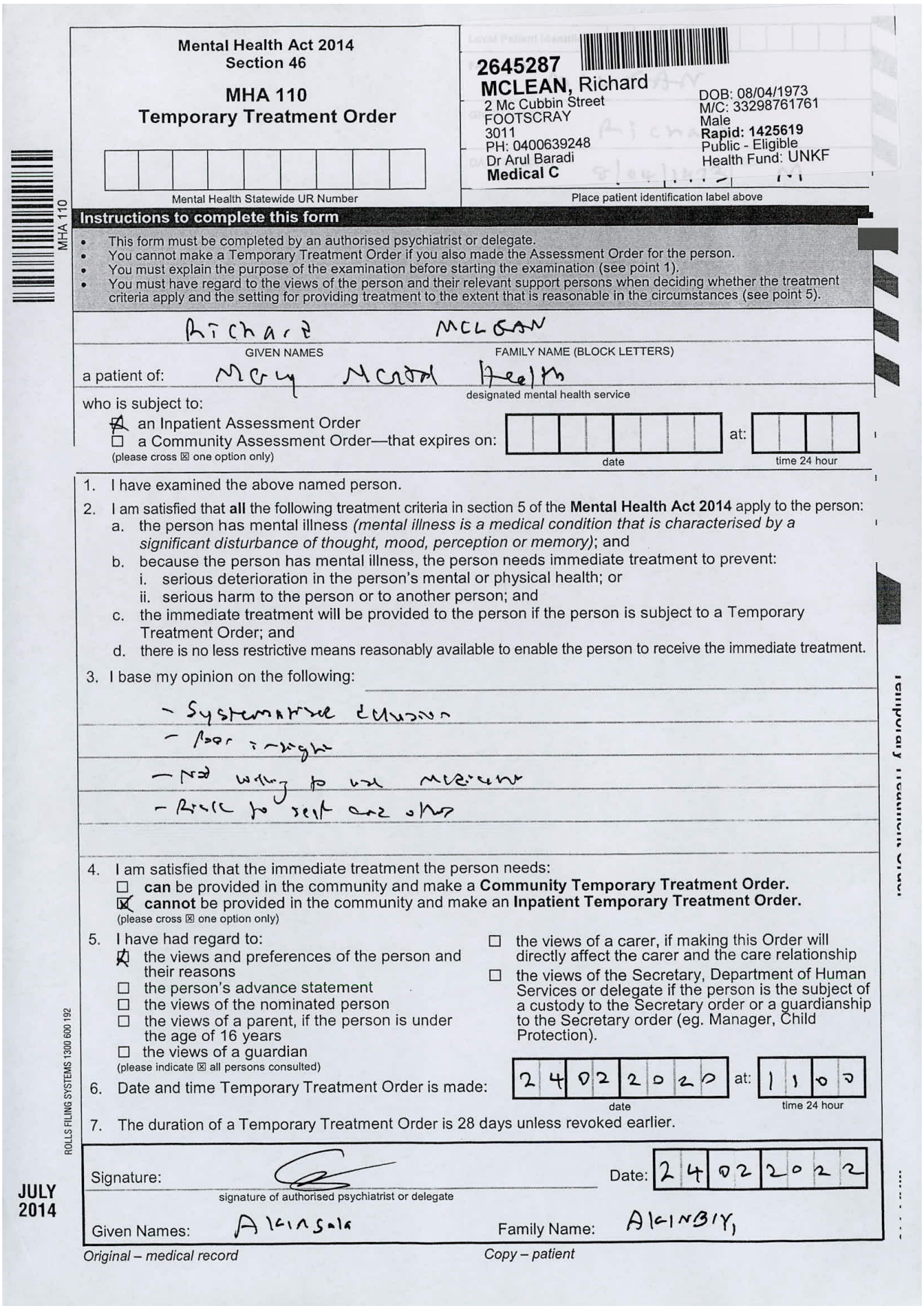
- consistently remind client of the rationale for being unable to access devices and that you do not consent to being recorded

- please ensure all policies are followed as this client is highly litigious and document information clearly in the clinical file as is likely to access this through FOI at some point

- be aware that this client may staff split in an attempt to obtain devices. Keep interactions simple, clear and with contact nurse only to avoid confusion or plan not being followed

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

3.9. Correspondence generic (FMH400001)



Mental Health Act 2014
Section 46

MHA 110
Temporary Treatment Order

Mental Health Statewide UR Number

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

Place patient identification label above

MHA 110

Instructions to complete this form

- This form must be completed by an authorised psychiatrist or delegate.
- You cannot make a Temporary Treatment Order if you also made the Assessment Order for the person.
- You must explain the purpose of the examination before starting the examination (see point 1).
- You must have regard to the views of the person and their relevant support persons when deciding whether the treatment criteria apply and the setting for providing treatment to the extent that is reasonable in the circumstances (see point 5).

Richard — GIVEN NAMES
MCLEAN — FAMILY NAME (BLOCK LETTERS)

a patient of: Mercy Mental Health
designated mental health service

who is subject to:
☒ an Inpatient Assessment Order
☐ a Community Assessment Order—that expires on: ____ (date) at: ____ (time 24 hour)
(please cross ☒ one option only)

1. I have examined the above named person.
2. I am satisfied that **all** the following treatment criteria in section 5 of the **Mental Health Act 2014** apply to the person:
   a. the person has mental illness *(mental illness is a medical condition that is characterised by a significant disturbance of thought, mood, perception or memory)*; and
   b. because the person has mental illness, the person needs immediate treatment to prevent:
      i. serious deterioration in the person's mental or physical health; or
      ii. serious harm to the person or to another person; and
   c. the immediate treatment will be provided to the person if the person is subject to a Temporary Treatment Order; and
   d. there is no less restrictive means reasonably available to enable the person to receive the immediate treatment.
3. I base my opinion on the following:
   - Systematic delusion
   - Poor insight
   - Not willing to use medication
   - Risk to self and others
4. I am satisfied that the immediate treatment the person needs:
   ☐ **can** be provided in the community and make a **Community Temporary Treatment Order.**
   ☒ **cannot** be provided in the community and make an **Inpatient Temporary Treatment Order.**
   (please cross ☒ one option only)
5. I have had regard to:
   ☒ the views and preferences of the person and their reasons
   ☐ the person's advance statement
   ☐ the views of the nominated person
   ☐ the views of a parent, if the person is under the age of 16 years
   ☐ the views of a guardian
   ☐ the views of a carer, if making this Order will directly affect the carer and the care relationship
   ☐ the views of the Secretary, Department of Human Services or delegate if the person is the subject of a custody to the Secretary order or a guardianship to the Secretary order (eg. Manager, Child Protection).
   (please indicate ☒ all persons consulted)
6. Date and time Temporary Treatment Order is made: 24/02/2022 (date) at: 1100 (time 24 hour)
7. The duration of a Temporary Treatment Order is 28 days unless revoked earlier.

Signature: _____ (signature of authorised psychiatrist or delegate) Date: 24/02/2022

Given Names: Alainsala Family Name: Alainby

JULY 2014

ROLLS FILING SYSTEMS 1300 600 192

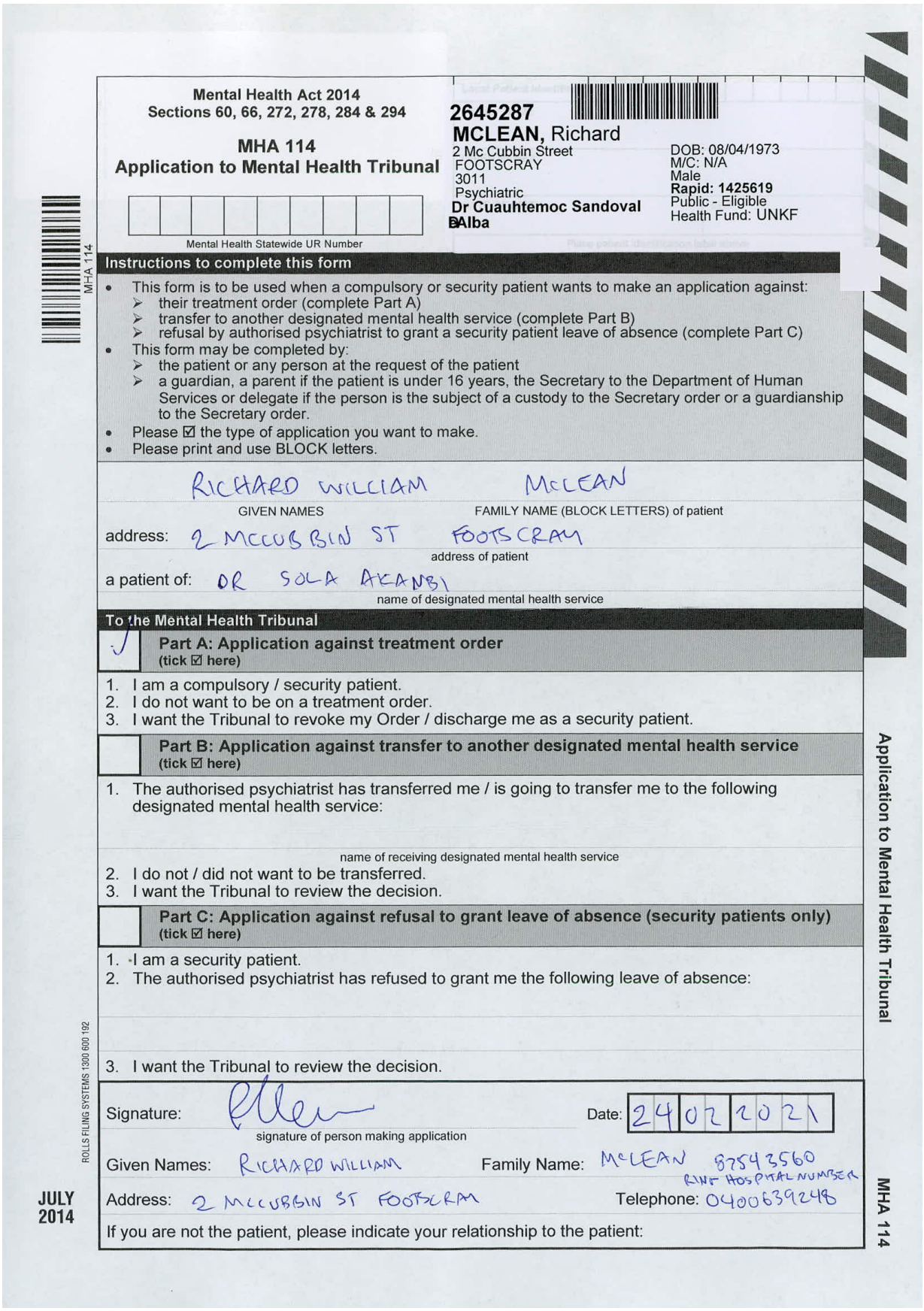
Original – medical record

Copy – patient

Temporary Treatment Order

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

3.10. Correspondence generic (FMH400001)



Mental Health Act 2014
Sections 60, 66, 272, 278, 284 & 294

MHA 114
Application to Mental Health Tribunal

Mental Health Statewide UR Number

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Cuauhtemoc Sandoval Alba
DOB: 08/04/1973
M/C: N/A
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

MHA 114

Instructions to complete this form

- This form is to be used when a compulsory or security patient wants to make an application against:
  - their treatment order (complete Part A)
  - transfer to another designated mental health service (complete Part B)
  - refusal by authorised psychiatrist to grant a security patient leave of absence (complete Part C)
- This form may be completed by:
  - the patient or any person at the request of the patient
  - a guardian, a parent if the patient is under 16 years, the Secretary to the Department of Human Services or delegate if the person is the subject of a custody to the Secretary order or a guardianship to the Secretary order.
- Please ☑ the type of application you want to make.
- Please print and use BLOCK letters.

RICHARD WILLIAM — GIVEN NAMES
MCLEAN — FAMILY NAME (BLOCK LETTERS) of patient

address: 2 MCCUBBIN ST FOOTSCRAY
address of patient

a patient of: DR SOLA AKANBI
name of designated mental health service

To the Mental Health Tribunal

☑ **Part A: Application against treatment order**
(tick ☑ here)

1. I am a compulsory / security patient.
2. I do not want to be on a treatment order.
3. I want the Tribunal to revoke my Order / discharge me as a security patient.

☐ **Part B: Application against transfer to another designated mental health service**
(tick ☑ here)

1. The authorised psychiatrist has transferred me / is going to transfer me to the following designated mental health service:

name of receiving designated mental health service

2. I do not / did not want to be transferred.
3. I want the Tribunal to review the decision.

☐ **Part C: Application against refusal to grant leave of absence (security patients only)**
(tick ☑ here)

1. I am a security patient.
2. The authorised psychiatrist has refused to grant me the following leave of absence:

3. I want the Tribunal to review the decision.

Signature: [signature] Date: 24 02 2021
signature of person making application

Given Names: RICHARD WILLIAM Family Name: MCLEAN 8754 3560
RMH HOSPITAL NUMBER

Address: 2 MCCUBBIN ST FOOTSCRAY Telephone: 0400639248

If you are not the patient, please indicate your relationship to the patient:

ROLLS FILING SYSTEMS 1300 600 192

JULY 2014

Application to Mental Health Tribunal

MHA 114

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

3.11. Correspondence generic (FMH400001)

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

Next steps

After completing this form:

- **send** a copy of this form to the Mental Health Tribun
  - Email: mht@mht.vic.gov.au
  - Fax: 9032 3223
  - Mail: Level 30, 570 Bourke Street, Melbourne 3000; or
  - **ask** a member of staff at the mental health service to send the application to the Tribunal.
- **get** more information from the Tribunal:
  - Tel: 9032 3200
  - Tel: 1800 242 703 (toll free)
  - Web www.mht.vic.gov.au

Privacy statement

The information collected on this form will be used by the Mental Health Tribunal to schedule a hearing. The Tribunal will notify you and the designated mental health service that a hearing has been scheduled. It will request the service to provide information about you and your treatment. The Tribunal will use this information to help it decide your application. The exchange of information between the Tribunal and the designated mental health service is authorised under the **Mental Health Act 2014**.

The Tribunal will keep your information secure and not disclose it for any other purpose unless it is required by a law. You can access information held about you by the Tribunal by contacting the Tribunal at the address above.

10-32 - I have not had a vape

10-32 - I have not had dinner or a phone call or phone last night - had everything taken off me.

10-15am - psychiatrist upheld order without seeing evidence

10-33 My dog (therapy) has been locked in all night They said they would take care of that but no one has.

I received (don't know what I where this is - I have been set up or he is complicant (with the (set up conspiracy).

MHA 114

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

4. Admission Episode:10896416:MHS

4.1. Visual Obs (FMH521500)

Mercy Health
Care first
Mercy Health

UR No:

Family Name: Mclean

Given Name Richard

DOB: 8/4/1973 Sex: M

Address:
(if no UR)

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

VISUAL OBSERVATIONS

Date: 23/2/21

(B)

FMH521500

Category of observation	Visual observation	Level	Activity
1 – Low Risk	*Routine*	*1*	*Settled, minimal agitation*
2 – Medium Risk	*Hourly*	*2*	*Mildly aroused, pacing, still willing to talk reasonably, or withdrawn/isolated*
3 – High Risk	*30/60*	*3*	*Moderately aroused, agitated, disorganised, becoming more vocal and unreasonable, or hostile*
4 – ICA High Risk	*15/60*	*4*	*Highly aroused, grossly disorganised and possibly distressed or fearful*
5 - Specialled High Risk	*1:1 Constant Supervision*	*5*	*Highly aroused, violent towards self, others or property*
Client Description:		*X*	*Client not sighted, document reason in clinical notes*
		0	*Asleep* **with A, B or C MUST have TWO of A, B & C documented e.g. 0 A C**
		A *B* *C*	*A - Regular respiratory movements* *B - Change in sleeping position* *C - Audible respiratory sounds*

Overall Risk Assessment rating in past 72 hours ☐

Client observation level 1-5 ☐

Exact Date & Time	Arousal Level	Print Name & Designation	Exact Date & Time	Arousal Level	Print Name & Designation
2348	1	C Bate RN			

BINDING MARGIN – NO WRITING

P1751 V2 02/17

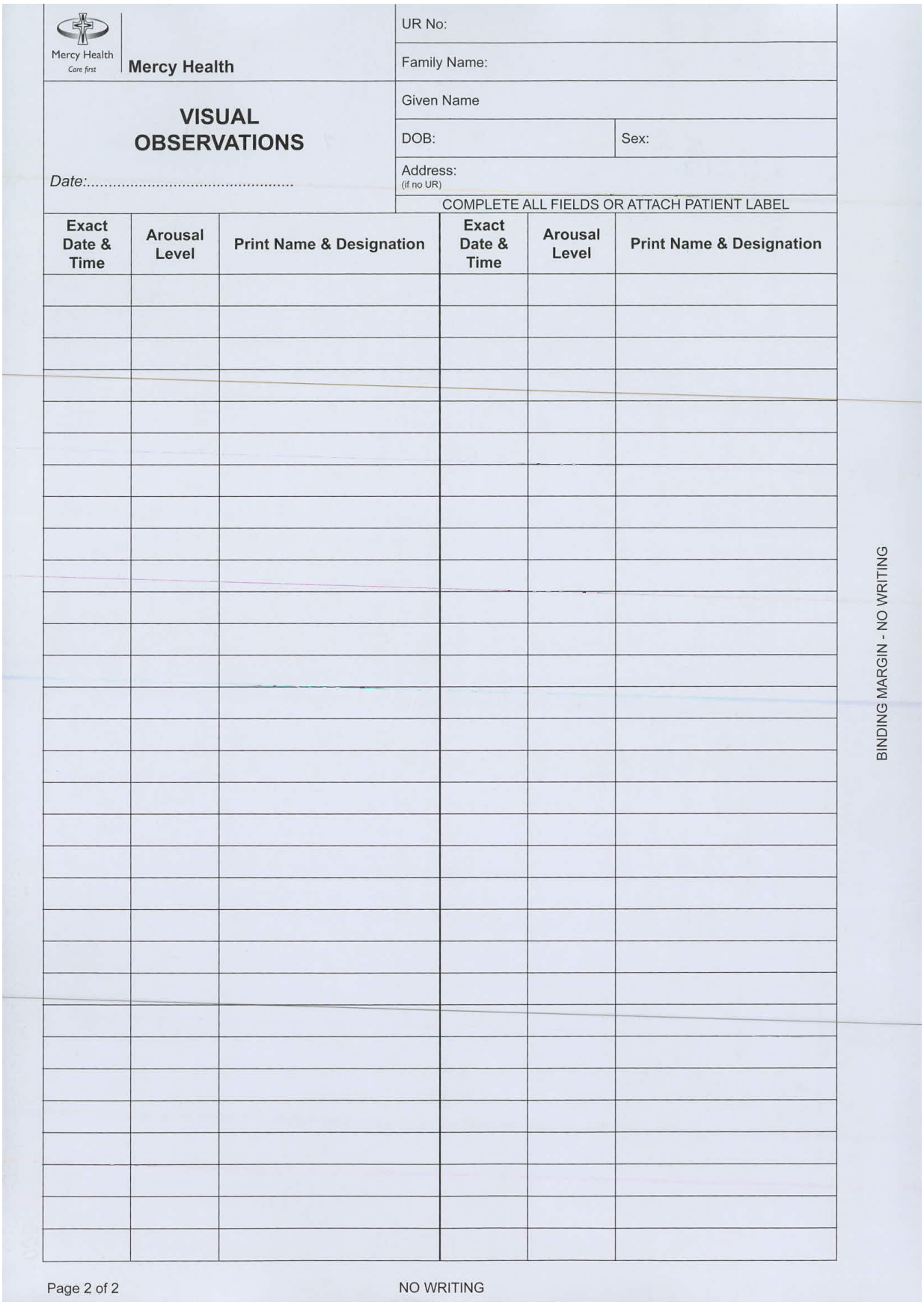
VISUAL OBSERVATIONS ED 0044 AD 2900

NO WRITING

Page 1 of 2

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

4.2. Visual Obs (FMH521500)



Mercy Health
Care first

Mercy Health

UR No:

Family Name:

Given Name

DOB:

Sex:

Address:
(if no UR)

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

VISUAL OBSERVATIONS

Date:.....

Exact Date & Time	Arousal Level	Print Name & Designation	Exact Date & Time	Arousal Level	Print Name & Designation

4.3. Visual Obs (FMH521500)

Mercy Health
Care first

Mercy Health

1/3

VISUAL OBSERVATIONS

Date: 24/2/21

13

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Cuauhtemoc Sandoval De Alba
DOB: 08/04/1973
M/C: N/A
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

Address:
(if no UR)

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH521500

BINDING MARGIN – NO WRITING

Category of observation	Visual observation	Level	Activity
1 – Low Risk	*Routine*	*1*	*Settled, minimal agitation*
2 – Medium Risk	*Hourly*	*2*	*Mildly aroused, pacing, still willing to talk reasonably, or withdrawn/isolated*
3 – High Risk	*30/60*	*3*	*Moderately aroused, agitated, disorganised, becoming more vocal and unreasonable, or hostile*
4 – ICA High Risk	*15/60*	*4*	*Highly aroused, grossly disorganised and possibly distressed or fearful*
5 - Specialled High Risk	*1:1 Constant Supervision*	*5*	*Highly aroused, violent towards self, others or property*
Client Description:		*X*	*Client not sighted, document reason in clinical notes*
		0	*Asleep* **with A, B or C MUST have TWO of A, B & C documented e.g. 0 A C**
		A *B* *C*	*A - Regular respiratory movements* *B - Change in sleeping position* *C - Audible respiratory sounds*

Overall Risk Assessment rating in past 72 hours ☐

Client observation level 1-5 ☐

Exact Date & Time	Arousal Level	Print Name & Designation	Exact Date & Time	Arousal Level	Print Name & Designation
0000	1	C Bako RPT	30	1	B Jones
30	1	C Bako RPT	0800	1	B Jones
0100	1	C Bako RPT	30	1	B Jones
30	OAC	C Bako RPT	0900	1	B Jones
0200	OAC	C Bako RPT	30	1	B Jones
30	OAC	C Bako RPT	1000	1	B Jones
0300	OAC	C Bako RPT	30	1	B Jones
30	OAC	B Kaur RN	1100	1	B Jones
0400	OAC	B Kaur RN	30	1	B Jones
30	OAC	A. Pham RN	1200	1	B Jones
0500	OAC	A. Pham RN	30	1	B Jones
30	OAC	A. Pham RN	1300	1	B Jones
0600	OAC	A. Pham RN	30	1	B Jones
30	1	A. Pham RN	1400	1	B Jones
0700	1	A. Pham RN	30	1	B Jones

VISUAL OBSERVATIONS ED 0044 AD 2900

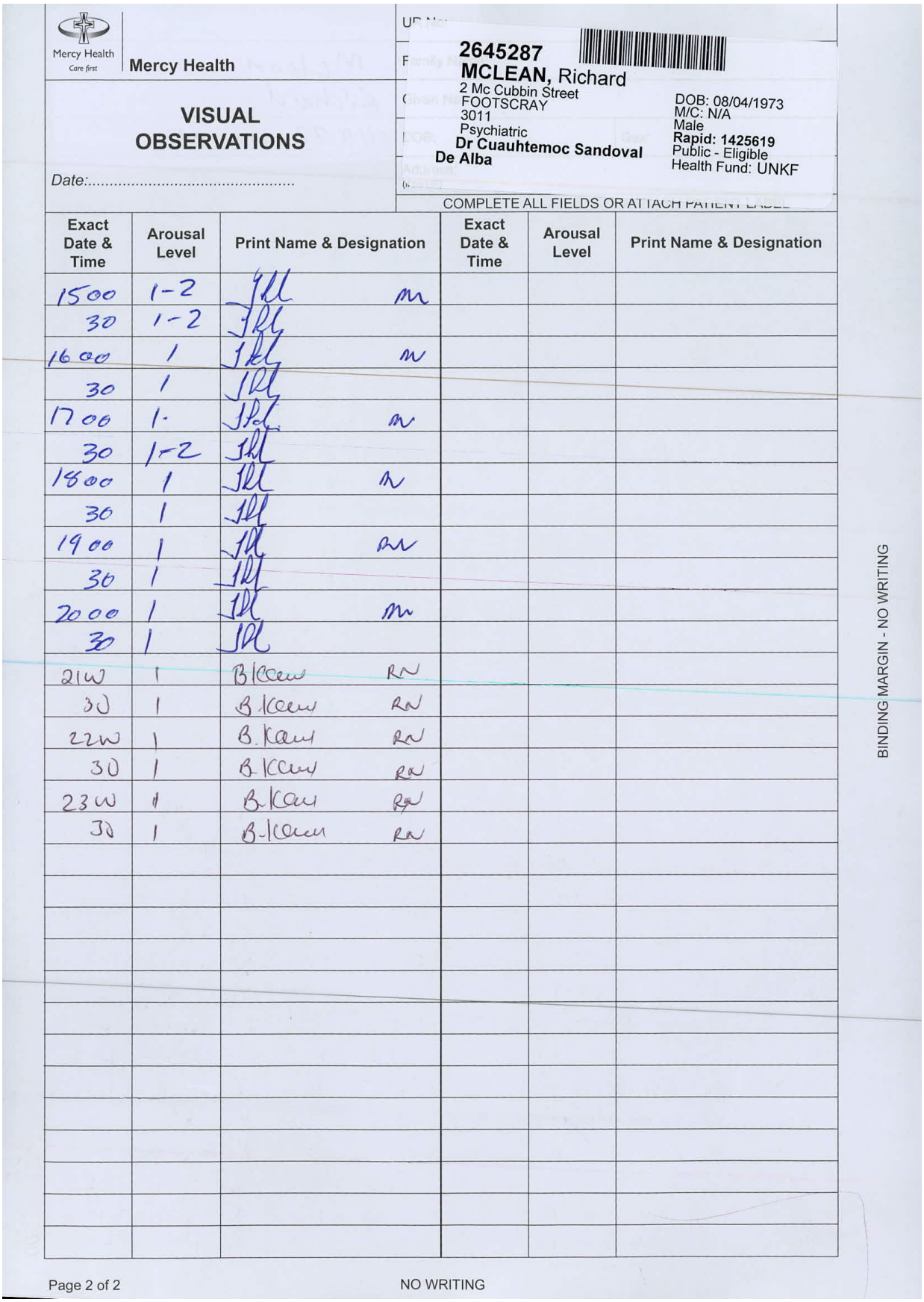
P1751 V2 02/17

NO WRITING

Page 1 of 2

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

4.4. Visual Obs (FMH521500)



Mercy Health
Care first

Mercy Health

VISUAL OBSERVATIONS

Date:.....

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Cuauhtemoc Sandoval De Alba
DOB: 08/04/1973
M/C: N/A
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Exact Date & Time	Arousal Level	Print Name & Designation		Exact Date & Time	Arousal Level	Print Name & Designation
1500	1-2	JKL	M			
30	1-2	JKL				
1600	1	JKL	M			
30	1	JKL				
1700	1	JKL	M			
30	1-2	JKL				
1800	1	JKL	M			
30	1	JKL				
1900	1	JKL	M			
30	1	JKL				
2000	1	JKL	M			
30	1	JKL				
21W	1	B Keen	RN			
30	1	B Keen	RN			
22W	1	B Keen	RN			
30	1	B Keen	RN			
23W	1	B Keen	RN			
30	1	B Keen	RN			

BINDING MARGIN - NO WRITING

Page 2 of 2

NO WRITING

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4.5. Visual Obs (FMH521500)

Mercy Health
Care first

Mercy Health 13

UR No:

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Cuauhtemoc Sandoval De Alba
DOB: 08/04/1973
M/C: N/A
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

VISUAL OBSERVATIONS

Date: 25/12/21

FMH521500

Category of observation	Visual observation	Level	Activity
1 – Low Risk	*Routine*	*1*	*Settled, minimal agitation*
2 – Medium Risk	*Hourly*	*2*	*Mildly aroused, pacing, still willing to talk reasonably, or withdrawn/isolated*
3 – High Risk	*30/60*	*3*	*Moderately aroused, agitated, disorganised, becoming more vocal and unreasonable, or hostile*
4 – ICA High Risk	*15/60*	*4*	*Highly aroused, grossly disorganised and possibly distressed or fearful*
5 - Specialled High Risk	*1:1 Constant Supervision*	*5*	*Highly aroused, violent towards self, others or property*
Client Description:		*X*	*Client not sighted, document reason in clinical notes*
		0	*Asleep* **with A, B or C MUST have TWO of A, B & C documented e.g. 0 A C**
		A *B* *C*	*A - Regular respiratory movements* *B - Change in sleeping position* *C - Audible respiratory sounds*
Overall Risk Assessment rating in past 72 hours ☐		*Client observation level 1-5* ☐	

Exact Date & Time	Arousal Level	Print Name & Designation	Exact Date & Time	Arousal Level	Print Name & Designation
0000	OAC	C Bako RPT	0900		
30	OAC	C Bako RPT	0930		
0100	OAC	C Bako RPT	1000		
30	OAC	C Bako RPT	30		
0200	OAC	C Bako RPT	1000		
30	OAC	B.Kaur RN	30		
0300	OAC	B.Kaur RN	1100		
30	OAC	B.Kaur RN	30		
0400	OAC	B.Kaur RN	1200		
30	OAC	B.Kaur RN	30		
0500	OAC	B.Kaur RN	1300		
0530	OAC	B.Kaur RN	30		
0600	OAC	B.Kaur RN			
0700	OAC	B.Kaur RN			
~~0500~~ 30 M	1				

BINDING MARGIN – NO WRITING

VISUAL OBSERVATIONS ED 0044 AD 2900

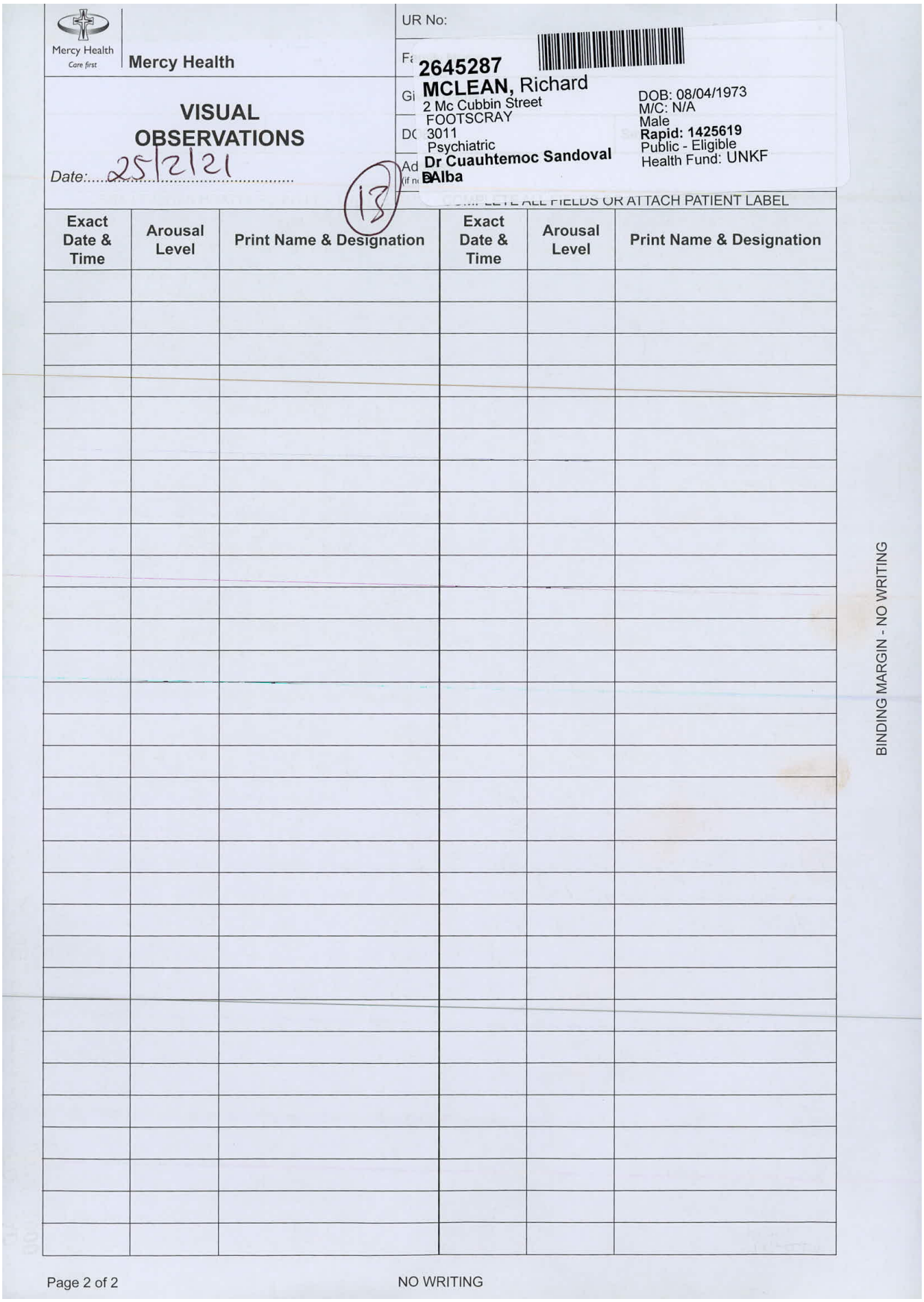
P1751 V2 02/17

NO WRITING

Page 1 of 2

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

4.6. Visual Obs (FMH521500)



Mercy Health
Care first

Mercy Health

UR No:

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Cuauhtemoc Sandoval
BAlba
DOB: 08/04/1973
M/C: N/A
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

VISUAL OBSERVATIONS

Date: 25/2/21

(13)

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Exact Date & Time	Arousal Level	Print Name & Designation	Exact Date & Time	Arousal Level	Print Name & Designation

4.7. Visual Obs (FMH521500)

Mercy Health
Care first

Mercy Health

VISUAL OBSERVATIONS

Date: 04.03.2021

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

(if no UR)

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH521500

BINDING MARGIN – NO WRITING

Category of observation	Visual observation	Level	Activity
1 – Low Risk	Routine	1	Settled, minimal agitation
2 – Medium Risk	Hourly	2	Mildly aroused, pacing, still willing to talk reasonably, or withdrawn/isolated
3 – High Risk	30/60	3	Moderately aroused, agitated, disorganised, becoming more vocal and unreasonable, or hostile
4 – ICA High Risk	15/60	4	Highly aroused, grossly disorganised and possibly distressed or fearful
5 - Specialled High Risk	1:1 Constant Supervision	5	Highly aroused, violent towards self, others or property
		X	Client not sighted, document reason in clinical notes
		0	Asleep **with A, B or C** **MUST have TWO of A, B & C documented** **e.g. 0 A C**
		A B C	A - Regular respiratory movements B - Change in sleeping position C - Audible respiratory sounds

Client Description:
47 yr old caucasian male wearing denim shorts and black t-shirt, has tattoos on R/arm

Overall Risk Assessment rating in past 72 hours ☐

Client observation level 1-5 ☐

Exact Date & Time	Arousal Level	Print Name & Designation	Exact Date & Time	Arousal Level	Print Name & Designation
1745	1	Cecile RN	2130	1	Dufault RPN
1800	1	Cecile RN	45	1	Dufault RPN
15	1	Cecile RN	2200	1	Dufault RPN
30	1	Cecile RN	15	1	Dufault RPN
45	1	Simer RN	30	1	Dufault RPN
1900	1	Simer RN	45	1	Dufault RPN
15	1	Simer RN	2300	1	Dufault RPN
30	1	Simer RN	15	1	Dufault RPN
45	1	Simer RN	30	OAC	Dufault RPN
2000	1	Simer RN	45	OAC	Dufault RPN
15	1	Jasmeet RPN		5/3/21	
30	1	Jasmeet RPN	0000	OAC	Dufault RPN
45	1	Jasmeet RPN	15	OAC	Dufault RPN
2100	1	Jasmeet RPN	30	OAC	Dufault RPN
15	1	Dufault RPN	45	OAC	Dufault RPN

VISUAL OBSERVATIONS ED 0044 AD 2900

P1751 V2 02/17

NO WRITING

Page 1 of 2

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

4.8. Visual Obs (FMH521500)

Mercy Health
Care first

Mercy Health

UR No:

2645287

MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

VISUAL OBSERVATIONS

Date: 5/3/21

Exact Date & Time	Arousal Level	Print Name & Designation	Exact Date & Time	Arousal Level	Print Name & Designation
0100	OAC	Dufak RPN	45	1	Lm RN
15	OAL	Dufak RPN	0900	1	Lm RN
30	1	Dufak RPN	15	1	Lm RN
45	OAC	Dufak RPN	30	1	Lm RN
0200	OAC	Dufak RPN	45	1	Lm RN
15	OAC	F.Oyeme EEN	1000	1	Lm RN
30	OAC	F.Oyeme EEN	15	1	Lm RN
45	OAC	F.Oyeme EEN	30	1	Lm RN
0300	OAC	F.Oyeme EEN	45	1	Lm RN
15	OAL	Dufak RPN	1100	1	Lm RN
30	OAC	Dufak RPN	15	1	Lm RN
45	OAC	Dufak RPN	30	1	Lm RN
0400	OAC	Dufak RPN	45	1	Lm RN
15	OAL	Dufak RPN	1200	1	Lm RN
30	OAC	Dufak RPN	15	1	Lm RN
45	OAC	Dufak RPN	30	1	Lm RN
0500	OAL	Dufak RPN	45	1	Lm RN
15	OAC	Dufak RPN	1300	1	Lm RN
30	1	Dufak RPN	15	1	Jasmeet RPN
45	OAC	Dufak RPN	30	1	Jasmeet RPN
0600	OAC	Dufak RPN	45	1	Jasmeet RPN
15	OAL	Dufak RPN	1400	1	Jasmeet RPN
30	OAC	Dufak RPN	15	1	Jasmeet RPN
45	OAC	Dufak RPN	30	1	Jasmeet RPN
0700	OAC	Dufak RPN	45	1	Jasmeet RPN
15	OAC	Dufak RPN	1500	1	Jasmeet RPN
30	OAL	Lm RPN	15	1	Jasmeet RPN
45	OAL	Lm RPN	30	1	Jasmeet RPN
0800	OAL	Lm RPN	45	1	Jasmeet RPN
15	OAL	Lm RPN	1600	1	Jasmeet RPN
30	1	Lm RPN	15	1	Jasmeet RPN

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Page 2 of 2

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4.9. Visual Obs (FMH521500)

Mercy Health
Care first

Mercy Health

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Akinsola Akinbiyi

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

VISUAL OBSERVATIONS

Date: 05/03/2021

FMH521500

BINDING MARGIN – NO WRITING

Category of observation	Visual observation	Level	Activity
1 – Low Risk	Routine	1	Settled, minimal agitation
2 – Medium Risk	Hourly	2	Mildly aroused, pacing, still willing to talk reasonably, or withdrawn/isolated
3 – High Risk	30/60	3	Moderately aroused, agitated, disorganised, becoming more vocal and unreasonable, or hostile
4 – ICA High Risk	15/60	4	Highly aroused, grossly disorganised and possibly distressed or fearful
5 - Specialled High Risk	1:1 Constant Supervision	5	Highly aroused, violent towards self, others or property
Client Description:		X	Client not sighted, document reason in clinical notes
		0	Asleep **with A, B or C** **MUST have TWO of A, B & C documented** **e.g. 0 A C**
		A B C	A - Regular respiratory movements B - Change in sleeping position C - Audible respiratory sounds

Overall Risk Assessment rating in past 72 hours ☐

Client observation level 1-5 ☐

Exact Date & Time	Arousal Level	Print Name & Designation	Exact Date & Time	Arousal Level	Print Name & Designation
1630	1	Jasmeet RPN	2015	1	Jasmeet RPN
45	1	Jasmeet RPN	30	1	Jasmeet RPN
1700	1	Jasmeet RPN	45	1	Jasmeet RPN
15	1	Jasmeet RPN	2100	1	Jasmeet RPN
30	1	Jasmeet RPN	15	1	JN RSE
45	1	Jasmeet RPN	30	1	JN "
1800	1	Jasmeet RPN	45	1	JN "
15	1	[illegible] RPN	2200	1	JN PSA
30	1	[illegible] RPN	15	1	JN "
45	1	[illegible] RPN	30	1	JN "
1900	1	[illegible] RPN	45	1	JN "
15	1	[illegible] RPN	2300	1	JN "
30	1	[illegible] RPN	15	OAC	JN
45	1	Jasmeet RPN	30	OAC	JN PS
2000	1	Jasmeet RPN	45	OAC	JN R

VISUAL OBSERVATIONS ED 0044 AD 2900

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Page 1 of 2

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4.10. Visual Obs (FMH521500)

Mercy Health
Care first

Mercy Health

VISUAL OBSERVATIONS

Date:.....

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Akinsola Akinbiyi

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

(if no UR)

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Exact Date & Time	Arousal Level	Print Name & Designation	Exact Date & Time	Arousal Level	Print Name & Designation

4.11. Visual Obs (FMH521500)

Mercy Health
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Mercy Health

UR No:

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Akinsola Akinbiyi

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

VISUAL OBSERVATIONS

Date: 06/03/21

8

FMH521500

BINDING MARGIN – NO WRITING

Category of observation	Visual observation	Level	Activity
1 – Low Risk	Routine	1	Settled, minimal agitation
2 – Medium Risk	Hourly	2	Mildly aroused, pacing, still willing to talk reasonably, or withdrawn/isolated
3 – High Risk	30/60	3	Moderately aroused, agitated, disorganised, becoming more vocal and unreasonable, or hostile
4 – ICA High Risk	15/60	4	Highly aroused, grossly disorganised and possibly distressed or fearful
5 - Specialled High Risk	1:1 Constant Supervision	5	Highly aroused, violent towards self, others or property
Client Description:		X	Client not sighted, document reason in clinical notes
		0	Asleep **with A, B or C** **MUST have TWO of A, B & C documented** **e.g. 0 A C**
		A B C	A - Regular respiratory movements B - Change in sleeping position C - Audible respiratory sounds

Overall Risk Assessment rating in past 72 hours ☐

Client observation level 1-5 ☐

Exact Date & Time	Arousal Level	Print Name & Designation	Exact Date & Time	Arousal Level	Print Name & Designation
0000	1	JN PSEN	0345	OAC	JN PSEN
15	1	JN PSEN	0400	OAB	JN "
30	OAC	JN PSE	15	OAC	JN "
45	OAC	JN	30	OAC	JN
0100	OAC	JN PSEN	45	OAC	JN
15	1	JN	0500	OAC	JN "
30	1	JN	15	OAC	JN PSEN
45	OAC	JN	30	OAC	JN PSEN
0200	OAC	JN PSEN	45	1	JN PSEN
15	OAB	JN	0600	OAC	JN "
30	OAC	JN	15	OAC	JN "
45	OAC	JN	30	OAC	JN "
0300	OAC	JN PSEN	45	OAC	JN PSEN
15	1	JN PSEN	0700	OAC	JN PSEN
30	OAB	JN PSEN	15	OAC	

VISUAL OBSERVATIONS ED 0044 AD 2900

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Page 1 of 2

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4.12. Visual Obs (FMH521500)

Mercy Health
Care first

Mercy Health

8

VISUAL OBSERVATIONS

Date: 06/03/21

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Akinsola Akinbiyi

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

(if no UR)

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Exact Date & Time	Arousal Level	Print Name & Designation	Exact Date & Time	Arousal Level	Print Name & Designation
0730	1	Jevons	1515	1	Jevons
45	1	Jevons	30	1	Jevons
0800	1	Jevons	45	1	Jevons
15	1	Jevons	1600	1	Jevons
30	1	Jevons	15	1	Jevons
45	1	Jevons	30	1	Jevons
0900	1	Jevons	45	1	Jevons
15	1	Jevons	1700	1	Jevons
30	1	Jevons	15	1	Jevons
45	1	Jevons	30	1	Jevons
1000	1	Jevons	45	1	Jevons
15	1	Jevons	1800	1	Jevons
30	1	Jevons	15	1	Jevons
45	1	Jevons	30	1	Jevons
1100	1	Jevons	45	1	Jevons
15	1	Jevons	1900	OAC	Jevons
30	1	Jevons	15	OAC	Jevons
45	1	Jevons	30	OAC	Jevons
1200	1	Jevons	45	OAC	Jevons
15	1	Jevons	2000	OAC	Jevons
30	1	Jevons	15	1	Jevons
45	1	Jevons	30	1	Jevons
1300	1	Jevons	45	1	Jevons
15	1	Jevons	2100	1	Jevons
30	1	Jevons	15	1	Shalom RMO
45	1	Jevons	30	1	Shalom
1400	1	Jevons	45	1	Shalom
15	1	Jevons	2200	1	Shalom
30	1	Jevons	15	1	Shalom
45	1	Jevons	30	1	Shalom
1500	1	Jevons	45	1	Shalom

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Page 2 of 2

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4.13. Visual Obs (FMH521500)

Mercy Health
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Mercy Health

(8)

UR No:

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Akinsola Akinbiyi

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

VISUAL OBSERVATIONS

Date: 07/3/21

FMH521500

BINDING MARGIN – NO WRITING

Category of observation	Visual observation	Level	Activity
1 – Low Risk	*Routine*	*1*	*Settled, minimal agitation*
2 – Medium Risk	*Hourly*	*2*	*Mildly aroused, pacing, still willing to talk reasonably, or withdrawn/isolated*
3 – High Risk	*30/60*	*3*	*Moderately aroused, agitated, disorganised, becoming more vocal and unreasonable, or hostile*
4 – ICA High Risk	*15/60*	*4*	*Highly aroused, grossly disorganised and possibly distressed or fearful*
5 - Specialled High Risk	*1:1 Constant Supervision*	*5*	*Highly aroused, violent towards self, others or property*
Client Description:		*X*	*Client not sighted, document reason in clinical notes*
		0	*Asleep* **with A, B or C** **MUST have TWO of A, B & C documented** **e.g. 0 A C**
		A *B* *C*	*A - Regular respiratory movements* *B - Change in sleeping position* *C - Audible respiratory sounds*
Overall Risk Assessment rating in past 72 hours ☐		*Client observation level 1-5* ☐	

Exact Date & Time	Arousal Level	Print Name & Designation	Exact Date & Time	Arousal Level	Print Name & Designation
2300	OAL	Shalom RSM			
15	OAL	Shalom			
30	OAL	Shalom			
45	OAL	Shalom			

VISUAL OBSERVATIONS ED 0044 AD 2900

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Page 1 of 2

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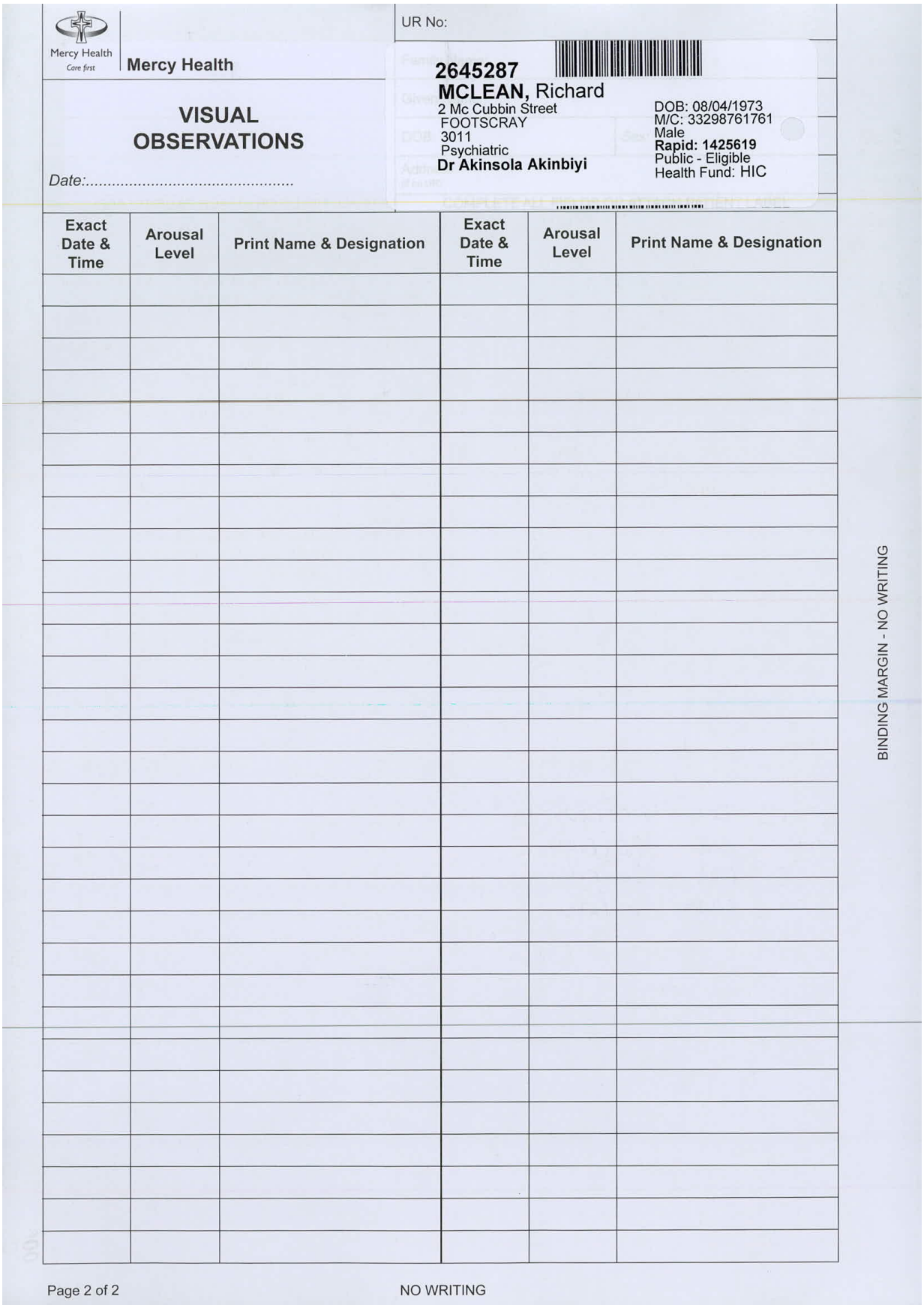
4.14. Visual Obs (FMH521500)



Mercy Health
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Mercy Health

UR No:

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Akinsola Akinbiyi

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

VISUAL OBSERVATIONS

Date:.....

Exact Date & Time	Arousal Level	Print Name & Designation	Exact Date & Time	Arousal Level	Print Name & Designation

4.15. Visual Obs (FMH521500)

Mercy Health
Care first

Mercy Health

UR No:

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Akinsola Akinbiyi

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

VISUAL OBSERVATIONS

Date:.....

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH521500

Category of observation	Visual observation	Level	Activity
1 – Low Risk	Routine	1	Settled, minimal agitation
2 – Medium Risk	Hourly	2	Mildly aroused, pacing, still willing to talk reasonably, or withdrawn/isolated
3 – High Risk	30/60	3	Moderately aroused, agitated, disorganised, becoming more vocal and unreasonable, or hostile
4 – ICA High Risk	15/60	4	Highly aroused, grossly disorganised and possibly distressed or fearful
5 - Specialled High Risk	1:1 Constant Supervision	5	Highly aroused, violent towards self, others or property
Client Description:		X	Client not sighted, document reason in clinical notes
		0	Asleep **with A, B or C** **MUST have TWO of A, B & C documented** **e.g. 0 A C**
		A B C	A - Regular respiratory movements B - Change in sleeping position C - Audible respiratory sounds

Overall Risk Assessment rating in past 72 hours ☐

Client observation level 1-5 ☐

Exact Date & Time	Arousal Level	Print Name & Designation	Exact Date & Time	Arousal Level	Print Name & Designation
0000	1	Shalom RPN	0345	OAC	Shalom
15	1	Shalom RPN.	0400	OAC	Fola RPN
30	1	Shalom RPN	15	OAC	Fola RPN
45	1	Shalom RPN	30	OAC	Fola RPN
0100	1	Shalom	45	OAC	Fola RPN
15	1	Shalom	0500	OAC	Fola RPN
30	OAC	Shalom	15	OAC	Fola RPN
45	OAC	Shalom	30	OAC	Fola RPN
0200	OAC	Shalom	45	OAC	Shalom RPN
15	OAC	Shalom	0600	OAC	Shalom
30	OAC	Shalom	15	OAC	Shalom
45	OAC	Shalom	30	OAC	Shalom
0300	OAC	Shalom	45	OAC	Shalom
15	OAC	Shalom	0700	OAC	Shalom
30	OAC	Shalom	15	OAC	Shalom

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VISUAL OBSERVATIONS ED 0044 AD 2900

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4.16. Visual Obs (FMH521500)

Mercy Health
Care first

Mercy Health

UR No:

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Akinsola Akinbiyi

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

VISUAL OBSERVATIONS

Date: 07/03/21

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Exact Date & Time	Arousal Level	Print Name & Designation	Exact Date & Time	Arousal Level	Print Name & Designation
0130	OAC	Thenjiwe RPN	1530	1	RN
45	OAC	Thenjiwe RPN	1600	1	RN
0800	1	Thenjiwe RPN	30	1	RN
15	1	Thenjiwe RPN	1700	1	RN
30	1	Thenjiwe RPN	30	1	RN
45	1	Thenjiwe RPN	1800	1	RN
0900	1	Thenjiwe RPN	30	1	RN
15	1	Thenjiwe RPN	1900	1	RN
30	1	Thenjiwe RPN	30	1	Jelaude RN
45	OAC	Thenjiwe RPN	2000	1	Jelaude RN
1000	OAC	Thenjiwe RPN	30	1	Jelaude RN
15	OAC	Thenjiwe RPN	2100	1	RN
30	OAC	Thenjiwe RPN	30	1	Sumu RPN
45	1	Thenjiwe RPN	2200	1	Sumu RPN
1100	1	Thenjiwe RPN	30	1	Sumu RPN
15	1	Thenjiwe RPN	2300	1	Fola RPN
30	1	Thenjiwe RPN	30	OAC	Fola RPN
45	1	Thenjiwe RPN			
1200	1	Thenjiwe RPN			
15	1	Thenjiwe RPN			
30	1	Thenjiwe RPN			
45	1	Thenjiwe RPN			
1300	1	RN			
15	1	RN			
30	1	RN			
45	1	RN			
1400	1	RN			
15	1	RN			
30	1	RN			
45	1	RN			
1500	1	RN			

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Page 2 of 2

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4.17. Visual Obs (FMH521500)

Mercy Health
Care first

Mercy Health

(8)

UR No: 2645287

MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Akinsola Akinbiyi

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

VISUAL OBSERVATIONS

Date: 8/3/2021

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH521500

BINDING MARGIN – NO WRITING

Category of observation	Visual observation	Level	Activity
1 – Low Risk	*Routine*	*1*	*Settled, minimal agitation*
2 – Medium Risk	*Hourly*	*2*	*Mildly aroused, pacing, still willing to talk reasonably, or withdrawn/isolated*
3 – High Risk	*30/60*	*3*	*Moderately aroused, agitated, disorganised, becoming more vocal and unreasonable, or hostile*
4 – ICA High Risk	*15/60*	*4*	*Highly aroused, grossly disorganised and possibly distressed or fearful*
5 - Specialled High Risk	*1:1 Constant Supervision*	*5*	*Highly aroused, violent towards self, others or property*
Client Description:		*X*	*Client not sighted, document reason in clinical notes*
		0	*Asleep* **with A, B or C MUST have TWO of A, B & C documented e.g. 0 A C**
		A *B* *C*	*A - Regular respiratory movements* *B - Change in sleeping position* *C - Audible respiratory sounds*

Overall Risk Assessment rating in past 72 hours ☐

Client observation level 1-5 ☐

Exact Date & Time	Arousal Level	Print Name & Designation	Exact Date & Time	Arousal Level	Print Name & Designation
0000	OAC	Sumudu MAN	0000	OAC	HNdlou RPN
30	OAC	Sumu RPN	30	OAC	HNdlou RPN
0100	OAC	Sumu RPN	0900	1	HNdlou RPN
30	OAC	Sumu RPN	30	1	HNdlou RPN
0200	OAC	Sumu RPN	1000	1	HNdlou RPN
30	OAC	Fola RPN	30	1	HNdlou RPN
0300	OAC	Fola RPN	1100	1	HNdlou RPN
30	OAC	Fola RPN	30	1	HNdlou RPN
0400	OAC	Fola RPN	1200	1	HNdlou RPN
30	OAC	Fola RPN	30	1	HNdlou RPN
0500	OAC	Fola RPN	1300	1	HNdlou RPN
30	OAC	Shalom RPN	30	1	HNdlou RPN
0600	OAC	Shalom RPN	1400	1	H Ibu RPN
0730	1	HNdlou RPN	30	OAC	H. Ibu RPN
0800	1	HNdlou RPN	1500	OAC	H. Ibu RPN

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4.18. Visual Obs (FMH521500)

Mercy Health
Care first

Mercy Health

UR No:

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(if n

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Akinsola Akinbiyi

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

VISUAL OBSERVATIONS

Date:.....

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Exact Date & Time	Arousal Level	Print Name & Designation	Exact Date & Time	Arousal Level	Print Name & Designation
1600	1	H-Blom RPN			
30	1	H-Blom RPN			
1700	1	H-Blom RPN			
30	1	H-Dm			
1800	1	H-Bm			
30	1	H Dm			
1900	1	H Bm			
30	1	H Bm			
2000	1	H Bm			
30	1	H Bm			
2100	1	H Bm			
30	OAC	Sarath: KS RPN			
2200	OAC	Sarath: KS RPN			
30	OAC	Sarath: KS RPN			
2300	OAB	Sarath: KS RPN			
30	1	JN PS			

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Mercy Health
Care first

4.19. Visual Obs (FMH521500)

Mercy Health Care first

Mercy Health

8

UR No: 2[illegible]5287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Akinsola Akinbiyi
DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

VISUAL OBSERVATIONS

Date: 9/3/21

FMH521500

BINDING MARGIN – NO WRITING

Category of observation	Visual observation	Level	Activity
1 – Low Risk	Routine	1	Settled, minimal agitation
2 – Medium Risk	Hourly	2	Mildly aroused, pacing, still willing to talk reasonably, or withdrawn/isolated
3 – High Risk	30/60	3	Moderately aroused, agitated, disorganised, becoming more vocal and unreasonable, or hostile
4 – ICA High Risk	15/60	4	Highly aroused, grossly disorganised and possibly distressed or fearful
5 - Specialled High Risk	1:1 Constant Supervision	5	Highly aroused, violent towards self, others or property
Client Description:		X	Client not sighted, document reason in clinical notes
		0	Asleep **with A, B or C** **MUST have TWO of A, B & C documented** **e.g. 0 A C**
		A B C	A - Regular respiratory movements B - Change in sleeping position C - Audible respiratory sounds

Overall Risk Assessment rating in past 72 hours ☐

Client observation level 1-5 ☐

Exact Date & Time	Arousal Level	Print Name & Designation	Exact Date & Time	Arousal Level	Print Name & Designation
0000	1	Fola RPN	0800	1	HNdlan
30	OAC	Fola RPN	1830	1	HNdlan
0150	OAC	Fola RPN	0900	1	HNdlan
30	OAC	JN PSW	30	1	HNdlan
0200	OAC	JN PSW	1000	1	HNdlan
30	OAB	Sarath:k.s RPN	30	1	HNdlan
0300	OAC	Sarath:k.s RPN	1100	1	HNdlan
30	OAB	Sarath:k.s RPN	30	1	HNdlan
0400	OAC	Sarath:k.s RPN	1200	1	HNdlan
30	OAC	Sarath:k.s RPN	30	DISCharge	
0500	OAB	Sarath:k.s RPN			
30	OAC	Fola RPN			
0600	OAC	Fola RPN			
0700	OAC	HNdlan RPN			
0730	1	HNdlan RPN			

VISUAL OBSERVATIONS ED 0044 AD 2900

P1751 V2 02/17

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Page 1 of 2

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4.20. Visual Obs (FMH521500)

Mercy Health
Care first

Mercy Health

VISUAL OBSERVATIONS

Date:.....

UR No:
Family
Given
DOB
Address
(if

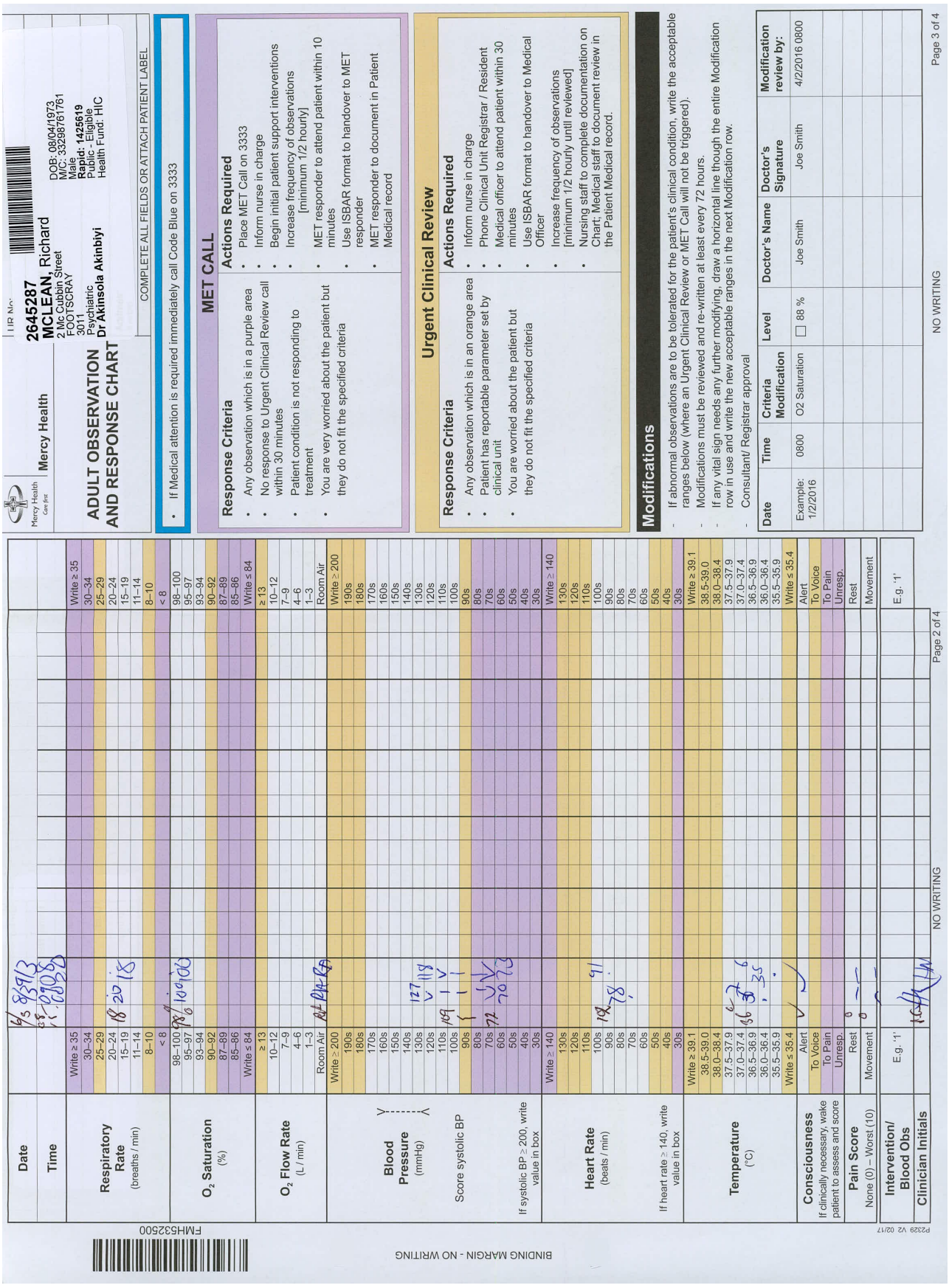
5287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Akinsola Akinbiyi
DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Exact Date & Time	Arousal Level	Print Name & Designation	Exact Date & Time	Arousal Level	Print Name & Designation

Mercy Health
Care first

4.21. Adult Obs & Response Chart (FMH532500)



Mercy Health
Care first

UR No: 2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Akinsola Akinbiyi
DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

ADULT OBSERVATION AND RESPONSE CHART

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

- If Medical attention is required immediately call Code Blue on 3333

MET CALL

Response Criteria	Actions Required
• Any observation which is in a purple area • No response to Urgent Clinical Review call within 30 minutes • Patient condition is not responding to treatment • You are very worried about the patient but they do not fit the specified criteria	• Place MET Call on 3333 • Inform nurse in charge • Begin initial patient support interventions • Increase frequency of observations [minimum 1/2 hourly] • MET responder to attend patient within 10 minutes • Use ISBAR format to handover to MET responder • MET responder to document in Patient Medical record

Urgent Clinical Review

Response Criteria	Actions Required
• Any observation which is in an orange area • Patient has reportable parameter set by clinical unit • You are worried about the patient but they do not fit the specified criteria	• Inform nurse in charge • Phone Clinical Unit Registrar / Resident • Medical officer to attend patient within 30 minutes • Use ISBAR format to handover to Medical Officer • Increase frequency of observations [minimum 1/2 hourly until reviewed] • Nursing staff to complete documentation on Chart; Medical staff to document review in the Patient Medical record.

Modifications

- If abnormal observations are to be tolerated for the patient's clinical condition, write the acceptable ranges below (where an Urgent Clinical Review or MET Call will not be triggered).
- Modifications must be reviewed and re-written at least every 72 hours.
- If any vital sign needs any further modifying, draw a horizontal line though the entire Modification row in use and write the new acceptable ranges in the next Modification row.
- Consultant/ Registrar approval

Date	Time	Criteria Modification	Level	Doctor's Name	Doctor's Signature	Modification review by:
Example: 1/2/2016	0800	O2 Saturation	☐ 88 %	Joe Smith	Joe Smith	4/2/2016 0800

NO WRITING

Page 3 of 4

Page 2 of 4

NO WRITING

BINDING MARGIN - NO WRITING

FMH532500

P2229 V2 02/17

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

DOB: 08-Apr-1973 URNO: 2645287 (MHS)
McLean, Richard

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

Mercy Health
Care first

Mercy Health

ADULT OBSERVATION AND RESPONSE CHART

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Akinsola Akinbiyi
DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

Address:
(if no UR)

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Other Observation Charts In Use

- ☐ Alcohol
- ☐ Daily Fluid Balance
- ☐ Diabetes Record
- ☐ Neurological
- ☐ Neurovascular
- ☐ Pain/Analgesia Observation
- ☐ Peripheral Intravenous Record
- ☐ _____
- ☐ _____

General Instructions

» You must record a full set of observations:
- On admission
- At a frequency appropriate for the patient's clinical state
- If the patient is deteriorating or an observation is in a shaded area
- Whenever you are concerned about the patient.

» When graphing observations, place a dot (•) in the centre of the box which includes the current observation in its range of values and connect it to the previous dot with a straight line. For blood pressure, use the symbol indicated on the chart.

» Whenever an observation falls within a shaded area, you must initiate the actions required for that colour, unless a modification has been made (see overleaf).

» If observations fall within both purple and orange coloured areas for the same time period, the actions required for the purple area apply.

Blood Observations

B	Baseline
15	15 minutes post commencement
H	Hourly observations
C	On Completion

Frequency of Observations/ Monitoring Plan

Monitoring Plan must be reviewed daily

Refer to Adult Observation and Response Chart Procedure for Frequency of Observation instructions/minimum standard

If Rapid Response system is activated, Frequency of Observations will be as per Adult Escalation of Care Procedure

Date:	(e.g.) 6/4/14						
Frequency of Observations:	2/24 or QID						
Name / Designation:	Smith RN						

ADULT OBSERVATION AND RESPONSE CHART AD 4420

NO WRITING

Mercy Health
Care first

Mercy Health

ADULT OBSERVATION AND RESPONSE CHART

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Akinsola Akinbiyi
DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Interventions Associated With Abnormal Vital Signs

If you administer an intervention, record here and note number in Intervention row over page in appropriate time column.

Reference Number	Intervention (initial if required)
1	
2	
3	
4	
5	
6	
7	
8	

Urgent Clinical Review Requests

Review requested Date / / Time : Dr Name & Designation

Specify reason:

Review requested Date / / Time : Dr Name & Designation

Specify reason:

Review requested Date / / Time : Dr Name & Designation

Specify reason:

Additional Observations

Date							
Time							
Weight (kg)							
Bowels							
Urinalysis	Specific gravity						
	pH						
	Leukocytes						
	Blood						
	Nitrite						
	Ketones						
	Bilirubin						
	Urobilinogen						
	Protein						
	Glucose						

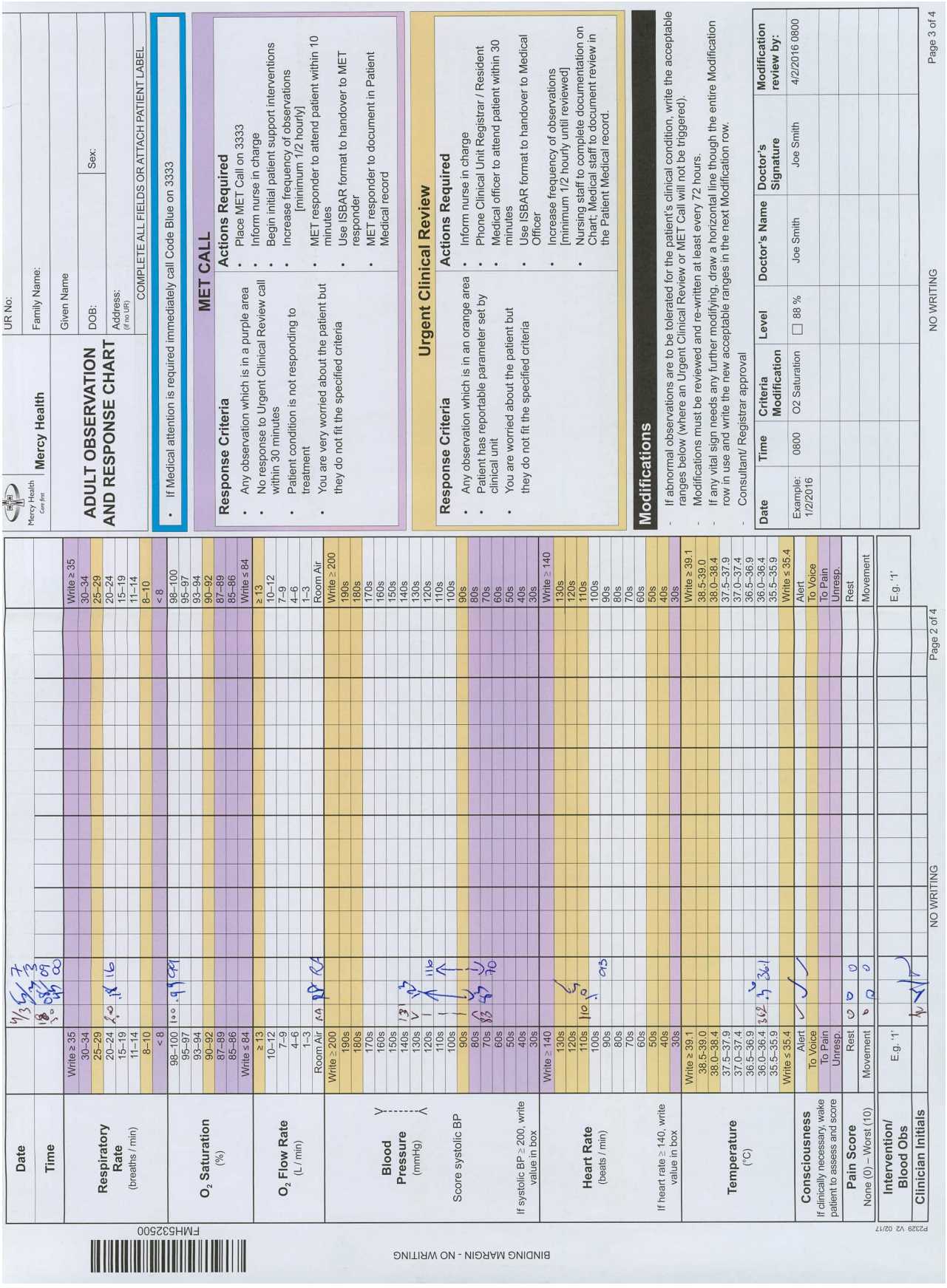
BINDING MARGIN - NO WRITING

NO WRITING

Mercy Health
Care first

Mercy Health
Care first

4.23. Adult Obs & Response Chart (FMH532500)



Mercy Health
Care first

ADULT OBSERVATION AND RESPONSE CHART

UR No:
Family Name:
Given Name
DOB: Sex:
Address: (if no UR)
COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

- If Medical attention is required immediately call Code Blue on 3333

MET CALL

Response Criteria	Actions Required
• Any observation which is in a purple area • No response to Urgent Clinical Review call within 30 minutes • Patient condition is not responding to treatment • You are very worried about the patient but they do not fit the specified criteria	• Place MET Call on 3333 • Inform nurse in charge • Begin initial patient support interventions • Increase frequency of observations [minimum 1/2 hourly] • MET responder to attend patient within 10 minutes • Use ISBAR format to handover to MET responder • MET responder to document in Patient Medical record

Urgent Clinical Review

Response Criteria	Actions Required
• Any observation which is in an orange area • Patient has reportable parameter set by clinical unit • You are worried about the patient but they do not fit the specified criteria	• Inform nurse in charge • Phone Clinical Unit Registrar / Resident • Medical officer to attend patient within 30 minutes • Use ISBAR format to handover to Medical Officer • Increase frequency of observations [minimum 1/2 hourly until reviewed] • Nursing staff to complete documentation on Chart; Medical staff to document review in the Patient Medical record.

Modifications

- If abnormal observations are to be tolerated for the patient's clinical condition, write the acceptable ranges below (where an Urgent Clinical Review or MET Call will not be triggered).
- Modifications must be reviewed and re-written at least every 72 hours.
- If any vital sign needs any further modifying, draw a horizontal line though the entire Modification row in use and write the new acceptable ranges in the next Modification row.
- Consultant/ Registrar approval

Date	Time	Criteria Modification	Level	Doctor's Name	Doctor's Signature	Modification review by:
Example: 1/2/2016	0800	O2 Saturation	☐ 88 %	Joe Smith	Joe Smith	4/2/2016 0800

NO WRITING

Page 3 of 4

Date		4/3	5/3	7/3
Time		18:00	05:00	09:00
Respiratory Rate (breaths / min)	Write ≥ 35			
	30–34			
	25–29			
	20–24	20	18	16
	15–19			
	11–14			
	8–10			
	< 8			
O_2 Saturation (%)	98–100	100	99	99
	95–97			
	93–94			
	90–92			
	87–89			
	85–86			
	Write ≤ 84			
O_2 Flow Rate (L / min)	≥ 13			
	10–12			
	7–9			
	4–6			
	1–3			
	Room Air	RA	RA	RA
Blood Pressure (mmHg)	Write ≥ 200			
	190s			
	180s			
	170s			
	160s			
	150s			
	140s	132	132	
	130s			118
	120s			
	110s			
	100s			
Score systolic BP	90s			
	80s	82	82	70
	70s			
	60s			
	50s			
	40s			
	30s			
If systolic BP ≥ 200, write value in box	Write ≥ 140			
Heart Rate (beats / min)	130s			
	120s		120	
	110s		110	
	100s			
	90s			
	80s			
	70s			
	60s			
	50s			
	40s			
	30s			
If heart rate ≥ 140, write value in box	Write ≥ 39.1			
Temperature (°C)	38.5–39.0			
	38.0–38.4			
	37.5–37.9			
	37.0–37.4			
	36.5–36.9	36.7	36.7	
	36.0–36.4			
	35.5–35.9			
	Write ≤ 35.4			
Consciousness If clinically necessary, wake patient to assess and score	Alert	✓	✓	
	To Voice			
	To Pain			
	Unresp.			
Pain Score None (0) – Worst (10)	Rest	0	0	
	Movement	0	0	
Intervention/ Blood Obs	E.g. '1'			
Clinician Initials				

NO WRITING

Page 2 of 4

BINDING MARGIN - NO WRITING

FMH532500

P2329 V2 02/17

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

DOB: 08-Apr-1973 URNO: 2645287 (MHS)
McLean, Richard

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

Mercy Health
Care first

Mercy Health

UR No: 2645287
Family Name: MCLEAN
Given Name: Richard
DOB: 08/04/1973 Sex:
Address: 2 Mc Cubbin Street FOOTSCRAY 3011
PH: 0400639248
Dr Arul Baradi
Medical C
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

ADULT OBSERVATION AND RESPONSE CHART

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Other Observation Charts In Use

- ☐ Alcohol
- ☐ Daily Fluid Balance
- ☐ Diabetes Record
- ☐ Neurological
- ☐ Neurovascular
- ☐ Pain/Analgesia Observation
- ☐ Peripheral Intravenous Record
- ☐ _____
- ☐ _____

General Instructions

» You must record a full set of observations:
- On admission
- At a frequency appropriate for the patient's clinical state
- If the patient is deteriorating or an observation is in a shaded area
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» Whenever an observation falls within a shaded area, you must initiate the actions required for that colour, unless a modification has been made (see overleaf).

» If observations fall within both purple and orange coloured areas for the same time period, the actions required for the purple area apply. 116/70 93 36 99

Blood Observations

B	Baseline
15	15 minutes post commencement
H	Hourly observations
C	On Completion

Frequency of Observations/ Monitoring Plan

Monitoring Plan must be reviewed daily

Refer to Adult Observation and Response Chart Procedure for Frequency of Observation instructions/minimum standard

If Rapid Response system is activated, Frequency of Observations will be as per Adult Escalation of Care Procedure

Date:	(e.g.) 6/4/14						
Frequency of Observations:	2/24 or QID						
Name / Designation:	Smith RN						

NO WRITING

ADULT OBSERVATION AND RESPONSE CHART AD 4420

Mercy Health
Care first

Mercy Health

UR No:
Family Name:
Given Name
DOB: Sex:
Address:
(if no UR)

ADULT OBSERVATION AND RESPONSE CHART

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Interventions Associated With Abnormal Vital Signs

If you administer an intervention, record here and note number in Intervention row over page in appropriate time column.

Reference Number	Intervention (initial if required)
1	
2	
3	
4	
5	
6	
7	
8	

Urgent Clinical Review Requests

Review requested Date / / Time : Dr Name & Designation

Specify reason:

Review requested Date / / Time : Dr Name & Designation

Specify reason:

Review requested Date / / Time : Dr Name & Designation

Specify reason:

Additional Observations

Date		
Time		
Weight (kg)		
Bowels		
Urinalysis	Specific gravity	
	pH	
	Leukocytes	
	Blood	
	Nitrite	
	Ketones	
	Bilirubin	
	Urobilinogen	
	Protein	
	Glucose	

BINDING MARGIN - NO WRITING

NO WRITING

Mercy Health
Care first

4.25. CPF Inpatient Checklist (FMH539600)

Mercy Health
Care first
Mercy Health

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Akinsola Akinbiyi
DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

CPF INPATIENT CHECKLIST

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH539600

BINDING MARGIN - NO WRITING

Discharge Information

Ward: Cmb.

Discharge Date: 9/3/2021.

Discharge Summary Completed: Yes ☑ No ☐

Statistical Discharge: Yes ☐ No ☐

Discharge Summary Faxed: Yes ☑ No ☐

To Ward: _____

Length of Stay > 10 days: Yes ☐ No ☑

Planned Follow-up

Planned Re-admission: Yes ☐ No ☑

Expected Date: _____

Outpatient Appointment:

Planned: Yes ☐ No ☑

Date (if known): _____

OR

Expected: Yes ☐ No ☑

48 Hours ☐ 1 week ☐ 2 weeks ☐

Compensable Status

Is the patient a private or ineligible patient: Yes ☑ No ☐

Record Compilation

All paperwork removed from folders and trays for this patient? Yes ☑ No ☐

Patient Labels on every page? Yes ☑ No ☐

Compiled by: ISABEL

Date Compiled: 9/3/2021.

HIS USE ONLY

Date Received:

Stamp here

HIS Received
11 MAR 2021

Prepped by: Chelsea

Date Prepped: 15/03/21

P2801 02/20

CPF INPATIENT CHECKLIST AD 4925

NO WRITING

Page 1 of 1

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5. Admission Episode:10894962:MHS

5.1. eDischargeSummaryMercy (FMH503101)

Clinical Handover eDischarge Summary

Mercy Health

Mercy Health
Care first

Hospital Site:

☑ Werribee Mercy Hospital 300 Princes Highway, Werribee Vic 3030 Ph: (03) 8754 3000

UR No		
2645287		
First Name	***Surname***	
Richard	McLean	
Date of Birth	***Phone Number***	
Sunday April 8, 1973	04 0063 9248	
Address	***Postcode***	
2 Mc Cubbin Street Footscray	3011	

Admission Details

Admission Date	***Discharge Date***	***Discharge Destination:***	***Clinical Unit***
Thursday February 25, 2021	Thursday March 4, 2021	Home/private residence	Baradi DR Arul

Aboriginal and/or Torres Strait Islander status	***Spoken language***	***Interpreter required***
Not stated		

Diagnosis

Principal Diagnosis	***Secondary Diagnosis***
Hypovolemic shock secondary to self inflicted LFT ACF wound	Acute psychosis

Reason for presentation

- 47M MET call in mental health unit
- Patient found collapsed on ground in shower, evidence of self harm to left cubital fossa (sustained by using vape machine) à 'squirting' blood. suicide attempt, note present

At MET call:

A – Patent, own

B – Spontaneous ventilation, SPO2 ?70% - Requiring high flow via non-rebreather on arrival

C – HR 120, BP unrecordable, cold, clammy, diaphoretic, unable to palpate arterial pulses

D - GCS E1V2M6 on arrival – rousable, groaning, calling out

E – Laceration to left anterior cubital fossa, no ongoing bleeding, superficial. No other obvious injuries.

Unable to site peripheral venous access for fluid, transferred to ICU

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Progress in ICU:

- Femoral CVC inserted
- 3L IVF stat
- Commenced noradrenaline, vasopressin
- R) radial art line inserted, IDC
- GCS improved over time to 15/15

Clinical Summary

 # Hypovolemic Shock

- Hypovolemia secondary to blood loss from laceration on Left cubital fossa
- Started on noradrenaline and vasopressin with blood pressure monitoring
- Hydrocortisone given 50mg QID for 48 hours and ceased
- Transfused three units PRBC on 26/2/21. Hemoglobin monitored (Hb on 27/2 – 87)
- High BSL (22.5) – managed with insulin infusion on 25/2 and subsequently stopped. Not a known diabetic. HbA1c 5.5. Latest BSL 5.9
- Elevated lactate 13 on ICU admission. Given 4 litres of fluid resuscitation. Normalised to 3
- No ECG changes or chest pain. Troponin inc from 48 -> 431 -> 160. (Corrected Qtc – 410 Normal)
- CT Angiography – Brain, Chest, Abdo/Pelvis and Left Arm done – no significant abnormality reported as a cause for severe hypotension

#Staphylococcus lugdunesis Bacteraemia

- Discussed with ID, likely 2ndary to contamination during insertion of CVC line in setting of severe hypotension
- Taken from CVC line on admission. CVC line removed
- Detected in aerobic bottle blood culture 25/2/2021
- Sensitive to Flucloxacillin, vancomycin, penicillin. No resistance
- Commenced on cephazolin 2g 8hrly
- ID plan, patient for 2 weeks of IV Cephazolin initially, this was changed to oral flucloxacillin and planned the same for 2 weeks total
- Endocarditis screen –ve. TTE showed LVEF60% normal left ventricular size and wall thickness. Hyperdynamic LV systolic function.

 # Left cubital fossa lacerations

- Sustained with glass from vape machine
- Received ADT booster in ICU

- Left arm CT angiogram- superficial injury. No hematoma/ pseudo-aneurysm
- Started on Tazocin. Changed to Cefazolin on 26/2
- Suturing done in ICU, by ICU resident on 26/2/2021. 7 nylon suture in situ
- Reviewed by surgical team on 27/02/21

#AKI, likely ATN 2ndary to hypoperfusion

- Cr168 on admission, normalized to 82 on discharge
- Had proteinuria and glycosuria on dipstick urine
- Spot Urine A/CR ratio 8.4, likely elevated secondary to AKI
- For ongoing monitoring of renal function

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

 # High BSL

- BSL 23 on arrival
- Not a known diabetic. HbA1c 5.5
- Commenced insulin infusion, BSLs improved and normalised

#Schizophrenia and suicidal ideation

- On involuntary treatment order

Assessed as ongoing high risk of suicide, requires ongoing admission to the mental health unit for assessment and treatment once medically cleared

Does the patient have any Co-Morbidities?

☑ Yes

☑ Drug dependence and or harmful use

☑ Mental Health

Other

Pregnancy related Co-Morbidities

☑ No

Paediatric related Co-Morbidities

☑ No

Did the patient have any Complications

☑ No

Procedures

Were any Procedures performed?

☑ Yes

sutures

Allergies or Adverse Reactions

Does the patient have any Allergies or Adverse Reactions?

☑ No known drug allergies

Medication on Discharge

Medication	***Dose/Route /Frequency***	***Status***	***Quantity Supplied***
flucloxacillin	po 500mg QID	New	

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

	Reason or Indication
	staph lugdensis

Has the patient been counselled on their Medication?

☑ Yes

Ceased Medication and reason for cessation

Were any medications ceased during this admission?

☑ Nil Medication ceased

Guardian, Medical Treatment Decision-Maker(s) or an Advance Care Directive

Does the patient have a Guardian, Medical Treatment Decision-Maker(s) or an Advance Care Directive?

☑ Yes

Does the patient have a guardian appointed by the Victorian Civil and Administrative Tribunal (VCAT) to make decisions on medical treatment?

☑ No/Not applicable

Does the patient have a Medical Treatment Decision-Maker(s)?

☑ Discussed during admission

Does the patient have an Advance Care Directive?

☑ Discussed during the admission

Advice and or Request to GP

1. Ongoing wound reviews. Sutures to remain insitu for 7 days and arm needs to be kept straight for 7 days. For review of ?Removal of sutures Fri 5/3/2021
2. Monitor Hemoglobin and blood pressure for any further drop
3. Chase subsequent blood culture and sensitivty reports for growth of any other organisms
4. Continue oral flucloxacillin 500mg Q6hrly till 2 weeks are completed from 25/02/2021
5. Ongoing consultation with psychiatry unit

Alerts and or Risks

Alerts/Risks (infectious risk, falls, pressure injuries, delirium, malnutrition, social alerts, special needs)

☑ Yes

Any social alerts should be discussed clearly with patient

Follow-up Appointments

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

Description (GP/OPD/Other)	When (Date/Weeks)	Reason

Instructions provided to the Patient

Handout provided to patient
☑ No

Recipients of Discharge Summary

Name:	Address:	Fax:
DR Gp Unknown	,	

Author or Completing Clinician

Completing Doctor

Given Name
Kim

Family Name
Vo

Provider Number
5374685B

Treating Team

Treating Team contact details (Registrar Hospital number)
medical

Author	Date	Time
Shruthi Subramanyan - Unknown designation	4/3/2021	15:58:11

This Document is eDS Sent

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

5.2. Pt Registration Form (FMH501000)

PATIENT REGISTRATION FORM

Mercy Health
Care first

FMH501000

UR: 2645287

Patient Surname: McLean

Patient Forename: Richard (Mr)

Patient Demographic Details

Surname	Forename	Date Of Birth	Age
McLean	Richard	08/04/1973	47 years, 10 months

Marital Status	Sex	Country Of Birth
Not stated/inadequately described	Male	Not Stated

Address	Contact Details
2 Mc Cubbin Street, FOOTSCRAY, 3011	Home: 0400639248 Work: Mobile:

Religion	ATSI Status	Language spoken
Not Specified	NOT Aboriginal or TSI	English

Patient Contact/Personal Carers

Next of Kin	Relationship	Contact Telephone Number
Jodie Bongetti	Sister	Home: 0427309049. Mobile:

GP Details

Surname	Forename	Address	Phone / Fax	Preference	Role
Unknown	Gp	,	P: F:	Primary	Not Specified

Insurance Details

Administrative Category for this admission:	Public - Eligible
Medicare Number	**Medicare Number Expiry Date**
33298761761	__/__/____
Health Care Card Number	**DVA Number**

Health Fund	Health Fund Number	Insurance Plan
Medicare Australia	0	Not Applicable

Admission & Discharge Details

Admission Date	Admission Time	Admitting Ward
Thursday 25th February 2021	2:26 pm	W CMC ICU

Admission Source	Admission Clinician	Clinician Specialty
Statistical Admission	Dr Arul Baradi	Medical C

Discharge Date	Discharge Time	Discharge To

Post Discharge

Compiled By:	Date:		
Coded By:	Date:	DRG:	WIES:
Audited By:	Date:	DRG:	WIES:
Audit Type:			

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

5.3. Pt Registration Form (FMH501000)

FMH501000

PATIENT REGISTRATION FORM

Mercy Health
Care first

UR: 2645287

Patient Surname: McLean

Patient Forename: Richard (Mr)

Patient Demographic Details

Surname	Forename	Date Of Birth	Age
McLean	Richard	08/04/1973	47 years, 10 months

Marital Status	Sex	Country Of Birth
Not stated/inadequately described	Male	Not Stated

Address	Contact Details
2 Mc Cubbin Street, FOOTSCRAY, 3011	Home: 0400639248 Work: Mobile:

Religion	ATSI Status	Language spoken
Not Specified	NOT Aboriginal or TSI	English

Patient Contact/Personal Carers

Next of Kin	Relationship	Contact Telephone Number
Doug McLean + Brad (Brother) 0435033888	Father	Home: 0416104587. Mobile:

GP Details

Surname	Forename	Address	Phone / Fax	Preference	Role
Unknown	Gp	,	P: F:	Primary	Not Specified

Insurance Details

Administrative Category for this admission:	Public - Eligible
Medicare Number	**Medicare Number Expiry Date**
33298761761	__/__/____
Health Care Card Number	**DVA Number**

Health Fund	Health Fund Number	Insurance Plan
Insurance Fund Unknown	0	Insured - Level Unknown

Admission & Discharge Details

Admission Date	Admission Time	Admitting Ward
Thursday 25th February 2021	2:26 pm	W CMC ICU
Admission Source	**Admission Clinician**	**Clinician Specialty**
Statistical Admission	Dr Arul Baradi	Medical C
Discharge Date	**Discharge Time**	**Discharge To**

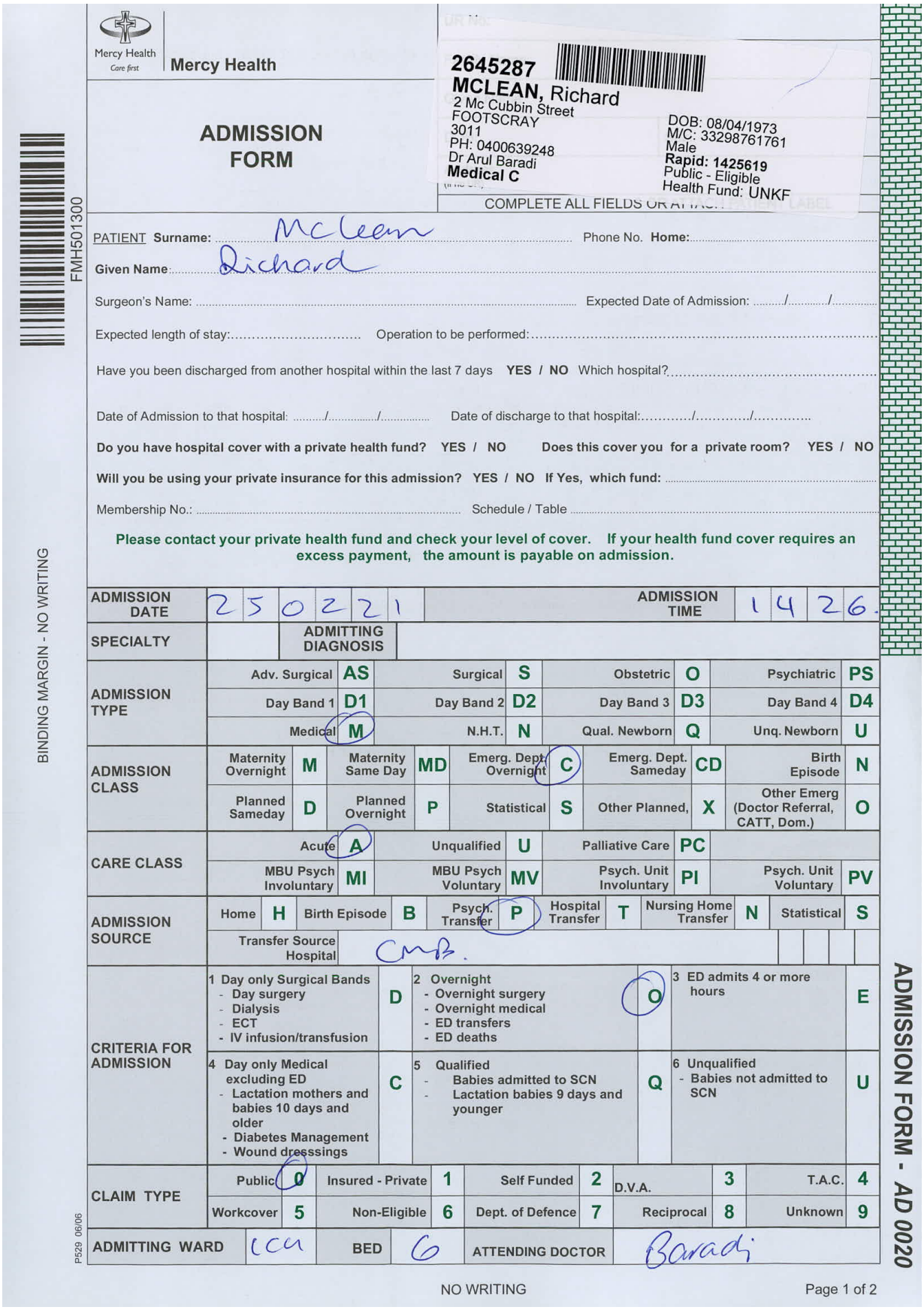
Post Discharge

Compiled By:	Date:		
Coded By:	Date:	DRG:	WIES:
Audited By:	Date:	DRG:	WIES:
Audit Type:			

Sister 0427309049

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

5.4. Adm Form (FMH501300)



Mercy Health
Care first

Mercy Health

ADMISSION FORM

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

COMPLETE ALL FIELDS OR AFFIX PATIENT LABEL

FMH501300

PATIENT **Surname:** McLean Phone No. **Home:**

Given Name Richard

Surgeon's Name: Expected Date of Admission: ../../..

Expected length of stay:..... Operation to be performed:.....

Have you been discharged from another hospital within the last 7 days **YES / NO** Which hospital?.....

Date of Admission to that hospital: ../../.. Date of discharge to that hospital:../../..

Do you have hospital cover with a private health fund? YES / NO Does this cover you for a private room? YES / NO

Will you be using your private insurance for this admission? YES / NO If Yes, which fund:

Membership No.: Schedule / Table

Please contact your private health fund and check your level of cover. If your health fund cover requires an excess payment, the amount is payable on admission.

ADMISSION DATE	2 5 0 2 2 1						**ADMISSION TIME**	1 4 2 6							
SPECIALTY		**ADMITTING DIAGNOSIS**													
ADMISSION TYPE	Adv. Surgical	AS	Surgical	S	Obstetric	O	Psychiatric	PS							
	Day Band 1	D1	Day Band 2	D2	Day Band 3	D3	Day Band 4	D4							
	Medical	M	N.H.T.	N	Qual. Newborn	Q	Unq. Newborn	U							
ADMISSION CLASS	Maternity Overnight	M	Maternity Same Day	MD	Emerg. Dept. Overnight	C	Emerg. Dept. Sameday	CD	Birth Episode	N					
	Planned Sameday	D	Planned Overnight	P	Statistical	S	Other Planned,	X	Other Emerg (Doctor Referral, CATT, Dom.)	O					
CARE CLASS	Acute	A	Unqualified	U	Palliative Care	PC									
	MBU Psych Involuntary	MI	MBU Psych Voluntary	MV	Psych. Unit Involuntary	PI	Psych. Unit Voluntary	PV							
ADMISSION SOURCE	Home	H	Birth Episode	B	Psych. Transfer	P	Hospital Transfer	T	Nursing Home Transfer	N	Statistical	S			
	Transfer Source Hospital	CMB.													
CRITERIA FOR ADMISSION	1 Day only Surgical Bands - Day surgery - Dialysis - ECT - IV infusion/transfusion	D	2 Overnight - Overnight surgery - Overnight medical - ED transfers - ED deaths	O	3 ED admits 4 or more hours	E									
	4 Day only Medical excluding ED - Lactation mothers and babies 10 days and older - Diabetes Management - Wound dresssings	C	5 Qualified - Babies admitted to SCN - Lactation babies 9 days and younger	Q	6 Unqualified - Babies not admitted to SCN	U									
CLAIM TYPE	Public	0	Insured - Private	1	Self Funded	2	D.V.A.	3	T.A.C.	4					
	Workcover	5	Non-Eligible	6	Dept. of Defence	7	Reciprocal	8	Unknown	9					
ADMITTING WARD	ICU	**BED**	6	**ATTENDING DOCTOR**	Baradi										

BINDING MARGIN - NO WRITING

P529 06/06

ADMISSION FORM - AD 0020

NO WRITING

Page 1 of 2

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5.5. Adm Form (FMH501300)

FORM OF ELECTION - TO BE COMPLETED AT THE TIME OF ADMISSION FOR PUBLIC & COMPENSABLE PATIENTS

I (name)..... **of** (address).....

.....**elect for myself / on behalf of** (patient name).....

to be treated as a private (chargeable) Patient, Public (non-chargeable) Patient or Compensable / Veterans' Affairs Patient as indicated below, and that provided I (or my agent) has made an informed decision, my initial ELECTION STATUS cannot be changed unless for those unforeseen circumstances outlined below in *Victorian Public Hospitals – Information for Patients.*

PRIVATE (CHARGEABLE) PATIENT

Choosing option 1, 2 below or both will make me a private patient and I will be responsible for all charges listed below. I understand that my health insurance policy may not fully cover the costs incurred by treatment as a private patient.

(a) hospital accommodation fees
(b) charges for medical services, including diagnostic services performed and / or ordered by the doctor of my choice or by other medical practitioners to whom I may be referred.
(c) prosthesis, and
(d) dental services

Option 1. I elect to be treated by Dr. ☐
Option 2. I elect to be treated in a single room Yes / No ☐

Signed			
Relationship to patient *(if applicable)*		**Date**	

PUBLIC (NON-CHARGEABLE) PATIENT OR

Choosing this option will mean that I am not charged for the medical, hospital, prosthetic or dental services provided to me and that I will be treated by a doctor(s), nominated by the hospital

Signed			
Relationship to patient *(if applicable)*	Covid period.	**Date**	

COMPENSABLE / VETERANS' AFFAIRS OR

I (name) ..... **declare that in the event of the rejection of a claim for hospitalisation**

As a Workcover; ☐ **Department of Veterans' Affairs;** ☐
Common Law Damages ☐ **Criminal Injuries Compensation;** ☐
Transport Accident Commission ☐ **Other** *(please specify)* ☐

Patient I elect to be classified as a private patient ☐ **public patient** ☐

I have read the information for Patients and I understand the consequences of this election.

Signed			
Relationship to patient *(if applicable)*		**Date**	

DECLARATION BY HOSPITAL EMPLOYEE AS WITNESS

I, (print name)	Rebecca	**have witnessed the above election**	
Signed & Position	Clerk ICU	**Date**	25/2/21

Victorian Public Hospitals – Information for Patients

The Medicare Agreement between the Commonwealth and State governments requires that **YOU** (or your agent) **ELECT** to be treated as a **PUBLIC** (non-chargeable) or **PRIVATE** (chargeable) patient upon admission to this hospital.

YOU (or your agent, having made an **INFORMED** election to be treated as a **PUBLIC** or **PRIVATE** patient upon admission to this hospital, cannot change your initial election status except for the following "unforeseen" circumstances:

(i) A change in medical circumstances, i.e. patients who are admitted for a particular procedure but are found to have co-morbidities not evident at admission, or develop complications;
(ii) A change in social circumstances, i.e. change in income status resulting in an inability to pay (loss of job, or bankruptcy).

A PUBLIC PATIENT

- will be treated by doctors nominated by the hospital;
- cannot choose a specific doctor to provide his / her medical treatment, and
- will not be charged for medical or hospital services.

YOU CANNOT choose to be a **PUBLIC PATIENT** if you elect

- to be treated by a doctor of you own choice; or
- to occupy a bed in a single room.

A PRIVATE PATIENT

- will be treated by his / her nominated doctor(s) provided that the doctor(s) has the right to practise at the hospital; and
- will be **RESPONSIBLE FOR THE PAYMENT** of the hospital accommodation fees, charges for all medical services and prosthesis and dental fees.

PRIVATE HEALTH INSURANCE

- **DOES NOT** stop you from electing to be a public patient:
- **WILL COVER** full cost of shared ward accommodation but may not fully cover costs (depending on level of cover) if single room accommodation is chosen and you are not in a "front end deductible scheme";
- Medicare will cover 75% of the Commonwealth Medical Benefits Schedule fee for the medical services provided to private patients while in hospital and private health insurance will cover the remaining 25% of the fee;
- where a doctor charges a fee which exceeds the Commonwealth Medical Benefits Schedule fee, the patient will be responsible for paying the difference between the fee charged by the doctor and the Schedule fee.

COMPENSABLE PATIENTS

If you are, or may be, entitled to, (or have already received) compensation, damages or other benefits in respect of the injury, illness or disease for which you are receiving hospital care and medical treatment, all fees and charges may be met by your compensation. This includes, for example, compensation under the *Accident Compensation Act 1985 (Vic), Transport Accident Act 1986 (Vic), Criminal Injuries Compensation Act 1983 (Vic), Safety Rehabilitation and Compensation Act 1988 (Cth),* or a claim for damages at common law.

VETERANS' AFFAIRS PATIENTS

Veterans' Affairs patients are those for whom the Department of Veterans' Affairs has agreed to accept responsibility for hospital charges for the condition for which you are being admitted.

BINDING MARGIN - NO WRITING

Page 2 of 2 NO WRITING

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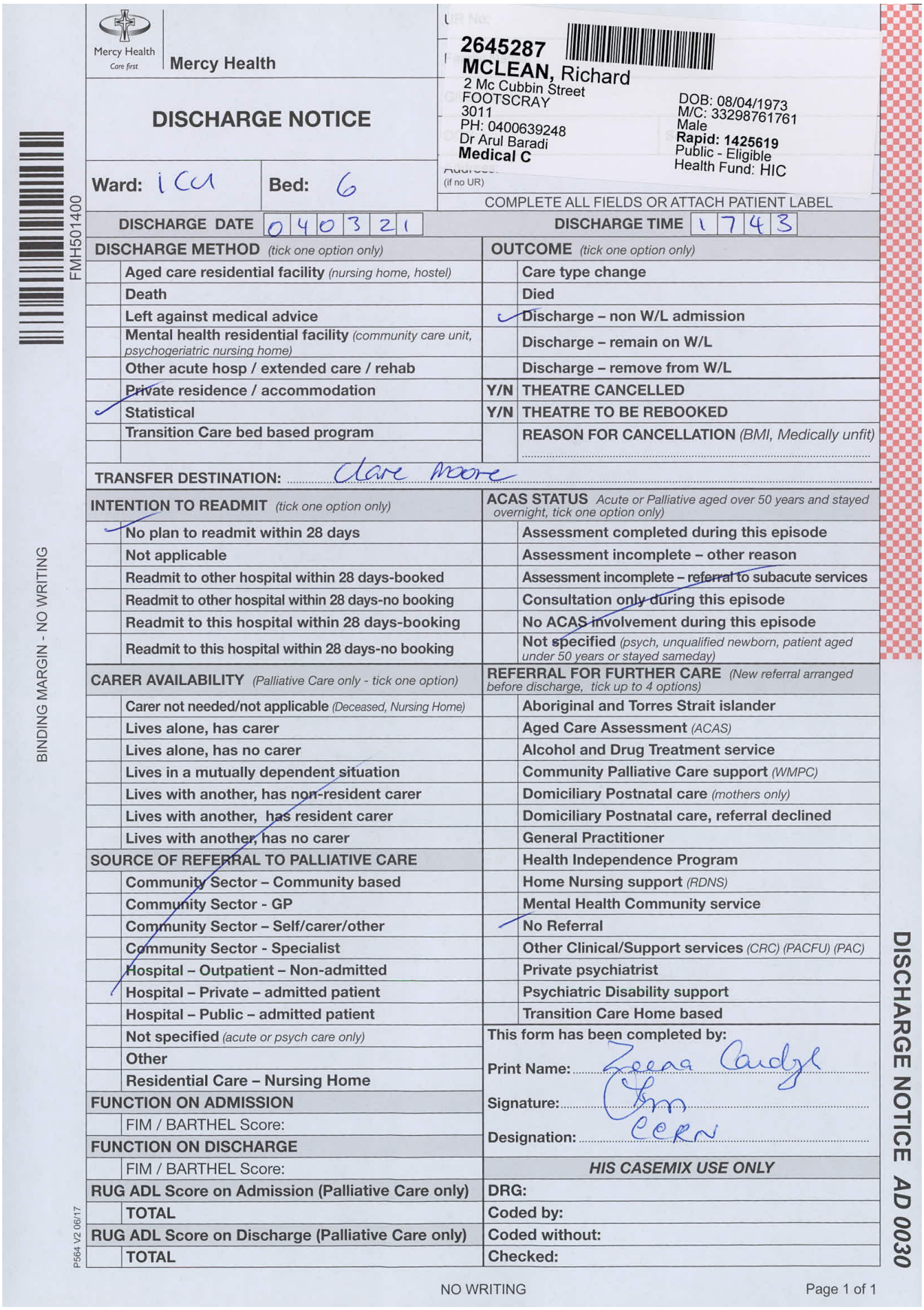
5.6. DC Notice (FMH501400)



Mercy Health
Care first

Mercy Health

DISCHARGE NOTICE

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C
DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

Ward: ICU Bed: 6

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

DISCHARGE DATE 04/03/21 DISCHARGE TIME 1743

DISCHARGE METHOD *(tick one option only)*	OUTCOME *(tick one option only)*
Aged care residential facility *(nursing home, hostel)*	Care type change
Death	Died
Left against medical advice	✓ Discharge – non W/L admission
Mental health residential facility *(community care unit, psychogeriatric nursing home)*	Discharge – remain on W/L
Other acute hosp / extended care / rehab	Discharge – remove from W/L
Private residence / accommodation	Y/N THEATRE CANCELLED
✓ Statistical	Y/N THEATRE TO BE REBOOKED
Transition Care bed based program	REASON FOR CANCELLATION *(BMI, Medically unfit)*

TRANSFER DESTINATION: clare moore

INTENTION TO READMIT *(tick one option only)*	ACAS STATUS *Acute or Palliative aged over 50 years and stayed overnight, tick one option only)*
✓ No plan to readmit within 28 days	Assessment completed during this episode
Not applicable	Assessment incomplete – other reason
Readmit to other hospital within 28 days-booked	Assessment incomplete – referral to subacute services
Readmit to other hospital within 28 days-no booking	Consultation only during this episode
Readmit to this hospital within 28 days-booking	No ACAS involvement during this episode
Readmit to this hospital within 28 days-no booking	Not specified *(psych, unqualified newborn, patient aged under 50 years or stayed sameday)*

CARER AVAILABILITY *(Palliative Care only - tick one option)*	REFERRAL FOR FURTHER CARE *(New referral arranged before discharge, tick up to 4 options)*
Carer not needed/not applicable *(Deceased, Nursing Home)*	Aboriginal and Torres Strait islander
Lives alone, has carer	Aged Care Assessment *(ACAS)*
Lives alone, has no carer	Alcohol and Drug Treatment service
Lives in a mutually dependent situation	Community Palliative Care support *(WMPC)*
Lives with another, has non-resident carer	Domiciliary Postnatal care *(mothers only)*
Lives with another, has resident carer	Domiciliary Postnatal care, referral declined
Lives with another, has no carer	General Practitioner
SOURCE OF REFERRAL TO PALLIATIVE CARE	Health Independence Program
Community Sector – Community based	Home Nursing support *(RDNS)*
Community Sector - GP	Mental Health Community service
Community Sector – Self/carer/other	✓ No Referral
Community Sector - Specialist	Other Clinical/Support services *(CRC) (PACFU) (PAC)*
Hospital – Outpatient – Non-admitted	Private psychiatrist
Hospital – Private – admitted patient	Psychiatric Disability support
Hospital – Public – admitted patient	Transition Care Home based
Not specified *(acute or psych care only)*	This form has been completed by:
Other	Print Name: Zeena Cadyl
Residential Care – Nursing Home	Signature:
FUNCTION ON ADMISSION	Designation: CORN
FIM / BARTHEL Score:	
FUNCTION ON DISCHARGE	***HIS CASEMIX USE ONLY***
FIM / BARTHEL Score:	DRG:
RUG ADL Score on Admission (Palliative Care only)	Coded by:
TOTAL	Coded without:
RUG ADL Score on Discharge (Palliative Care only)	Checked:
TOTAL	

FMH501400

BINDING MARGIN - NO WRITING

DISCHARGE NOTICE AD 0030

P564 V2 06/17

NO WRITING

Page 1 of 1

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5.7. IP Leave of absence plan IP MMH (FMH501800)

Mercy Health
Care first

Mercy Health

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Cuauhtemoc Sandoval
BAlba

DOB: 08/04/1973
M/C: N/A
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

INPATIENT LEAVE OF ABSENCE PLAN – INPATIENT MENTAL HEALTH

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH501800

Inpatient Leave of Absence has been authorised
from 24/02/21 **to** 04/03/21

This leave has been granted subject to the following conditions:

- You are to be accompanied by: HGL – accomp. by staff
- You are to return at the time agreed upon with ward staff.
- Conditions of Leave: Hospital Ground leave, accompanied by staff, as per nursing staff

A copy of the Leave of Absence form (MHA120) will be provided to you if you are a Compulsory Patient.

Senior Staff member approving Leave:

Signature: ..... Print Name: M. Abraham

Designation: Psych Reg Date: 24/02/21

Patient Statement

I ..... understand that I have been given leave from

..... ward with the conditions listed above.

I have discussed and understand my leave plan and what to do if I need help while I am away from the ward.

I have been given the 'Taking Leave' brochure and have discussed the content with a staff member.

I will sign in and out on the leave register on the back of this form every time I take leave within the period specified above.

I understand that if I do not return as planned that the ward staff will contact me on .....
(phone number, contact details). If the ward staff cannot contact me, I understand that they may contact my Nominated Person, Next of Kin and Victoria Police and I may be reported as a missing person.

Signed ..... Date: .....

Accompanying Adult Statement

I understand that it is a condition of the leave granted to the patient mentioned above and that he/she is accompanied by a responsible adult whilst on approved leave.

I have discussed my role with staff from the ward and I have received the Taking Leave brochure.

I am aware of the above plan if I require help while away from the ward and I am able to follow this if required.

I understand that we are required to return at a time specified on the leave register on the back of this form and that I may be contacted if we do not return as planned.

Staff member who discussed leave with client

Signature: ..... Print Name: .....

Designation: ..... Date: .....

Interpreter:

I have explained the above information to the Client in a language that could be understood.

Signature: ..... Print Name: .....

Date: .....

BINDING MARGIN – NO WRITING

Community Reviewed
Making information easier to read.

P2113 01/15 V2

INPATIENT LEAVE OF ABSENCE PLAN AD 0065

NO WRITING

Page 1 of 2

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5.8. IP Leave of absence plan IP MMH (FMH501800)

Mercy Health
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Mercy Health

INPATIENT LEAVE OF ABSENCE PLAN – INPATIENT MENTAL HEALTH

UR No:
2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C
Address:
(if no UR)
DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Inpatient Mental Health – Leave Register

Date	Discussed with ANUM (Print ANUM Name)	Staff Details – I have discussed leave with patient & accompanying adult (Print name & sign)	Accompanying adult – I understand the conditions of the leave (Print name and sign)	Accompanying adult – Phone number	Time Out	Planned time to return	Patient Initials	Actual time of return	Patient Initials

BINDING MARGIN - NO WRITING

Page 2 of 2

NO WRITING

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DOB: 08-Apr-1973 URNO: 2645287 (MHS)
McLean, Richard

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Mercy Health
Care first

Mercy Health

UR No:

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

BLOOD PRODUCT PRESCRIPTION

ROUTINE TRANSFUSIONS ARE NOT TO OCCUR OVERNIGHT

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH507300

BINDING MARGIN - NO WRITING

TRANSFUSION HISTORY

Has the patient previously received blood transfusion/s and/or blood product administration? ☐ Yes ☐ No

If yes, what blood products were administered? List: _____

Has the patient had a previous blood transfusion reaction? ☐ Yes ☐ No

If yes, comment: _____

CLINICAL INDICATION(S) FOR THE ADMINISTRAION BLOOD PRODUCTS

If the patient does not meet these criteria, consider carefully the need for the blood / blood products and the risk they carry to the patient.

RED CELLS
CODE Clinical Indication
R1: Critical Bleeding
R2: Severe anaemia (Hb <70)
R3: Anaemia (70-100g/L) with ongoing blood loss
R4: Anaemia (70-100g/L) with S/S impaired O2 transport
R5: Bone marrow failure parameters
→ 1 unit _____
→ 2 units _____
→ 3 units _____

PLATELETS
CODE Clinical Indication
P1: Critical Bleeding with thrombocytopenia
P2: Surgery (maintain platelet count <50x10^9/L)
P3: Prophylaxis in bone marrow failure without risk factors (platelet count <10x10^9/L)
P4: Prophylaxis in bone marrow failure with risk factors (platelet count <20x10^9/L)
P5: Platelet function disorder

FRESH FROZEN PLASMA (FFP)
CODE Clinical Indication
F1: Critical bleeding with INR >1.5
F2: Pre-operative coagulopathy with INR >1.5
F3: Liver disease in the presence of bleeding and abnormal coagulation
F4: Anticoagulation Reversal
F5: Acute DIC in the presence of bleeding and abnormal coagulation
F6: Coagulation Inhibitor Deficiencies

CRYOPRECIPITATE
CODE Clinical Indication
C1 Haemorrhage with fibrinogen deficiency <1.5g/L
C2 Obstetric haemorrhage with fibrinogen <2.0g/L
C3 Acute DIC
C4 Trauma
C5 Invasive procedure & fibrinogen deficiency

ALBUMIN
CODE Clinical Indication
A1 Critically ill/associated with shock
A2 Therapeutic plasmapheresis
A3 Paracentesis of ascites
A4 Other: _____

OTHER PRODUCTS / INDICATION
CODE Clinical Indication
01 As per IVIg criteria
02 Outside IVIg criteria:
Reason: _____
03 Prothrombinex
04 Other *(specify)*:
_____

PRE-TRANSFUSION CHECKLIST

1) Check **consent** has been completed prior to collection of blood product
2) Identify and confirm **patient's name** (including spelling) and **date of birth (DOB)** verbally
3) Identify and confirm **patient's name, DOB and UR number** on wristband, blood product, prescription form and transfusion laboratory issue form
4) Check **blood group** on Australian Red Cross Service label and patient label on the blood product and verify with transfusion laboratory issue form and the patient
5) Check **donation number** on both the Australian Red Cross Blood Service label and patient label on the blood product & verify against transfusion laboratory issue form
6) Check **expiry date** of product and bag inspected for leaks at ports or seams and evidence of discolouration

In signing for the blood product administered, each staff member is confirming they independently completed the correct checking process.

MONITORING REQUIRED - COMPLETE VITAL SIGNS

(Temp/Pulse/BP/Resp rate/02 Sat) must be recorded during:
- Prior to each unit of blood / blood product
- 15 minutes post commencement of each product
- Hourly for the duration of the transfusion
- At the completion of each unit of blood / blood product

Observe the patient closely for the first 15 minutes of transfusion for adverse events and observe the patient throughout the transfusion. Ask the patient to report any concerns to the nursing staff immediately.

P1902 05/20

Mercy Health
Care first

Mercy Health

UR No:

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
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Health Fund: UNKF

BLOOD PRODUCT PRESCRIPTION

ROUTINE TRANSFUSIONS ARE NOT TO OCCUR OVERNIGHT

DO NOT COMMENCE TRANSFUSION UNLESS CONSENT HAS BEEN COMPLETED

	DATE	BLOOD PRODUCT	INDICATION CODE	DOSE	RATE	MEDICAL OFFICER	TRANSFUSION REACTION? IF YES COMPLETE PAGE 4
1	25/2/21	PRBC	R3	one unit	1/24	M Blackwood.	
	DONATION NUMBER		START	FINISH	CHECKED BY:	ADMINISTERED BY:	
	3113061		26/2/21 0630	0740		Cafo	
2	26/2/21	PRBC	R3	one unit	1/24	HTN	
	DONATION NUMBER		START	FINISH	CHECKED BY:	ADMINISTERED BY:	
	3105320		26/2/21 1325	1430		Josi Wagn.	
3	26/2/21	PRBC	R3	ONE UNIT	1/24	SHRUTI	
	DONATION NUMBER		START	FINISH	CHECKED BY:	ADMINISTERED BY:	
	3109625		1845	~~1845~~ 1945	SH N.rum	Josi Wagn.	
4	DATE	BLOOD PRODUCT	INDICATION CODE	DOSE	RATE	MEDICAL OFFICER	TRANSFUSION REACTION? IF YES COMPLETE PAGE 4
	DONATION NUMBER		START	FINISH	CHECKED BY:	ADMINISTERED BY:	
5	DATE	BLOOD PRODUCT	INDICATION CODE	DOSE	RATE	MEDICAL OFFICER	TRANSFUSION REACTION? IF YES COMPLETE PAGE 4
	DONATION NUMBER		START	FINISH	CHECKED BY:	ADMINISTERED BY:	
6	DATE	BLOOD PRODUCT	INDICATION CODE	DOSE	RATE	MEDICAL OFFICER	TRANSFUSION REACTION? IF YES COMPLETE PAGE 4
	DONATION NUMBER		START	FINISH	CHECKED BY:	ADMINISTERED BY:	
7	DATE	BLOOD PRODUCT	INDICATION CODE	DOSE	RATE	MEDICAL OFFICER	TRANSFUSION REACTION? IF YES COMPLETE PAGE 4
	DONATION NUMBER		START	FINISH	CHECKED BY:	ADMINISTERED BY:	
8	DATE	BLOOD PRODUCT	INDICATION CODE	DOSE	RATE	MEDICAL OFFICER	TRANSFUSION REACTION? IF YES COMPLETE PAGE 4
	DONATION NUMBER		START	FINISH	CHECKED BY:	ADMINISTERED BY:	
9	DATE	BLOOD PRODUCT	INDICATION CODE	DOSE	RATE	MEDICAL OFFICER	TRANSFUSION REACTION? IF YES COMPLETE PAGE 4
	DONATION NUMBER		START	FINISH	CHECKED BY:	ADMINISTERED BY:	
10	DATE	BLOOD PRODUCT	INDICATION CODE	DOSE	RATE	MEDICAL OFFICER	TRANSFUSION REACTION? IF YES COMPLETE PAGE 4
	DONATION NUMBER		START	FINISH	CHECKED BY:	ADMINISTERED BY:	

Mercy Health
Care first

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

Mercy Health
Care first

Mercy Health

UR No:

Family Name:

Given Name:

DOB: Sex:

Address:

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C
DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

BLOOD PRODUCT PRESCRIPTION

ROUTINE TRANSFUSIONS ARE NOT TO OCCUR OVERNIGHT

PRESCRIPTION 1 OF 1

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

CONSENT TO THE ADMINISTRATION OF BLOOD AND BLOOD PRODUCTS

Prior to the transfusion or administration of blood and blood products, the risks and benefits must be explained to the patient or person legally responsible so that they can make an informed decision. Where indicated, a qualified interpreter must be used. Patients or their Medical Treatment Decision Maker (MTDM) should be offered written information.

BLOOD MANAGEMENT PLAN

Does this patient have a Blood Management Plan? ☐ Yes ☑ No

If yes, the current Blood Management Plan and treatment requirements need to be reviewed and adhered to.

DURATION OF CONSENT

Admitted Patient = valid for current admission only ☑

Transfusion Dependent Patient = valid for 12 months from the date of signing ☐

(Copy to be retained in current medical file)

PATIENT CONSENT AND CLINICIAN DECLARATION

Medical Officer / Clinician declaration and signature:

I have explained to the patient /person legally responsible for the patient:

- The purpose of giving the blood/blood product
- The type of blood/blood products to be administered
- The possible risks associated with administration
- The risk from non-administration
- Available alternatives

A qualified interpreter was present: ☐ Yes ☐ No ☑ Not required

The transfusion/administration **HAS BEEN AGREED** to by the: ☑ Patient ☐ MTDM ☐ Parent/Guardian (if Child)

Written information has been given: ☑ Yes ☐ No ☐ Declined by the patient

Signature: _____ **Print name:** 25/2/21 **Date:** / /

Patient/Guardian declaration and signature:

The purpose and type of blood/blood products to be administered, the possible risks associated with administration, the risk from non-administration and available alternatives have been explained to me. I was given the opportunity to ask questions and my questions have been satisfactorily answered. I CONSENT to the administration of blood/blood products as may be found to be necessary during the course of my medical treatment this admission.

Signature: _____ **Print name:** _____ **Date:** / /

PATIENT INCAPABLE / UNABLE TO CONSENT TO TRANSFUSION

Patient unable to consent Medical Officer Declaration and signature:

I have not been able to explain the risks and benefits of administration as blood/blood products were administered urgently/as life-saving treatment AND the patient's conscious state was impaired because of:

☐ Illness or Injury ☐ Sedation ☐ Anaesthesia ☐ Other (specify):

Signature: _____ **Print name:** _____ **Date:** / /

The patient/legal representative for the patient was notified of the transfusion on: **Date:** / /

TRANSFUSION DEPENDENT PATIENT (Medical Day Stay Use)

The treating nurse must sight and acknowledge the original consent and have this confirmed by a second staff member before the treatment commences.

Nurse 1 Signature: _____ **Nurse 2 Signature:** _____ **Date:** / /

If required for a condition other than for the patient's regular transfusion, then a new consent must be obtained.

NO WRITING

BLOOD PRODUCT PRESCRIPTION AD 0870

Mercy Health
Care first

Mercy Health

UR No:

Family Name:

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C
DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

TRANSFUSION REACTION REPORT

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

BLOOD TRANSFUSION REACTION MANAGEMENT AND RE

1) RECOGNIZE:

Transfusion reactions range from mild allergies and febrile reactions to life threatening acute haemolysis as a results of ABO incompatibility.

Signs & Symptoms: (please tick)

Minor	Major	Presumed Reaction Type
☐ Fever (>1C over baseline or >38C)	☐ Tachycardia	☐ Febrile non-haemolytic transfusion reaction
☐ Chills	☐ Hypotension	☐ Allergy / Anaphylaxis
☐ Urticaria	☐ Dyspnoea	☐ Bacterial Sepsis
☐ Nausea / Vomiting	☐ Chest Pain / Lumbar Pain	☐ Acute haemolytic transfusion reaction
☐ Other: _____	☐ IV Site Pain / Bleeding	☐ Transfusion Related Acute Lung Injury (TRALI)
	☐ Haemoglobinuria	☐ Fluid Overload: _____
	☐ Other: _____	

2) RESPOND:

In the event of a transfusion reaction call for assistance to help you with the following:

1. STOP the transfusion
2. STABILISE the patient and check their vital signs
3. NOTIFY medical officer. MET Call if applicable
4. INSERT new IV cannula if able
5. ADMINISTER medication to treat the reaction
6. REMAIN with the patient until the reaction resolves
7. DOCUMENT the transfusion reaction
8. CHECK that it is the right blood for the right patient
9. COMPLETE the laboratory's reaction report
10. CONTACT the laboratory for further instructions

Concurrent Therapy (Simultaneous infusion of other fluids:)

If YES specify: _____

Bedside Clerical Check

Correct Patient	☐ Yes	☐ No	Correct Rate	☐ Yes	☐ No
Correct Blood Transfused	☐ Yes	☐ No	Allergies	☐ Yes	☐ No

3) REPORT

Complete this Transfusion Reaction Report for all actual and suspected transfusion reactions. Contact the blood bank for advice.

Date of Transfusion: ___/___/___	**Time transfusion commenced:**	**Time transfusion ceased:**
	Temperature:	Temperature:
	Pulse Rate:	Pulse Rate:
	Respiratory Rate:	Respiratory Rate:
	Blood Pressure:	Blood Pressure:

Indication for Transfusion / Diagnosis:

Blood Product + Volume infused:	Donation Number:

4) REQUIREMENTS FOR BLOOD BANK INVESTIGATION

Please contact Blood Bank to confirm the collection of the following samples for the investigation of the transfusion reaction.
Please specify "suspected transfusion reaction" on the the request form: (Send a completed copy of this form to Bloodbank)

A) 1 x 8.5ml SST (yellow tube)
B) 1 x 4ml EDTA (purple tube)
C) 2 x 6 ml EDTA (pink tube)
D) Transfused packs and unit being administered
E) Blood Cultures
F) Urine Sample when able - if delayed send as Suspected Transfusion Reaction

DO NOT RETURN ANY EXPOSED SHARPS WITH THE UNITS BACK TO BLOOD BANK

5) REVIEW

Clinical Outcome: Recovered? ☐ Yes ☐ No **Report sent to Bloodbank:** ☐ Yes ☐ No

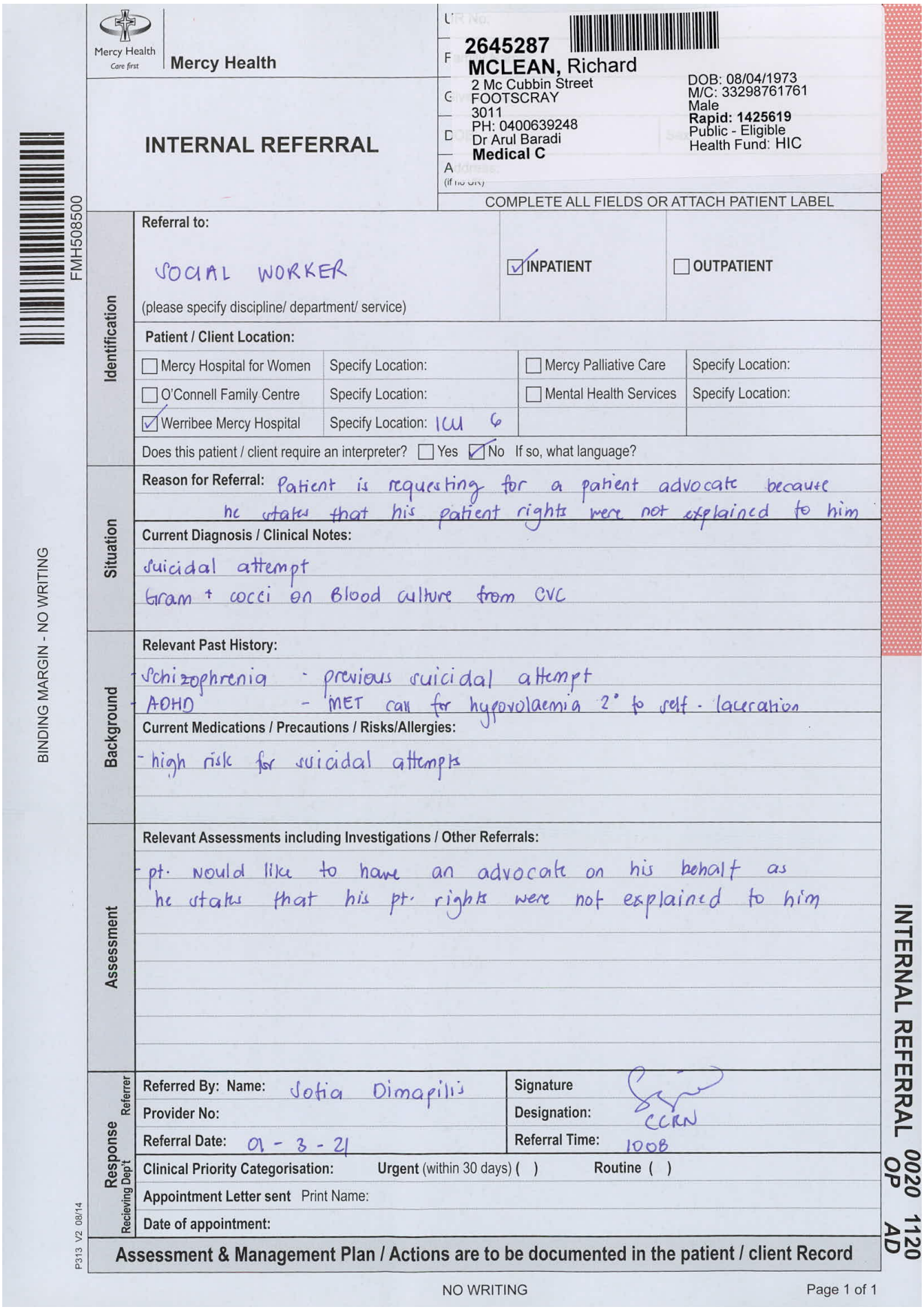
Medical Review By: Name: Dr _____ Contact No _____

Reported By: Name _____ Signature _____ Date: _____

BINDING MARGIN - NO WRITING

NO WRITING

5.11. Internal Referral (FMH508500)



Mercy Health
Care first
Mercy Health

INTERNAL REFERRAL

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH508500

BINDING MARGIN - NO WRITING

Identification

Referral to: SOCIAL WORKER (please specify discipline/ department/ service)

☑ INPATIENT ☐ OUTPATIENT

Patient / Client Location:

☐ Mercy Hospital for Women	Specify Location:	☐ Mercy Palliative Care	Specify Location:
☐ O'Connell Family Centre	Specify Location:	☐ Mental Health Services	Specify Location:
☑ Werribee Mercy Hospital	Specify Location: ICU 6		

Does this patient / client require an interpreter? ☐ Yes ☑ No If so, what language?

Situation

Reason for Referral: Patient is requesting for a patient advocate because he states that his patient rights were not explained to him

Current Diagnosis / Clinical Notes:
suicidal attempt
Gram + cocci on Blood culture from CVC

Background

Relevant Past History:
- Schizophrenia - previous suicidal attempt
- ADHD - MET call for hypovolaemia 2° to self-laceration

Current Medications / Precautions / Risks/Allergies:
- high risk for suicidal attempts

Assessment

Relevant Assessments including Investigations / Other Referrals:
- pt. would like to have an advocate on his behalf as he states that his pt. rights were not explained to him

Response

Referrer
Referred By: Name: Sofia Dimapilis — Signature — Designation: CCRN
Provider No: — Referral Date: 01-3-21 — Referral Time: 1008

Recieving Dep't
Clinical Priority Categorisation: Urgent (within 30 days) ( ) Routine ( )
Appointment Letter sent Print Name:
Date of appointment:

Assessment & Management Plan / Actions are to be documented in the patient / client Record

INTERNAL REFERRAL 0020 OP 1120 AD

P313 V2 08/14

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Page 1 of 1

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5.12. Periop Clinical Handover Checklist (FMH513400)

Mercy Health
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Mercy Health

UR No: **2645287**

MCLEAN, Richard
2 Mc Cubbin Street
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3011
PH: 0400639248
Dr Arul Baradi
Medical C

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PERIOPERATIVE CLINICAL HANDOVER CHECKLIST

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH513400

BINDING MARGIN - NO WRITING

Each item to be completed and discussed theatre check in with Ward Nurse / Midwife and theatre / PACU Nurse

	WARD TO THEATRE	COMMENTS
I	Does the patient have 2 identification bands on	☐ Yes ☐ No
	Does the patient have a known allergy / sensitivity *medication chart?)* — bee stings augmentin	☑ Yes ☐ No ____ *(if Yes is it documented on*
	Interpreter required ☐ Yes ☑ N/A Language spoken:_____	Interpreter present ☐ Yes ☐ No
S	Is the operation consent form signed	☐ Yes ☐ No
	Is the anaesthetic consent form signed	☐ Yes ☐ No
	Procedure side & site confirmed with patient	☐ Yes ☐ No
	Procedure site marked *(if applicable)*	☐ Yes ☐ No
	Is the patient fasted? Time fasted from 12 midnight	☑ Yes ☐ No
B	Is the patient a Diabetic: ☐ Yes ☑ No type 1 / type 2 / GDM BSL recorded 6.2 @ 0520	mmol/L
	Date of LNMP ____/____/____ ☐ N/A Urine BHCG ☐ +ve ☐ -neg	
	Patient's height/weight recorded ☐ Yes ☐ No BMI calculated ☐ Yes ☐ No Vital signs recorded ☐ Yes ☐ No	
	Discuss relevant past history/co morbidities	☑ Yes ☐ No
A	Has the patient had bowel preparation / enema *(circle & document result)*	☐ Yes ☑ No ☐ N/A
	Has a premedication been given	☐ Yes ☑ No ☐ N/A
	VTE prophylaxis (TEDS / SCD / Anticoagulation therapy)	☑ Yes ☐ No ☐ N/A
	Blood group / G & H / X-match	☑ Yes ☐ No ☐ N/A
	Fetal heart rate - fetal scalp electrode on-/-off	☐ Yes ☐ No ☑ N/A
	Are there any packs insitu	☐ Yes ☑ No
	Dentures / bridgework / caps / crowns / loose teeth	☐ Yes ☐ No
	Prosthesis insitu	☐ Yes ☐ No ☐ N/A
	- Type / location	
R	X-rays with patient	☐ Yes ☐ No ☐ N/A
	Make-up / nail varnish / acrylic nails / hairpins removed	☐ Yes ☐ No ☐ N/A
	Jewellery removed / taped	☐ Yes ☐ No ☐ N/A
	Luggage: locker / sent to ward / in ward / PACU / with patient/family	☐ Yes ☐ No
	Fluid Balance Chart / Medication Chart / Health questionnaire in patient file	☐ Yes ☐ No
	Other special needs or requirements (e.g glasses / hearing aid / restricted movement)	☐ Yes ☐ No
	Patient invited to ask questions	☐ Yes ☐ No

	Name	Signature	Designation	Date
Ward				/ /
Theatre				/ /

PERIOPERATIVE CLINICAL HANDOVER CHECKLIST AD 1860

P1923 V2 05/17

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Page 1 of 2

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5.13. Periop Clinical Handover Checklist (FMH513400)

Mercy Health
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Mercy Health

UR No:

Family Name:

Given Name

DOB: Sex:

Address:
(if no UR)

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

PERIOPERATIVE CLINICAL HANDOVER CHECKLIST

Each item to be completed and discussed at the bedside with Ward Nurse/ Midwife and theatre / PACU Nurse

	PACU TO WARD/DAY SURGERY UNIT	COMMENTS
I	Does the patient have 1 identification band on ☐ Yes ☐ No	
	Does the patient have a known allergy / sensitivity ☐ Yes ☐ No	
S	**Discuss**	
	o Surgical procedure ☐ Yes ☐ No	
	o Any Intraoperative complications / events ☐ Yes ☐ No	
	o Type of Anaesthetic & medications administered intraoperatively / PACU ☐ Yes ☐ No	
B	Discuss relevant past history/co morbidities ☐ Yes ☐ No	
A	**Discuss**	
	o PACU vital signs and alerts noted ☐ Yes ☐ No	
	o Observations documented on ward chart (ie MORC/ORC/VICTOR) ☐ Yes ☐ No	
	o Any altered MET/Clinical review criteria ☐ Yes ☐ No	
	o Pain / analgesia status / PONV ☐ Yes ☐ No	
	o PACU discharge criteria ☐ Yes ☐ No	
	o Urinary status / IDC insitu ☐ Yes ☐ No	
	o Wound / dressings / drain tube / packs insitu ☐ Yes ☐ No *(circle relevant)*	
	o Is the patient a Diabetic: type 1 / type 2 / GDM BSL recorded ☐ Yes ☐ No *(circle relevant)*	
R	Operation report, post operative orders reviewed ☐ Yes ☐ No	
	Anaesthetic orders reviewed ☐ Yes ☐ No	
	Checked Fluid Balance Chart / IV order status ☐ Yes ☐ No	
	IV site checked and dated ☐ Yes ☐ No	
	Discuss medications ordered	
	☐ VTE prophylaxis	
	☐ Analgesia ☐ Yes ☐ No	
	☐ Anti-emetics	
	☐ Regular medications	
	Ensure patient belongings are with patient ☐ Yes ☐ No	
	Other special needs or requirements (e.g glasses / hearing aids) ☐ Yes ☐ No	
	Patient invited to ask questions ☐ Yes ☐ No	

	Name	Signature	Designation	Date
PACU				/ /
Ward				/ /

BINDING MARGIN - NO WRITING

Page 2 of 2

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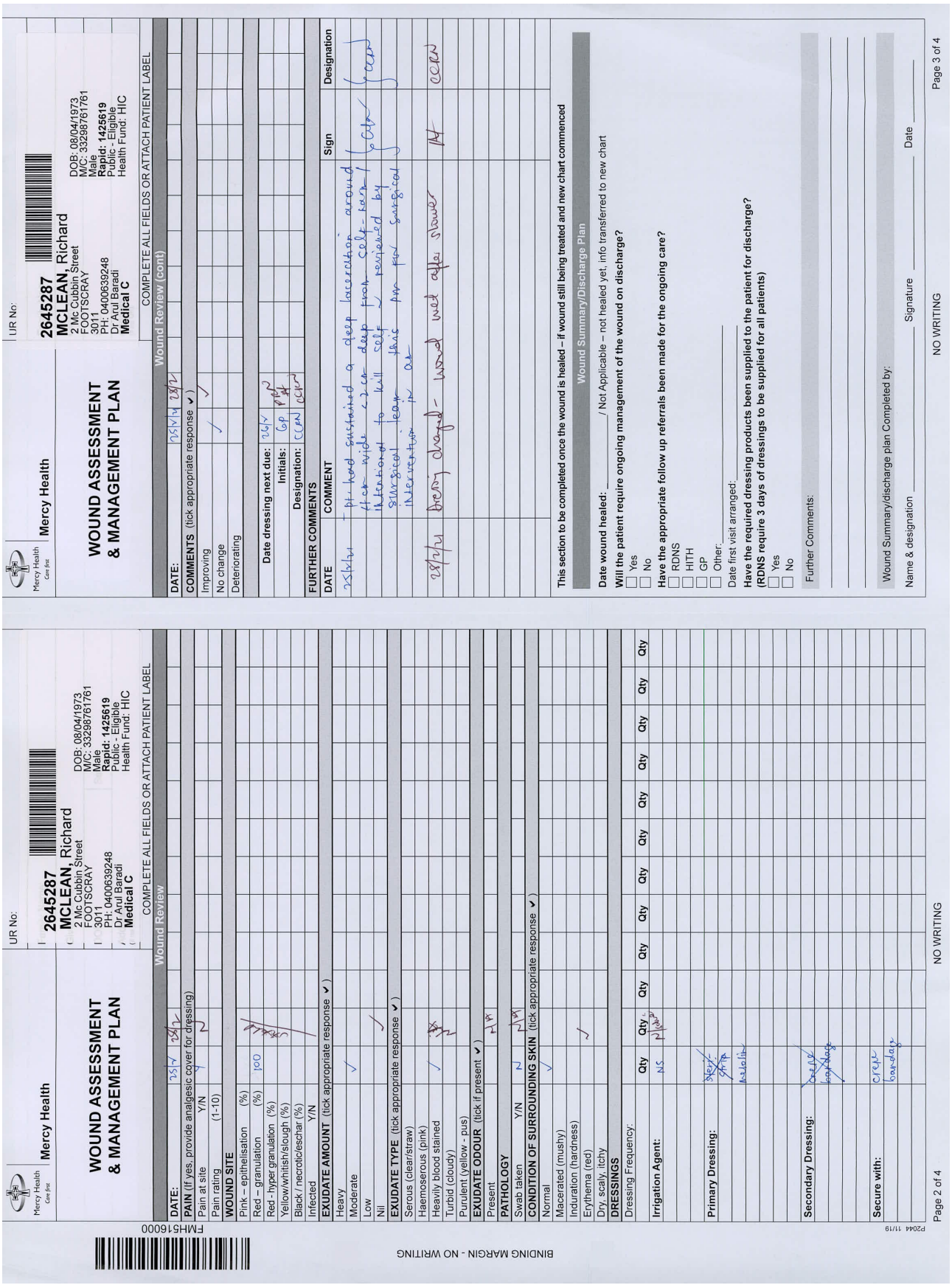
5.14. Wound Ass & Management Plan (FMH516000)



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Mercy Health
Care first

5.15. Wound Ass & Management Plan (FMH516000)

Mercy Health
Care first

URNO:

F 2645287
MCLEAN, Richard
2 Mc Cubbin Street
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PH: 0400639248
Dr Arul Baradi
Medical C

G
D
A

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

WOUND ASSESSMENT & MANAGEMENT PLAN

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Location: L cubital fossa

Draw below the location and extent of the wound.

RIGHT FOOT

LEFT FOOT

Type:
☐ Pressure Ulcer ☐ Skin Cancer ☐ Skin tear ☐ Burn
☐ Arterial Ulcer ☐ Skin Graft ☐ Surgical wound ☑ Laceration
☐ Venous Ulcer ☐ Abrasion ☐ Puncture ☐ Other

Factors that may reduce healing for this resident

Intrinsic (Patient)
☐ Poor Mobility ☐ Smoker ☐ Diabetes
☐ Incontinence ☐ Obesity ☐ Poor Nutrition

Extrinsic (Environment)
☐ Exudate ☐ Slough ☐ Necrotic tissue
☐ Infection ☐ Pressure ☐ Shearing

Shape and Size
Draw the shape and state length and width in cm.

Assessment

Skin tear severity:
☐ Category 1a
☐ Category 1b
☐ Category 2a
☐ Category 2b
☐ Category 3

Wound Stage / Pressure Ulcer Severity:
☐ Stage 1 – Non blanchable erythema
☐ Stage 2 – Partial thickness skin loss
☐ Stage 3 – Full thickness skin loss (subcutaneous fat visible but no underlying bone, tendons or muscle visible)
☐ Stage 4 – Full thickness tissue loss (underlying bone, tendon or muscle visible)
☐ Unstageable – depth unknown
☐ Suspected deep tissue injury –depth unknown

Date_____
Length: ____cm Width: ____cm Depth: ____cm

If larger area is required, please attach grid to chart

NO WRITING

Page 1 of 4

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Health Fund: HIC

WOUND ASSESSMENT & MANAGEMENT PLAN

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

WOUND ASSESSMENT & MANAGEMENT PLAN ***AD 2170***

Wound Photographs

Date:	Date:	Date:
Date:	Date:	Date:
Date:	Date:	Date:

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Page 4 of 4

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5.16. Progress Report (FMH516500)

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Health Fund: HIC

Patient Progress

Date & Time: 25/02/2021 12:22:45 PM

DRAFT - last modified: 25/02/2021 12:23:00 PM

3011 Footscray
VIC
Australia

Home Phone : 0400639248 **Work Phone :**

Address 2 Mc Cubbin Street
3011 Footscray
VIC
Australia

Home Phone (AH) 0400639248 **Work Phone (BH)**

Statewide UR: 1425619
Date of Entry: 25/02/2021
Time of Entry: 12:12
Clinician: Mustafa Abuqlam
Designation: Registrar
Team: Clare Moore Building Inpatient Team

25/02/21 1145

Psych CWR – Akinbiyi, Abuqlam

Worried about his dog not being fed at home.

Informed him that we can't discharge him home at this point as we believe he is mentally unwell and is not safe to be discharged.

Complaining about medications not being given on time.

States that he just wants to be at home with his dog.

Expresses paranoia that he is not being given his dexamphetamine because "it's coming from the top" and Dr Hogan (Private Psychiatrist).

States that the treating psychiatrist is keeping Richard in hospital to protect the GP that Richard has issues with.

Richard asked if he can provide evidence of the conspiracy theory.

MSE:
Irritated affect

Psychomotor agitation

Title: Psychiatrist Review **Page No.** 1
Created By: Mustafa Abuqlam (Registrar)
Printed By: Bryan Dimabuyo (Nurse)

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5.17. Progress Report (FMH516500)

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MCLEAN, Richard
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Health Fund: HIC

Patient Progress

Date & Time: 25/02/2021 12:22:45 PM

DRAFT - last modified: 25/02/2021 12:23:00 PM

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Home Phone : 0400639248 **Work Phone :**

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3011 Footscray
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Australia

Home Phone (AH) 0400639248 **Work Phone (BH)**

Paranoid ideation

Plan:

1. Mobile phone use ONLY supervised. Allow supervised access to phone to email the letter from Dr Hogan (Private Psychiatrist) to reception, and also to organise getting a friend/family to feed his dog.
2. Collateral history from private psychiatrist Dr Hogan --> Add-On 1215 Contacted the clinic that Dr Hogan works at, they informed me that Richard has not been a patient of Dr Hogan's ever, and last saw a psychiatrist there Dr Anna Mogilevski many years ago who has since retired from clinical practice.
3. Ceasing dexamphetamine due to no evidence currently of him being prescribed it and no clear indication
4. If the dog can't be fed by his friends/family, then call the police/RSPCA by 3pm to rescue the dog
5. Refer to case mx – contact nurse to do please
6. EDD 11/03/21

AbuqlaM

Title: Psychiatrist Review **Page No.** 2
Created By: Mustafa Abuqlam (Registrar)
Printed By: Bryan Dimabuyo (Nurse)

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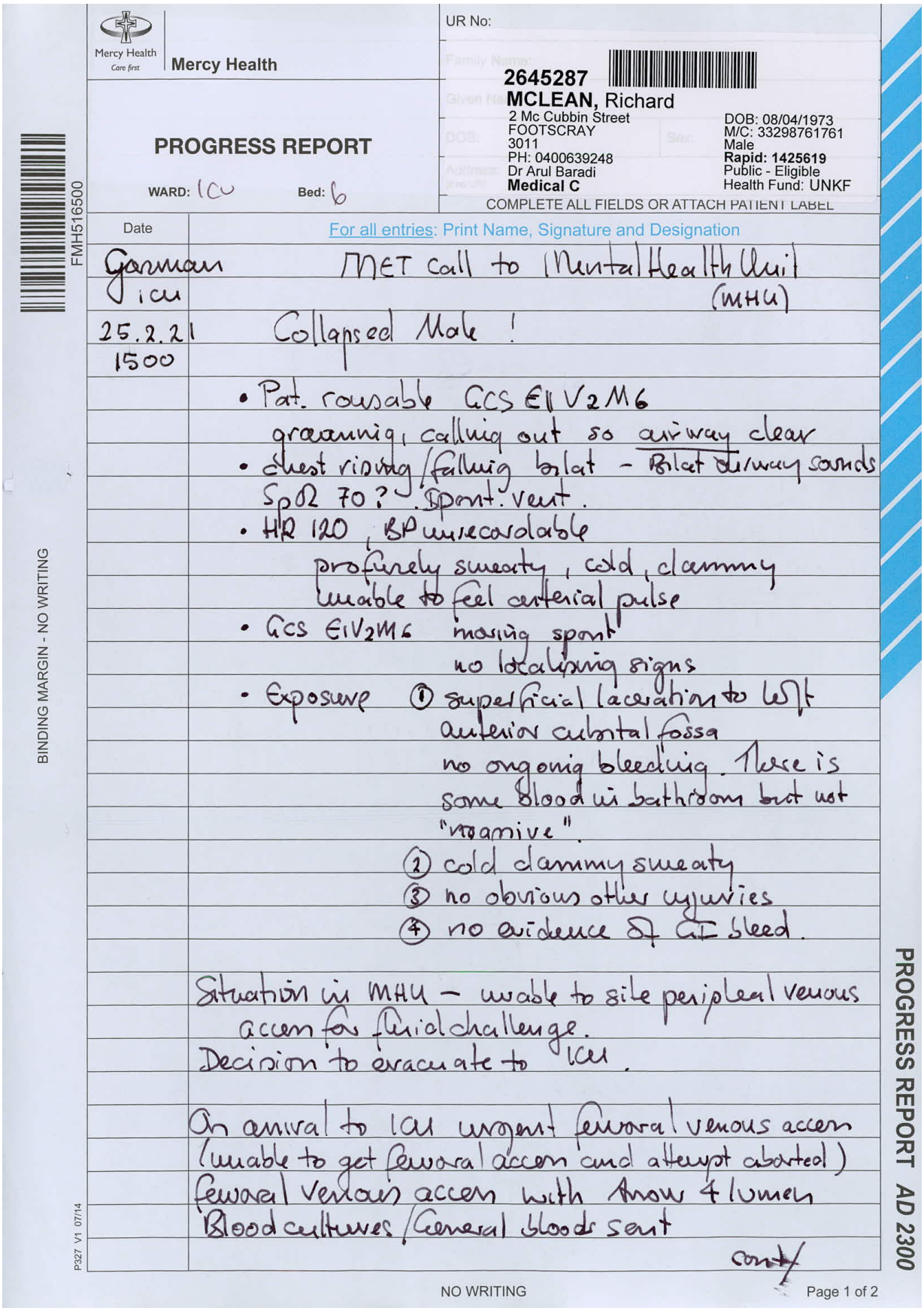
5.18. Progress Report (FMH516500)



Mercy Health
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Mercy Health

UR No:

2645287
MCLEAN, Richard
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Health Fund: UNKF

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

PROGRESS REPORT

WARD: ICU Bed: 6

FMH516500

BINDING MARGIN - NO WRITING

Date

For all entries: Print Name, Signature and Designation

Garman ICU

MET call to Mental Health Unit (MHU)

25.2.21 1500

Collapsed Male !

- Pat. rousable GCS E4 V2 M6 groaning, calling out so airway clear
- chest rising/falling bilat - Bilat airway sounds SpO2 70? Spont. vent.
- HR 120, BP unrecordable profusely sweaty, cold, clammy unable to feel arterial pulse
- GCS E4V2M6 moving spont no localising signs
- Exposure ① superficial laceration to left anterior cubital fossa no ongoing bleeding. There is some blood in bathroom but not "massive".
  ② cold clammy sweaty
  ③ no obvious other injuries
  ④ no evidence of GI bleed.

Situation in MHU – unable to site peripheral venous accen for fluid challenge.
Decision to evacuate to ICU.

On arrival to ICU urgent femoral venous accen (unable to get femoral accen and attempt aborted) femoral venous accen with 4row 4lumen
Blood cultures/General bloods sent

cont/

P327 V1 07/14

NO WRITING

PROGRESS REPORT AD 2300

Page 1 of 2

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5.19. Progress Report (FMH516500)

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UR No:

2645287
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M/C: 33298761761
Male
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Public - Eligible
Health Fund: UNKF

PROGRESS REPORT

WARD: ICU Bed: 6

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Date	For all entries: Print Name, Signature and Designation

German
ICU

Cont.

25.2.21 Venous blood gas shows pH 7.1
Glu 23
Lact 13
HCO3 4
Na 134
K 4.4
AG 21
Hb 129
Cl 101
Δ gap −6

Interestingly no hx of DM
no abdo pain
abdo soft, BS present
no guarding.
all limb equally perfusing

Plan: (I) fluid loading
(II) ackapid infusion
(III) noradrenalin IVI

• Blood cultures sent

↓

Pat. has improved slowly
invasive arterial pressure to 100/(40)(70) on 10 mcg/min of norad.
GCS 15/15 – conversant

↓

Plan: repeat ABG
Dr Atherale has taken over
− ABx
− ADT
− CXR
− BC
− Urin MCS

LKG

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Page 2 of 2

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5.20. Progress Report (FMH516500)

2645287
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Rapid: 1425619
Public - Eligible
Health Fund: HIC

Patient Progress

Date & Time: 25/02/2021 4:14:34 PM
Completed: 25/02/2021 4:21:00 PM

2 Mc Cubbin Street
3011 Footscray
VIC
Australia

Home Phone : 0400639248 **Work Phone :**

Address 2 Mc Cubbin Street
3011 Footscray
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Australia

Home Phone (AH) 0400639248 **Work Phone (BH)**

Statewide UR: 1425619
Date of Entry: 25/02/2021
Time of Entry: 16:06
Clinician: Bryan Dimabuyo
Designation: RN Psych Div1
Team: Clare Moore Building Inpatient Team

Entry:

Case note entry of met call

Staff noticed nurse call bell for room 113. Staff called out to ask if patient was okay, no response. Staff asked for male colleague to check on client.
Staff noticed client laying on the floor, with a wound near L) radial artery, profusely squirting blood. Code grey activated, colleagues responded with a met call subsequently being called. Client was able to verbalise his name and his current whereabouts. Became abusive towards staff. Met-call team attended and client was transferred to level 4 Medical ward ICU.

Identified broken pieces of glasses on floor from a vape/e-cigarette, possibly could have been used in self harm/suicide attempt. Also found a suicide note, not legible.
Attempted to contact NOK, nil answer at this stage.

All belongings packed and stored in blue cupboard next to Nursing station pod 3

ROUTINE RISK ASSESSMENT					
Mark with an "X" if applicable					
Risk Categories	**PHx**	**L**	**M**	**H**	**Rationale / Comments (mandatory)**
Suicidality / Self Harm (Inpatient staff to consider removal of ligature risks if M or H and document in clinical file)	x			x	Attempted suicide 6 years ago. Client attempted suicide this shift with suicide note left.

Title: Nursing – PM Shift Entry
Created By: Bryan Dimabuyo (Nurse)
Printed By: Bryan Dimabuyo (Nurse)

Page No. 1

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5.21. Progress Report (FMH516500)

2645287
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Patient Progress

Date & Time: 25/02/2021 4:14:34 PM
Completed: 25/02/2021 4:21:00 PM

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Risk of self -neglect /accidental self-harm (homeless, personal hygiene, food/fluid intake)			x		Diet and fluids well tolerated Moderate risk of losing accommodation.
Violence/ harm to others		x			**verbally abusive towards staff**
Risk of violence, harm or neglect to infant/child (consider CPS notification if M or H)		x			nil child
Risk of Vulnerability/Harm from others (physical, sexual, emotional or financial)				x	currently vulnerable due to distress, potentially giving personal belongings away
Risk of absconding/ non - adherence with treatment				x	high risk of absconding due to suicidal thoughts and plans.
Risk of Impulsivity/ level of distress	x			x	distress ++ suicide attempt
Current substance use, intoxication or withdrawal - Yes or No			x		reports recreational drug use ?drug use this shift
Physical health issues or adverse effects from treatment		x			nil reported
Other (falls risk, use of restrictive interventions, PRN medication)				x	client t/f to ICU medical ward, remains on TTO due to intense suicidal thoughts.

Delete inappropriate answer (Yes or No)

Factors to consider		Rationale / Comments (mandatory)
Is the patient's level of risk highly changeable?	Yes	Guarded,
Is the reliability of this risk assessment low? (Poor engagement, conflicting info, etc.)	Yes	Superficially engaging
Are tipping points present? (personal stress, recent death, end of relationship)	Yes	loss of relationship with psychiatrist
Are there significant protective/ resilience factors present?	No	

If high changeability = Yes or low reliability = Yes, then the overall risk rating should increase by one level (i.e. medium to high)

Title: Nursing – PM Shift Entry **Page No.** 2
Created By: Bryan Dimabuyo (Nurse)
Printed By: Bryan Dimabuyo (Nurse)

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5.22. Progress Report (FMH516500)

Mercy Health
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Mercy Health

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Health Fund: UNKF

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH516500

BINDING MARGIN - NO WRITING

PROGRESS REPORT

WARD: IGU Bed: 6

Date	For all entries: Print Name, Signature and Designation
	ICU ADMISSION SUMMARY
Date 25/02/21	**UR: 2645287 NAME: Richard MCLEAN DOB: 08/04/1973**
18:42	**Admitting Consultant:** Dr Athavale **Admitting Registrar:** Dr Nanji
	Home Team / Admitting Consultant: General Medicine **Reason for Admission:** Collapse and shock
	History of Presenting Complaint: - 47M MET call in mental health unit - Patient found collapsed on ground in shower, evidence of self harm to left cubital fossa (sustained by using vape machiene) → 'squirting' blood. suicide attempt, note present
	At MET call: A – Patent, own B – Spontaneous ventilation, SPO2 ?70% - Requiring high flow via non-rebreather on arrival C – HR 120, BP unrecordable, cold, clammy, diaphoretic, unable to palpate arterial pulses D - GCS E1V2M6 on arrival – rousable, groaning, calling out E – Laceration to left anterior cubital fossa, no ongoing bleeding, superficial. No other obvious injuries.
	Unable to site peripheral venous access for fluid, transferred to ICU
	Progress in ICU: - Femoral CVC inserted - 3L IVF stat - Commenced noradrenaline, vasopressin - R) radial art line inserted, IDC - GCS improved over time to 15/15
	B/G of being in the mental health unit for 2 days, on assessment order, due to deteriorating mental health, increasing suspiciousness and delusions.
	Past Medical History: As per psych notes Schizophrenia ADHD Previous suicide attempts Opportunistic methamphetamine and amphetamine misuse
	Nil Hx diabetes
	Medications: Did not disclose full list On transfer from psych:
	ALLERGIES: Bees Augmentin - Vomiting
	Social / Functional: Isolated Has PhD in art therapy Pet dog
	Resuscitation Status: Full Resuscitation – GOC A

Page 1 of 3

Werribee Mercy Hospital
300-310 Princes Highway, Werribee Victoria 3030 Telephone: (03) 9216 8888 Facsimile: (03) 9216 8777
mercyhealth.com.au ABN 74 762 230 429

P327 V1 07/14

Compassion Hospitality Respect Innovation Stewardship Teamwork

NO WRITING

Page 1 of 2

PROGRESS REPORT AD 2300

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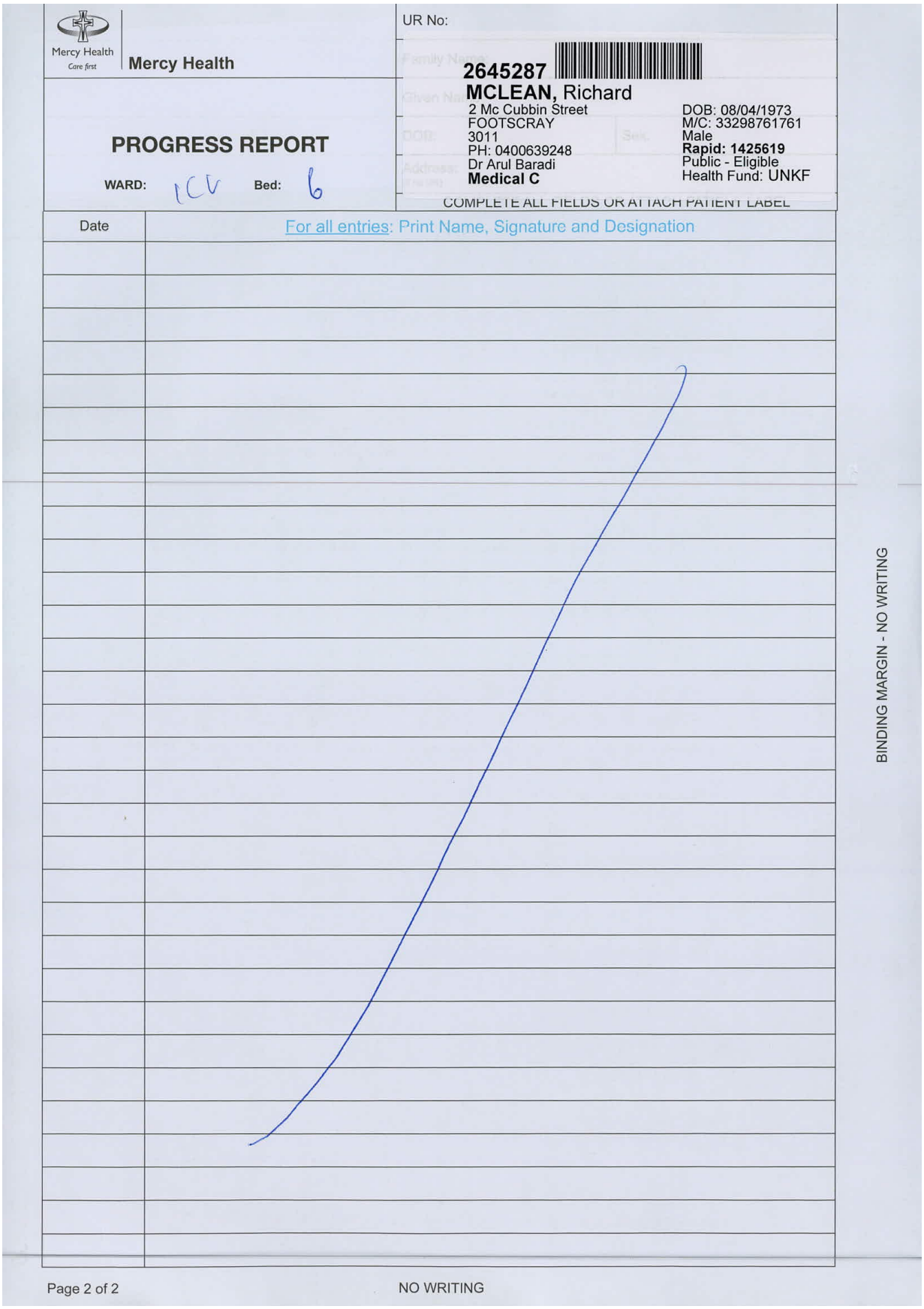
5.23. Progress Report (FMH516500)



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Public - Eligible
Health Fund: UNKF

PROGRESS REPORT

WARD: ICV Bed: 6

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Date	For all entries: Print Name, Signature and Designation

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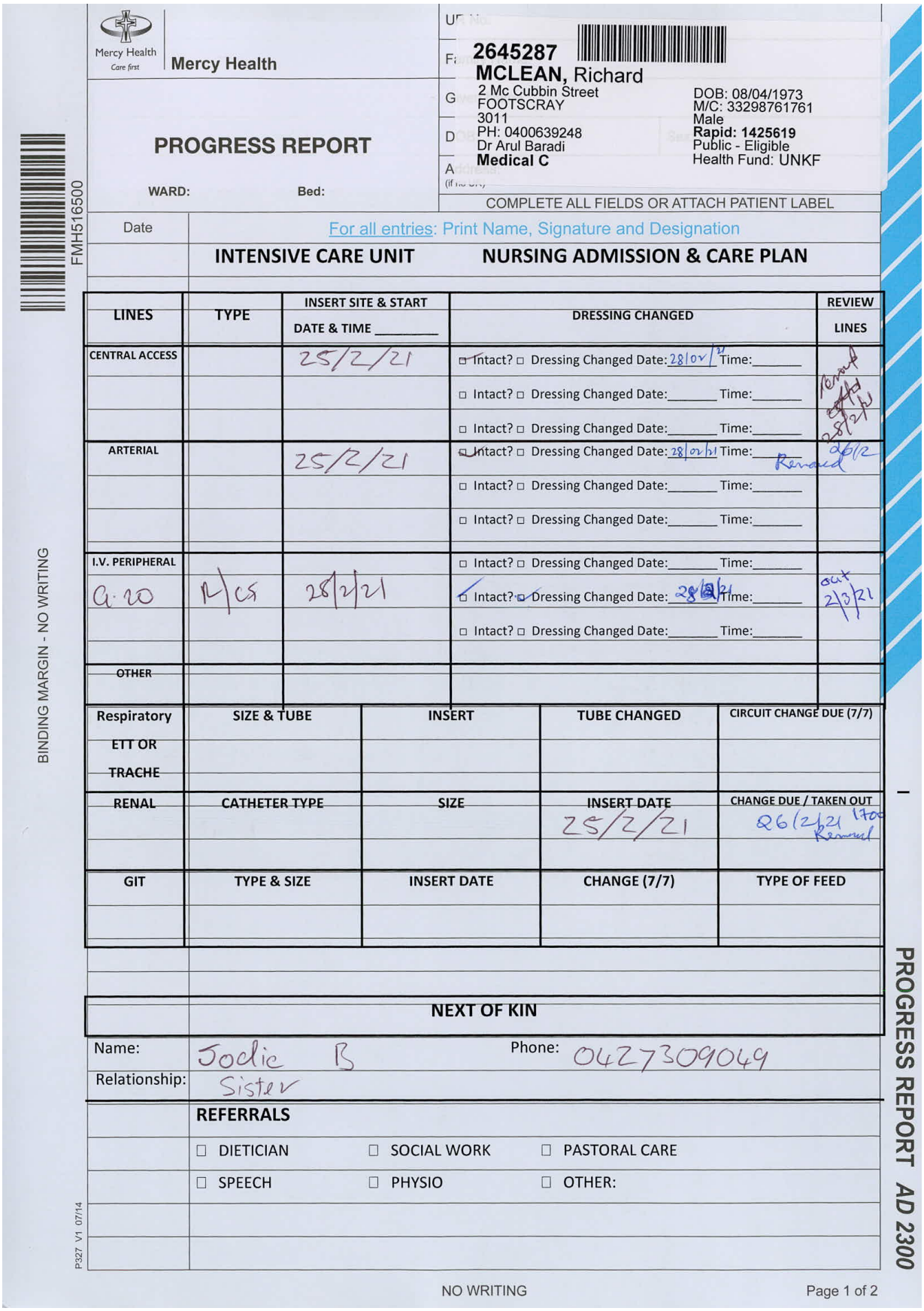
5.24. Progress Report (FMH516500)



Mercy Health
Care first

Mercy Health

UR
F: 2645287
MCLEAN, Richard
G: 2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C
DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

PROGRESS REPORT

WARD: Bed:

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

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Date | For all entries: Print Name, Signature and Designation

INTENSIVE CARE UNIT NURSING ADMISSION & CARE PLAN

LINES	TYPE	INSERT SITE & START DATE & TIME	DRESSING CHANGED	REVIEW LINES
CENTRAL ACCESS		25/2/21	☐ Intact? ☐ Dressing Changed Date: 28/02/21 Time:_____	Removed 28/2/21
			☐ Intact? ☐ Dressing Changed Date:_____ Time:_____	
			☐ Intact? ☐ Dressing Changed Date:_____ Time:_____	
ARTERIAL		25/2/21	☐ Intact? ☐ Dressing Changed Date: 28/02/21 Time: Removed	26/2
			☐ Intact? ☐ Dressing Changed Date:_____ Time:_____	
			☐ Intact? ☐ Dressing Changed Date:_____ Time:_____	
I.V. PERIPHERAL			☐ Intact? ☐ Dressing Changed Date:_____ Time:_____	
G 20	R/CS	28/2/21	☑ Intact? ☐ Dressing Changed Date: 28/2/21 Time:_____	out 2/3/21
			☐ Intact? ☐ Dressing Changed Date:_____ Time:_____	
~~OTHER~~				

Respiratory ~~ETT OR TRACHE~~	SIZE & TUBE	INSERT	TUBE CHANGED	CIRCUIT CHANGE DUE (7/7)

RENAL	CATHETER TYPE	SIZE	INSERT DATE	CHANGE DUE / TAKEN OUT
			25/2/21	26/2/21 1700 Removed

GIT	TYPE & SIZE	INSERT DATE	CHANGE (7/7)	TYPE OF FEED

NEXT OF KIN

Name: Jodie B Phone: 0427309049

Relationship: Sister

REFERRALS

☐ DIETICIAN ☐ SOCIAL WORK ☐ PASTORAL CARE

☐ SPEECH ☐ PHYSIO ☐ OTHER:

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5.25. Progress Report (FMH516500)

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PROGRESS REPORT

WARD: Bed:

UR No:

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INTENSIVE CARE UNIT NURSING ADMISSION & CARE PLAN

ALLERGIES: **RESUSCITATION STATUS:** Full.

Bee stings
Augmentin

RELEVANT PAST HISTORY: schizophrenia
ADHD
previous suicide attempts
opportunistic methamphetamine and amphetamine misuse

REASON FOR ADMISSION/CURRENT HISTORY: - collapse and shock
* met call in mental health unit, found collapsed on ground in shower, evidence of self-harm in (L) OF (sustained using vape machine)

☑	☑ GLASSES	HEARING AIDS: ☐ L ☐ R	DENTURES: ☐ TOP ☐ BOTTOM
PATIENT	☐ HANDBAG/BAG	☑ WALLET	☑ MOBILE PHONE/CHARGER
BELONGINGS	☐ WATCH	☐ FOOTWEAR	☐ OWN MEDICATION
LABELLED	☑ JEWELLERY ..earrings L+R silver..		☐ VALUABLES WITH SECURITY
ON ICU	☑ OTHER ..additional charger candles..		
ADMISSION			

DATE	MAJOR EVENTS & PATIENT PROGRESS

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5.26. Progress Report (FMH516500)

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PROGRESS REPORT

WARD: ICU Bed: 6

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Date	For all entries: Print Name, Signature and Designation

Examination: After fluid resus

A: Patent, talking

B: RR 23, SPO2 100% on 2L via nasal prongs

C: HR 120, BP 103/63 (75) on 25 norad and 0.6 vasopressin
ECG: Sinus rhythm, normal axis, no ST or T wave changes
Peripherally cool

D: Alert, interactive, GCS 15

E: R) femoral CVC, R) radial arterial line, IDC

L) elbow 1 large and two small lacerations, not actively bleeding, neurovascularly intact peripherally, weak peripheral pulses but confirmed on Doppler.

Chest clear anteriorly
HSDNM
Abdomen SNT, BS +ve
Calves SNT, no oedema

Investigations:
Initial VBG: pH 7.1, glucose 23, lactate 13, bicarb 4, Na 134, K+ 4.4, AG 21, Hb 129, Cl 101, anion gap -6

1700PM ABG pH 7.33/ pO2 113 / pCO2 39.5 / HCO3 20.7/ Lactate 3.1
Na 133/ K 4.4/ Cl 95/ CorCa 2.31/ Mg 1.16/ PO4 2.60
Creat 168/ Urea 4.1/ eGFR 41
Bili 11/ ALP 77/ GGT 64 / ALT 63/ AST 47/ Albumin 33
Hb 121/ Plt 243/ WCC 6.8 / Neut 4.4
CK 99

Issues:
 # Shock of unknown origin
- ?hypovolemia secondary to blood loss from laceration
- High BSL, high lactate
- No ECG changes or chest pain, awaiting troponin
- No clear focus of infection

 # Left cubital fossa lacerations
- Sustained with glass from vape machiene
- Received ADT booster in ICU
- Plan for xray to check for retained glass
- Awaiting Gen Surg review and CTA of left arm

 # High BSL
- BSL 23 on arrival
- On insulin infusion, BSLs improving to 13.3

ICU Plan (D/W Consultant):
1) Admit ICU
2) Monitor BSL, lactate, urine output
3) Aim MAP >65 – titrate vasopressors
4) Hydrocortisone 50mg QID
5) One unit PRBC
6) Ongoing IVF – Hartmann's at 100mL per hour
7) IVABx – Tazocin TDS
8) Chase bloods including trop and CK, chase blood and urine cultures

Page 2 of 3

Werribee Mercy Hospital
300-310 Princes Highway, Werribee Victoria 3030 Telephone: (03) 9216 8888 Facsimile: (03) 9216 8777
mercyhealth.com.au ABN 74 762 230 429

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Compassion Hospitality Respect Innovation Stewardship Teamwork

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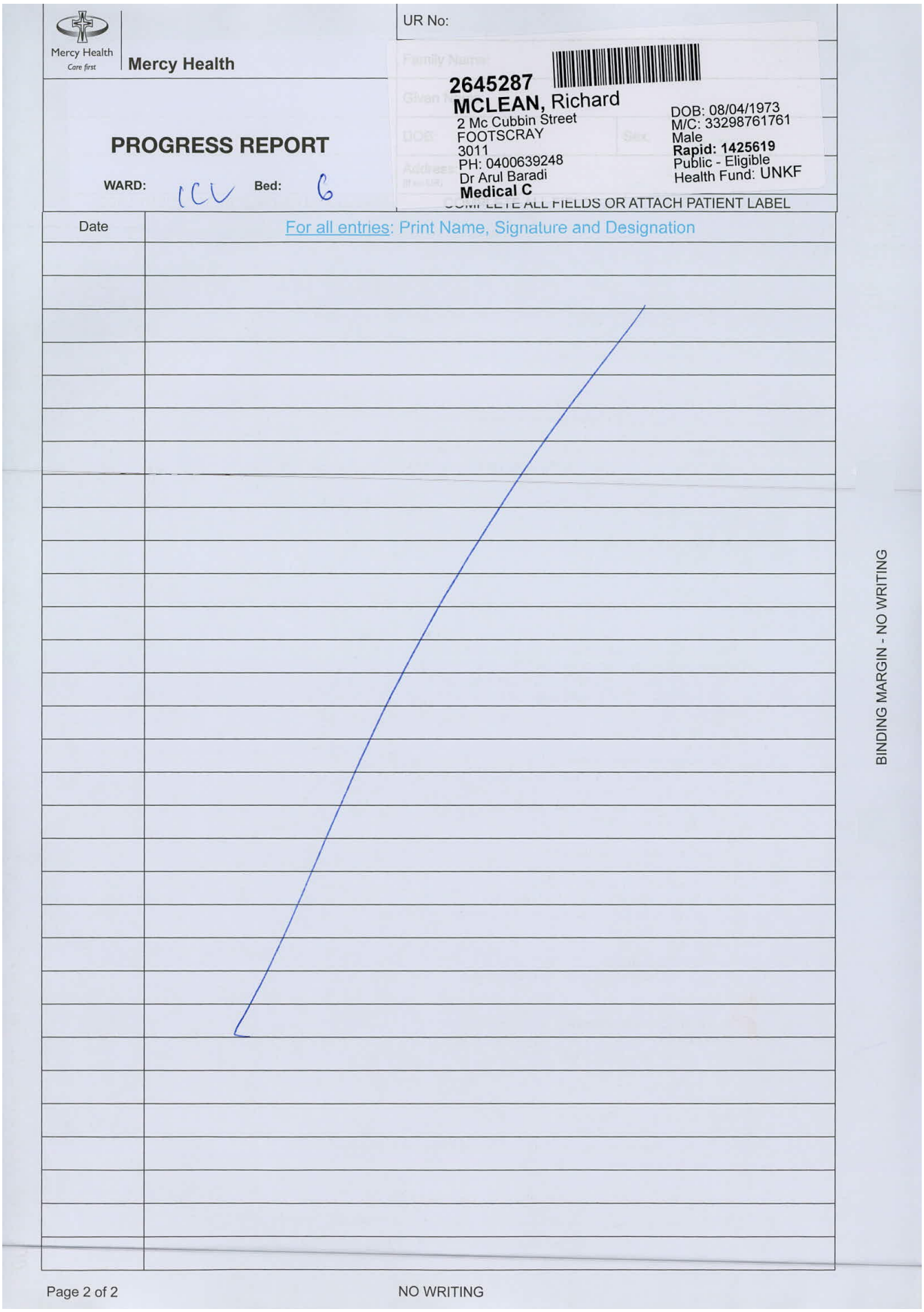
5.27. Progress Report (FMH516500)



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PROGRESS REPORT

WARD: ICU Bed: 6

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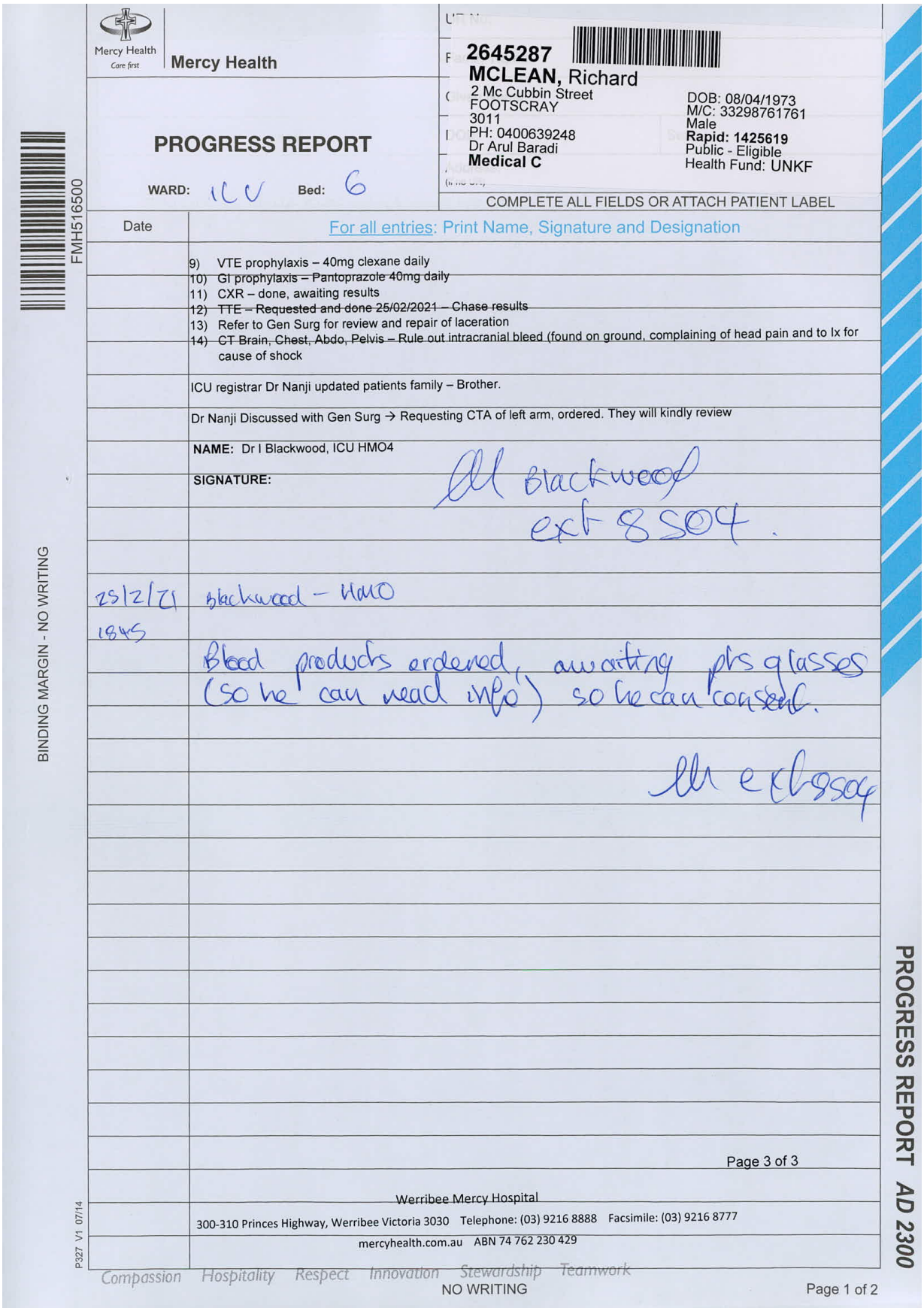
5.28. Progress Report (FMH516500)



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PROGRESS REPORT

WARD: ICU Bed: 6

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Date	For all entries: Print Name, Signature and Designation
	9) VTE prophylaxis – 40mg clexane daily 10) GI prophylaxis – Pantoprazole 40mg daily 11) CXR – done, awaiting results 12) TTE – Requested and done 25/02/2021 – Chase results 13) Refer to Gen Surg for review and repair of laceration 14) CT Brain, Chest, Abdo, Pelvis – Rule out intracranial bleed (found on ground, complaining of head pain and to Ix for cause of shock
	ICU registrar Dr Nanji updated patients family – Brother.
	Dr Nanji Discussed with Gen Surg → Requesting CTA of left arm, ordered. They will kindly review
	NAME: Dr I Blackwood, ICU HMO4 **SIGNATURE:** I Blackwood ext 8504.
25/2/21 1845	Blackwood – HMO Blood products ordered, awaiting pts glasses (so he can read info) so he can consent. I Blackwood ext 8504

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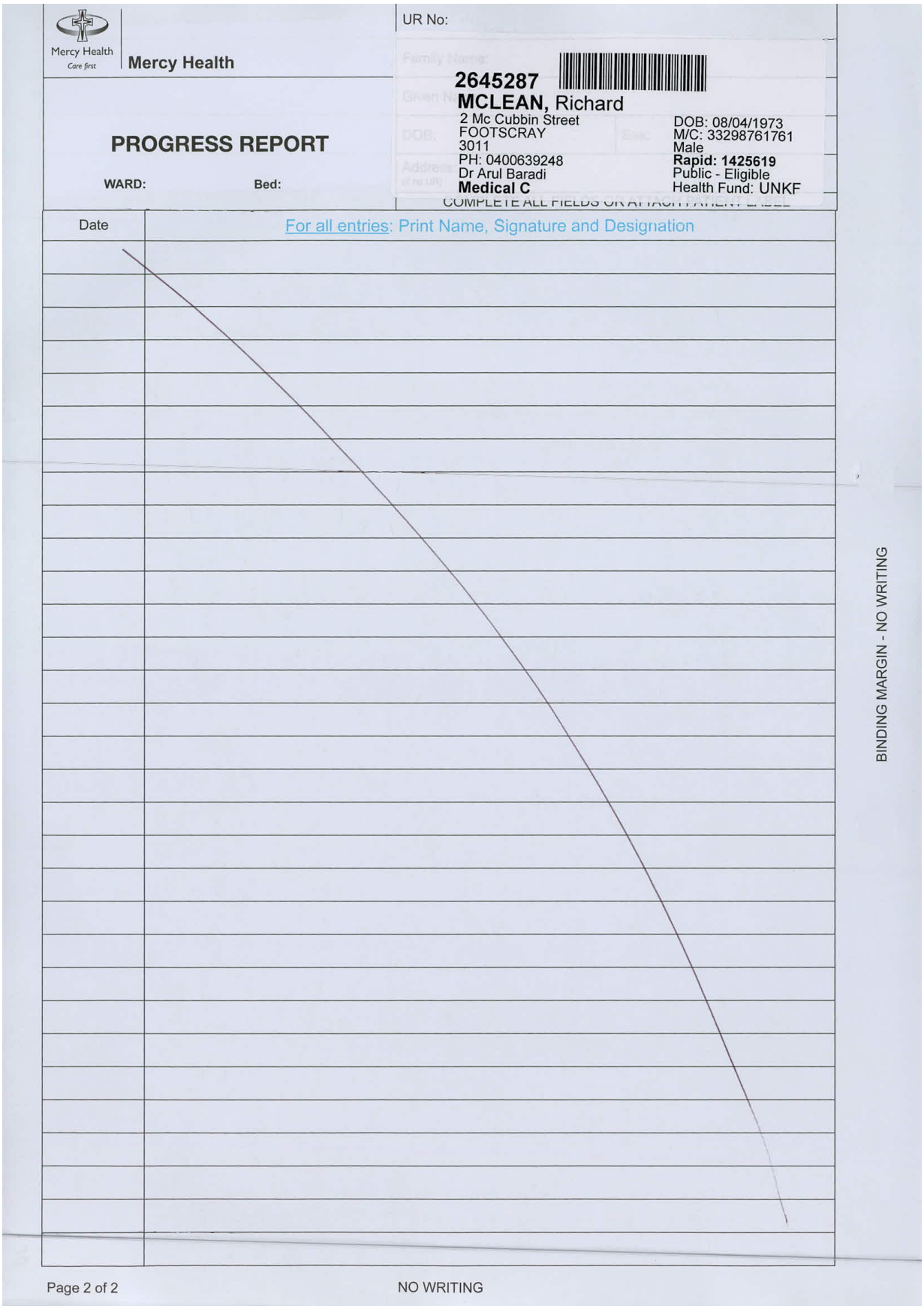
5.29. Progress Report (FMH516500)



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PROGRESS REPORT

WARD: **Bed:**

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5.30. Progress Report (FMH516500)

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PROGRESS REPORT

WARD: ICU Bed: 6

FMH516500

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Date	For all entries: Print Name, Signature and Designation
25/12/21	Surgical Resident Cover (I. Malthi)
2015	ATSP RE: left antecubital fossa laceration - self inflicted.
	57M current psychiatric patient - pt reluctant to elaborate as to presentation. Consented to photograph wound.
	Today broke "vape" pen and attempted suicide in the shower by "bleeding out." Reports bleeding for 30 minutes - unsure amount.
	Reports minimal pain over laceration. Also reports mild chest pain and dizziness when moving in bed. Able to move arm, flex + extend, supinate and pronate.
	Currently on IV Tazocin - as per ICU.
O/E	OBS: normal, afebrile. BP initially low, on noradrenaline. Alert + oriented - responds. GCS 15/15.
	Biceps tendon intact.

	R	U	M	AIN	PIN
M	-	-	-	-	-
S	-	-	-	-	

Pulse present. (Radial). Normal cap refill.

Plan/ 1. Review photograph w/ surg reg.
2. CTA
3. NBM.
4. Confirm ongoing plan w/ surg reg.

#62919.

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5.31. Progress Report (FMH516500)

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COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Date	For all entries. Print Name, Signature and Designation
25/12 2030pm	Nursing entry :- Patient assessment & Safety checks completed. Patient received from mental health unit @ 1400pm after meal, CVC & art line inserted & handover received @ 1630pm - GCS = 15/15 - GCS improved PEARL = 3+. Good power to all limbs, wound to L) CF - 3 wounds - self inflict wounds - & noticed heavy bleeding from wound site @ 1615pm, around 150mls blood - Pressure applied & dressing changed - ICU on duty doctors reviewed, ? for stitching & surgeons reviewed @ 2030pm, please see plan. Russ score 0 & Pain score 0/10. (2) Resp → Chest xray done, CXR on NIL @ 2L of oxygen & currently on RA SpO2 > 98%, Resp rate = 14 to 18/mt, ABG's @ 1900pm - pH = 7.39, pCO2 = 35.8, pO2 = 92.2 cHCO3 = 21.3, BE = -2.6. Good spontaneous cough & nil expectorated. (3) CVS → ST → SR 90 to 130 bpm, Rockers active - On Noradrenaline @ 20 → 25 mcg/hr & Vasopressin @ 0.8 units/hr to keep MAP > 70 mm of Hg. Systolic BP ~~→~~ on is maintaining inbetn 100 to 140mm of Hg. R) femoral CVC in situ. & R) Radial art line - in situ. All 4 extremities, pulses present good capillary refill, doppler checked to L) arm & R) dockets - (4) Afebrile - hypothermic on admission & Bairhugger → & temperature improved to 36.7°C at the time of report. & Bairhugger is stopped. Lactate was 14.0 (cont)

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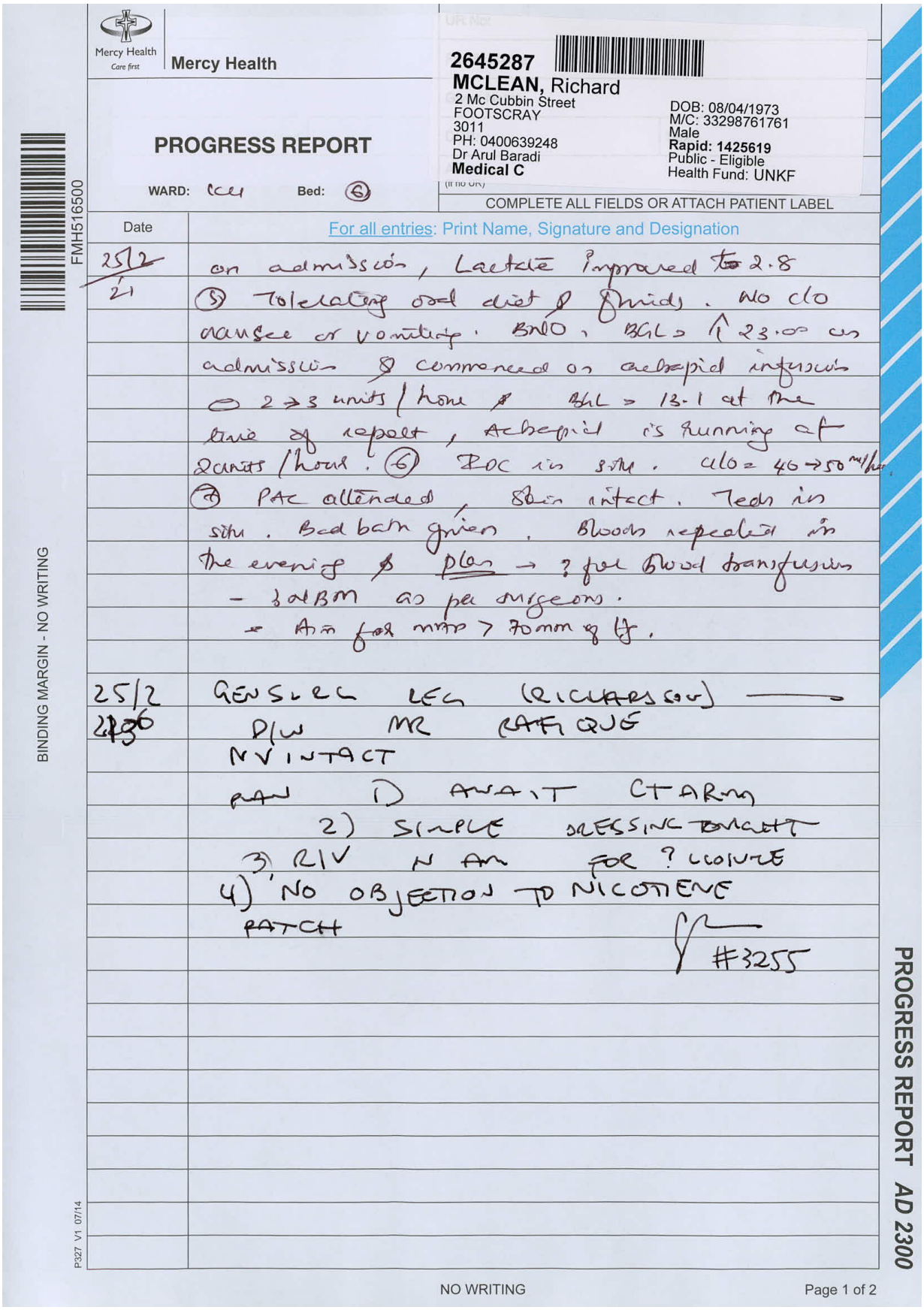

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PROGRESS REPORT

WARD: ICU Bed: 6

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Date	For all entries: Print Name, Signature and Designation
25/2 21	on admission, Lactate Improved to 2.8 ⑤ Tolerating oral diet & fluids. NAD clo nausea or vomiting. BNO, BGL ↑ 23.0 on admission & commenced on actrapid infusion @ 2→3 units/hour & BGL = 13.1 at the time of repeat, Actrapid is running at 2 units/hour. ⑥ IDC in situ. clo = 40→50 ml/hr. ⑦ PAC attended, Skin intact. Tegs in situ. Bad burn given. Bloods repeated in the evening & plan → ? for Blood transfusion - 3 of BM as per surgeons. - Aim for mane > 70mm of Hg.
25/2 2230	GENERAL LEG (RICHARDSON) —→ P/W MR RAFIQUE NV INTACT PLAN 1) AWAIT CT ARM 2) SIMPLE DRESSING TO LEFT 3) R/V N AM FOR ? CLOTTLE 4) NO OBJECTION TO NICOTIENE PATCH #3255

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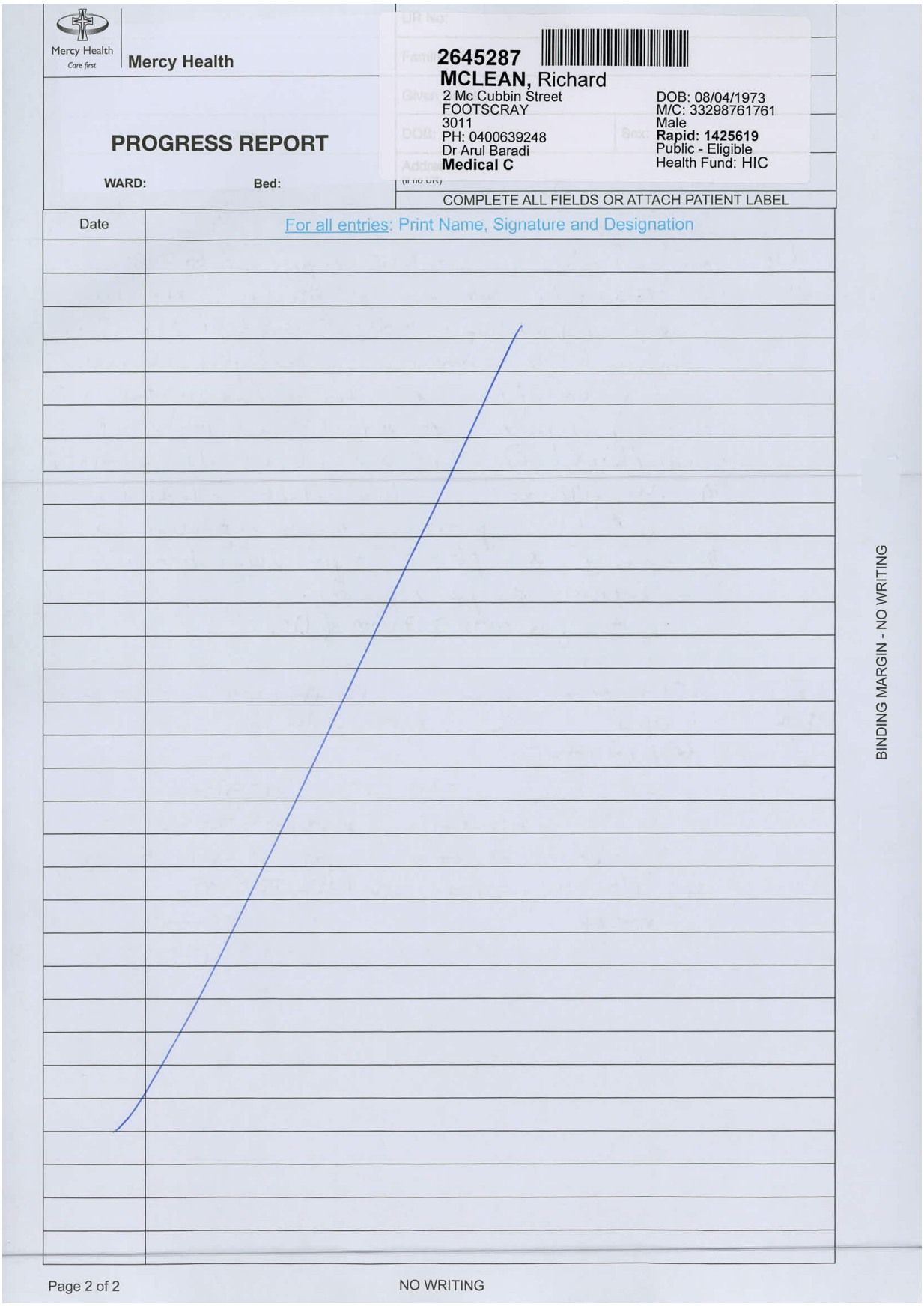
5.33. Progress Report (FMH516500)



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PROGRESS REPORT

WARD: Bed:

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COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Date	For all entries: Print Name, Signature and Designation

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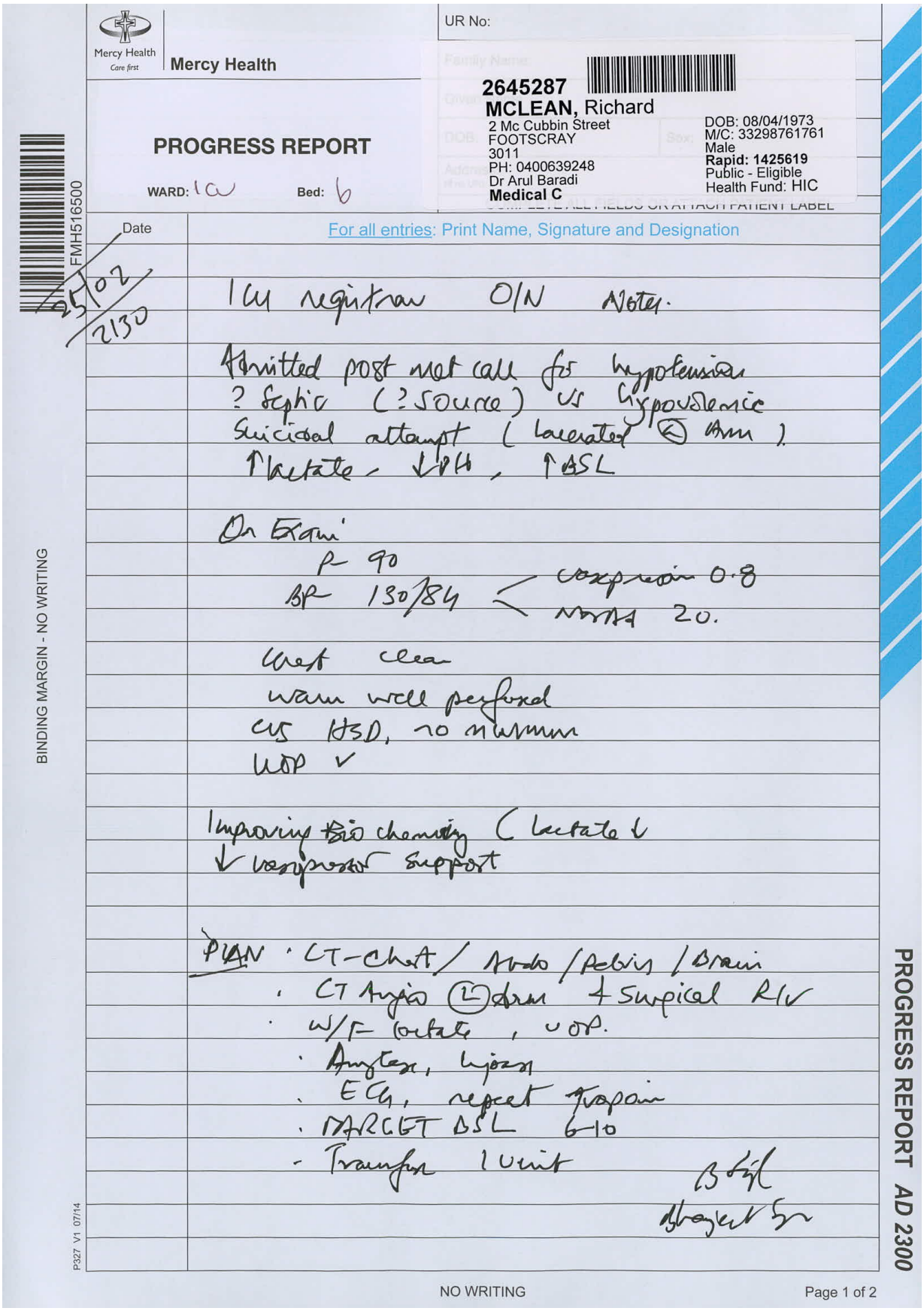
5.34. Progress Report (FMH516500)



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PROGRESS REPORT

WARD: ICU Bed: 6

FMH516500

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Date

For all entries: Print Name, Signature and Designation

25/02
2130

ICU registrar O/N Notes.

Admitted post met call for hypotension
? Septic (? Source) vs Hypovolemic
Suicidal attempt (lacerated ⊗ Arm)
↑ lactate - ↓PH, ↑BSL

On Exam
P 90
BP 130/84 < oxygen 0.8
Nasal 20.

Chest clear
warm well perfused
CVS HSD, no murmurs
UOP ✓

Improving bio chemistry (Lactate ↓
↓ vasopressor support

PLAN · CT-Chest / Abdo / Pelvis / Brain
· CT Angio ⊗ arm + Surgical R/V
· W/F lactate, UOP.
· Analgesia, Injury
· ECG, repeat Troponin
· TARGET BSL 6-10
· Transfer 1 unit

Registrar Sign

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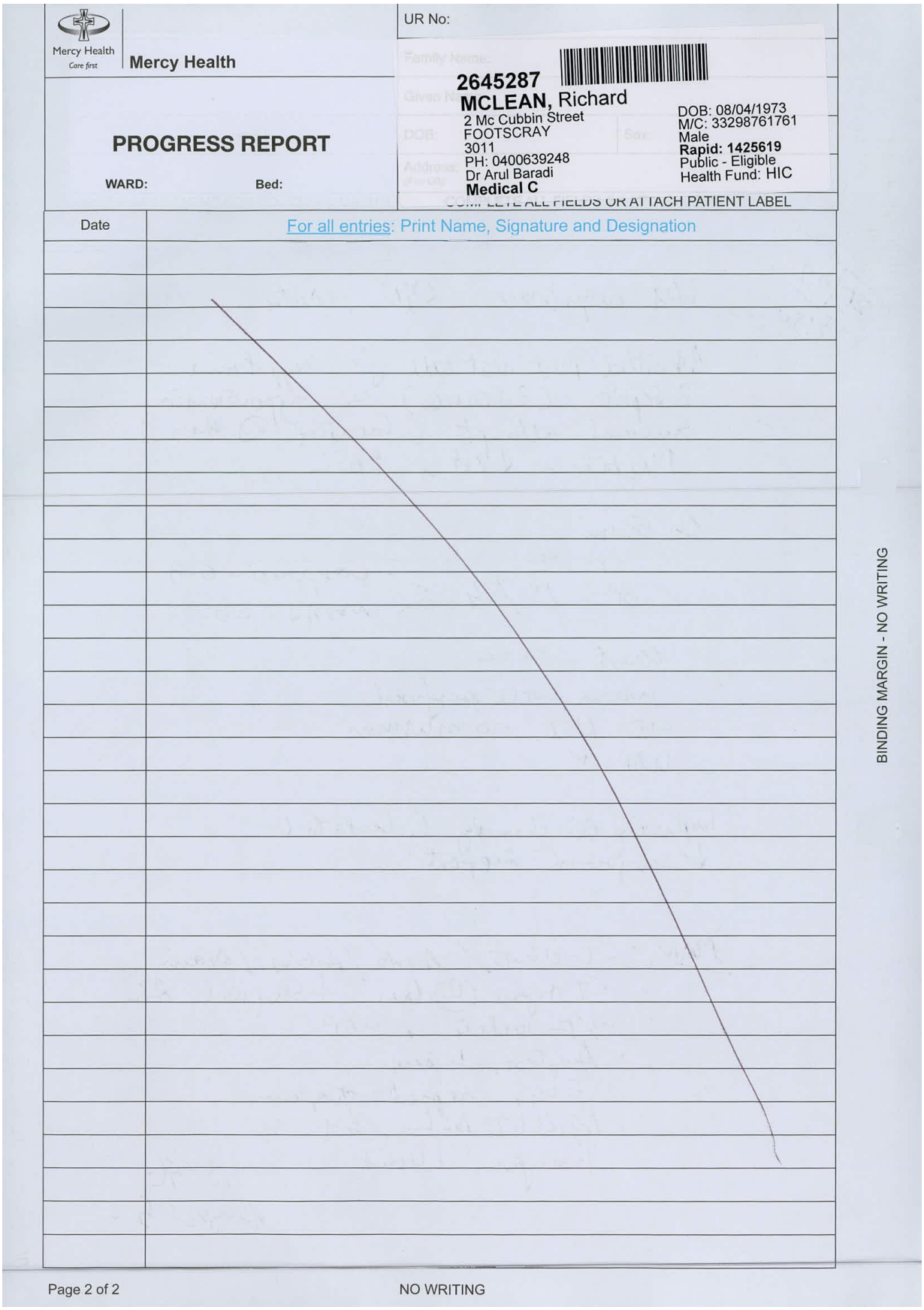
5.35. Progress Report (FMH516500)



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PROGRESS REPORT

WARD: Bed:

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5.36. Progress Report (FMH516500)

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PROGRESS REPORT

WARD: ICU Bed: 6

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

BINDING MARGIN - NO WRITING

Date	For all entries: Print Name, Signature and Designation
26/02/21	Gen Med Admission (Shan for Baradi)
0030	
	47M MET call in ED – hypotension, LOC post self harm to L) cub fossa, req norca.
	PMHx Schizophrenia ADHD Prev suicide attempts
	Meds – from ED Amisulpride 600mg nocte Paroxetine 20mg mane Tenofovir/emtricitabine (PREP) Diazepam 10mg nocte Dexamfetamine 30mg mane
	SHx Socially isolated Has pet dog Reports being ?homeless Prev methamphetamine use
	Allergies Bees, augmentin (NKV) Penicillin
	HOPC · on AO, in mental health unit last 2/7 for deteriorating mental health, paranoia, delusions · Today – MET – found collapsed on ground in shower – self harm to L) cub fossa, some blood in bathroom but no large amount & no ongoing bleeding. Denies other self harm / OD. – reported vape exploding – used glass – cold, clammy, BP unrecordable, HR 120 – GCS 9 (E1 V2 M1) at time – unable to gain peripheral access in ED → +/- IVU – Fem venous access gained, R) radial art line · pH 7.1 Lactate 13 BSL 23 HCO3 4 ~~BE~~ AG 21.

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5.37. Progress Report (FMH516500)

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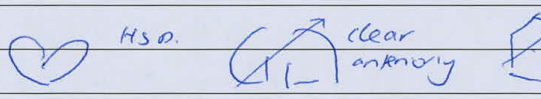
PROGRESS REPORT

WARD: ICU Bed: 6

Date	For all entries: Print Name, Signature and Designation
	· commenced noradr 2
	actrapid infusion
	Fluid load. 3L
	IV tot, hydrocort 50mg QID.
	→ GCS 15/15. BP stabilised
	pH 7.38 Lactate 2.
	· Reviewed by surg - NVI.
	· ADT booster
	· CTA ordered, awaiting report.
	· NBM
	O/E: Patient agitated, tangential on r/v
	Paranoid ++
	Req reassurance ++ to allow exam
	Fixated on missing antidepressant doses.
	 HSD. clear ankmoly soft SC.
	warm, well perfused. GCS 15. Looks well
	L) arm bandaged - as per ICU exam
	1X large, 2X small lac not actively bleeding
	NVI. weak pulse confirmed on doppler.
	IX FBE (N) Hb 129 → 84 post 4L IVT
	UEC 133/4.4/168 eGFR 41 Ur 44.
	CMP - PO42.6
	LFTs (N)
	VBG now normalised
	Urine: protein ++ gluc ++

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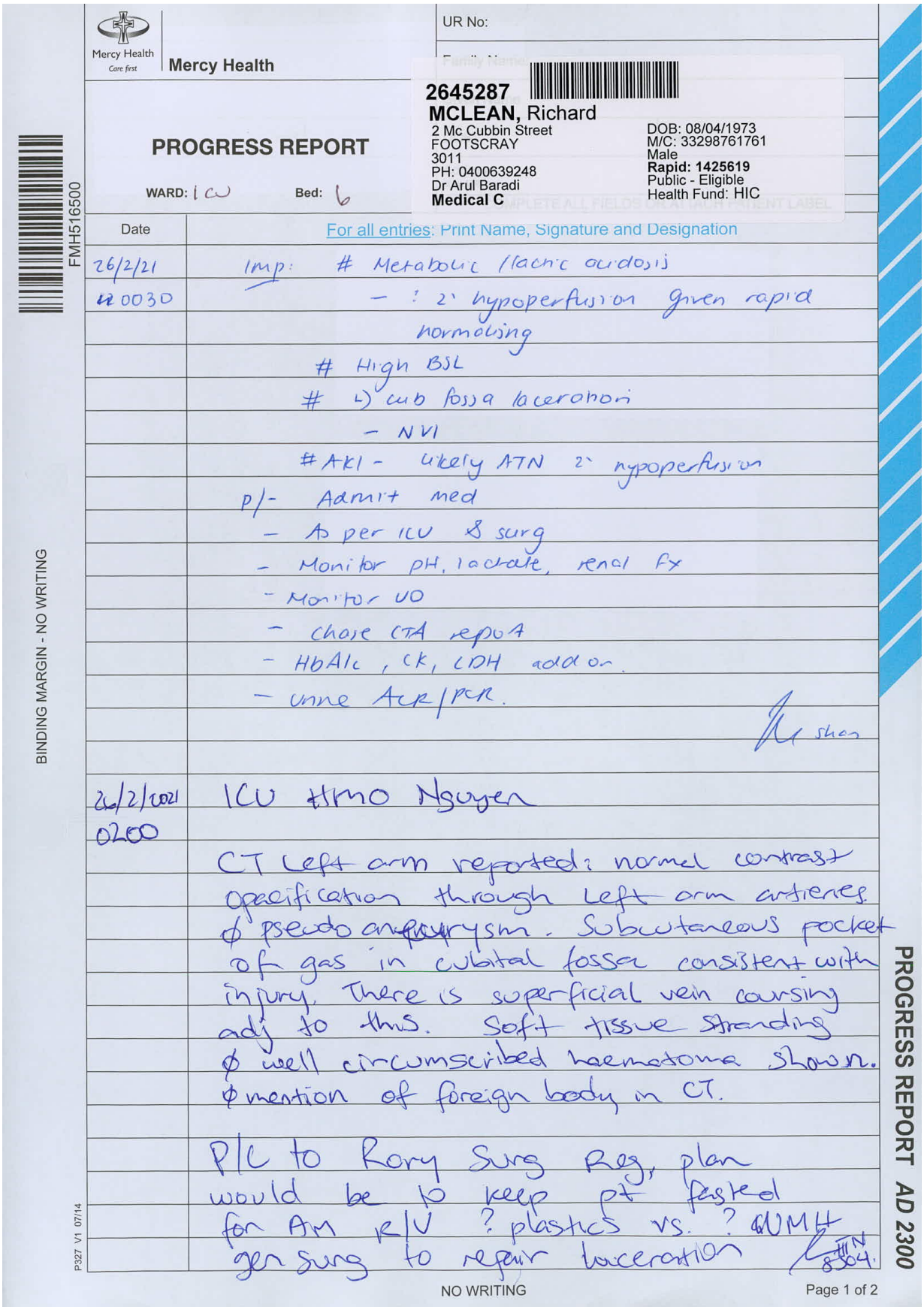
5.38. Progress Report (FMH516500)



Mercy Health
Care first
Mercy Health

UR No:

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

PROGRESS REPORT

WARD: ICU Bed: 6

FMH516500

BINDING MARGIN - NO WRITING

Date	For all entries: Print Name, Signature and Designation
26/2/21 0030	Imp: # Metabolic /lactic acidosis – ? 2° hypoperfusion given rapid normalising # High BSL # L) cub fossa lacerations – NVI # AKI – likely ATN 2° hypoperfusion P/- Admit med – As per ICU & surg – Monitor pH, lactate, renal fx – Monitor UO – chase CTA report – HbA1c, CK, LDH add on. – Urine ACR/PCR.
26/2/2021 0200	ICU Hmo Nguyen CT Left arm reported: normal contrast opacification through Left arm arteries ∅ pseudoaneurysm. Subcutaneous pocket of gas in cubital fossa consistent with injury. There is superficial vein coursing adj to this. Soft tissue stranding ∅ well circumscribed haematoma shown. ∅ mention of foreign body in CT. P/C to Rory Surg Reg, plan would be to keep pt fasted for AM R/V ? plastics vs. ? GUMH gen surg to repair laceration

PROGRESS REPORT AD 2300

P327 V1 07/14

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Page 1 of 2

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5.39. Progress Report (FMH516500)

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PROGRESS REPORT

WARD: ICU Bed: 6

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Date	For all entries: Print Name, Signature and Designation
26/2/21	
0350	Falls Score ---> 3 High falls risk Y N Clutter Free Space ✓ Appropriate Lighting ✓ Call bell & items in reach ✓ Non- slip socks Supervision in SOOB ✓ CAM ICU Completed ✓ Braden Score ----> 20 PI Prevention Strategies ✓ Malnutrition Screening 0 Date- 26/2/21 Time 0350 Sign Cfan
	SKIN INSPECTION Date: 26/2/21 Time: 0350 Consumer participated in care planning: Yes
26/2/21	nursing NIC - bedside safety checks done, alarm settings in monitor
0500	checked; escorted to ct for CTscan of brain, abdo, arm,
	pelvis ~ came c vs is the ICU reg.
	CNS: patient alert and orientated, GCS 15, RASS +1, PEARL 3+,
	CAM-ICU - negative, all 4 limbs equal strength and sensation
	intact; monitored for pain on the laceration site (L CF)
	and chest pain
	CVS: normotensive, maintaining MAP 70 to mmHg, on vasopressor
	support, c noradrenaline @ 8 mcg/min meaning down and
	vasopressin started @ 0.8 units/hr then 0.4... 0.0 then
	ceased @ 0330, HR- monitored ST/SR ϕ ectopics noted,
	warm and well- perfused, slightly febrile @ 37.7°C
	c CVC @ R femoral c x 4 lumens, distal lumen connected
	to CVP, c IA @ R radial - zeroed, CSL maintenance @ 100ml/hr
	finished @ 0100 ~ ICU resident verbalized not to have more IVF
	to follow, dve medication given as per AD 4890
	Respi: chest clear, good and equal air entry down to the
	bases, Or saturation >95% on RA; ϕ SOB or ↑ WOB
	noted, dry cough @ times → continuation

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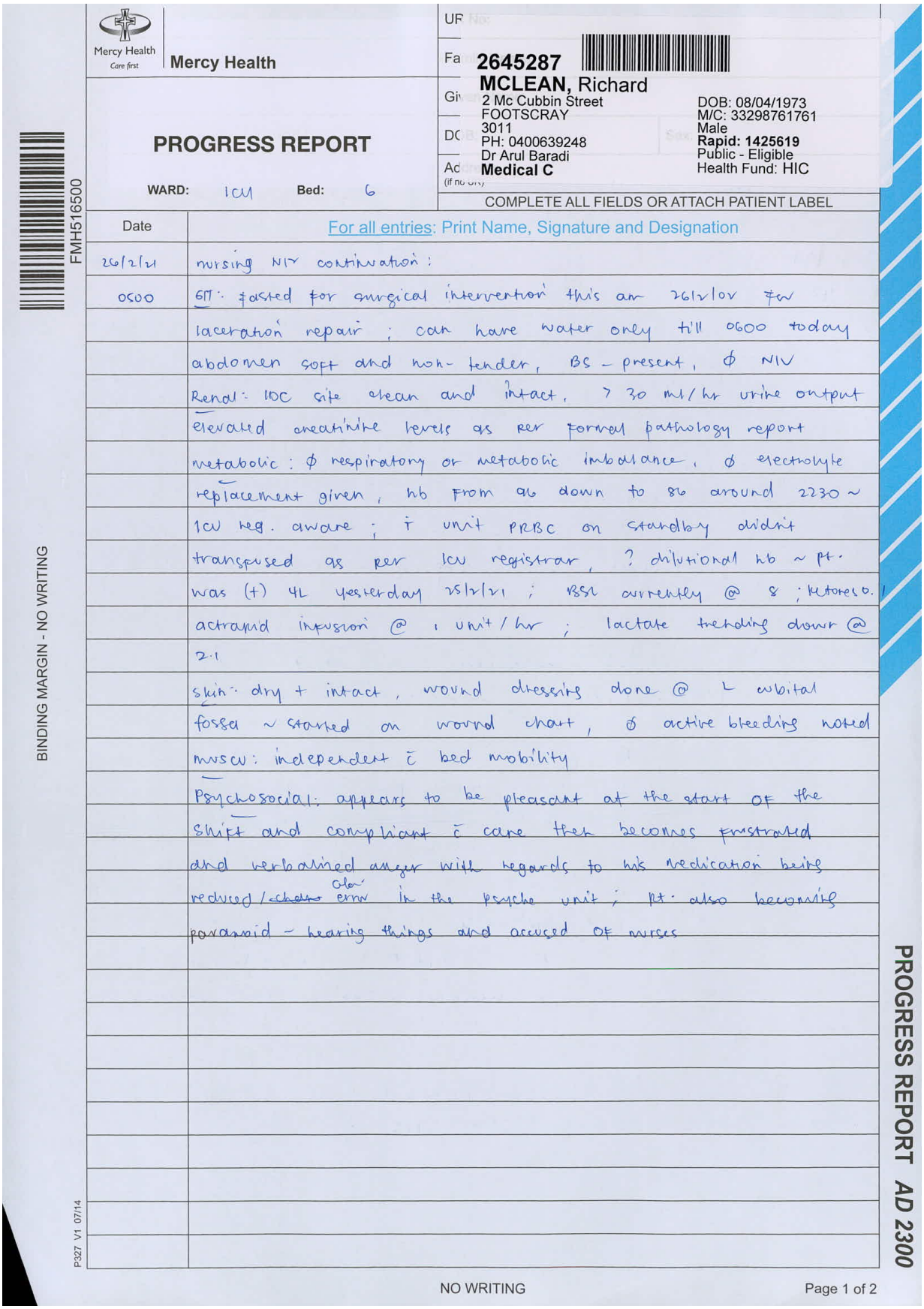
5.40. Progress Report (FMH516500)



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COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

PROGRESS REPORT

WARD: ICU Bed: 6

FMH516500

BINDING MARGIN - NO WRITING

Date	For all entries: Print Name, Signature and Designation
26/2/21	nursing NIV continuation :
0500	GIT : fasted for surgical intervention this am 26/2/21 for laceration repair ; can have water only till 0600 today
	abdomen soft and non- tender, BS - present, ∅ NIV
	Renal : IDC site clean and intact, > 30 ml/hr urine output
	elevated creatinine levels as per formal pathology report
	metabolic : ∅ respiratory or metabolic imbalance, ∅ electrolyte
	replacement given, hb from 96 down to 86 around 2230 ~
	ICU reg. aware ; 1 unit PRBC on standby did not
	transpused as per ICU registrar, ? dilutional hb ~ pt.
	was (+) 4L yesterday 25/2/21 ; BSL currently @ 8 ; ketones 0.
	actrapid infusion @ 1 unit/hr ; lactate trending down @ 2.1
	skin : dry + intact, wound dressing done @ L cubital
	fossa ~ started on wound chart, ∅ active bleeding noted
	musculo : independent ↄ bed mobility
	Psychosocial : appears to be pleasant at the start of the
	shift and compliant ↄ care then becomes frustrated
	and verbalised anger with regards to his medication being
	revised / ~~chains~~ clear error in the psyche unit ; pt : also becoming
	paranoid - hearing things and accused of nurses

PROGRESS REPORT AD 2300

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5.41. Progress Report (FMH516500)

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PROGRESS REPORT

WARD: ICU Bed: 6

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Date	For all entries: Print Name, Signature and Designation
26/2/24 09:42	AM ICU CWR (Kadam et al) Issues. ① Unknown origin - SHOCK. Elevated lactate Elevated BSL. No ecg changes ; FeNO report awaited, Trop ↑ Cultures sent. ② (L) Subtotal forna lacerations. Suicidal ideation. CT angio (L) arm → superficial injury. ③ Hyperglycemia. On insulin infusion. Glu 6.2. Not a known diabetic. A - own. B - SpO_2 - 95% Room air. RR - 15 Chest - Clear, No added sounds. C - HR - 114. BP - 100/60 (on noradrenaline) Peripheries warm, CRT - 2secs. HSDNM. D - Alert, oriented GCS - 15/15.

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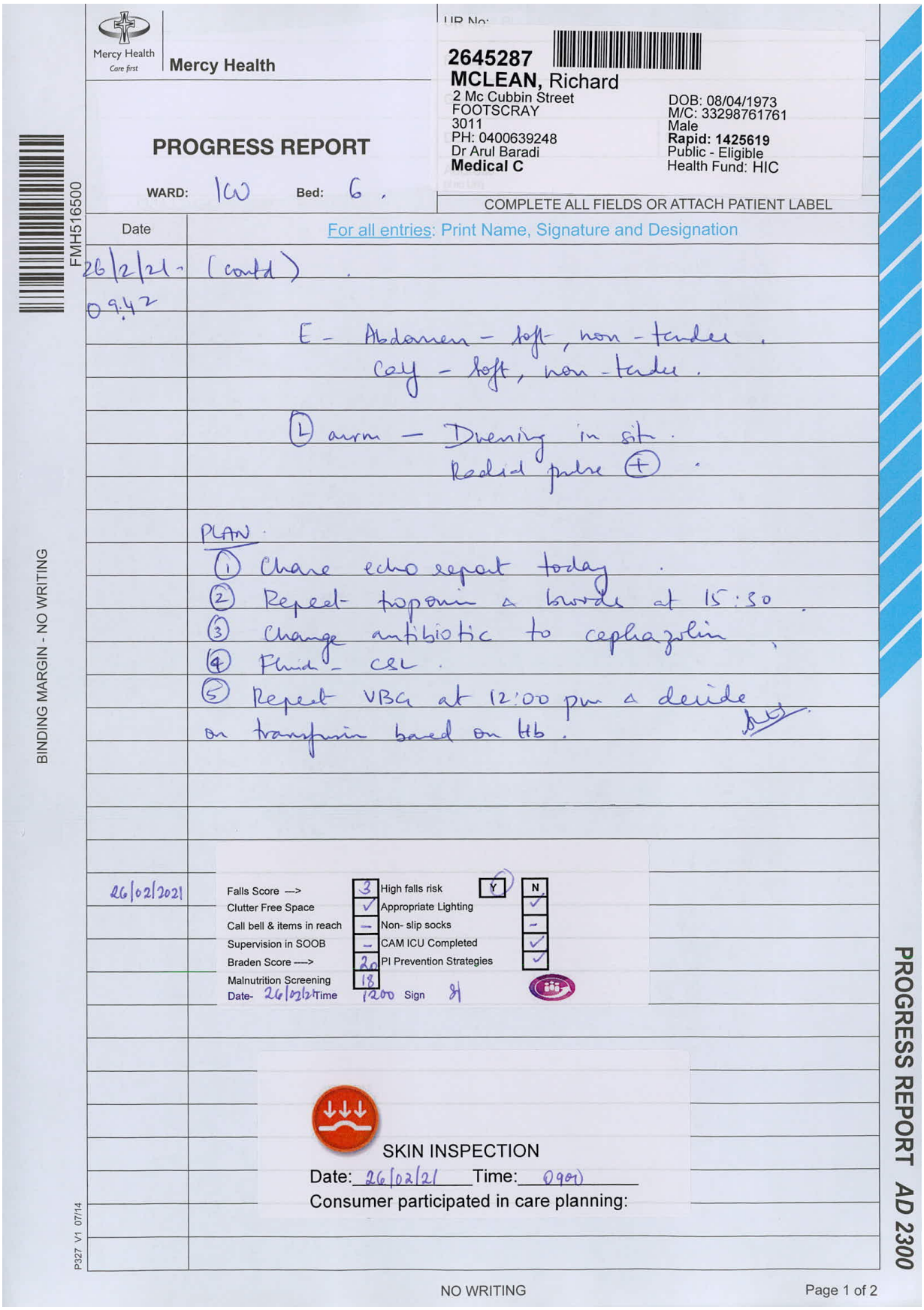
5.42. Progress Report (FMH516500)



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Health Fund: HIC

PROGRESS REPORT

WARD: ICU Bed: 6.

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH516500

BINDING MARGIN - NO WRITING

Date	For all entries: Print Name, Signature and Designation
26/2/21 - 0942	(contd) . E - Abdomen - soft, non-tender. Calf - soft, non-tender. (L) arm - Dressing in situ. Radial pulse (+). PLAN. (1) Chase echo report today. (2) Repeat troponin x 2 more at 15:30. (3) Change antibiotic to cephazolin. (4) Fluids - CSL. (5) Repeat VBG at 12:00 pm & decide on transfusion based on Hb.

26/02/2021

Falls Score --->	3	High falls risk	Y	N
Clutter Free Space	✓	Appropriate Lighting		✓
Call bell & items in reach	–	Non- slip socks		–
Supervision in SOOB	–	CAM ICU Completed		✓
Braden Score ---->	20	PI Prevention Strategies		✓
Malnutrition Screening	18			

Date- 26/02/21 Time 1200 Sign

SKIN INSPECTION
Date: 26/02/21 Time: 0901
Consumer participated in care planning:

P327 V1 07/14

PROGRESS REPORT AD 2300

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Page 1 of 2

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5.43. Progress Report (FMH516500)

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PROGRESS REPORT

WARD: ICU Bed: 6

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(if no UR)

DOB: 08/04/1973
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Male
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Public - Eligible
Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Date	For all entries: Print Name, Signature and Designation
26/02/2021 1300	Nursing: Handover received from NO staff Resus: Full CNC: Patient was initially RASS 0, calm, GCS 15, Camc-) most of the morning has been RASS +1, agitated and kept on saying that his mobile phone was taken from him. Reassured. Minimal pain on L) arm. Normal power & equal limb strength upper & lower limbs. NV obs on L) arm - intact - full sensation, warm & good capillary refill. Resp: Chest clear. Saturating well on RA - 98-100%. CVS: Warm & peripherally well perfused. On SR - nil ectopics. Initially on Noradrenaline 5mcg/min. Weaned to 3, ↓ 2 and stopped @ 1100. MAP ≥ 65. Pulses palpable on radial & pedal. Arterial line on R) radial - zeroed - flushing well. GIT: Abdomen soft - non tender BS ✓. On NBM - awaiting surgical team ? suture of L) arm wound. Nil nausea/vomiting. IVF Add: CVC on R) femoral insitu - x 4 lumens - all working & patent. Metabolic: ABG done this morning. Hb 80 - 2nd "u" of PRBC - on hold until 12pm ABG - to decide if will transfuse. K - 3.9 - replaced this morning. Mg 20mmol given as well. Mg 0.71. Phosphate 1.17. ABG @ midday - Na 139 K - 3.9 BSL 6.5 Lactate 1.3 Hb 78 - reviewed. 2nd "u" PRBC commenced @ 1325 hrs. Renal: IDC insitu draining to large amounts of urine. Social: Patient started to become agitated & suspicious @ around 10am - post R/V by Psych team - wanting to speak to "in charge" and wanting to use the phone to call his sister - done w/ supervision. Patient is for supervised visitor - sister is aware. Reassurance given. All regular medications provided by pharmacy & given to patient - more settled afterwards. Skin: Patient independent w/ turns in bed. Skin dry & intact - nil pressure area. Wound dressing on L) arm - still dry & intact Plan: For bloods & troponin @ 1530, awaiting surgical team for suturing of wound. DeMesa, Sandy RN

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Page 2 of 2 NO WRITING

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5.44. Progress Report (FMH516500)

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Health Fund: HIC

Australia

Home Phone 0400639248
Work Phone

Patient Progress

From 26/02/2021 **To** 26/02/2021

26/02/2021 12:25 PM : - [Medical Officer Review] (DRAFT - last modified 26/02/2021 01:06 PM)

Statewide UR: 1425619
Date of Entry: 26/02/2021
Time of Entry: 12:10
nician: Jeril Thomas
Designation: RN Grade 5
Team: Werribee Mercy Hospital Consultation Liaison Psychiatry Service

Status:
ITTO

Referral received from inpatient psychiatry team to review above consumer currently admitted to ICU after serious attempt to self-harm with intent to end his life.

47 male longstanding history of schizophrenia, admitted due to increasing threats and plan to end his life on the b/g of injustices done to him through previous treatments under the MHA. Consumer has negative outlook towards MHS system. Ongoing paranoia and animosity towards all hospital staff especially psychiatric teams. . Has history of being litigious through phone recording often without consent, using calculating behaviour in order to meet own needs and possible staff splitting. File review suggest Vape pen, shoe string was utilised and suicide letter was also found.

Brief discussion with ICU team who have advised that attempt was lethal and still requires surgical repair. Currently not medically cleared. Management plan in ICU notes for nurse to follow. Medication chart completed according to CMB med charts. Advised ICU to focus of containment and consistency in his management. Minimal negotiations around psychiatric management, follow management plan provided. Visitors are allowed by visitation must be supervised. Aim for swift medical clearance and is for transfer. Advised client will require ICA bed given risk of repeating self-harm or suicide.

Present:
Dr John Dahl
Jeril Thomas
Medical Student

Richard lying in bed was fast asleep and was easily woken up. All three clinicians introduced ourselves and Richard agreeable to engage. Richard able to recall and understand why he was brought into ICU. Showed insight into his action however remains to be supportive of this. Richard reported he thought this was the only was 'out of that place'. He reported feeling locked up. He referred to himself as being 'jailed'.

Printed by : Jeril Thomas **Printed on : 26/02/2021 1:06:21 PM**

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5.45. Progress Report (FMH516500)

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Australia

Patient Progress

From 26/02/2021 **To** 26/02/2021

Home Phone 0400639248
Work Phone

Richard acknowledged concerns from community regarding his plan to end his life on his Bday hence admission was warranted to prevent this. Richard abruptly stopped the assessment and asked all 3 clinicians credentials again. This was given again to him. Richard refused to talk any further as he felt that CL team were recording. Referred to CL team as 'Part of the system'. CL team provided some further information around role and reason for review today in order to ease anxiety and confusion, however this did not help.

olored with Richard around reason for self-harm. He explained feeling very frustrated with treating team. Reported feelings of not being heard. Richard wanting discharge and this was not approved. Richard did not express any remorse around his action. Continued to justify as his right and has no issues in doing it again. Reported that he is satisfied with his life, has not issues with dying however will hold the 'system' accountable. In order to air his frustrations CL team attempted to elicit his quarrels however he quickly stopped conversation and demanded telephone and Vape and t-shirt. Explained same management applies as he was in CMB-IPU and that these wont be provided. Richard strongly asked to leave, refused to talk and interview was terminated. As CL team walking out Richard yelled 'You didn't show me any empathy'.

Mental Status Examination
Presentation / Appearance: 47 Male, Tattoo on both arms, medium build, lying in bed
Behaviour / Psycho-motor Activity: Nil PMR or PMA.
Affect / Mood: refused to elaborate on mood. Affect irritable.
Speech: Normal, coherent and spontaneous.
ought Disturbance (stream / flow of speech and thoughts): NFTD, linear stream.
ought Content (major themes, delusions): Presents with longstanding paranoia and mis-trust towards mental health system and staff. Persecutory views around system trying to silence him. Unable to elaborate why the system was doing this to him.
Perceptual Disturbances (hallucinations etc): Denied and none detected.
Insight / Judgement: Imapired
Cognition / Concentration / Consciousness / Memory: Intact

Overall Risk: HIGH
Risk of Suicide and self-harm
Risk of Absconding
Moderate Risk of Aggression
Moderate risk of non-compliance towards treatment

Plan:
1. Follow management plan strictly, Aim for consistent and containment of behaviours. Minimal negotiations. Focus on immediate care needs. Limit multiple care givers.
2. Aim for swift medical clearance and transfer to CMB. Upgraded risk status. Requires ICA bed once cleared.
3. Supervised visits with family. Belongings to stay in CMB and can be provided after transfer.

Printed by : Jeril Thomas **Printed on : 26/02/2021 1:06:35 PM**

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5.46. Progress Report (FMH516500)

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Patient Progress

From 26/02/2021 **To** 26/02/2021

Home Phone 0400639248
Work Phone

4. Clear and transparent communication. Firm limit and boundary setting.
5. Nil planned review for the weekend. On-call psychiatric service can be utilised.
6. If transferred to medical ward then will require 1:1 RPN.

\FT In Progress - [Jeril Thomas] - [Nurse]

Printed by : Jeril Thomas **Printed on :** **26/02/2021 1:07:01 PM**

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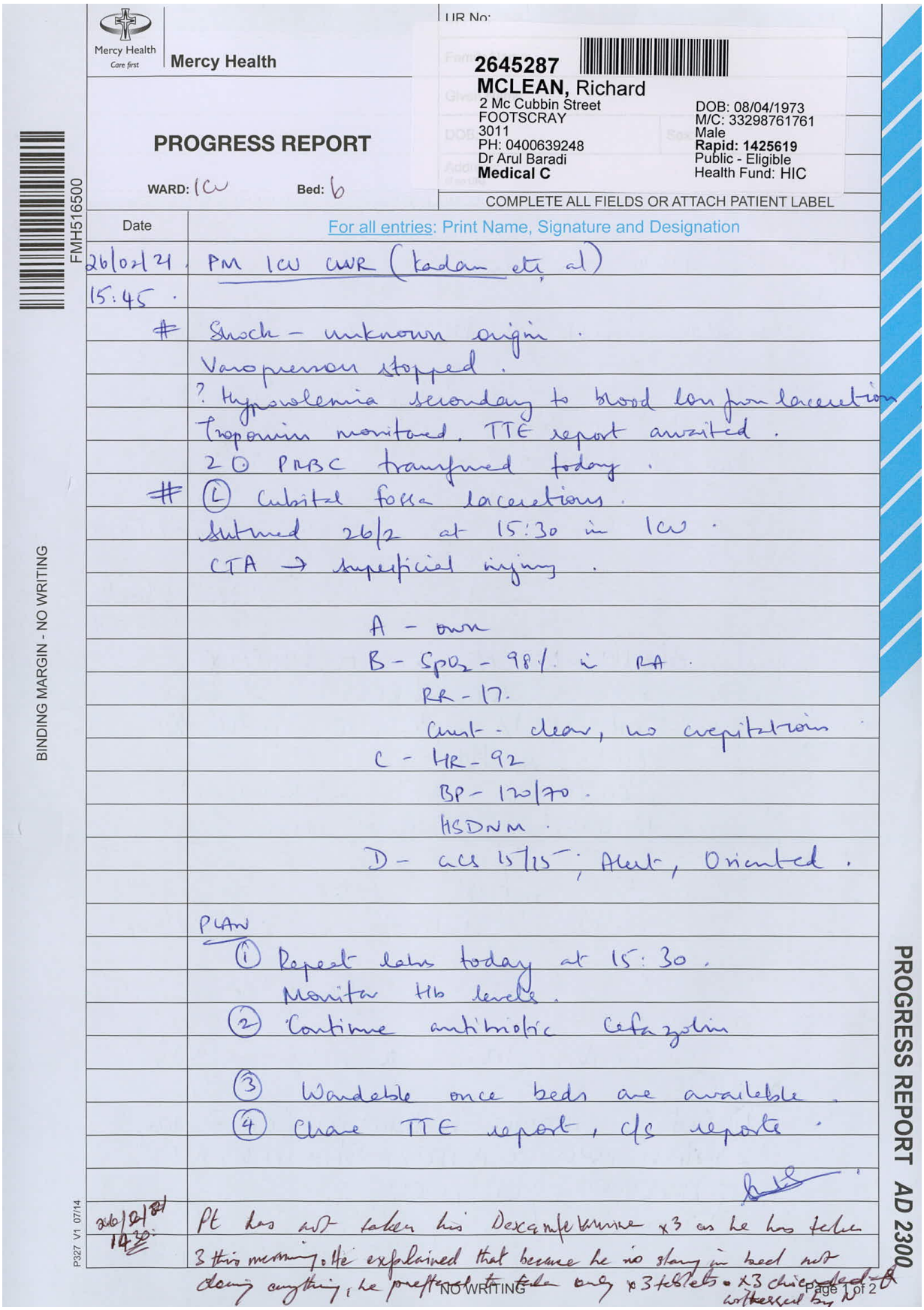
5.47. Progress Report (FMH516500)



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PROGRESS REPORT

WARD: ICU Bed: 6

FMH516500

BINDING MARGIN - NO WRITING

PROGRESS REPORT AD 2300

P327 V1 07/14

Date	For all entries: Print Name, Signature and Designation
26/02/24 15:45	PM ICU CWR (Kadam et al)
#	Shock – unknown origin.
	Vasopressor stopped.
	? Hypovolemia secondary to blood loss from laceration
	Troponins monitored. TTE report awaited.
	2 U PRBC transfused today.
#	(L) Cubital fossa lacerations.
	Sutured 26/2 at 15:30 in ICU.
	CTA → superficial injury.
	A – own
	B – SpO2 – 98% in RA.
	RR – 17.
	Chest – clear, no crepitations.
	C – HR – 92
	BP – 120/70.
	HSDNM.
	D – ACS 15/15; Alert, Oriented.
	PLAN
	(1) Repeat labs today at 15:30. Monitor Hb levels.
	(2) Continue antibiotic Cefazolin
	(3) Wardable once beds are available.
	(4) Chase TTE report, c/s reports.
26/2/24 1430	Pt has not taken his Dexamfetamine x3 as he has taken 3 this morning. He explained that because he is staying in bed not doing anything, he prefers not to take only x3 tablets. x3 chicken ~~ordered~~ ordered by N

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Page 1 of 2

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5.48. Progress Report (FMH516500)

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PROGRESS REPORT

WARD: ICU Bed: 6

Date	For all entries: Print Name, Signature and Designation
26/2 1600	ICU HMO4 - Blackwood
	Procedure supervised by Dr Nanji - ICU reg
	Suturing of left antecubital fossa lacerations - CT angio of L) arm - no vascular injury - neurovascularly intact.
	O/E: multiple jaggard lacerations clean no ooze / active bleeding
	sterile tech Aseptic technique throughout. 2mLs of 1% xylocaine injected for local anaesthesia. cleaned ū sterile normal saline irrigated ū normal saline. 7 x sutures used to repair lacerations (3-0 Nylon). Nil immediate complications, no bleeding on pack. Sutures dressed ū betadine & bandaged
	Plan: 1) Sutures need to remain in for 7 days 2) Arm needs to remain straight (NO bending) for 7days 3) wound R/V by psych medical team. Blackwood ext 8504

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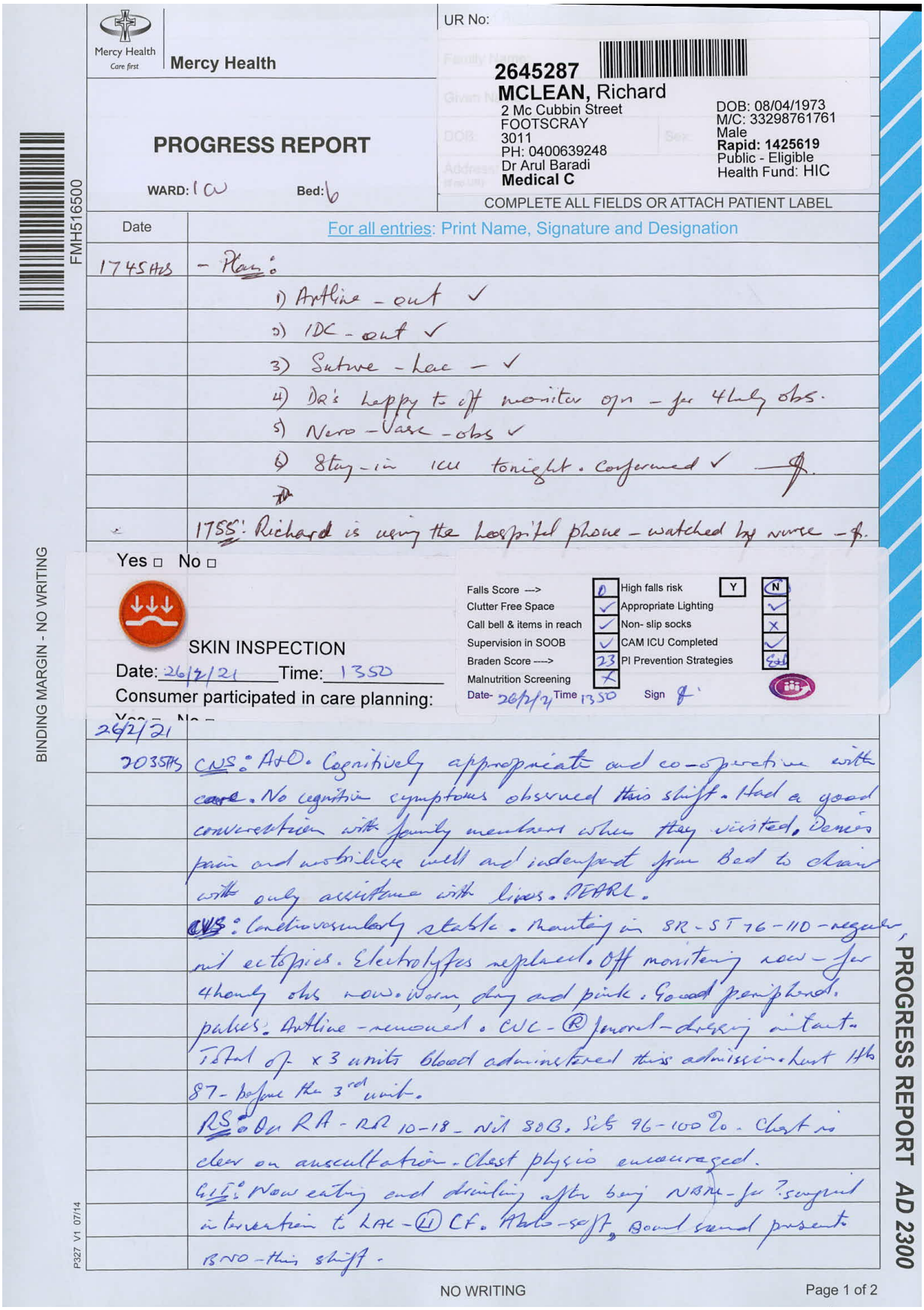
5.49. Progress Report (FMH516500)



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PROGRESS REPORT

WARD: ICU Bed: 6

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Date	For all entries: Print Name, Signature and Designation
1745Hrs	- Plan: 1) Artline - out ✓ 2) IDC - out ✓ 3) Suture - Lac - ✓ 4) Dr's happy to off monitor ogn - for 4hrly obs. 5) Neuro - Vasc - obs ✓ 6) Stay - in ICU tonight. Conferred ✓ —ℱ. 1755: Richard is using the hospital phone - watched by nurse - ℱ.

Yes ☐ No ☐

SKIN INSPECTION
Date: 26/2/21 Time: 1350
Consumer participated in care planning:
Yes – No –

Falls Score --->	0	High falls risk	Y	N
Clutter Free Space	✓	Appropriate Lighting		✓
Call bell & items in reach	✓	Non- slip socks		X
Supervision in SOOB	✓	CAM ICU Completed		✓
Braden Score ---->	23	PI Prevention Strategies		Gel
Malnutrition Screening	X			

Date- 26/2/21 Time 1350 Sign ℱ

26/2/21	
2035Hrs	CNS: A+O. Cognitively appropriate and co-operative with care. No cognitive symptoms observed this shift. Had a good conversation with family members when they visited. Denies pain and mobilises well and independent from Bed to chair with only assistance with lines. PEARL. CVS: Cardiovascularly stable. Monitor in SR - ST 76-110 - regular, nil ectopics. Electrolytes replaced. Off monitor now - for 4hourly obs now. Warm, dry and pink. Good peripheral pulses. Artline - removed. CVC - ® femoral - dressing intact. Total of x3 units blood administered this admission. Last Hb 87 - before the 3rd unit. RS: On RA - RR 10-18 - Nil SOB. Sats 96-100%. Chest is clear on auscultation. Chest physio encouraged. GIT: Now eating and drinking after being NBM - for ?surgical intervention to LAC - (1) Cf. Halo - self, Bowel sound present. BNO - this shift.

P327 V1 07/14

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PROGRESS REPORT AD 2300

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Page 1 of 2

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5.50. Progress Report (FMH516500)

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PROGRESS REPORT

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Date	For all entries: Print Name, Signature and Designation
	Renal: IDC – removed. Voided post removal.
	RT Wound: Sutured by DR. Trobel under aseptic procedure. (see doctors notes). Dry dressing and bandaged.
	Integument: Generally intact. — JWanginbeta Jotirfini Wanginbeta CCRN.

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Page 2 of 2 NO WRITING

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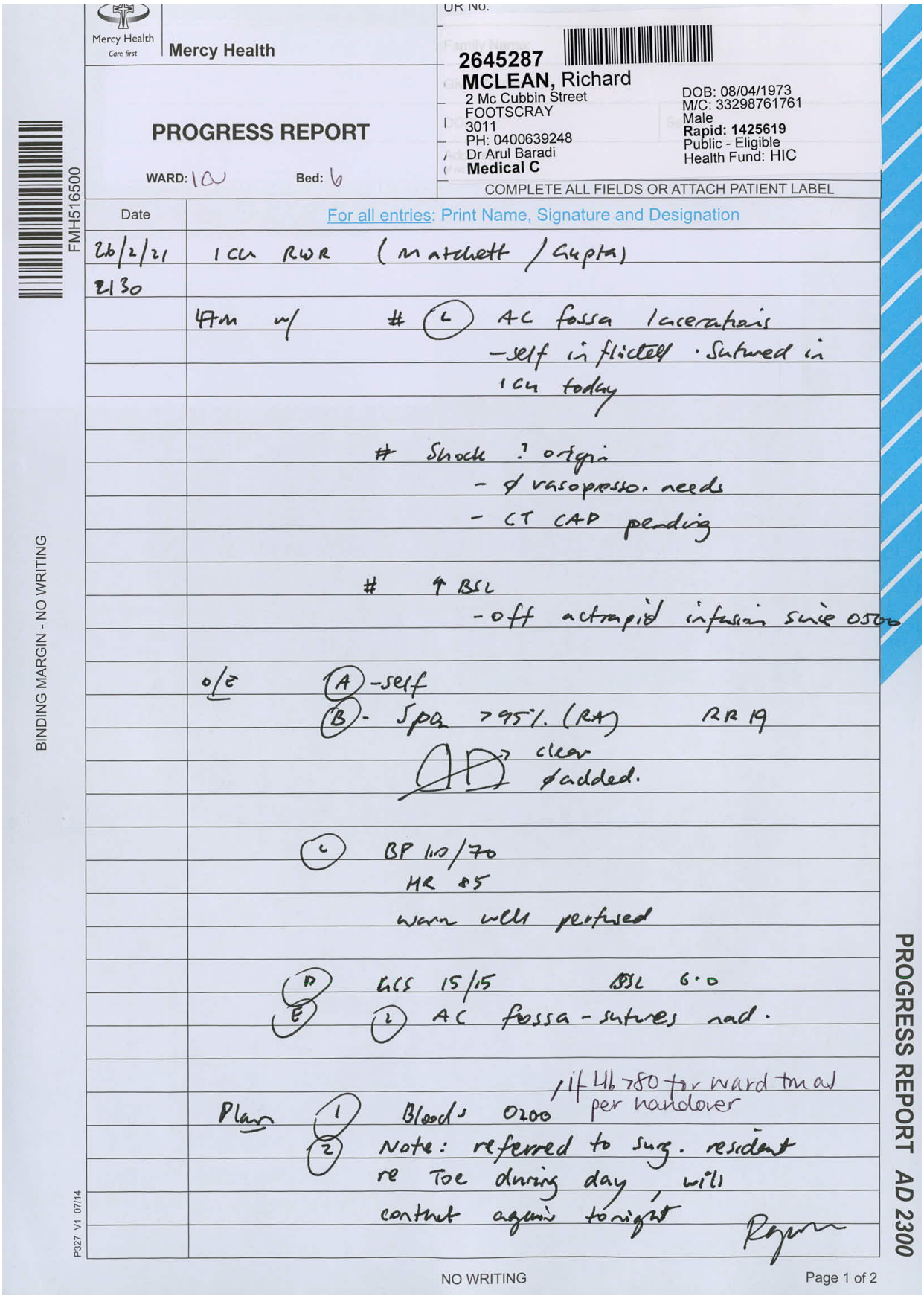
5.51. Progress Report (FMH516500)



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PROGRESS REPORT

WARD: ICU Bed: 6

FMH516500

BINDING MARGIN - NO WRITING

Date	For all entries: Print Name, Signature and Designation
26/2/21 2130	ICU RWR (Matchett / Gupta)
	47M w/ # (L) AC fossa lacerations - self in flicted · Sutured in ICU today
	# Shock ? origin - ↓ vasopressor needs - CT CAP pending
	# ↑ BSL - off actrapid infusion since 0500
	o/e (A) - self (B) - SpO2 >95% (RA) RR 19 clear ↓ added.
	(C) BP 110/70 HR 85 warm well perfused
	(D) GCS 15/15 BSL 6.0 (E) (2) AC fossa - sutures rad.
	Plan (1) Bloods 0200 if Hb <80 for ward tm as per handover (2) Note: referred to surg. resident re Toe during day, will continue again tonight Regan

PROGRESS REPORT AD 2300

P327 V1 07/14

NO WRITING

Page 1 of 2

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5.52. Progress Report (FMH516500)

Mercy Health
Care first

Mercy Health

UR No:

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

PROGRESS REPORT

WARD: ICU Bed: 6

Date	For all entries: Print Name, Signature and Designation
	Yes ☐ No ☐
	SKIN INSPECTION Date: 26/2/21 Time: 2000 Consumer participated in care planning: Yes ✓ No ☐

Falls Score --->	3	High falls risk	Y / N
Clutter Free Space	✓	Appropriate Lighting	✓
Call bell & items in reach	✓	Non- slip socks	
Supervision in SOOB	ND	CAM ICU Completed	✓
Braden Score ---->	23	PI Prevention Strategies	✓
Malnutrition Screening			

Date- 26/2/21 Time 2000 Sign PS

27/2/21 0500	NURSING NOTE 46 yo male admitted for haemodynamic support post collapse and ? Shock, Self harm. Allergies, Bee stings, Augmentin. Fluid resus. CNS: Alert + oriented, appropriate and co-operative overnight. GCS 15, PEARL 3↑. Comm -ve, RASS 0. Denies pain. Regular analgesia given. Slept well overnight. CVS: monitoring ceased at 2000 as per ICU team. for 4 hrly visual obs. + NV obs on (L) hand. Cwms well perfused, CRT < 3 sec throughout, Nil oedema, CVC on (Rt) Fem vein flushed and patent, only brown lumen yielding blood, 4 lumens present NAD. Nil PIVC. Resp: chest clear, Nil Resp distress Nil ↑WOB, SpO2 > 95% on RA. GIT: tolerates fluids o/w. Abdo SNT. BS +ve. BNO. Renals: PUSB Continent. Renal function monitored, improving. Integ: Skin NAD, Sacrum + heels NAD. Indep ↑ POC, RIB all night, Dx on (LT) ACF NAD intact, Nil ooze o/w clear plan from surgical team. Meds: Bloods sent at 0300 hrs., Hb: 87, Electrolytes monitored. Other: Nil suicidal ideation or behaviour issues. Overnight ——— PS Dee ——— Blue Rinse Cwm

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Page 2 of 2 NO WRITING

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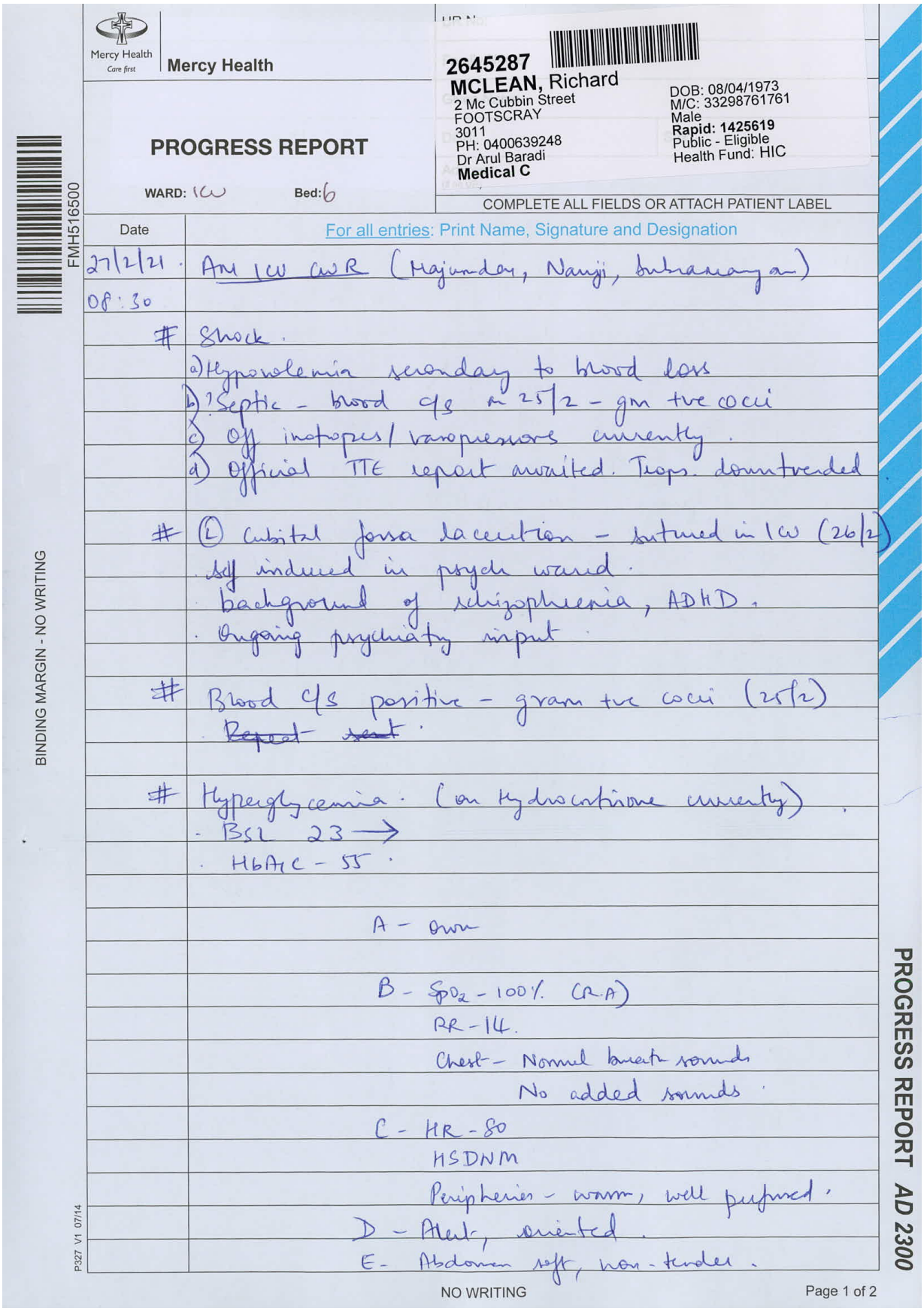
5.53. Progress Report (FMH516500)



Mercy Health
Care first

Mercy Health

PROGRESS REPORT

WARD: ICU Bed: 6

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COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH516500

BINDING MARGIN - NO WRITING

Date	For all entries: Print Name, Signature and Designation
27/2/21	AM ICU CWR (Majumdar, Nanji, Subramanyan)
OP: 30	
#	Shock.
	a) Hypovolemia secondary to blood loss
	b) ? Septic - blood c/s on 25/2 - gm +ve cocci
	c) Off inotropes / vasopressors currently.
	d) Official TTE report awaited. Trops. downtrended.
#	(L) Cubital fossa laceration - sutured in ICU (26/2)
	Adj induced in psych ward.
	background of schizophrenia, ADHD.
	Ongoing psychiatry input.
#	Blood c/s positive - gram +ve cocci (25/2)
	Repeat ~~sent~~.
#	Hyperglycemia. (on hydrocortisone currently)
	- BSL 23 →
	- HbA1C - 55.
	A - Own
	B - SpO_2 - 100% (RA)
	RR - 14.
	Chest - Normal breath sounds
	No added sounds.
	C - HR - 80
	HSDNM
	Peripheries - warm, well perfused.
	D - Alert, oriented.
	E - Abdomen soft, non-tender.

PROGRESS REPORT AD 2300

P327 V1 07/14

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Page 1 of 2

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5.54. Progress Report (FMH516500)

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Health Fund: HIC

PROGRESS REPORT

WARD: ICU Bed: 6

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Date	For all entries: Print Name, Signature and Designation
27/2/24. 8:30 Am	AM ICU ward (contd). ECG → Corrected QTc - 410 (Normal) (L) arm → wound dressing in situ. No leakage. PLAN - (1) D/w a transfer under general surgery Dr Rafique's team - considering patient has blood cs positivity & will require IV antibiotics. Full reports awaited (2) To review and cease hydrocortisone tomorrow (total duration 48 hours) (3) Vancomycin 2g stat dose to be given today. Chase final blood cs reports. (4) Wandable. Subs.

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Page 2 of 2 NO WRITING

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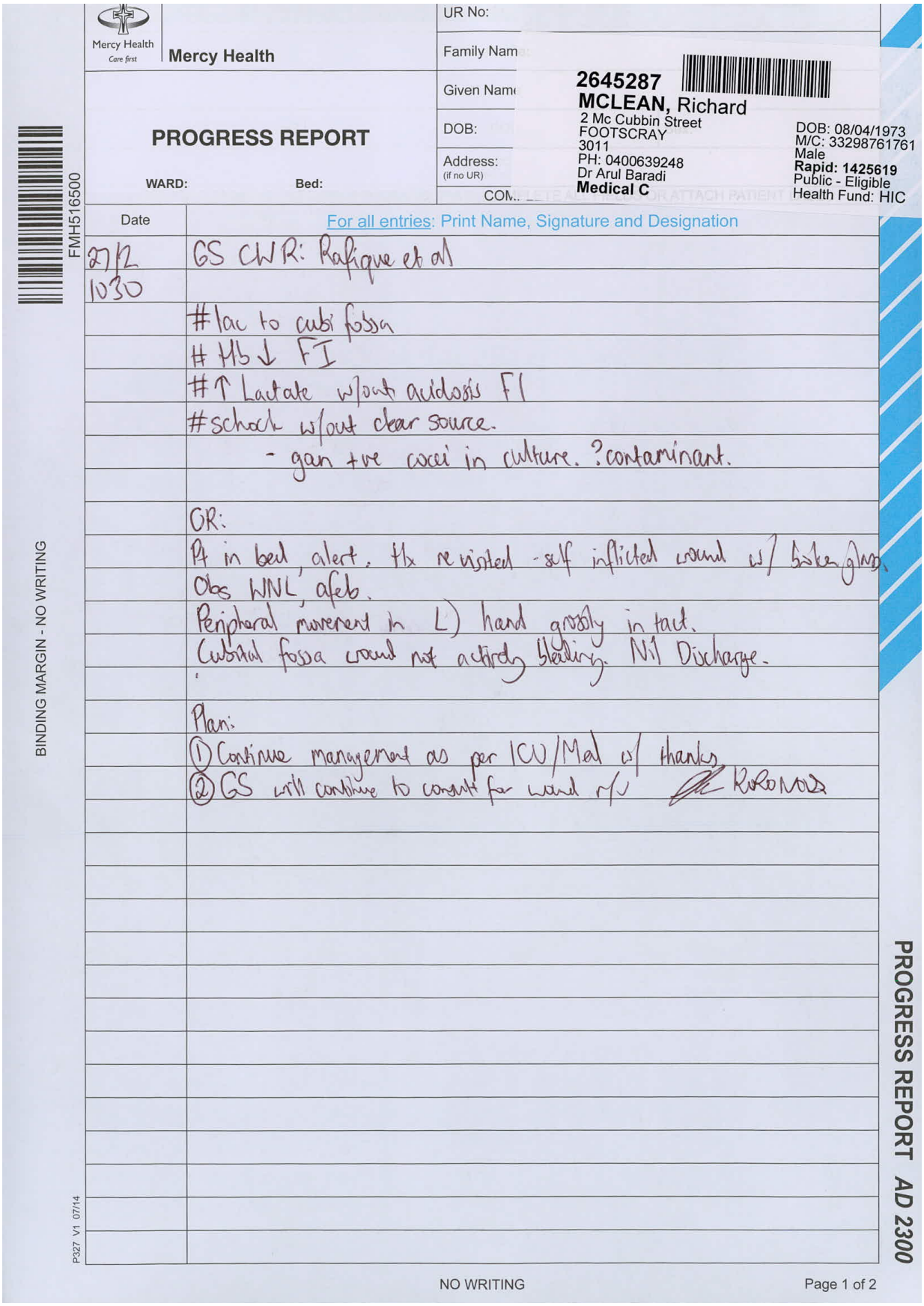
5.55. Progress Report (FMH516500)



Mercy Health
Care first

Mercy Health

UR No:

Family Nam

Given Nam

DOB:

Address:
(if no UR)

2645287
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PH: 0400639248
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Male
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Health Fund: HIC

PROGRESS REPORT

WARD: Bed:

FMH516500

Date	For all entries: Print Name, Signature and Designation
27/12 1030	GS CWR: Rafique et al
	#lac to cubi fossa
	# Hb ↓ FI
	#↑ Lactate w/out acidosis FI
	#shock w/out clear source.
	- gan +ve cocci in culture. ?contaminant.
	OR:
	Pt in bed, alert. Hx resisted - self inflicted wound w/ broken glass.
	Obs WNL, afeb.
	Peripheral movement in L) hand grossly intact.
	Cubital fossa wound not actively bleeding. Nil Discharge.
	Plan:
	① Continue management as per ICU/Med c/f thanks
	② GS will continue to consult for wound r/v

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5.56. Progress Report (FMH516500)

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UR No: 2645287

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2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

PROGRESS REPORT

WARD: Bed:

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Date	For all entries: Print Name, Signature and Designation

5.57. Progress Report (FMH516500)

Mercy Health
Care first

Mercy Health

UR No:

2645287
MCLEAN, Richard
2 Mc Cubbin Street
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3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

PROGRESS REPORT

WARD: 7 cen Bed: 6

FMH516500

BINDING MARGIN - NO WRITING

Date	For all entries: Print Name, Signature and Designation
27/2/21 1745	NSg AM: patient ē collapse and shock ēn Mental health cent. Pull rescue. CNS: Alert. Oreinted GCS-15. RASS-0. CAM Scre Neg Mobēlises ēndependently. On assessment Orde. CVS: Normotensive Vitally Stable afebrile. ϕ oedema. RJ femoral ccdc x4 Wcreen patent. Flushed. Resp: Own spont- breaths. Maintains Sats on RA. ϕ SOB /A Fed NOB. GIT: ASNT. Eating & drinking well. BxIO. Renal: PUIB. ϕ issues. Metabolic: electrolytes replaced as documented. BSL - NAD. Wnty: Hygiene attended. Independent - PAE. Social: patient pleasant, certainty about mobile being locked up. Explanation given regarding the same. patient constantly talks over the hospital phone to friends and family. All queshions answered. Pcce team updated NOK. — Thm CCRN

Yes ☐ No ☐

SKIN INSPECTION
Date: 27/2/21 Time: 1600
Consumer participated in care planning:
Yes ☑ No ☐

Falls Score --->	0	High falls risk	Y	N
Clutter Free Space	✓	Appropriate Lighting		✓
Call bell & items in reach	✓	Non- slip socks		✓
Supervision in SOOB	✓	CAM ICU Completed		✓
Braden Score --->	16	PI Prevention Strategies		✓
Malnutrition Screening	✓			

Date- 27/2 Time 1600 Sign

P327 V1 07/14

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Page 1 of 2

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5.58. Progress Report (FMH516500)

Mercy Health
Care first

Mercy Health

PROGRESS REPORT

WARD: Rees Bed: 6

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2645287
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PH: 0400639248
Dr Arul Baradi
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DOB: 08/04/1973
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Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Date	For all entries: Print Name, Signature and Designation
27/2/21 1900	Add on: patient's belongings from mental health ward received @ 1900hrs. patient wants to use mobile phone. As per Rees Consultant pt is OK to use. mobile phone still @ mental health ward. Advised PSA to bring @. Other belongings will be handed over to PN night staff to document. — [illegible] Add on: Left arm LA dressing changed. Sutures intact. — [illegible]
27.2.21 19.45	Liliana Sousa Nanji (IW Reg) Staphylococcus lugdunensis on blood culture from 25/2 Discussed w/ ID Consultant Dr Darby: - likely source is laceration - also need to look for infective endocarditis, osteomyelitis, etc - continue Cephazolin - happy with single dose of vancomycin given he's well - chase sensitivities tomorrow (small percentage are resistant to cephazolin) - daily blood cultures - happy to keep CVC and art line in Mainak informed and agreed with above Liliana Nanji IW

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Page 2 of 2 NO WRITING

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5.59. Progress Report (FMH516500)

40260

Mercy Health

Patient Progress

Date & Time: 27/02/2021 6:11:31 PM

Completed: 27/02/2021 6:12:00 PM

2645287

UR Number :
Surname : McLean
First Name : Richard
DOB : 8/04/1973 **Gender** Male
Address : 2 Mc Cubbin Street
3011 Footscray
VIC
Australia

Home Phone : 0400639248 **Work Phone :**

Address 2 Mc Cubbin Street
3011 Footscray
VIC
Australia

Home Phone (AH) 0400639248 **Work Phone (BH)**

Statewide UR: 1425619
Date of Entry: 27/02/2021
Time of Entry: 18:11
Clinician: Mustafa Abuqlam
Designation: Registrar
Team: Clare Moore Building Inpatient Team

27/02/21 1515 (Retrospective)
Psych Reg Note – Mustafa Abuqlam

Richard is currently in ICU.

Spoke with ICU Reg (Liliana) + ICU Consultant:
Has apparently had x3 bags of packed red blood cells, and 5L of IV fluids. They believe the low haemoglobin of ~89 is likely dilutional.
CT of left arm shows intact neurovasculature.
He has grown a gram +ve cocci in his blood culture → ?contaminant vs staph aureus bacteraemia, source is possibly from his suicide attempt with a pen. Currently on IV ABx.
Will likely need to be on surg ward for ~3 more days for IV ABx.

On Review:
Richard appeared to be updating his blog before I entered.
Richard has a long list of things he would like to happen, including seeing social work and having his mobile phone back so he can organise to have his lawn mown and bills paid.
He states his dog is under the care of someone now.
Richard had a notepad in front of him with a list of more than 10 requests.
Richard occasionally wrote what the interviewer was saying on his notepad.
He says:

- "My life will fall apart if I don't get the phone.
- "You're a mediocre person, I'm an up and down person"
- "Some people's perceptions are broader than others"
- Asked the interviewer to document he said "Why am I in here" and regarding the withholding of his dexamphetamine, "is that malicious?"

Title: Medical Officer Review **Page No.** 1
Created By: Mustafa Abuqlam (Registrar)
Printed By: Jeril Thomas (Nurse)

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5.60. Progress Report (FMH516500)

40260

Mercy Health

Patient Progress

Date & Time: 27/02/2021 6:11:31 PM

Completed: 27/02/2021 6:12:00 PM

2645287

UR Number :
Surname : McLean
First Name : Richard
DOB : 8/04/1973 **Gender** Male
Address : 2 Mc Cubbin Street
3011 Footscray
VIC
Australia

Home Phone : 0400639248 **Work Phone :**

Address 2 Mc Cubbin Street
3011 Footscray
VIC
Australia

Home Phone (AH) 0400639248 **Work Phone (BH)**

- "People are trying to kill me", in reference to the treating psychiatrist.
- "You're trying to lock me up"

When asked about suicidal thoughts, he stated "I choose not to answer".
When asked about angry thoughts, he stated he has angry thoughts towards the psychiatric doctors who reviewed him prior to his suicide attempt, including the interviewer.

At a certain point he refused to answer any further questions and stated "Get the fuck out".
Interview terminated early.

Med Chart:
Patient now having his dexamphetamine

MSE:
Intense eye contact
Psychomotor agitation (restless legs)
Speech increased in rate and volume
Agitated affect, congruent with thought content
Not obviously responding to internal stimuli
Fast thought stream, circumstantial speech.
Thought content – fixated on getting mobile phone back. Angry thoughts towards many of his treating doctors, especially the psychiatric doctors. Chose not to answer re: suicidal ideation.
Insight/Judgment – impaired
Cognition – grossly intact

Impression: High risk of harm to self, moderate risk of damage to own reputation, otherwise low risk

Discussed with Dr Lograsso re: mobile phone use
- Can return mobile phone to Richard
- If he voice/video records patients or staff, then highly consider confiscating it due to concerns of confidentiality and privacy of others

Informed the ICU consultant of above

Title: Medical Officer Review **Page No.** 2
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Printed By: Jeril Thomas (Nurse)

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Mercy Health
Care first

5.61. Progress Report (FMH516500)

40260

Mercy Health

Patient Progress

Date & Time: 27/02/2021 6:11:31 PM

Completed: 27/02/2021 6:12:00 PM

2645287

UR Number :
Surname : McLean
First Name : Richard
DOB : 8/04/1973 **Gender** Male
Address : 2 Mc Cubbin Street
3011 Footscray
VIC
Australia

Home Phone : 0400639248 **Work Phone :**

Address 2 Mc Cubbin Street
3011 Footscray
VIC
Australia

Home Phone (AH) 0400639248 **Work Phone (BH)**

Plan:

- Continue CL Psych reviews
- Social work to kindly see on Monday
- Can return mobile phone to Richard. If he voice/video records patients or staff, then highly consider confiscating it due to concerns of confidentiality and privacy of others.
- As per Dr Lograsso – When eventually coming back to psych ward, should be in ICA with 1-1 nurse special due to risk of harm to self
- Advised patient that he can request his medical records through the freedom of information act

AbuqlaM

Title: Medical Officer Review **Page No.** 3
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Printed By: Jeril Thomas (Nurse)

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5.62. Progress Report (FMH516500)

2645287
MCLEAN, Richard
2 Mc Cubbin Street
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3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

Assessment

Date & Time: 23/02/2021 9:45:39 PM
Completed: 24/02/2021 12:37:00 AM
Staff Name: MachemB

Address 2 Mc Cubbin Street
3011 Footscray
VIC
Australia

Home Phone : 0400639248 **Work Phone :**

Address 2 Mc Cubbin Street
3011 Footscray
VIC
Australia

Home Phone (AH) 0400639248 **Work Phone (BH)**

Screening Register

Screening Detail

Presenting Problem:

Introduction:
Clinician Name: Nicole Withers
Position: P3
Date: 14/2/2021
Time: 11:45

Situation
REFERRAL REASON AND / OR REASON FOR CALL:
P/C from Allanah; 0403117361
Concerned friend, reported to be a mental health therapist. Clarified is an NDIS art therapist.
States that she is worried that Richard's MH may be declining as has noticed on his website that he is escalating in distress and listing suicide dates with increased "venom" towards former GP and health care professionals.
States that he is "paranoid" about being refused services.
Making reference to ASIO
tting up $5 million fundraisers on Facebook.
reports he has increased stress: lost job as NDIS peer worker (as misrepresenting himself allegedly as therapist) and also has had car repossessed.
States Richard to be usually "quite vocal" but concerned that he is now placing himself in danger of vulnerability and deterioration.
Spoke of anonymity in referral, however, when explaining need to disclose if asked re referrer details (as cold calls not appropriate) Allanah agreed to be named.

Relevant Background
Current legal status: VOL

CMI/Master care Search:
Nil CMI hx.
Referred to CM in Sept 2020, not accepted for CM.

Diagnosis: As per file: Schizophrenia (disputed) and ADHD
ALERTS: Nil formal listed
Current Medications: did not disclose full medication list. Stated he is on Dexamphetamine and this is primary treatment.
Relevant Medical Hx: Did not disclose.

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Printed By: Bryan Dimabuyo (Nurse)

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5.63. Progress Report (FMH516500)

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Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

Assessment

Date & Time: 23/02/2021 9:45:39 PM
Completed: 24/02/2021 12:37:00 AM
Staff Name: MachemB

Address 2 Mc Cubbin Street
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Home Phone : 0400639248 **Work Phone :**

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Current supports: GP and PP but states poor therapeutic relationship with same.

Relevant Psychosocial Factors: isolated. Hx of childhood sexual abuse, reported limited friendships, invalidated by medical professionals. Financial debt: $50K

Relevant Risk Hx : Hx of suicide attempt via overdose 3 years ago. States now has plan to kill himself (did not disclose concrete suicide plan) on his birthday (8/4) this year.

Substance abuse current use and relevant Hx: opportunistic methamphetamine and amphetamine misuse reported.

Assessment –
MENTAL STATE EXAMINATION:

Behaviour: cooperative with underlying irritability
Speech: Slight pressure, talking over writer at times. Difficult to distract.
Reported Mood: ventilated at length re: marginalization, and longstanding anxiety, grief and pain.
Thought Form: Nil clear FTD

Thought Content:
troduced self and reason for call. Willing to speak to writer.
poke at length regarding "conspiracy to silence" him and marginalisation by health professionals.
Spoke of intention to kill himself on 8/4 as he does not feel he can live in a world "like this" anymore.
Spoke of medical professionals as the "right wing of shamanism" and that Psychiatrists are the "Nazi's of them all", focusing in on the perceived power imbalance in appointments and belief that medications are designed to dull people to make them normal.
States that he is a "hedonist" and that he accepts that some of his actions are done to "self medicate" reported anxiety, grief and pain, describing long history of loss, abuse (since childhood), suicide of friend, invalidation by family, LGBTI discrimination, poor relationships etc.
Reported to be a mental health advocate for 20+ years and that he is "famous" having written a book about his experiences.
States his website: **richmclean.com.au** outlines his frustration with services to date, explaining he had been victim of medical negligence by former GP and that his attempts to pursue claim legally have been blocked by those in power with cover up of situation. States that he advised GP of plan to overdose 3 years ago and was ignored, given medication which he then used to overdose. Believes there is a large cover up/conspiracy by government, including AHPRA up to minster for health and he can no longer obtain legal supports to assist him as he is "blacklisted".
States he also is struggling to obtain any health care supports due to this interaction with GP and subsequent issues with PP Dr David Horgan. (all listed on his website publically with voice recordings of alleged inappropriate interactions).
Spoke then of former partner, who is allegedly working for ASIO and despite this is a drug dealer with extensive tax

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Printed By: Bryan Dimabuyo (Nurse)

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5.64. Progress Report (FMH516500)

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Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

PROGRESS REPORT

WARD: ICU Bed: 6

FMH516500

BINDING MARGIN - NO WRITING

Date	For all entries: Print Name, Signature and Designation
27/2/21 2100	ICU RWR PM (Matchett et al).
	47M w/ # Shock ? origin - no vasopressor needs now
	# (L) AC fossa lacerations - self inflicted - sutured in ICU ~~27~~ 26/2/21
	# Staph lugdunesis bacteremia - culture on 25/2 - subsequent cult. (-ve) - on cephazolin . START vanc given today - source: likely from lacerations
	# ↑ BSL - on hydrocortisone 50mg OD
	O/E (A) -self (B) - SpO_2 97% RA RR 17 clear ✓added.
	(C) BP 140/90 HR 90
	(D) GCS 15/15
	(E) (L) AC fossa lac (sutured)

PROGRESS REPORT AD 2300

P327 V1 07/14

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Page 1 of 2

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PROGRESS REPORT

WARD: ICU Bed: 6

Date	For all entries: Print Name, Signature and Designation
27/2/21	Plan (1) Daily cultures as per ID. (2) TOE (monday). (3) Chase culture speciation. (4) wardable Rpgpt.
28/2/21 0500	NSCN ND: Pt day 2 ICU post intentional self harm laceration & shock. CND = CVS IS PEARL, Pt denies pain, alert & oriented - restless initially though pleasant & not agitated. Given regular & PRN diazepam & pt slept well past. Pt on TTO (MTHA 11d). Has voiced plans to be compliant & medications & not currently planning further self harm. CVS = Afebrile & vitally stable on Nora cbs. R/J femoral CVC insitu - dressing secured CTH F x 4 lumens Pt cont IVABX & blood cultures sent - Lactate 2.2 on VBG, Hb 92. Resp = Maintaining airway, Chest clear, SpO_2 >92% RA, nil cough/SOB. GIT = good diet, nil nausea. BO mane 27/2. Renal = UOP drinking adequately, FBC ceased. Nil edema. Integ = 4cm laceration dressed - distal neurovasc obs stable, NAD. Otherwise skin DTL. Self attending PAC. Mob = Pt independent on feet. Guessing CCA

Falls Score --->
Clutter Free Space
Call bell & items in reach
Supervision in SOOB
Braden Score --->
Malnutrition Screening
Date- 27/2 Time 2000

		Y	N
1	High falls risk		
✓	Appropriate Lighting		
✓	Non- slip socks		
✓	CAM ICU Completed		
23	PI Prevention Strategies		
0			

Sign

Yes ☐ No ☐

SKIN INSPECTION
Date: 27/2/21 Time: 2000
Consumer participated in care planning:
Yes ☑ No ☐ several areas inspection not attempted

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Page 2 of 2

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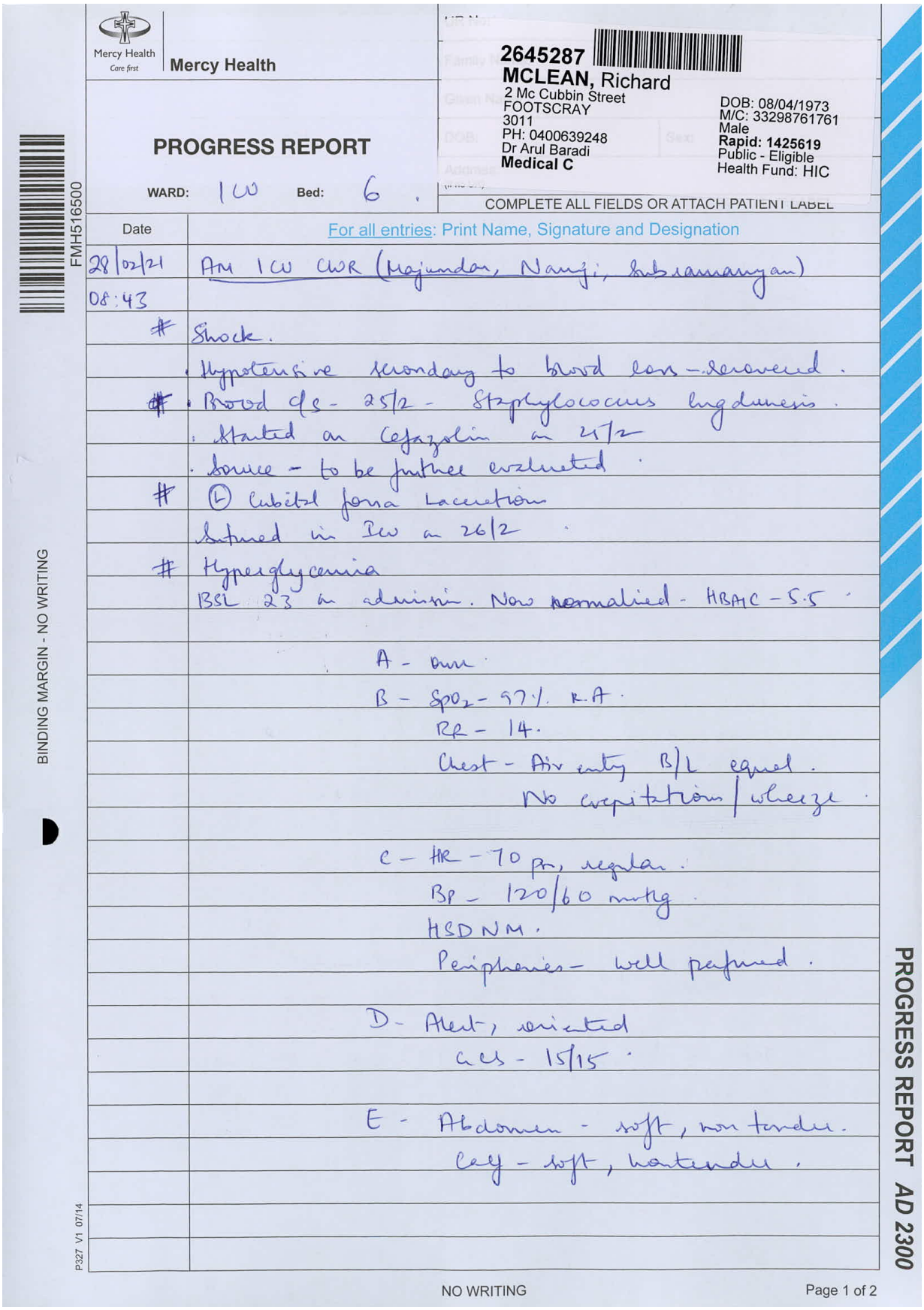
5.66. Progress Report (FMH516500)



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Mercy Health

PROGRESS REPORT

WARD: ICU Bed: 6

2645287
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FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH516500

BINDING MARGIN - NO WRITING

Date	For all entries: Print Name, Signature and Designation
28/02/21 08:43	AM ICU CWR (Majumdar, Nanji, Subramanyan)
#	Shock.
	• Hypotensive secondary to blood loss - recovered.
#	• Blood c/s - 25/2 - Staphylococcus lugdunesis.
	• Started on Cefazolin on 25/2
	• Source - to be further evaluated.
#	(L) Cubital fossa Laceration
	Sutured in ICU on 26/2.
#	Hyperglycemia
	BSL 23 on admission. Now normalised - HBA1C - 5.5.
	A - Own.
	B - SpO_2 - 97% RA.
	RR - 14.
	Chest - Air entry B/L equal.
	No crepitations / wheeze.
	C - HR - 70 bpm, regular.
	BP - 120/60 mmHg.
	HSDNM.
	Peripheries - well perfused.
	D - Alert, oriented
	GCS - 15/15.
	E - Abdomen - soft, non tender.
	Leg - soft, non tender.

PROGRESS REPORT AD 2300

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5.67. Progress Report (FMH516500)

Mercy Health
Care first

Mercy Health

PROGRESS REPORT

WARD: Bed:

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2645287
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Date	For all entries: Print Name, Signature and Designation
28/2/21. 08:43	AM ICU WR (contd) - Bedside ophthalmoscopy (undilated) LE & RE – Normal cup & disc. No edema No flame shaped hemmhages. Normal venous pulsation visible Palate – No petechiae. Skin – Clinically no foci of infection newly identified. (L) arm laceration – healing in situ. no soakage. PLAN: (1) Chase final culture & sensitivity reports. Continue cefazolin. CVC out today. Insert IVC. (2) Repeat c/s from the cannula (3) Discuss with ID regarding further evaluation for source (Skin likely) – TOE, Bronchoscopy etc. (ongoing consultation) (4) To modify antibiotics ~~regarding~~ based on final cultures & to decide on duration (5) Cease hydrocortisone. Wandable. The patient has been counselled regarding his condition & blood culture growth of staphylococcus lugdunensis. Gc's were clearly explained to him.

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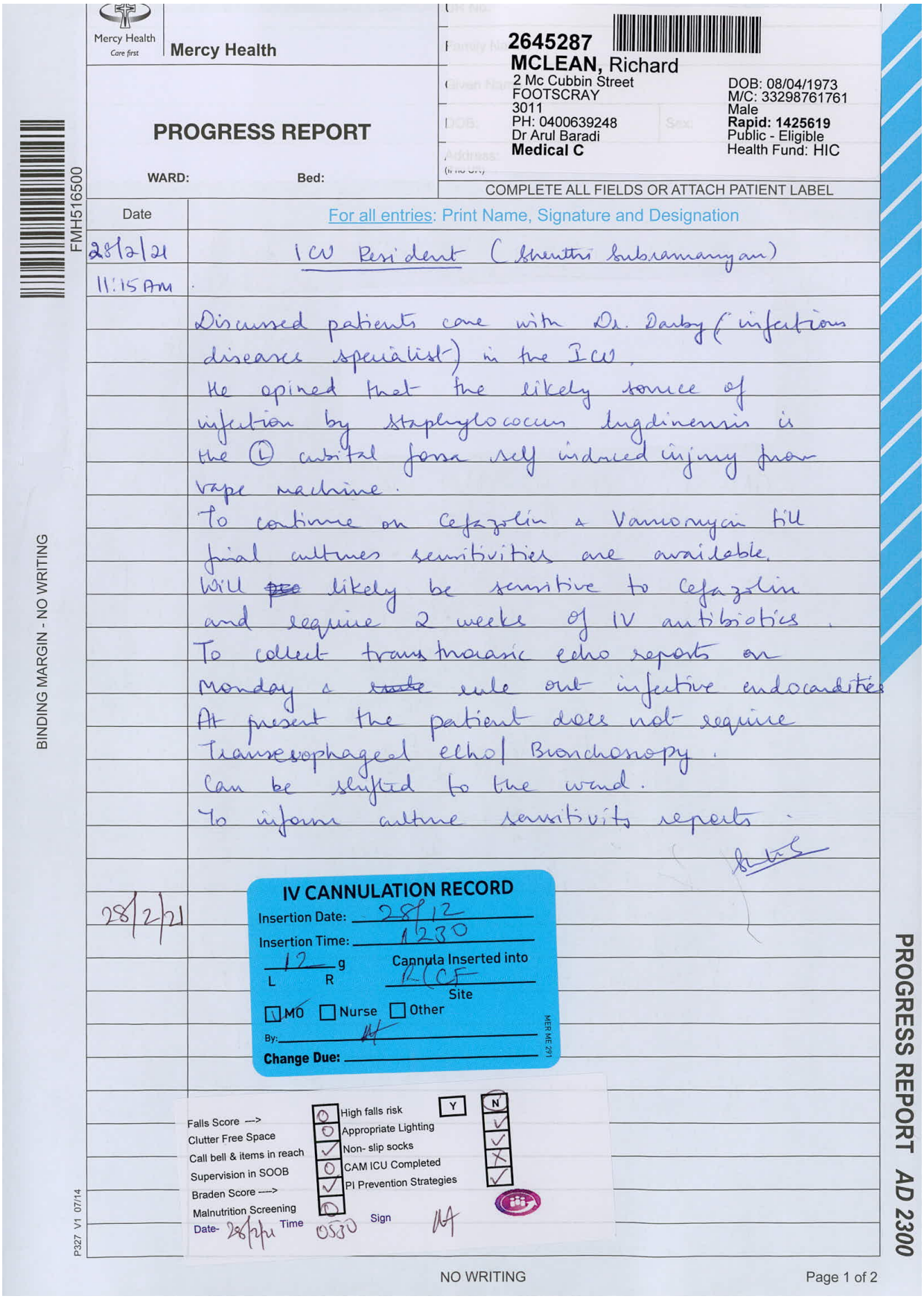
5.68. Progress Report (FMH516500)



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PROGRESS REPORT

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Date	For all entries: Print Name, Signature and Designation
28/2/21 11:15 AM	ICU Resident (Shruthi Subramanyam) Discussed patients care with Dr. Darby (infectious diseases specialist) in the ICU. He opined that the likely source of infection by staphylococcus lugdinensis is the (L) cubital fossa self induced injury from vape machine. To continue on Cefazolin & Vancomycin till final cultures sensitivities are available. Will ~~pt~~ likely be sensitive to Cefazolin and require 2 weeks of IV antibiotics. To collect transthoracic echo reports on Monday & ~~route~~ rule out infective endocarditis At present the patient does not require Transesophageal echo/ Bronchoscopy. Can be shifted to the ward. To inform culture sensitivity reports. Shruthi
28/2/21	**IV CANNULATION RECORD** Insertion Date: 28/2 Insertion Time: 1230 12 g L R Cannula Inserted into R CF Site ☑ MO ☐ Nurse ☐ Other By: Change Due: MER ME 291
	Falls Score ---> 0 Clutter Free Space 0 Call bell & items in reach ✓ Supervision in SOOB 0 Braden Score ---> ✓ Malnutrition Screening 0 Date- 28/2/21 Time 0530 Sign High falls risk Y N Appropriate Lighting ✓ Non- slip socks ✓ CAM ICU Completed N/A PI Prevention Strategies ✓

PROGRESS REPORT AD 2300

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5.69. Progress Report (FMH516500)

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PROGRESS REPORT

WARD: Bed:

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Date	For all entries: Print Name, Signature and Designation
28/2/24 1536	Nursing Notes Day Shift: Admitted on 25/2/24 - found collapsed in shower in mental health unit. fall risks. - Trademark sock in place.

SKIN INSPECTION
Date: 28/2/24 Time: 1530
Consumer participated in care planning:
Yes ☑ No ☐

CNS: CNS: GCS - 15; alert and oriented. Ambulant independently. Resp: On room air. O2 sats 100%. resp rate 15/min. Nil devices. CVS: Pink, warm cap cpil brisk. R/J femoral CVC removed. R/J CF canula @ 2L inserted. Patent. H/R - stable 64bpm BP mildly hypotensive this afternoon. ≈ 158 systolic. Patient reports has been hypotensive in the past. GU: Voiding in the toilet. Drinking well fluids, tea and able tea fluids. Nil obvious oedema. GIT: Tolerated diet. BGL 6.9. Metabolic: Afebrile. Social Visited by his mum and dad. Brought for him clothes and food. Nil contraband (drugs) noted on the things brought from home. Others: Behaviours: Patient calm, talking about when he has a chance. Compliant with medications. Requested coffee. Patient escorted to cafeteria with myself (nurse) and security personal x1 to buy coffee and get money from his ATM. Happy to be able to buy coffee. Has been on the phone most of the day; speaking to friends regarding his condition. Patient wants a social worker and a Lawyer. Informed he can get social worker tomorrow. Also wants HES claim form filled up for his income protection. Patient has been spoken to by myself and doctor to wait tomorrow for confirmation of g

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5.70. Progress Report (FMH516500)

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Date	For all entries: Print Name, Signature and Designation
28/2/21	NSg noted ct: duration of treatment. However patient still insist on having the form done today. I have informed him to wait tomorrow - to confirm duration of treatment and social worker. Xrays had been charged. Patient requested for stool as am. clean line has been issued for him to stool We ready. All fluids ceased ATOR — Renadale CCRN AT
Addl: 1629:	Had shower afternoon. 4) of dressing changed post shower. x 1 b/c soiled post cannulation — AT Renadale. Patient's Wallet has 17 aud — Renadale AT CCRN.
28/2/21 17:51	ICU Resident (Shwetha Subramanyam)
	Culture & sensitivity reports (blood) discussed with microbiology. They opined that Staph. lugdunensis was sensitive flucloxacillin and hence would be sensitive to Cephazolin & this does not require specific sensitivity testing any further. To stop Vancomycin and continue on Cephazolin.

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PROGRESS REPORT AD 2300

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Page 1 of 2

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5.71. Progress Report (FMH516500)

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COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Date	For all entries: Print Name, Signature and Designation
28/2/21 2030	ICU RMR PM (Markleff et al)-
	47 M w/ # ① Arc fusion ICU. # High BSLs # Staph. lugdunensis bacteraemia
	O/E (A) - self (B) - Spo2 97% (RA) - RR 16 clear. (C) - BP 140/90 - HR 60 (D) GCS 15/15
	Plan (1) wardable (2) Ax per day ICU MR

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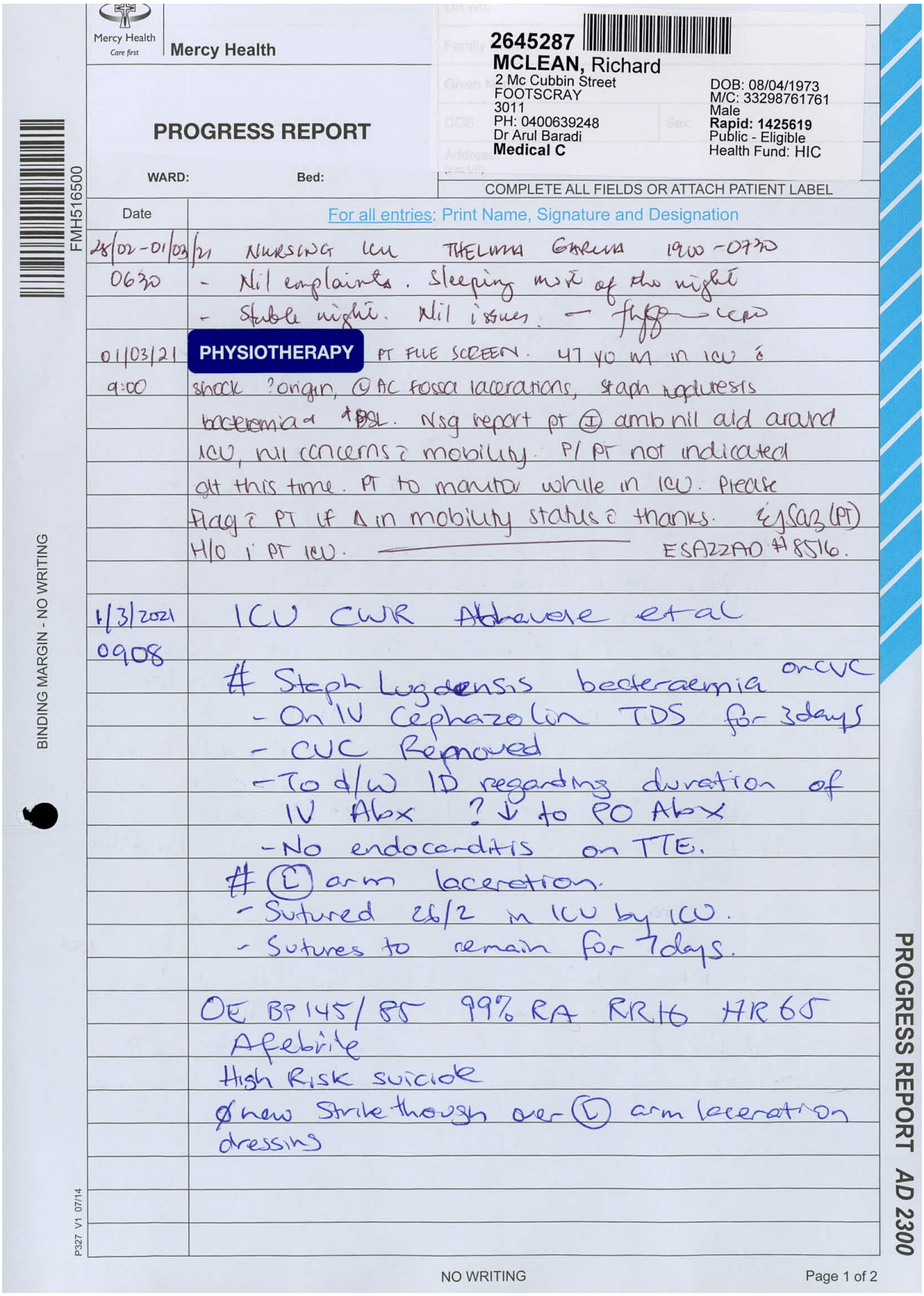
5.72. Progress Report (FMH516500)



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PROGRESS REPORT AD 2300

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Date	For all entries: Print Name, Signature and Designation
28/02-01/03/21 0630	NURSING ICU THELMA GARCIA 1900-0700 - Nil emplaints. Sleeping most of the night - Stable night. Nil issues. — fluff — ICU
01/03/21 9:00	PHYSIOTHERAPY PT FILE SCREEN. 47 yo M in ICU 6 shock ?origin, (L) AC fossa lacerations, staph saprophytis bacteremia c *BSL. Nsg report pt (I) amb nil aid around ICU, nil concerns ? mobility. P/ PT not indicated at this time. PT to monitor while in ICU. Please flag ? PT if Δ in mobility status ? thanks. EJSA23 (PT) H/O i' PT ICU. ESA22AO #8516.
1/3/2021 0908	ICU CWR Abhavale et al # Staph Lugdensis bacteraemia on CVC - On IV Cephazolin TDS for 3days - CVC Removed - To d/w ID regarding duration of IV Abx ? ↓ to PO Abx - No endocarditis on TTE. # (L) arm laceration. - Sutured 26/2 in ICU by ICU. - Sutures to remain for 7days. OE BP 145/85 99% RA RR16 HR 65 Afebrile High Risk suicide Ø new Strike through over (L) arm laceration dressing

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Page 1 of 2

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5.73. Progress Report (FMH516500)

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Date	For all entries: Print Name, Signature and Designation
	Plan 1. D/W ID regarding ? PO Abx 2. For ward, ongoing ~~review~~ IVT 3. Sutures R/V in 7 days ATN
	Addit P/C to Dr Darby, ~~re~~ already discussed yesterday. For 2wks IV Abx
	Pt raised concerns w/ nurses that his rights were not explained to him on admission P/C to CL & nurse Jeri, pt is admitted under involuntary treatment order ~~in Aug~~ currently & will need to go to a tribunal, likely difficulties • Jeri raised concerns about managing pt on med ward w/ a RPN special if patient requires 2wks of IV Abx
	Spoke to senior Med Reg Chun about elevated A/Cr Ratio (8.4) that was requested on Med admit note. Dr Chun said this proteinuria likely 2nd to AKI. ∅ further intervention req presently as renal fune has recovered & protein level now normalised ↓ ↑ respectively. plan Cont to monitor renal fune. ATN 8504.

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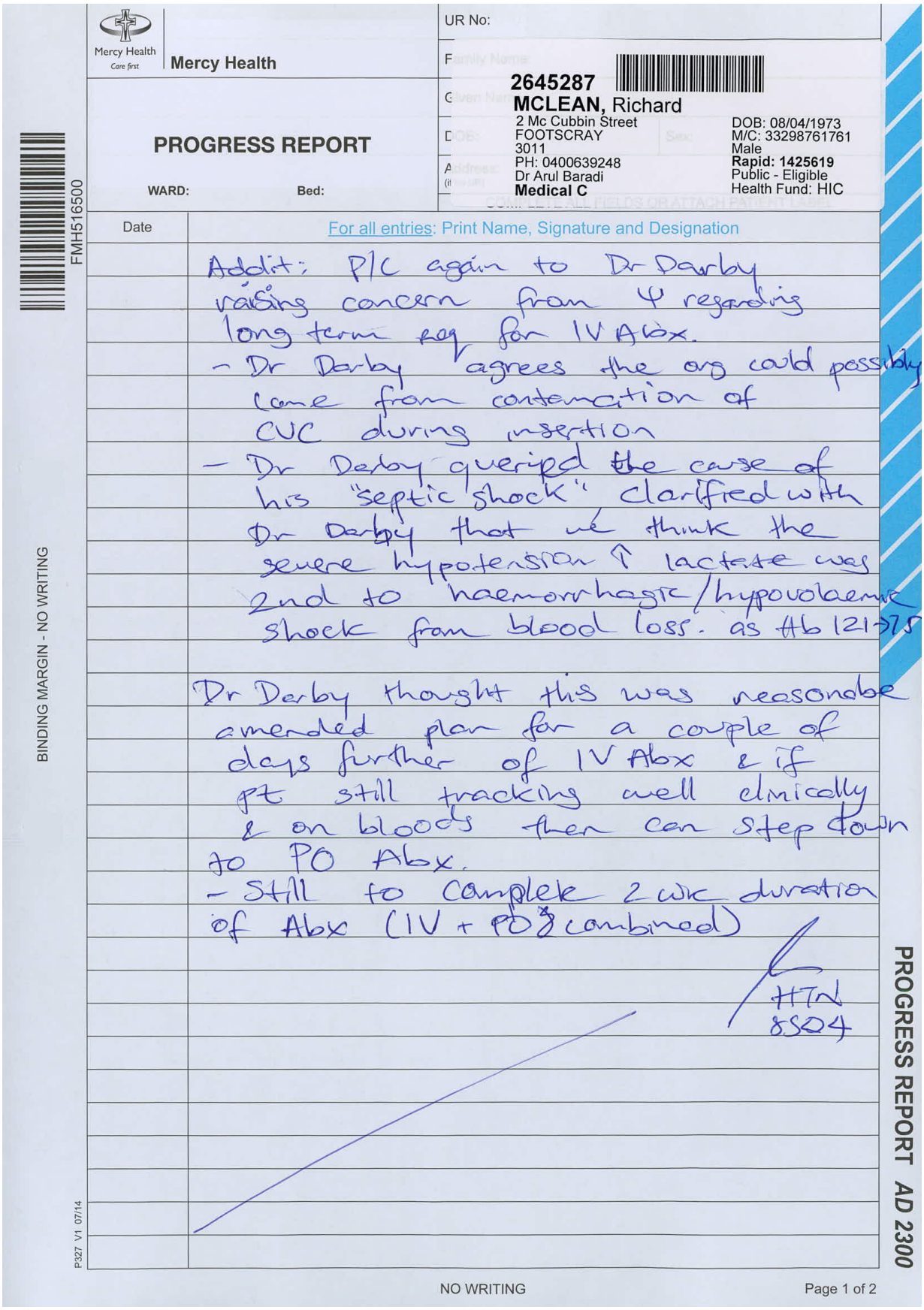
5.74. Progress Report (FMH516500)



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PROGRESS REPORT

WARD: Bed:

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Date	For all entries: Print Name, Signature and Designation

Addit: P/C again to Dr Darby raising concern from ψ regarding long term req for IV Abx.
- Dr Darby agrees the org could possibly come from contamination of CUC during insertion
- Dr Derby queried the cause of his "septic shock" clarified with Dr Darby that we think the severe hypotension ↑ lactate was 2nd to haemorrhagic/hypovolaemic shock from blood loss. as Hb 121→75

Dr Derby thought this was reasonable amended plan for a couple of days further of IV Abx & if pt still tracking well clinically & on bloods then can step down to PO Abx.
- Still to complete 2 wk duration of Abx (IV + PO combined)

HTN
8504

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Page 1 of 2

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5.75. Progress Report (FMH516500)

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PROGRESS REPORT

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COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Date	For all entries: Print Name, Signature and Designation
01-03-2021 1444	ICU Nursing Notes (0700-1900H) Post MET call for hypovolaemia, bleeding 2° to self-laceration; Pt. consent gained prior to Nursing care; Nursing assessment: CNS - GCS 15/15; Nil complaints of pain; Mobilising independently; Verbalized that he feels agitated this arvo & requested for diazepam; PRN diazepam given PO; CVS: Regular HR; BP is stable; Warm & well perfused peripheries; x1 P. IVC, patent & intact, I.V. access for IV Abx; Afebrile; FBE, UEC, CMP & Blood culture sent this AM; Plans for antibiotics are in place, can de-escalate to PO Abx if pt. continues to improve clinically; Respiratory: Own; SpO2 >98% on room-air; talking in full sentences; Nil signs of respiratory distress; G.I.: EATD; pt. stated that he opened his bowels this AM; Renal: Renal functions are better from AM bloods; PUIT; Skin: Dressing on (L) ACF is intact & no signs of active bleeding; Psychosocial: visited by sister, Jodie, today; seen by psyche services; Pt. was allowed by psyche doctor to use his phone given that he doesn't record staff / name staff in social media or any similar platform; Patient requested a patient advocate & would want a lawyer today; referred to social worker; Patient is cooperative towards his treatment / medications today; Explained medication actions & side effects to patient prior to administration; Plans: ① aim for ward bed; ② continue IV Abx for now; ③ social worker consult ——— SOFIA JOYCE DIMAPILIS CCRN
	Addit: pt. expresses concerns regarding intercom behind monitor screen & that someone can be recording him; Explained to him that the intercomm are used by nurses to ask for help if it is needed during covid times ——— SOFIA DIMAPILIS CCRN
1530H	seen by social worker ——— SOFIA DIMAPILIS CCRN

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5.76. Progress Report (FMH516500)

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FMH516500

PROGRESS REPORT

WARD: ICU Bed: 6

BINDING MARGIN - NO WRITING

Date	For all entries: Print Name, Signature and Designation
1/3/21	ICU cwk Dr Arulvale et al. 47M. admit w hypovolemic shock 2° lac. self inflicted Issues. # Staph. lugdunensis BC (F) 25/1/21. Sensitive to fluclox. - no raised inflammatory markers - ID → likely 2° left ACF lac. Suggests at least a few days of IV cefazolin on medical ward until clinical R/V → ? discharge to complete abx for a total duration of 2/52. # psychosis. - MH R/V today. - no acute Δ to meds - on ITO. under MH A. - MH will # Inappropriate social media posting - Amum, ↓ aware - escalated appropriately - patient counselled. Referred to social worker. - patient agreed not to post further until Wed (next MH R/V).

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PROGRESS REPORT AD 2300

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5.77. Progress Report (FMH516500)

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PROGRESS REPORT

WARD: ICU Bed: 6.

Date	For all entries: Print Name, Signature and Designation
1/3/21	Iai c/w/R Athavale c/o't.
	Plan.
	# Remain under MHA ITO.
	# Cont. IV Abx
	# Aim for medical ward bed
	# Chase blood cultures
	118503.
1/3/21 16:10	SOCIAL WORK initial contact SW referral w thanks for 47yo M p/w shock of unknown origin? Hypovolemia secondary to blood loss from laceration. SW referral re: patient advocate SW rev file note SHx, SW Met w pt at bedside Self ✓, roles, consent ✓, IDR, Discussed: - pt believes held against will, pt rights violated and not explained - pt discussed that he wanted support w rent - informed to consider centrelink loan by contacting centrelink/using MyGov account. als discussed getting rent (A) can also make a claim for this through MyGov

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5.78. Progress Report (FMH516500)

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FMH516500

PROGRESS REPORT

WARD: Bed:

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BINDING MARGIN - NO WRITING

Date	For all entries: Print Name, Signature and Designation
1/3/21	SOCIAL WORK — note cont —

- also offered housing support information re: crisis accom - pt declined
- offered emergency relief information - pt declined.
- pt asked if SW can be his advocate explained can note concerns but should contact the consumer liason officer to discuss concerns.
- informed pt to contact legal aid if requiring legal support
- informed pt can contact unison in seddon for housing support.
- pt informed his sexual abuse case that was rejected he didnt get outcome of SW explained to contact lawyer and seek to have outcome provided in writing as pt ? appealing.
- pts TV is making noise. SW heard a females voice come through speaker two times during conversation w pt. SW notified pts Nurse.
- pt wants letter from medical team addressed to HCF re: golden staff and how long will take to recover - doesnt want centrelink certificate.
- SW can not provide anything further to pt, pt not reading anything further from SW

Plan: (1) DIC from SW input, pt not reading ongoing SW input, not appropriate for SW.
(2) redirect pt to contact consumer liaison officer.

McPhail (SW) #3080

PROGRESS REPORT AD 2300

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5.79. Progress Report (FMH516500)

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COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Date	For all entries: Print Name, Signature and Designation
1/3/21 2025	ICU Night WR - Matchett / Ade
	# septic shock ? Staph. lugdunensis bacteraemia 2° self inflicted laceration
	# psychosis
	A own
	B Sats 98% on RA RR 16
	C BP 130/90 HR 70
	D GCS 15
	Plan: 1. transfer to ward when bed available. 2. Ut replacement

Ade
Reg

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5.80. Progress Report (FMH516500)

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COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

PROGRESS REPORT

WARD: icu Bed: 6

FMH516500

BINDING MARGIN - NO WRITING

Date	For all entries: Print Name, Signature and Designation
2/3/21 0430	NURSING NOTES (1900-0730) 47 yo male admitted c shock post self harm attempt. Not in isolation specialled 1:1 Pmh Reports. Allergies to Bee Sting, Augmentin, Specialled CNS: A+O, PEDRC 2+, GCS 15, Converting to full Sentence, Oppm -ve, RASS-0-, Uncooperative c care. Denies pain, Able to ambulate to and from toilet. CVS: Not for cardiac monitoring, bed blocked, currently on ward obs. Warm, well perfused CRT < 3 sec. Afebrile, PIVC on RFA CF. NSOP, P+P. Haemodynamically Stable Resp: Nil Resp distress N:T SOB ABOB. Chest clear. Bilateral equal AE on RA Currently SPO2 95% on RA CVS: tolerates fluids, Abdo SNTD BNO. This Shift. Pt Report: PMT Continent reports that he is passing goal amounts of urine. Renal functions monitored improved. for Bloods today this am. Integ: Indep c PDC, Nil PI, Noted dry skin lesions on both legs pt reports this as psoriasis (pre existing) Ready mobilise to toilet c distant supervision. Other psycho social: Nil pic or visitors O/N. Pt spoke to his mum on his own phone. Pt reported he is hearing voices from the black intercom in the room and "talking about me" and "swearing". Reassurance provided and explained the intercom used for communication during covid19 outbreak. Pt said "I don't want to hear it". NS disconnected the power of the intercom and turned it off. Pt settled and agreed to go back to sleep. Nil other behaviour issues O/N. Appeared to have — Po Free Continue

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PROGRESS REPORT AD 2300

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5.81. Progress Report (FMH516500)

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PROGRESS REPORT

WARD: ~~ICU~~ ICU Bed: 6.

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Date	For all entries: Print Name, Signature and Designation
2/3/21	NURSING ND CONTINUED ÷ good sleep overnight. Nil other social and psychological issues voiced by pt. PLANS: Continued 1:1 special due to self harm risk. a/w ward bed & Special ——— 135 Tee Dilcardia RN
0630 -	ECG done, pt declined blood Collection, wanting to ~~Stopp~~ Sleep, notified ICU doctors. Happy with a late Collection, will handover to Dm staff — PS Tee CERN
2/3/21.	ICU ~~Registrar~~ CLERK (ATTENDANT ET AL)
0911 -	
	✱ Septic Shock vs Hypovolaemic shock - S. lugdunensis bacteraemia - Stable inflammatory markers - Haemodynamically stable - nil supports. - Hb 88 (post x3 PRBC) yesterday. ✱ Psychosis. - Reviewed by ~~Psych~~ psych yesterday
	O/E Well today BP 140/90 HR 80 SpO2 100% RA
	PLAN ① IV Abx today ② ~~Start Rapid~~ For deescalation to PO Abx tomorrow ③ Psych review tomorrow. ④ Richard's concern about his treatment here at WMH will be escalated. BRANDON

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5.82. Progress Report (FMH516500)

40260

Mercy Health

Patient Progress

Date & Time: 1/03/2021 4:41:26 PM

DRAFT - last modified: 1/03/2021 9:24:00 PM

2645287

UR Number :
Surname : McLean
First Name : Richard
DOB : 8/04/1973 **Gender** Male
Address : 2 Mc Cubbin Street
3011 Footscray
VIC
Australia

Home Phone : 0400639248 **Work Phone :**

Address 2 Mc Cubbin Street
3011 Footscray
VIC
Australia

Home Phone (AH) 0400639248 **Work Phone (BH)**

Statewide UR: 1425619
Date of Entry: 01/03/2021
Time of Entry: 16:38
Clinician: Narayanan Manoj Kumar Muthangaparambil
Designation: Psychiatrist Medical Specialist
Team: Werribee Mercy Hospital Consultation Liaison Psychiatry Service

Reviewed Richard in ICU
Richard's sister Jodie was also present during the review
In attendance: Dr Manoj Kumar and Jeril Thomas(CLN)
Richard is a 47 year old man, admitted initially in the psychiatric unit last week on 23rd of february on an assessment order under MHA 2014, subsequently on a temporary treatment order in the context of suicidal risk risk in the community, currently treated in the ICU after he attempted to end his life by cutting his arm while he was in the inpatient unit.
Richard has been expressing suicidal thoughts and plans to end his life on his upcoming birthday in April through social media. His mental state has deteriorated in the past few weeks in the context of stop working for NDIS, car being repossessed, financial issues and stopped seeing his private psychiatrist.
He has been on multiple psychotropic medications including dexamphetamine tablets, Paroxetine and Amisulpride.
Richard has a long history of being treated for mental health issues.He has been diagnosed in the past with Schizophrenia and depression.He has been also diagnosed with ADHD and receiving dexamphetamine tablets(30-40mg daily) for the same. He has attempted overdsoe few years ago. Richard has experienced significant psychological trauma in early life, experienced sexual abuse, discrimination due to homosexuality, multiple losses and relationship loss, loosing a friend who committed suicide couple of years ago, financial problems and perceived lack of support and validation from others.He also has been having issues with one of the previous GPs as Richard felt that the doctor was medically negligent by prescribing medications to him on which he could overdose with. He has felt that there was lot of injustice happened towards him from various levels.He has contacted health minister Greg Hunt and Gordon Parker from beyond blue expressing his complaints.
On a positive note Richard had worked as a consumer consultant in the past and he has completed his Phd after working really hard on himself.
Richard had challenges with alcohol use in the past.

Title: Psychiatrist Review **Page No.** 1
Created By: Narayanan Manoj Kumar Muthangaparambil (Psychiatrist)
Printed By: Jeril Thomas (Nurse)

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5.83. Progress Report (FMH516500)

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Mercy Health

Patient Progress

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On examination today,
Richard recognised the writer from past contact and appropriately greeted us. He insisted to ask each of ours name in the beginning of the interview and wrote down the details of the conversation in the book. He was appeared paranoid that we are part of the hospital and at times he said he would not tell us all the information about his situation.He is appeared to be very angry that his medications (dexamphetamine) was stopped in the ward. He alleged that the suicidal attempt was co contributed by the way he was treated in the ward. He said he is going to take legal action against the hospital.
Richard did not want to elaborate on his beliefs about what led him to come to hospital. Richard gave the details of his website and asked us to read about what he has written there. It appeared that he believes that there are some real conspiracies going on against him. He said he is doing alright now and feels happy about the staff in ICU.He denied any current suicidal thoughts or plans. He was upset and angry when he was advised that he needs to continue staying in the hospital to complete treatment.He became threatening to take legal action against everyone involved in his treatment.
Richard has been posting on social media and on his website about his experiences in the hospital. He is advised about the need for respecting the privacy and confidentiality of the staff members who is treating him in the hospital. He is advised that this behaviour is not helping with his issues being addressed rather make the situation uncomfortable for staff to help him. He is advised that there could be restrictions for him to use mobile phone if he continue to post details of staff on social media or his website. He said he will refrain from doing it for the time being.

Mental Status Examination
Presentation / Appearance: Appropriately dressed and groomed
Behaviour / Psychomotor Activity:Increased Psychomotor activities
Affect / Mood:Appropriate, depressed, angry
Speech: Normal in volume and tone, goal directed
Thought Disturbance (stream / flow of speech and thoughts): No formal thought disorder elicited
Thought Content (major themes, delusions): Persecutory ideas about conspiracy against him
Perceived thoughts Of injustice towards him
Perceptual Disturbances (hallucinations etc): No perceptual abnormalities elicited
Insight / Judgement: Impaired judgement and limited insight about the risk of impulsive suicidal attempt and persecutory ideas.
Cognition / Concentration / Consciousness / Memory: Grossly intact, not tested in detail

Title: Psychiatrist Review **Page No.** 2
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5.84. Progress Report (FMH516500)

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ROUTINE RISK ASSESSMENT

Mark with an "X" if applicable

Risk Categories	PHx	L	M	H	Rationale / Comments (mandatory)
Suicidality / Self Harm (Inpatient staff to consider removal of ligature risks if M or H and document in clinical file)				x	
Risk of self -neglect /accidental self-harm (homeless, personal hygiene, food/fluid intake)				x	
Violence/ harm to others			x		
Risk of violence, harm or neglect to infant/child (consider CPS notification if M or H)		x			
Risk of Vulnerability/Harm from others (physical, sexual, emotional or financial)		x			
Risk of absconding/ non - adherence with treatment			x		
Risk of Impulsivity/ level of distress				x	
Current substance use, intoxication or withdrawal - Yes or No		x			
Physical health issues or adverse effects from treatment			x		
Other (falls risk, use of restrictive interventions, PRN medication)			x		

Delete inappropriate answer (Yes or No)

Factors to consider		Rationale / Comments (mandatory)
Is the patient's level of risk highly changeable?	**Yes** / No	
Is the reliability of this risk assessment low? (Poor engagement, conflicting info, etc.)	Yes / **No**	
Are tipping points present? (personal stress, recent death, end of relationship)	**Yes** / No	
Are there significant protective/ resilience factors present?	**Yes** / No	

Title: Psychiatrist Review **Page No.** 3
Created By: Narayanan Manoj Kumar Muthangaparambil (Psychiatrist)
Printed By: Jeril Thomas (Nurse)

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5.85. Progress Report (FMH516500)

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If high changeability = Yes or low reliability = Yes, then the overall risk rating should increase by one level (i.e. medium to high)	
OVERALL RISK RATING (LOW/ MEDIUM/ HIGH):	High
***RISK MANAGEMENT PLAN TO BE DOCUMENTED BELOW ***	

Impression:
Suicidal attempt
Significant impulsivity and depressive symptoms secondary to prominent cluster B personality issues
Schizophrenia
?ADHD

Plan (include strategies to manage risk):
1. Continue to have medical management for the suicidal attempt and infection
2. Continue on temporary treatment order under Mental Health Act 2014
3. Rights explained , including appeal to MHT , second opinion options and provided the rights booklet. He is advised about legal aid and options for legal help.
Richard is also referred to the social worker to help him with legal issues.
4. Sister , Jodie was advised about the treatment plan.She stated that Richard is not well and the family is concerned about him, supports hospital admission and treatment.
Jodie will organise a second opinion from an outside psychiatrist as Richard will trust that more than another psychiatrist in the hospital.
5. Richard is vaidated with his situation, allowed him to express his distress and feelings of injustice. He is advised that the hospital and treating team has the best interest for his safety and well being. He is advised that he needs to continue staying in the hospital to have ongoing treatment.
6. Medications to continue as charted
He needs to have a gradual reduction in the dexamphetamine tablets.
7. CL team to continue review him during his stay in the medical ward. He needs to have psychiatric admission after medical management.
8. 1:1 special nursing in the medical ward, watch for suicidal behaviour in the ward.
Please remove all objects from his vicinity which could be used as a self harm weapon.

Title: Psychiatrist Review **Page No.** 4
Created By: Narayanan Manoj Kumar Muthangaparambil (Psychiatrist)
Printed By: Jeril Thomas (Nurse)

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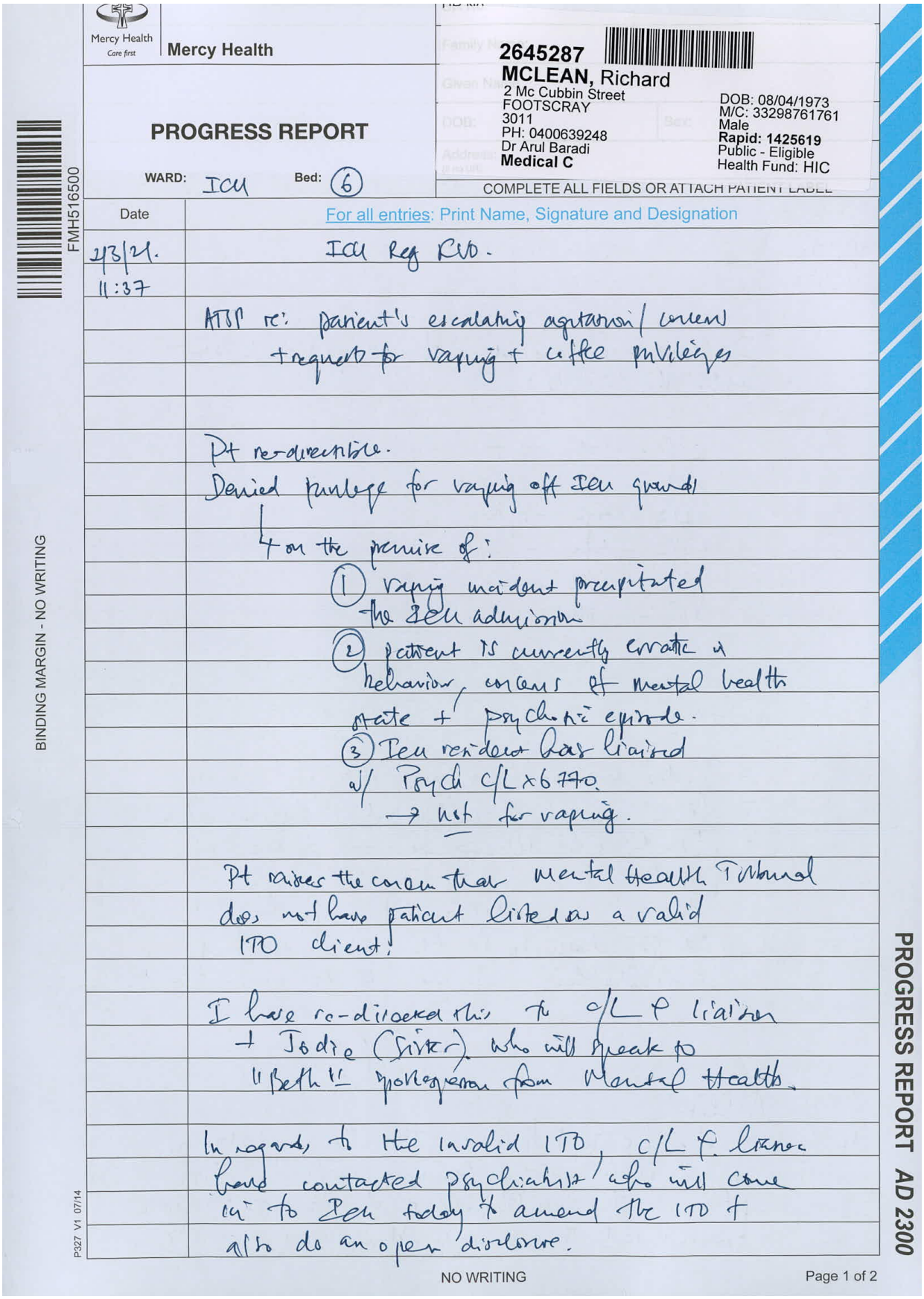
5.86. Progress Report (FMH516500)



Mercy Health
Care first

Mercy Health

PROGRESS REPORT

WARD: ICU Bed: 6

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH516500

BINDING MARGIN - NO WRITING

Date	For all entries: Print Name, Signature and Designation
23/2/21. 11:37	ICU Reg RVD.
	ATSP re: patient's escalating agitation / unsafe requests for vaping + coffee privileges
	Pt re-directible.
	Denied privilege for vaping off ICU grounds on the premise of:
	(1) vaping incident precipitated the ICU admission
	(2) patient is currently erratic in behaviour, concerns of mental health state + psychotic episode.
	(3) ICU resident has liaised w/ Psych c/L x6770. → not for vaping.
	Pt raises the concern that Mental Health Tribunal does not have patient listed as a valid ITO client.
	I have re-directed this to c/L & liaison + Jodie (Sister) who will speak to "Beth" progression from Mental Health.
	In regards to the invalid ITO, c/L & liaison have contacted psychiatrist who will come in to ICU today & amend the ITO + also do an open disclosure.

PROGRESS REPORT AD 2300

P327 V1 07/14

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Page 1 of 2

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5.87. Progress Report (FMH516500)

Mercy Health
Care first

Mercy Health

UR No:

2645287

MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
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Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

PROGRESS REPORT

WARD: Day Bed: 6

Date	For all entries: Print Name, Signature and Designation
2/3/21. 11:37	Ren note cont.
	I have also advised of the negative BC sent after the 25/2/21.
	25/2. 2nd → (–)
	25/2. 3rd → (–)
	26/2. 4th → (–)
	28/2 – 5th – (–).
	28/2 – 6th – (–).
	1/3 – 7th pending.
	Plan.
	– Jodie stay in for support of pt.
	– link in w/ pt advocate / liaison officer.
	– Psychiatrist assessment + discharge of ITO.
	– Any concerns, direct to myself x8503.
	– can step down to PO Abx if unit bed is available, as per Ren consultant.
	U 8503
2/3/21. 11:57	Addit: Jodie (sitter) believes the clozapine 30mg is doing some clinical benefit for the pt & that withholding of clozapine precipitated the suicide attempt U 8503.

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Page 2 of 2

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5.88. Progress Report (FMH516500)

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Mercy Health

PROGRESS REPORT

WARD: Bed:

2645287
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2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
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COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH516500

BINDING MARGIN - NO WRITING

Date	For all entries: Print Name, Signature and Designation
2/3/21 1530	Nursing 0700 - 1930. Nursed in ICU with septic shock, secandary to self inflicted laceration. CNS: GCS 15. Denies pain. Ongoing agitation and paranoia. Richard is upset because he feels he is well enough to go home and being kept against his will. CVS: 4/24 obs. Vitals stable. Afebrile. PIVC removed due to leaking, antibiotics changed to oral. Warm and well perfused. Resp: Spo2 >95% RA. No SOB or increased. GIT: Tolerating diet and fluids. Denies nausea. BO 1/3/21. Renal: PUIT, good amounts as per patient. Metabolic: No bloods taken this morning. Skin: Sutures L) arm from self harm laceration, dressing intact. Indep with PAC and mobility. Social: Visit from sister, update given. Plan: Await transfer to mental health unit. F.Huntley RN

SKIN INSPECTION
Date: 2/3/21 Time: 0900
Consumer participated in care planning:
Yes ☑ No ☐

Falls Score --->	2	High falls risk	Y	N
Clutter Free Space	✓	Appropriate Lighting		✓
Call bell & items in reach	✓	Non- slip socks		X
Supervision in SOOB	✓	CAM ICU Completed		✓ cam ICU negative
Braden Score --->	23	PI Prevention Strategies		X
Malnutrition Screening	0			

Date- 2/3 Time 1830 Sign FH.

P327 V1 07/14

PROGRESS REPORT AD 2300

NO WRITING

Page 1 of 2

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5.89. Progress Report (FMH516500)

Mercy Health
Care first

Mercy Health

PROGRESS REPORT

WARD: Bed:

2645287
MCLEAN, Richard
2 Mc Cubbin Street
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3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
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COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Date	For all entries: Print Name, Signature and Designation
2/3/21	ICU Night WR - Paul /Anu
2050	# acute psychosis
	# staph lugdunensis bacteraemia
	↳ △ PO flucloxacillin
	Plan:
	1. awaiting ward bed.
	2. 2/52 total Abx
3/3/21	CNS: Pt alert + Orientated. N.I Pain Reported
0520	Pt Polite + cooperative. N.I agitation. BEDRL
Nursing	CVS: 4-6 hrly Obs. Normotensive AFebrile
	N.I IV Access.
	Resp: Spont vent on RA SAO_2 99-100%.
	GIT: Tolerating Diet + Fluids. BNO. Abdo SNT
	Renal: Pt voiding in toilet.
	Skin: Intact. Dressings Dry + Intact.
	Plan: Meeting 3/3/21 to discuss Richards Plan.

SKIN INSPECTION
Date: 2/3/21 Time: 2200
Consumer participated in care planning:
Yes ☑ No ☐

Falls Score --->	✓	High falls risk	Y	N
Clutter Free Space	✓	Appropriate Lighting		✓
Call bell & items in reach	✓	Non- slip socks		✓
Supervision in SOOB	✓	CAM ICU Completed		✓
Braden Score --->	✓	PI Prevention Strategies		✓
Malnutrition Screening	✓			

Date- 2/3/21 Time 2200 Sign CA

CA (CNS Hutton)

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Page 2 of 2 NO WRITING

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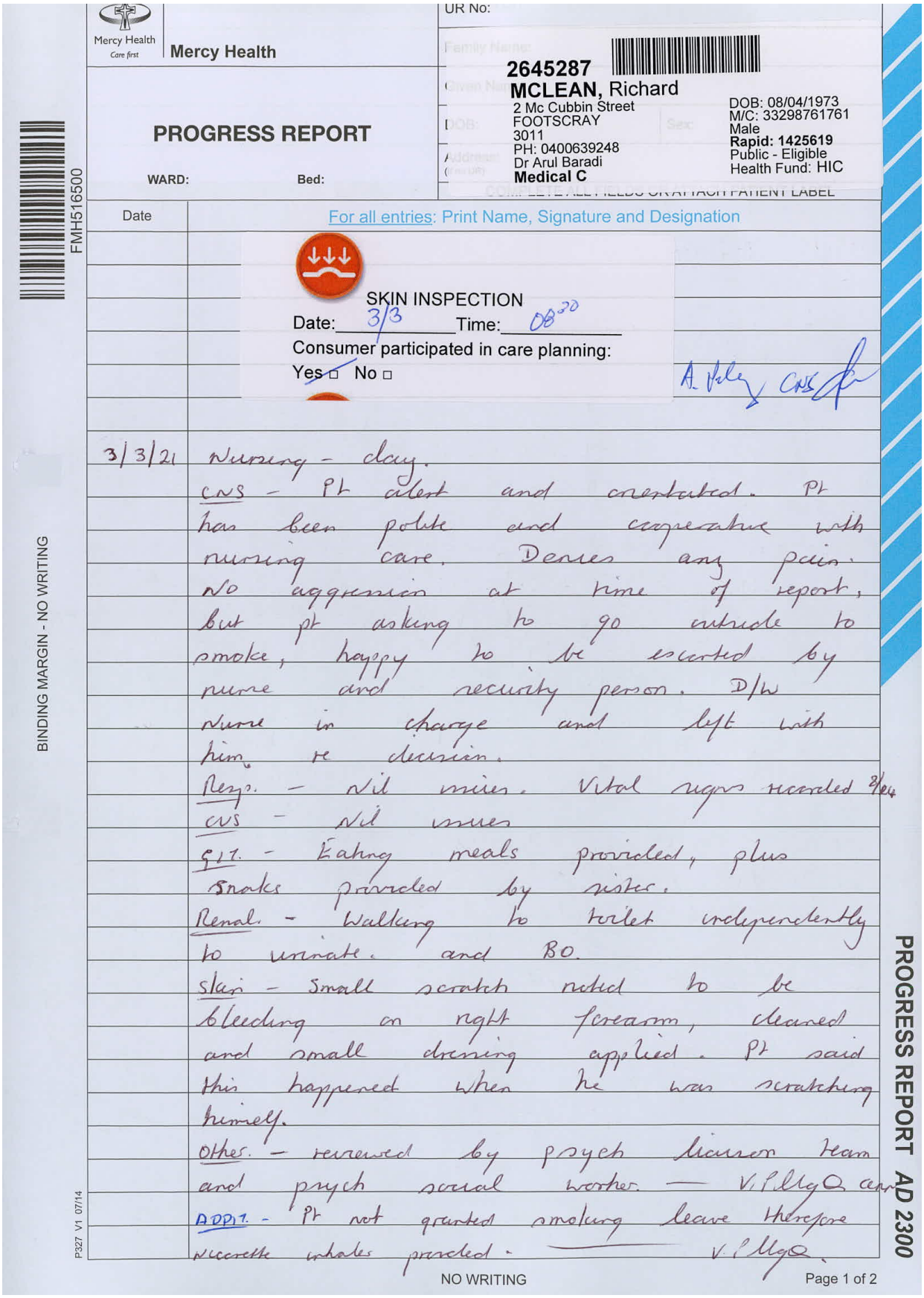
5.90. Progress Report (FMH516500)



Mercy Health
Care first

Mercy Health

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3011
PH: 0400639248
Dr Arul Baradi
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Male
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PROGRESS REPORT

WARD: Bed:

FMH516500

BINDING MARGIN - NO WRITING

Date | For all entries: Print Name, Signature and Designation

SKIN INSPECTION
Date: 3/3 Time: 0820
Consumer participated in care planning:
Yes ☒ No ☐

A. Villy CNS

3/3/21 Nursing - day.
CNS - Pt alert and orientated. Pt has been polite and cooperative with nursing care. Denies any pain. No aggression at time of report, but pt asking to go outside to smoke, happy to be escorted by nurse and security person. D/w Nurse in charge and left with him, re decision.
Resp. - Nil issues. Vital signs recorded 2/24
CVS - Nil issues
GIT. - Eating meals provided, plus snacks provided by sister.
Renal - Walking to toilet independently to urinate. and BO.
Skin - Small scratch noted to be bleeding on right forearm, cleaned and small dressing applied. Pt said this happened when he was scratching himself.
Others. - reviewed by psych liaison team and psych social worker. — V. Villy CNS
ADD'L - Pt not granted smoking leave therefore nicorette inhales provided. — V. Villy.

PROGRESS REPORT AD 2300

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Page 1 of 2

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5.91. Progress Report (FMH516500)

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3011
PH: 0400639248
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Medical C

DOB: 08/04/1973
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Public - Eligible
Health Fund: HIC

PROGRESS REPORT

WARD: Bed:

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Date	For all entries: Print Name, Signature and Designation
3/3/21.	Am ICU CWR (Alexander et al)
9:24.	
#	Acute psychosis . Ongoing psychiatry consultation.
#	Staphylococcus lugdunensis bacteraemia . On flucloxacillin presently. (total 2 weeks) . Afebrile.
	PLAN: Patient is medically cleared. Awaiting psych bed. Ongoing psychiatry input. Psychiatry to be the primary point of contact with the patient as per discussions Paul.
3/3/21	ICU Night WR - Paul / Ade
2039	# acute psychosis # staph lug. bacteraemia O/E: Obs WNL, afeb Plan: 1. as per psych Paul RMO

BINDING MARGIN - NO WRITING

Page 2 of 2 NO WRITING

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5.92. Progress Report (FMH516500)

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Mercy Health

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DRAFT - last modified: 3/03/2021 10:15:00 PM

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Address 2 Mc Cubbin Street
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Home Phone (AH) 0400639248 **Work Phone (BH)**

Statewide UR: 1425619
Date of Entry: 03/03/2021
Time of Entry: 14:22
Clinician: Narayanan Manoj Kumar Muthangaparambil
Designation: Psychiatrist Medical Specialist
Team: Werribee Mercy Hospital Consultation Liaison Psychiatry Service

Reviewed Richard in ICU
Sister, Jodie was present during the review
In attendance: Dr Manoj Kumar, Dr Johan Dahl, Jeril Thomas and medical student
Richard is now medically cleared to be transferred to psychiatric ward
Richard has been generally cooperative in the ICU
Richard took name and details of all the members of the treating team before proceeded with the interview.
He asked for recording the interview today but he is advised that we are not comfortable with recording the session.
Richard denied having any current thoughts or plans for self harm or suicide. He said he is ready to be discharged home and want to spend time with his dog.When he was asked about the suicidal plans he posted on his website that he is going to end his life on 08th of April , he said he was only expressing his frustration that no one is listening to his issues and he wanted people to listen and take him serious.He said he doesn't plan to commit suicide on 08th of April or afterwards. He wants lot of support in sorting out his financial issues, legal issues, accommodation issues and centrelink issues.
Richard described his experiences with various organisations, legal system , AHPRA, police, human rights commission, health commission etc being unhelpful.He felt that he was not listened to by these agencies. He said he is targetted by all these agencies and they are all linked and working against him. He talked about his ex partner being present in a murder scene and he knows about this.His expartner also do not want to give him his share of estate. Ex partner has contacts as his boss was in ASIO. Richard feels that the reason why he is not taken serious and people behave towards him is because he is re flagged in all these organisations.He is experiencing persecutory ideas in relation to his experiences.
Richard described his experiences with his GP Dr Whitaker and Dr Cooper and expressed his thoughts that they were behaving towards him those ways is because of the persecution against him and they are all are part of this what is going on. He incorporates his negative experiences with people as part of the

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persecutory theme. He felt that there is a plan for him to be denied of justice and make him end his life. He felt there is a lack of validation and listening to his grievances and also there is a persecutory plan linking all these organisations against him as they want to protect Dr Whitaker and his ex partner.
Richard has made some contacts with his private psychiatrist who was prescribing him dexamphetamine tablets and got some letters from Dr Horgan to support his diagnosis and treatment with dexamphetamine tablets before the hospital admission. Richard want to continue taking his dexamphetamine tablets and he doesn't believe that he is experiencing any psychotic symptoms.He said all his experiences are real and he has lot of evidence to prove it.
Richard also stated that he was sexually mistreated as a child and he reported this. He stated that his legal battle for this did not reach any success. He attributed that, this is also because of the persecution towards him.

Richard understand that he need to stay in the hospital. He has put lot of demands when he is admitted in the psychiatric ward. He wants to use mobile phone, wants to use waping, continue on dexamphetamine and want help from a lawyer.

Although Richard was talking about attempting self harm again if he is transferred to CMB, he denied any current thoughts of self harm or suicide. He wanted to know when he will be discharged from the the ward so that he can relax at home in company of his dog and friends.

Richard is advised that he needs to stay in the hospital for a period of time to observe his suicidal risk and further assessment and treatment of psychotic symptoms and suicidal thoughts. He is advised that it is possible that the dexamphetamine tablets could be contributing to his psychotic symptoms. Richard disagree with that and became upset about having any change with dexamphetamine tablets.

Impression

Prominent Cluster B personality(Borderline and Narcissistic) features, feelings of rejection, invalidation and extreme impulsivity

Significant anger issues related to perceived invalidation

Schizophrenia /Schizoaffective disorder diagnosed in the past

? Dexamphetamine induced worsening of psychotic symptoms

Plan

Title: Psychiatrist Review **Page No.** 2
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5.94. Progress Report (FMH516500)

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1. Transfer to psychiatric ward when becomes available in ICA, prefers Footscray, if not available CMB.

Needs ICA with special nursing to start with

2. Continue medications as charted and later consider reducing the dose of dexamphetamine while he is in the psychiatric ward after collaboratively discussing the plan with him. Reducing dexamphetamine dose to lower dose over a period of time could be a possible option if Richard is extremely resistant to stop using it.

3. Validate his feelings with appropriate boundaries and limit setting with consistent approach

4. Watch for suicidal behaviour, remove all objects which could be used as a potential aid for self harm when he is extremely impulsive and angry.

5. Further exploration of psychotic symptoms and management.

6. Phone call to Dr Horgan , previous psychiatrist, he believes that dexamphetamine medication has helped him to complete his Phd and supports ongoing use of the same. Dr Horgan acknowledges that Richard has a diagnosis of schizoaffective disorder but also feels that he has ADHD. Dr Horgan do not think dexamphetamine tablet has contributed to psychosis in him.

7. Sister Jodie was present during the review and aware of the plans for transfer to psychiatric ward and support the plan.

8. Please refer Richard for case management in the community.

9. Social work input during his stay in the ward

muthanm

Title: Psychiatrist Review **Page No.** 3
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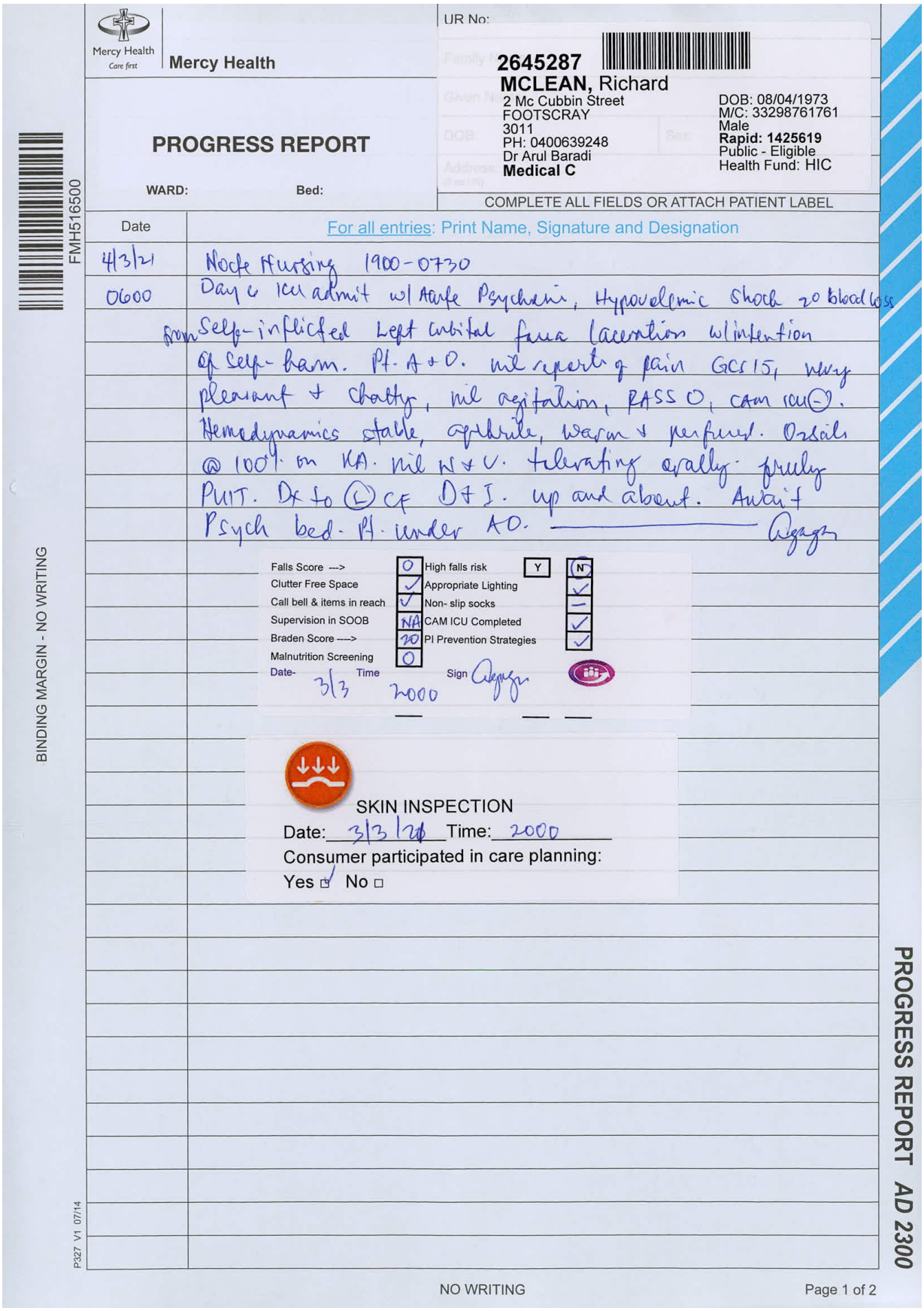
5.95. Progress Report (FMH516500)



Mercy Health
Care first

Mercy Health

UR No: 2645287

MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

PROGRESS REPORT

WARD: Bed:

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH516500

BINDING MARGIN - NO WRITING

Date	For all entries: Print Name, Signature and Designation
4/3/21	Night Nursing 1900-0730
0600	Day 6 ICU admit w/ Acute Psychosis, Hypovolemic Shock 2o blood loss from Self-inflicted Left orbital fossa laceration w/intention of self-harm. Pt. A+O. nil reports of pain GCS 15, very pleasant + chatty, nil agitation, RASS 0, CAM ICU (-). Hemodynamics stable, afebrile, warm + perfused. O2 sats @ 100% on RA. nil N+V. tolerating orally. fruity PUIT. Dx to (L) CF D+I. up and about. Await Psych bed. Pt. under AO. Agazzi

Falls Score --->	0	High falls risk	Y	N
Clutter Free Space	✓	Appropriate Lighting		✓
Call bell & items in reach	✓	Non- slip socks		—
Supervision in SOOB	NA	CAM ICU Completed		✓
Braden Score --->	20	PI Prevention Strategies		✓
Malnutrition Screening	0			

Date- 3/3 Time 2000 Sign Agazzi

SKIN INSPECTION
Date: 3/3/21 Time: 2000
Consumer participated in care planning:
Yes ☑ No ☐

PROGRESS REPORT AD 2300

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PROGRESS REPORT

WARD: ICU Bed: 6

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Date	For all entries: Print Name, Signature and Designation
4/3/21. 10:00	Am ICU CWR (Ahmad, Vo, Subramanyam)
#	Ante psycho. · Presently managed exclusively by psych team
#	~~Sepsis~~ Staphylococcus lugdunensis bacteremia on flucloxacillin
	No new symptoms clinically.
	O/E - All obs within normal limits
	PLAN -
	(1) Medically cleared - can be shifted to psych ward
	(2) Plan as per psych advice.
	(3) To R/W ± remove (L) arm sutures on 5/3/21.

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5.97. Progress Report (FMH516500)

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PROGRESS REPORT

WARD: Bed:

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH516500

BINDING MARGIN - NO WRITING

Date	For all entries: Print Name, Signature and Designation
4/3/21 12:45	SKIN INSPECTION Date: 4/3/21 Time: AM Consumer participated in care planning: Yes ☑ No ☐ Pt states pressure areas healthy. Not seen by staff. Seeace.

Falls Score --->	1	High falls risk	Y	N
Clutter Free Space	✓	Appropriate Lighting		✓
Call bell & items in reach	✓	Non- slip socks		✓ shoes
Supervision in SOOB	✓	CAM ICU Completed		✓
Braden Score ---->	23	PI Prevention Strategies		✓
Malnutrition Screening	✓			

Date- 4/3/21 Time 1245 Sign Seace.

Nursing notes (0700-1530) Susie Peace CCRN.
Medicly cleared awaiting bed in Y. Richard adjitated when first awoken as he wanted a "vape" and has been denied this for 6 days. Reasurance given that I was listening to his request and escalating it to the relevent people. Psych team have said that they still feel it would be unsafe for him to leave the ICU to have a vape. Richard given this information and offered further nicotine therapys. Richard requested name of "person responsible for holding me in solitory confinment." Dr Kumars name and hospital switch board number given.
Richard sat on balcony talking with myself to eleviate the feeling of being in solitory confinment. Currently RASS 0. Awaiting antibiotics from pharmacy.
Seace

PROGRESS REPORT AD 2300

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5.98. Progress Report (FMH516500)

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COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

PROGRESS REPORT

WARD: ICU Bed: 6

Date	For all entries: Print Name, Signature and Designation
4/3/21 1552	Nsg Pm: Patient alert, oriented. GCS - 15. Talks to people over the phone. Vitally stable afebrile. Mods given as charted. Well-tolerated diet and fluids. Nil nausea + vomiting. Patient reported. Nil concerns voiced atm. patient will be transferred to psych ward @ 1630hrs. Cardys. ECRN

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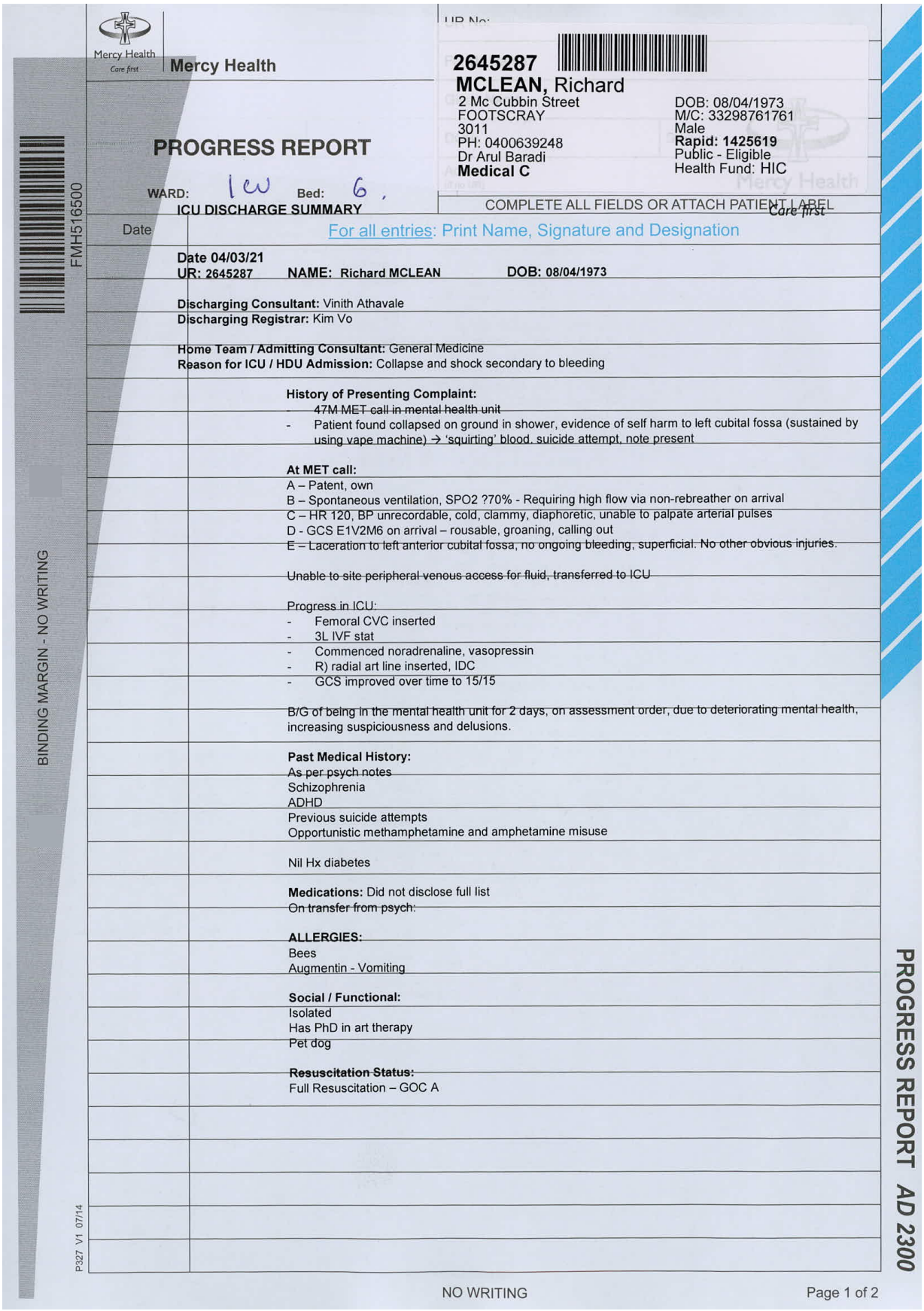
5.99. Progress Report (FMH516500)



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FMH516500

PROGRESS REPORT

WARD: ICW Bed: 6,
ICU DISCHARGE SUMMARY

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Date

For all entries: Print Name, Signature and Designation

Date 04/03/21
UR: 2645287 **NAME: Richard MCLEAN** **DOB: 08/04/1973**

Discharging Consultant: Vinith Athavale
Discharging Registrar: Kim Vo

Home Team / Admitting Consultant: General Medicine
Reason for ICU / HDU Admission: Collapse and shock secondary to bleeding

History of Presenting Complaint:
- 47M MET call in mental health unit
- Patient found collapsed on ground in shower, evidence of self harm to left cubital fossa (sustained by using vape machine) → 'squirting' blood, suicide attempt, note present

At MET call:
A – Patent, own
B – Spontaneous ventilation, SPO2 ?70% - Requiring high flow via non-rebreather on arrival
C – HR 120, BP unrecordable, cold, clammy, diaphoretic, unable to palpate arterial pulses
D - GCS E1V2M6 on arrival – rousable, groaning, calling out
E – Laceration to left anterior cubital fossa, no ongoing bleeding, superficial. No other obvious injuries.

Unable to site peripheral venous access for fluid, transferred to ICU

Progress in ICU:
- Femoral CVC inserted
- 3L IVF stat
- Commenced noradrenaline, vasopressin
- R) radial art line inserted, IDC
- GCS improved over time to 15/15

B/G of being in the mental health unit for 2 days, on assessment order, due to deteriorating mental health, increasing suspiciousness and delusions.

Past Medical History:
As per psych notes
Schizophrenia
ADHD
Previous suicide attempts
Opportunistic methamphetamine and amphetamine misuse

Nil Hx diabetes

Medications: Did not disclose full list
On transfer from psych:

ALLERGIES:
Bees
Augmentin - Vomiting

Social / Functional:
Isolated
Has PhD in art therapy
Pet dog

Resuscitation Status:
Full Resuscitation – GOC A

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PROGRESS REPORT AD 2300

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5.100. Progress Report (FMH516500)

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PROGRESS REPORT

ANZICS

WARD: ICU Progress Bed:

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

For all entries: Print Name, Signature and Designation

Date	
	# Hypovolemic Shock - Hypovolemia secondary to blood loss from laceration on Left cubital fossa - Started on noradrenaline and vasopressin with blood pressure monitoring - Hydrocortisone given 50mg QID for 48 hours and ceased - Transfused three units PRBC on 26/2/21. Hemoglobin monitored (Hb on 27/2 – 87) - High BSL (22.5) – managed with insulin infusion on 25/2 and subsequently stopped. Not a known diabetic. HbA1c 5.5. Latest BSL 5.9 - Elevated lactate 13 on ICU admission. Given 4 litres of fluid resuscitation. Normalised to 1.3 - No ECG changes or chest pain. Troponin inc from 48 -> 431 -> 160. (Corrected Qtc – 410 Normal) - CT Angiography – Brain, Chest, Abdo/Pelvis and Left Arm done – no significant abnormality reported as a cause for severe hypotension **#Staphylococcus lugdunesis Bacteraemia** - Discussed with ID, likely 2ndary to contamination during insertion of CVC line in setting of severe hypotension - Taken from CVC line on admission. CVC line removed - Detected in aerobic bottle blood culture 25/2/2021 - Sensitive to Flucloxacillin, vancomycin, penicillin. No resistance - Commenced on cephazolin 2g 8hrly - ID plan, patient for 2 weeks of IV Cephazolin initially, this was changed to oral flucloxacillin and planne the same for 2 weeks total - Endocarditis screen –ve. TTE showed LVEF60% normal left ventricular size and wall thickness. Hyperdynamic LV systolic function. **# Left cubital fossa lacerations** - Sustained with glass from vape machine - Received ADT booster in ICU - Left arm CT angiogram- superficial injury. No hematoma/ pseudo-aneurysm - Started on Tazocin. Changed to Cefazolin on 26/2 - Suturing done in ICU, by ICU resident on 26/2/2021. 7 nylon suture in situ - Reviewed by surgical team on 27/02/21 **#AKI, likely ATN 2ndary to hypoperfusion** - Cr168 on admission, normalized to 82 on discharge - Had proteinuria and glycosuria on dipstick urine - Spot Urine A/CR ratio 8.4, likely elevated secondary to AKI - For ongoing monitoring of renal function **# High BSL** - BSL 23 on arrival - Not a known diabetic. HbA1c 5.5 - Commenced insulin infusion, BSLs improved and normalised **#Schizophrenia and suicidal ideation** - On involuntary treatment order - Assessed as ongoing high risk of suicide, requires ongoing admission to the mental health unit for assessment and treatment once medically cleared **Plan**: 1. Ongoing wound reviews. Sutures to remain insitu for 7 days and arm needs to be kept straight for 7 days. For review of ?Removal of sutures Fri 5/3/2021 2. Monitor Hemoglobin and blood pressure for any further drop 3. Chase subsequent blood culture and sensitivty reports for growth of any other organisms 4. Continue oral flucloxacillin 500mg Q6hrly till 2 weeks are completed from 25/02/2021 5. Ongoing consultation with psychiatry unit

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Version 21.5 (Supports COMET) September 2018 ANZICS APD Data Collection Form

Page 2 of 8

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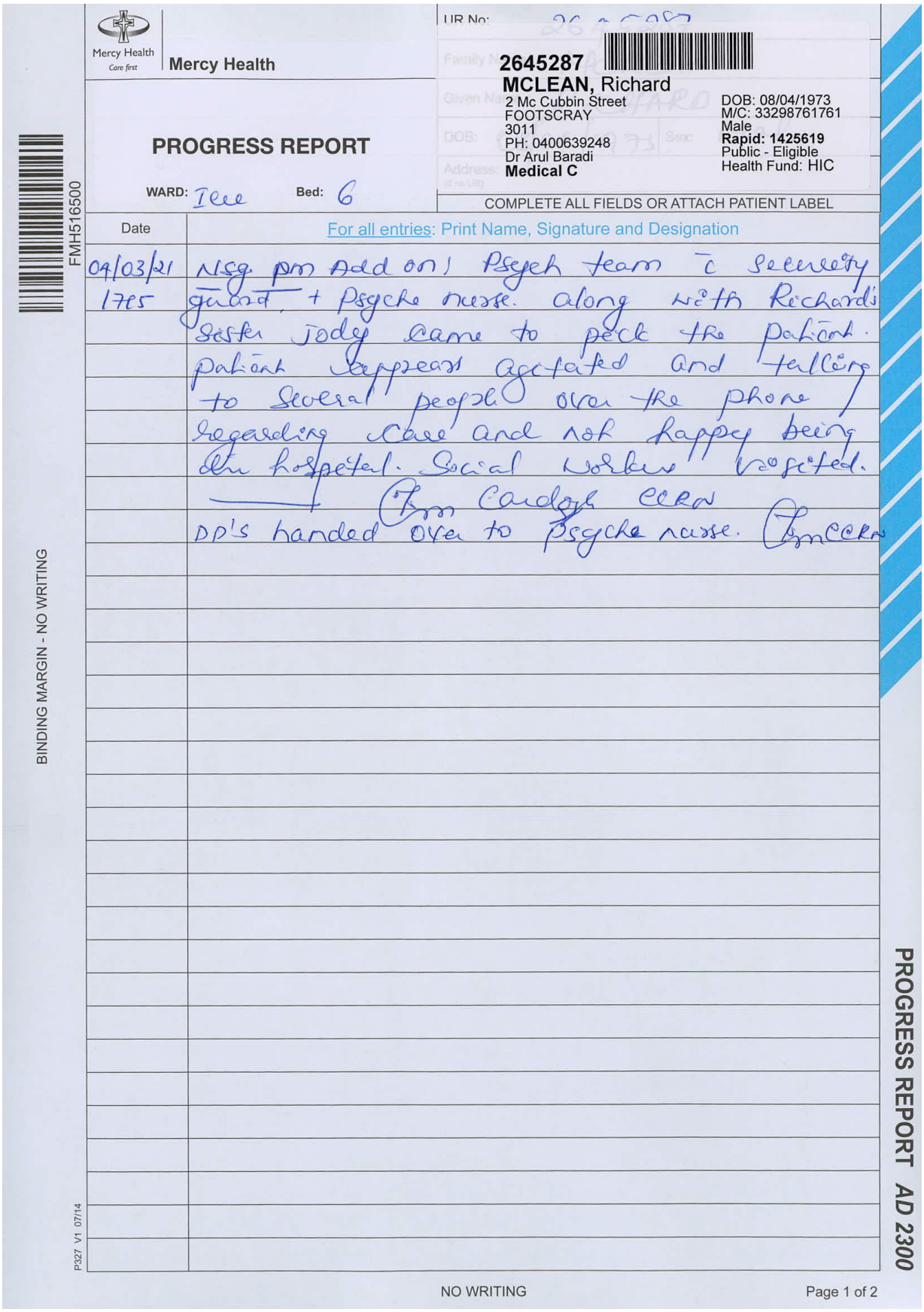
5.101. Progress Report (FMH516500)



Mercy Health
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UR No: 26 [illegible]

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PROGRESS REPORT

WARD: Tree Bed: 6

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH516500

BINDING MARGIN - NO WRITING

Date	For all entries: Print Name, Signature and Designation
09/03/21 1715	Nsg. pm Add on/ Psych team to see early guard + Psyche nurse. along with Richard's Sister Jody came to pick the patient. patient appears agitated and talking to several people over the phone regarding care and not happy being in hospital. Social worker notified. Tm Cardiac CCRN DD's handed over to Psyche nurse. Tm CCRN

PROGRESS REPORT AD 2300

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5.102. Progress Report (FMH516500)

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PROGRESS REPORT

WARD: **Bed:**

Date	For all entries: Print Name, Signature and Designation

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5.103. Risk Screening Tool (FMH517600)

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Health Fund: UNKF

RISK SCREENING TOOL

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH517600

BINDING MARGIN - NO WRITING

INFECTION CONTROL	NO	YES	ACTION REQUIRED
1. Does the patient have a current or previous diagnosis of Multi-Resistant Organism (MRO) infection or colonisation? E.g. VRE or MRSA ?			If yes • Implement Transmission Based Precautions (TBP)
2. Is the patient presenting with diarrhoea or vomiting?	✓		Refer to the TBP procedure or disease specific procedure on Prompt • Notify infection control
3. Is the patient presenting with any other known or suspected condition that requires isolation? (E.g. measles, pertussis, influenza, chickenpox etc.)	✓		
4. Has the patient had a previous diagnosis of Carbapenemase Producing Enterobacteriaceae (CPE)? ?			If yes • Implement TBP (isolation with strict contact precautions) • Take rectal swab, with evidence of faecal matter, for MRGN (multi resistant gram negative) screening • Notify infection control
5. Has the patient had an overnight stay in a hospital or aged care facility in the last 12 months?		✓	

If you answered 'yes' to any of the above, tick the strategies that have been implemented:

☐ Isolation room ☐ TBP signage ☐ Rectal swab taken ☐ I/C notified

- If after hours, notify I/C via phone message. I/C will respond on the next working day.

Name: _____ **Signature:** _____ **Designation:** _____

MEDICATION RISK SCREEN	NO	YES	ACTION REQUIRED
1. Is the patient taking any medication? (including prescription & complimentary medications)		✓	
2. Are any of the patient's own medications S8 or S11?		✓	If yes, they must be entered into register and locked in S8/S11 safe. If returned to family, document to whom they were given in clinical notes.
3. Does the patient meet two or more medication safety high risk & APINCHS criteria?		✓	If yes, please complete AD 1135; Pharmacy Referral form (for admitted patients only)

FALLS RISK SCREEN	NO	YES	ACTION REQUIRED
1. Does the patient take 3 or more prescription drugs?		✓	If yes to any, complete AD 2825; 6 Pack - Falls Assessment form
2. Is the patient over 65 years of age OR over 45 for Indigenous people?	✓		
3. Has the patient had a fall in the last 12 months?		✓	
4. Does the patient have any chronic medical condition/s affecting their balance and mobility? Eg. Cognitive impairment, Parkinson's Disease, Peripheral neuropathy, other neurological conditions.	✓		

Name: Elaiza Pelaw Signature: Date: 25/2/21 TIme: 2336

ALCOHOL AND OTHER DRUG SCREEN	NO	YES	ACTION REQUIRED
We routinely ask all patients about their alcohol and drug use.		✓	
1. Do you use alcohol, tobacco or other drugs?		✓	If yes, refer to the dual diagnosis nurse for further assessment or consultation.
2. Are you concerned about your alcohol, tobacco or other drug use?		✓	
3. Do you believe your attendance is related to alcohol, tobacco or other drug use?		✓	

Name: Elaiza Pelaw Signature: Date: 25/2/21 TIme: 2330

P2042 V3 09/18

RISK SCREENING TOOL AD 2515

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5.104. Risk Screening Tool (FMH517600)

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RISK SCREENING TOOL

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

PRESSURE INJURY RISK SCREEN	NO	YES	ACTION REQUIRED
1. Does the patient currently have 1 or more pressure injuries?	✓		If yes to any, complete AD 2875; Braden Pressure Injury Risk Assessment form
2. Does the patient need some or total assistance to move while in bed?	✓		
3. Is the patient confused, sedated or unconscious?	✓		
4. Does the patient have restricted mobility or sensation due to diabetes, stroke, PVD or other condition?	✓		
5. Is the patient over 65 years of age OR Aboriginal/TSI and over 45 years of age?	✓		

Name: Blaha Pelaez Signature: Date: 25/2/21 Time: 2325

DELIRIUM RISK ASSESSMENT	NO	YES	ACTION REQUIRED
1. Is the patient over 70 years of age?	✓		
2. Does the patient have visual impairment?	✓		
3. Does the patient have severe illness? (Including mental illness or depression, as identified by the nurse)		✓	
4. Does the patient have cognitive impairment?	✓		
5. Is the patient dehydrated? (Scanty, concentrate urine, fever, thirst, dry mucous membranes or raised creatinine/urea)	✓		
If your patient is greater than 70 years old and answered yes to at least one of the above risk factors = **RISK OF DELIRIUM**			If the patient is assessed at risk please follow Nursing and Medical Flow Chart for Delirium Recognition, Monitoring and Management.

ADVANCE CARE PLANNING	NO	YES	ACTION REQUIRED
1. Does the patient have an advance care plan? Including: • Both formally and informally documented written plans • A Refusal of Treatment Certificate • Verbal wishes documented by clinician on a Mercy Health Advance Care Planning Discussion Record			If yes, discuss with patient, family and team. Request a copy of the plan and file in legal divider in the medical record, or place at the front of the ward file. Place alert for advance care plan on clinical alert form (ALERT 0001) under 'other'. If no, provide Mercy Health Advance care planning brochure.
2. Does the patient have an appointed substitute decision maker? Including: • An Enduring Power of Attorney (Medical Treatment), • An Enduring Power of Attorney for personal matters (with authority to consent to healthcare), • Guardian appointed by VCAT with healthcare powers.			File as above. Place alert for substitute decision maker on the clinical alert form (ALERT 0001) under 'other' and record nature of decision maker. If no, provide Mercy Health advance care planning brochure.

Patient / Carer Education: Provide education information to inform the risks / prevention if patient identified as high risk for pressure injury / falls

	Previously Provided	Provided Now	Staff Initial	Date
Rights & Responsibilities	☐	☐		
Falls Prevention	☐	☐		
Pressure Injury Prevention	☐	☐		
_____	☐	☐		
_____	☐	☐		

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ADMISSION & SCREENING ASSESSMENT

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COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH519000

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CURRENT COMMUNITY SERVICES (Include date contacted regarding this admission)

☑ Home Help Service Provider: Garden Help	/ /	☐ HITH / MITH	/ /
☑ Meals on Wheels Service Provider: Nou	/ /	☐ Health Independence Programs (HIP)	/ /
☐ RDNS	/ /	☐ PACFU	/ /
☐ Community Palliative Care	/ /	☐ Other:	/ /
☐ Case Manager	/ /	☐ Other:	/ /

NEUROLOGICAL

Oriented to: ☑ Time ☐ Person ☑ Place

☐ History of cognitive impairment (Specify):

☐ Confused ☐ Drowsy ☐ Unconscious ☐ Agitated ☐ Restless

Sleep Pattern: ☐ Normal ☐ Insomnia | Sleep Aids/Medications used at home:

*** If confused / restless / agitated / drowsy – consider delirium and consult Medical Team**

*** If disoriented / confused / cognitively impaired / medication - complete MMSE (if trained). If not trained consult Medical Team**

NUTRITION AND HYDRATION

Admission Weight: **Kg**

Known - Food Allergies ☑ Intolerances ☐ Specify:

Usual Diet ☑ Regular ☐ Special dietary requirement Specify:
☐ Enteral ☐ Texture Modified - Puree/ Minced/ Soft (circle)

Usual Fluids ☐ Regular ☐ Thickened - Mild/ Moderate/ Extreme (circle)

Assistance with Meals:
☑ No ☐ Yes → ☐ Set Up Only ☐ Prompting ☐ Full assistance (refer Red Tray Program – WMH only)

Does the patient have difficulty swallowing? Yes ☐ No ☐ If Yes refer to Speech Pathology

Malnutrition Screening Tool (MST): (circle appropriate score to each question)

1. Has the Patient lost weight recently without trying?

No = 0 | Yes - 1-5kg = 1 | 6-10kg = 2 | 11-15kg = 3 | >15kg = 4 | Unsure = 2

2. Has the patient been eating poorly because of decreased appetite? No = 0 | Yes = 1

Total Score = 0 | **Score of 2 or higher – refer to Dietitian**

P1962 V1 09/14

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ADMISSION & SCREENING ASSESSMENT

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SELF-CARE

Does the patient require assistance with Personal Hygiene?

☑ No | ☐ Yes → ☐ Set Up ☐ Prompting ☐ Full Assistance

Does the patient require assistance with Mouth Care?

☑ No | ☐ Yes → ☐ Set Up ☐ Prompting ☐ Full Assistance

VTE

Risk Factors (tick as many as applicable)

- ☐ Age >60
- ☐ History of VTE
- ☐ History of ischemic stroke
- ☐ Acute on chronic lung disease
- ☐ Immobility
- ☐ Active cancer
- ☐ Congestive Cardiac Failure
- ☐ Medication with increased risk of thrombosis (estrogens, progesterone, raloxifene)

Contraindications for anti-embolitic stockings

- ☐ Localised dermatitis
- ☐ Gangrenous wound
- ☐ Recent skin graft
- ☐ Severe atherosclerosis
- ☐ Oedema of legs OR pulmonary oedema from CCF
- ☐ Extreme deformity of the leg
- ☐ Frail / Fragile skin
- ☐ Palliative Care

If risk factors are identified, and there are no contraindications then complete and commence mechanical VTE prophylaxis on medication chart. RMO to review re: chemical prophylaxis.

☐ **Complete Risk Screen on Medication Chart** | Completed by: | Date:

ELIMINATION / CONTINENCE

☐ Patient given Continence Guide (if applicable)

Urinary: Continent? ☑ Yes ☐ No → Aids:

If no, complete Continence Assessment and Management AD2970

☐ Nephrostomy ☐ Urostomy ☐ IDC/SPC (specify details below)

Reason for IDC/SPC: | Insertion Date: | Due to be changed:

Does the patient have any concerns regarding continence? ☐ No ☑ Yes (specify below)

☐ Leakage ☐ Urge / Stress ☐ Frequency ☐ Burning ☐ Odour ☐ Retention

Bowels Continent? ☑ Yes ☐ No → Aids:

If no, complete Continence Assessment and Management AD2970

Does patient suffer from or complain of: ☑ Constipation ☐ Diarrhoea

Bowels last open: 25/2/21 | Consistency:

☐ Colostomy ☐ Ileostomy Last Active:

Does the patient use aperients? ☐ No ☐ Yes → Product: Effective? ☐ No ☐ Yes

Commence Bowel Chart AD2980 for ALL patients

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ADMISSION & SCREENING ASSESSMENT

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Admission Date

☑ Check with patient that details on patient label are correct

Admitting Diagnosis:

Admitting Ward

☐ Has the appropriate ID band been used?
White ☐ Red ☑

☑ Is the patient aware of the reason for their admission?

☑ Charter of Healthcare Rights explained?

COMMUNICATION WITH NOK/FAMILY/CARER

☑ NOK details checked and documented

☑ NOK / family / carer aware of the admission

Person Notified: Jodie

Date Notified:

Notified by:

☐ Family / Carer involved in this Assessment

Carer Name (if applicable):

Relationship to patient:

LEGAL ORDERS

☑ Nil ☐ Advanced Care Directive

☐ General Power of Attorney ☐ Enduring Power of Attorney (Financial)

☐ Enduring Power of Attorney (Medical Treatment) ☐ Enduring Power of Guardianship

☐ Intervention Order ☐ Administration Order

☐ Other (Specify)

Copy Available? Yes ☐ No ☐

If No, Copy requested from:

Request by (Staff): Date:

RELEVANT PAST HISTORY

☐ Diabetes: ☐ Type 1 ☐ Type 2

☐ Heart Disease

☐ Lung Disease

☑ Mental Health

Other:

ALLERGIES / SENSITIVITIES ☐ No Known Allergies

Allergy / Sensitivity	Reaction
Bee stings	localised swelling
Augmentin	Vomiting.

Allergy / Sensitivity added to Clinical Alert Form ☑

Allergy/ Sensitivity added to Medication Chart ☑

PSYCHO – SOCIAL / SPIRITUAL / HOME SUPPORT

Usual Residence: Home alone

Will this be the discharge destination?

Yes ☑ No ☐

If No (specify):

Cultural / Spiritual needs and requests:

Social Issues:
(e.g. Financial, carers stress, legal issues, etc) Yes ☑ No ☐

Children under 18 yrs Yes ☐ No ☑
If Yes, refer to Social Work

COMMUNICATION

Preferred Language: English
(including Auslan for Hearing Impaired Clients)

Is an Interpreter required? Yes ☐ No ☑
(if Yes, document on Care Plan)

Are there comprehension difficulties? Yes ☐ No ☑

Specify:

Is the Patient visually impaired? Yes ☑ No ☐

Communication Aides:

☐ Hearing With Patient? Yes ☐ No ☐

☑ Glasses With Patient? Yes ☑ No ☐

☐ Other (specify, eg communication board):

ADMISSION & SCREENING ASSESSMENT AD 2780

NO WRITING

Mercy Health
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Mercy Health

2645287

MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

ADMISSION & SCREENING ASSESSMENT

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

ADMISSION FALLS ASSESSMENT

All patients must have falls assessment completed on admission. Please refer to 6 Pack Falls Assessment AD2825

☑ **Fall prevention plan commenced and documented on Functional Care Plan AD3085**

Completed By: Elaine Pelser Date: 25/2/21

ADMISSION SKIN ASSESSMENT AND PRESSURE INJURY

Pre-existing Pressure Areas at admission ☑ No ☐ Yes If Yes complete VHIMS and commence Wound Chart AD2170
Location:

☐ Stage 1 ☐ Stage 2 ☐ Stage 3 ☐ Stage 4 ☐ Unspecified ☐ Depth Unknown

Skin Inspection: (Check entire body focusing on bony prominences included sacral region, heels, ischial tuberosities and greater trochanters.)

☐ Erythema ☐ Localised heat ☐ Blanching response

☐ Oedema ☐ Induration ☐ Skin breakdown (wound chart required)

☐ Pressure Injury Prevention Risk Assessment completed on admission.
Refer Pressure Injury Risk Assessment Tool 2875

PATIENT MOBILITY RISK ASSESSMENT

	Yes	No
Does the patient weigh more than 100 kilos?		✓
Is the patient unable to move from sit to stand unaided ?		✓
Does the patient use mobility aids?		✓
Is the patient unable to walk without assistance?		✓
Is the patient's condition likely to change whilst undergoing treatment e.g. surgery?		✓
Does the patient's mobility fluctuate owing to disease process e.g. multiple sclerosis?		✓
Does the patient have any cognitive impairment?		✓
Does the patient need assistance in moving in bed?		✓
Does the current medication affect mobility?		✓

If you have answered yes to any of these questions then please complete Patient Mobility Risk Assessment AD2870 and refer to Physio

THIS ADMISSION AND SCREENING ASSESSMENT HAS BEEN COMPLETED BY:

Name: Elaine Pelser	**Designation:** CCRN	**Time:** 2315	**Date:** 25/2/21

REFERRALS REQUIRED ☐ No ☐ Yes Referrals completed ☐

BINDING MARGIN - NO WRITING

Mercy Health
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5.107. 6 Pack Falls Ass (FMH519900)

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6 PACK – FALLS ASSESSMENT

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COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH519900

BINDING MARGIN – NO WRITING

RISK ASSESSMENT PROTOCOL

1. Complete Risk Assessments at least daily and update each shift
2. Score according to their current characteristics.
   - On admission, circle relevant scores on the actual Risk Tools below. (Do not write subsequent changes on actual Tool as items circled are to reflect patient's condition on admission)
3. Use Risk Score and Professional knowledge to determine the Risk Level
4. Document details in dedicated sections inside the Care Plan, Pathway or Observation Chart
   - Ensure this Fall Risk Assessment Insert is kept with Care Plan at all times
5. Normal preventative clinical care must be provided for patients identified as being at low risk
6. For patients identified as being at increased risk:
   - Implement the most appropriate Prevention Strategies which should consist of the Falls Risk Alert Sign and AT LEAST one other of the strategies noted in the dedication section inside the Care Plan.

Consider referrals to other Disciplines

FALLS PREVENTION

RISK ASSESSMENT	Circle Scores here on admission	Score
1. Fall: Current admission?	Yes, Patient had a fall/s during current admission	3
2. Fall: within 12 months?	Yes, Patient had fall/s in the last 12 months	(1)
3. Mental State?	Yes, Patient is confused, agitated, intellectually challenged or impulsive (anxious, paranoid)	(1)
4. Mobility?	Yes, Patient needs supervision or assistance when mobilising	(1)
5. Impaired Balance?	Yes, Patient has impaired balance and/or hemiplegia	1
6. Age?	Yes, Patient is 80 years or older	1
7. Toileting?	Yes, Patient is in need of frequent toileting	1
8. Vision?	Yes, Patient is visually impaired so that everyday function is affected	1
9. Drug / Alcohol?	Yes, Patient presented with drug/ alcohol related problems	1
		3 0

PREVENTION STRATEGIES: Focus on strategies outlined in "Falls" box inside Care Plan

Date Tool above completed: 25 / 2 / 21 Name/ Signature: Blaha Reader Allen

Document details if change occurs: (Or if preferred, may use new risk assessment form)

- New score: 3 → 1 Date/Time: 28/2/21 Reason: Point 2 only.

Print Name: M. Greasley Signature: Designation: E.E.N.

- New score: _____ Date/Time: _____ Reason: _____

Print Name: _____ Signature: _____ Designation: _____

- New score: _____ Date/Time: _____ Reason: _____

Print Name: _____ Signature: _____ Designation: _____

The Northern Hospital Modified STRATIFY (TNH-STRATIFY)

Barker AL, Kamar J, Graco M, Lawlor V, Hill K. Adding value to the STRATIFY falls risk assessment in acute hospitals. J Adv Nurs 2011 Feb; 67(2):450-7

P1949 V1 07/14

6 PACK – FALLS ASSESSMENT AD 2825

NO WRITING

Page 1 of 1

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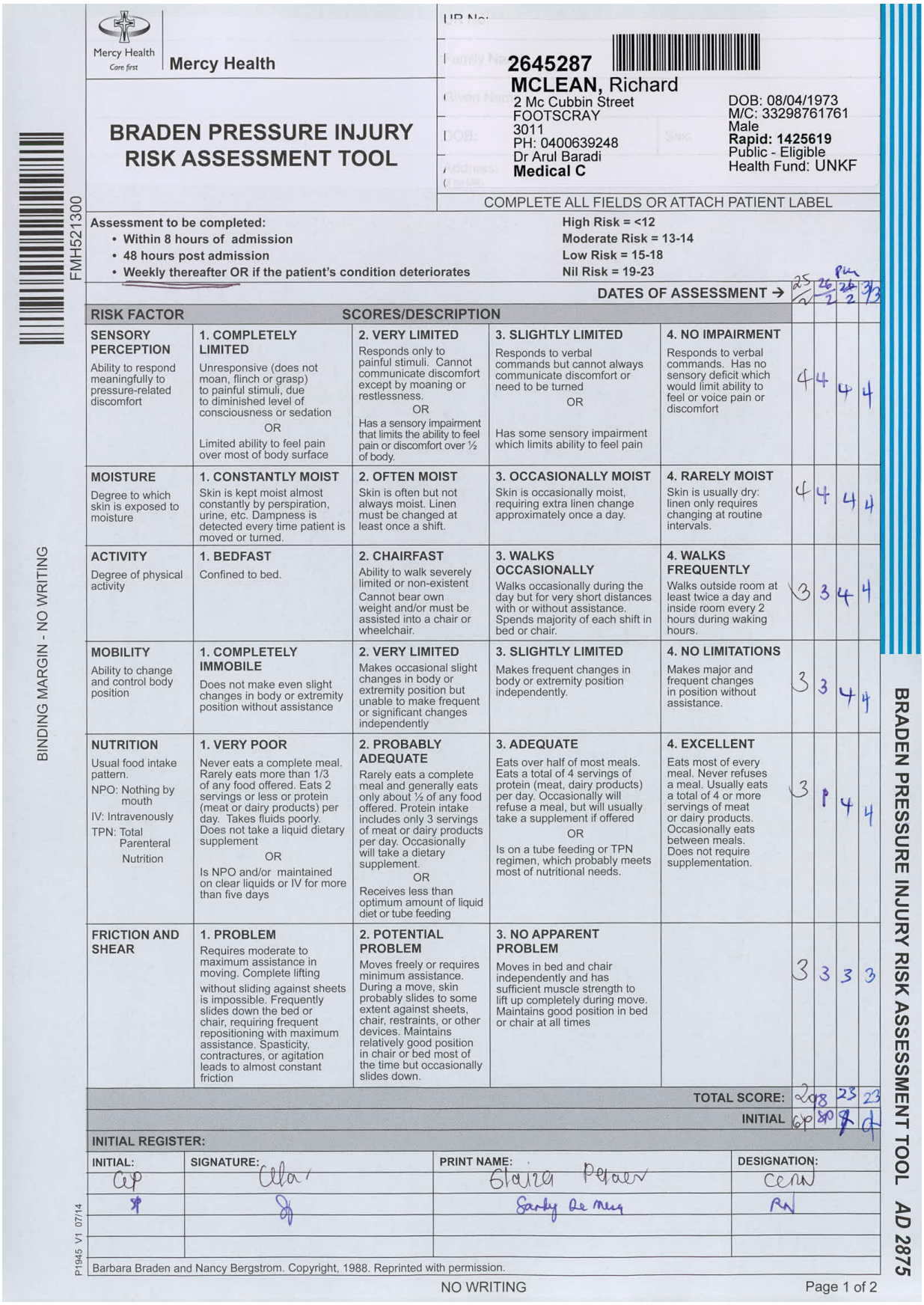
5.108. Braden Scale Pressure Injury Risk Ass Tool (FMH521300)



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COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

BRADEN PRESSURE INJURY RISK ASSESSMENT TOOL

FMH521300

Assessment to be completed:
- **Within 8 hours of admission**
- **48 hours post admission**
- **Weekly thereafter OR if the patient's condition deteriorates**

High Risk = <12
Moderate Risk = 13-14
Low Risk = 15-18
Nil Risk = 19-23

RISK FACTOR	SCORES/DESCRIPTION				DATES OF ASSESSMENT → 25/2 (Pm)	26/2	26/2	3/3
SENSORY PERCEPTION Ability to respond meaningfully to pressure-related discomfort	**1. COMPLETELY LIMITED** Unresponsive (does not moan, flinch or grasp) to painful stimuli, due to diminished level of consciousness or sedation OR Limited ability to feel pain over most of body surface	**2. VERY LIMITED** Responds only to painful stimuli. Cannot communicate discomfort except by moaning or restlessness. OR Has a sensory impairment that limits the ability to feel pain or discomfort over ½ of body.	**3. SLIGHTLY LIMITED** Responds to verbal commands but cannot always communicate discomfort or need to be turned OR Has some sensory impairment which limits ability to feel pain	**4. NO IMPAIRMENT** Responds to verbal commands. Has no sensory deficit which would limit ability to feel or voice pain or discomfort	4	4	4	4
MOISTURE Degree to which skin is exposed to moisture	**1. CONSTANTLY MOIST** Skin is kept moist almost constantly by perspiration, urine, etc. Dampness is detected every time patient is moved or turned.	**2. OFTEN MOIST** Skin is often but not always moist. Linen must be changed at least once a shift.	**3. OCCASIONALLY MOIST** Skin is occasionally moist, requiring extra linen change approximately once a day.	**4. RARELY MOIST** Skin is usually dry; linen only requires changing at routine intervals.	4	4	4	4
ACTIVITY Degree of physical activity	**1. BEDFAST** Confined to bed.	**2. CHAIRFAST** Ability to walk severely limited or non-existent Cannot bear own weight and/or must be assisted into a chair or wheelchair.	**3. WALKS OCCASIONALLY** Walks occasionally during the day but for very short distances with or without assistance. Spends majority of each shift in bed or chair.	**4. WALKS FREQUENTLY** Walks outside room at least twice a day and inside room every 2 hours during waking hours.	3	3	4	4
MOBILITY Ability to change and control body position	**1. COMPLETELY IMMOBILE** Does not make even slight changes in body or extremity position without assistance	**2. VERY LIMITED** Makes occasional slight changes in body or extremity position but unable to make frequent or significant changes independently	**3. SLIGHTLY LIMITED** Makes frequent changes in body or extremity position independently.	**4. NO LIMITATIONS** Makes major and frequent changes in position without assistance.	3	3	4	4
NUTRITION Usual food intake pattern. NPO: Nothing by mouth IV: Intravenously TPN: Total Parenteral Nutrition	**1. VERY POOR** Never eats a complete meal. Rarely eats more than 1/3 of any food offered. Eats 2 servings or less or protein (meat or dairy products) per day. Takes fluids poorly. Does not take a liquid dietary supplement OR Is NPO and/or maintained on clear liquids or IV for more than five days	**2. PROBABLY ADEQUATE** Rarely eats a complete meal and generally eats only about ½ of any food offered. Protein intake includes only 3 servings of meat or dairy products per day. Occasionally will take a dietary supplement. OR Receives less than optimum amount of liquid diet or tube feeding	**3. ADEQUATE** Eats over half of most meals. Eats a total of 4 servings of protein (meat, dairy products) per day. Occasionally will refuse a meal, but will usually take a supplement if offered OR Is on a tube feeding or TPN regimen, which probably meets most of nutritional needs.	**4. EXCELLENT** Eats most of every meal. Never refuses a meal. Usually eats a total of 4 or more servings of meat or dairy products. Occasionally eats between meals. Does not require supplementation.	3	P	4	4
FRICTION AND SHEAR	**1. PROBLEM** Requires moderate to maximum assistance in moving. Complete lifting without sliding against sheets is impossible. Frequently slides down the bed or chair, requiring frequent repositioning with maximum assistance. Spasticity, contractures, or agitation leads to almost constant friction	**2. POTENTIAL PROBLEM** Moves freely or requires minimum assistance. During a move, skin probably slides to some extent against sheets, chair, restraints, or other devices. Maintains relatively good position in chair or bed most of the time but occasionally slides down.	**3. NO APPARENT PROBLEM** Moves in bed and chair independently and has sufficient muscle strength to lift up completely during move. Maintains good position in bed or chair at all times		3	3	3	3
				TOTAL SCORE:	20	18	23	23
				INITIAL	GP	SP	[illegible]	[illegible]

INITIAL REGISTER:

INITIAL:	SIGNATURE:	PRINT NAME:	DESIGNATION:
GP	[signature]	Gloria Peraev	CERN
SP	[signature]	Sandy De Meg	RN

BINDING MARGIN - NO WRITING

BRADEN PRESSURE INJURY RISK ASSESSMENT TOOL AD 2875

P1945 V1 07/14

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5.109. Braden Scale Pressure Injury Risk Ass Tool (FMH521300)

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BRADEN PRESSURE INJURY RISK ASSESSMENT TOOL

UR No:
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COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

MANDATORY PRESSURE INJURY PREVENTION STRATEGIES (Implement for all patients)

- Assess & manage all bony prominences at risk of pressure damage
- Involve & educate patient and family on how they can prevent pressure injuries
- Maintain skin hygiene
- Assess mobility
- Use slide sheets when moving patient to eliminate shear & friction
- Assess nutritional risk & promote optimal nutrition

☐ **LOW RISK –** mandatory strategies implemented as above **Date:** **Signature:**

☐ **MODERATE/HIGH RISK –** Implement/continue prevention strategies as listed below

	STRATEGY IMPLEMENTATION DATE			
Address sensory perception	**TICK AS APPROPRIATE**			
Reposition patient at least 2 hourly or _____ (frequency specified) & document the position changes in the progress notes				
Encourage seated position changes every 15 minutes & limit sitting to <2 hours at a time, even when on a pressure relieving cushion				
Skin integrity				
Conduct comprehensive skin assessment twice daily for high risk patients				
Moisture				
Actively promote continence. Use an individualised toileting regime				
Skin care regime neutral pH soap and protective barrier creams as appropriate				
Assess for dry skin. Manage and treat skin to improve moisture balance				
Promote activity & mobility to relieve pressure				
Encourage bed mobility & early ambulation where possible				
Support surfaces implemented: refer to mattress selection guide Mattress surface:_____				
Pressure relieving cushion if sitting out of bed				
Head of the bed at 30 degrees unless medical condition contraindicates				
Elevate heels using pillow OR wedges				
Activate knee brake on bed				
Loose bed clothes OR bed cradle				
Apply protective dressing to heels/elbows to reduce sheer and friction				
Occupational therapy referral				
Physiotherapy referral				
Address nutrition				
Consult Dietitian to plan nutritional support				
Commence food chart				
Management of devices				
Minimise & eliminate pressure from medical devices such as IV lines, oxygen tubing- apply ear pressure relieving device				
Minimise & eliminate pressure from catheters- apply anchoring devices				
Minimise & eliminate pressure from nasogastric tubing				
Minimise and eliminate pressure for other devices Specify:_____				
Additional strategies				
INITIAL				

INITIAL REGISTER:

INITIAL:	SIGNATURE:	PRINT NAME:	DESIGNATION:

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Page 2 of 2

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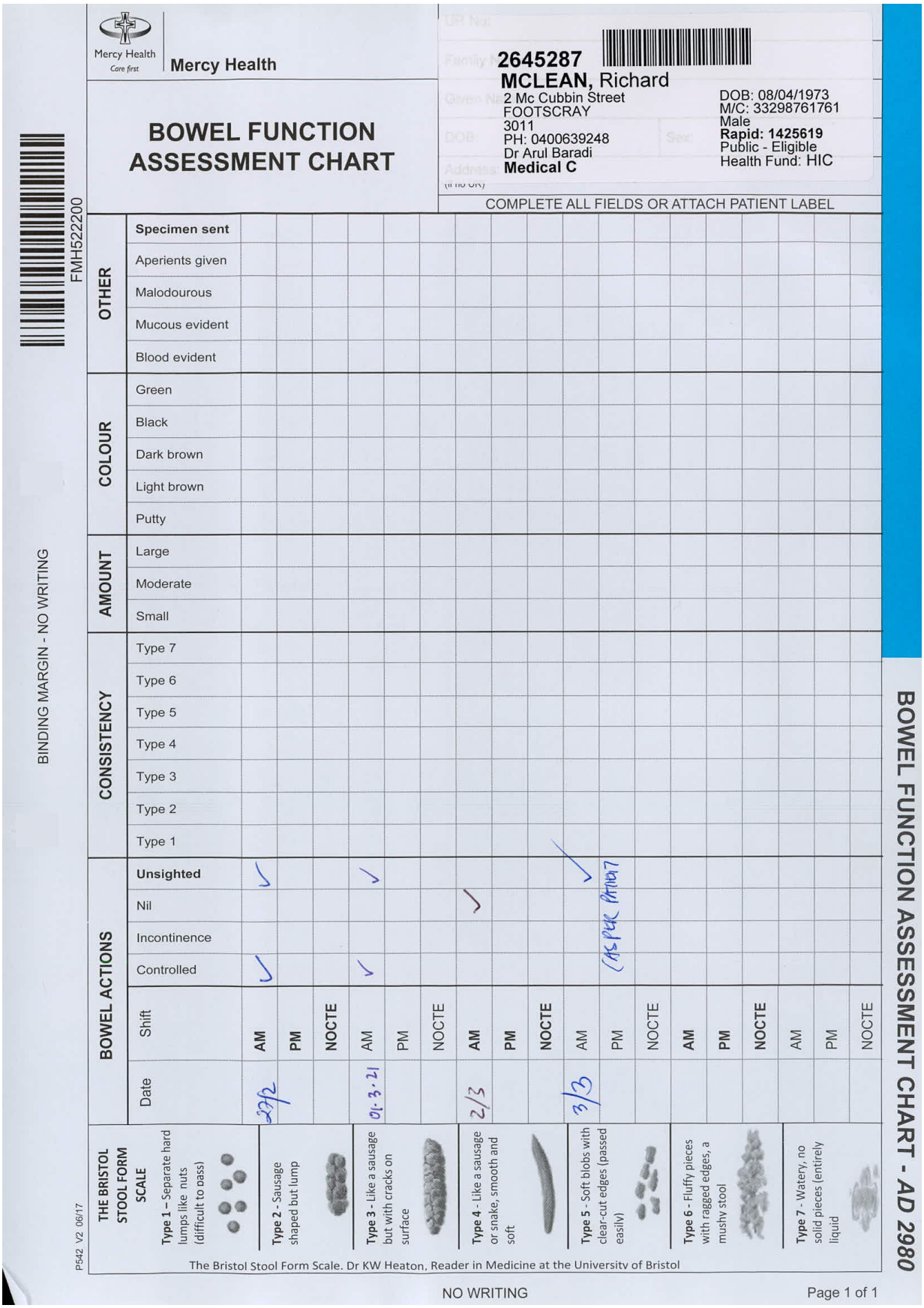
5.110. Bowel Function Ass Chart (FMH522200)



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BOWEL FUNCTION ASSESSMENT CHART

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Public - Eligible
Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH522200

BINDING MARGIN - NO WRITING

BOWEL ACTIONS	Date	22/2			01.3.21			2/3			3/3										
	Shift	**AM**	**PM**	**NOCTE**	AM	PM	NOCTE	**AM**	**PM**	**NOCTE**	AM	PM	NOCTE	**AM**	**PM**	**NOCTE**	AM	PM	NOCTE		
	Controlled	✓			✓							(As PRN Amiad)									
	Incontinence																				
	Nil							✓													
	Unsighted	✓			✓						✓										
CONSISTENCY	Type 1																				
	Type 2																				
	Type 3																				
	Type 4																				
	Type 5																				
	Type 6																				
	Type 7																				
AMOUNT	Small																				
	Moderate																				
	Large																				
COLOUR	Putty																				
	Light brown																				
	Dark brown																				
	Black																				
	Green																				
OTHER	Blood evident																				
	Mucous evident																				
	Malodourous																				
	Aperients given																				
	Specimen sent																				

THE BRISTOL STOOL FORM SCALE

Type 1 – Separate hard lumps like nuts (difficult to pass)

Type 2 - Sausage shaped but lump

Type 3 - Like a sausage but with cracks on surface

Type 4 - Like a sausage or snake, smooth and soft

Type 5 - Soft blobs with clear-cut edges (passed easily)

Type 6 - Fluffy pieces with ragged edges, a mushy stool

Type 7 - Watery, no solid pieces (entirely liquid

The Bristol Stool Form Scale. Dr KW Heaton, Reader in Medicine at the University of Bristol

P542 V2 06/17

BOWEL FUNCTION ASSESSMENT CHART - *AD 2980*

NO WRITING

Page 1 of 1

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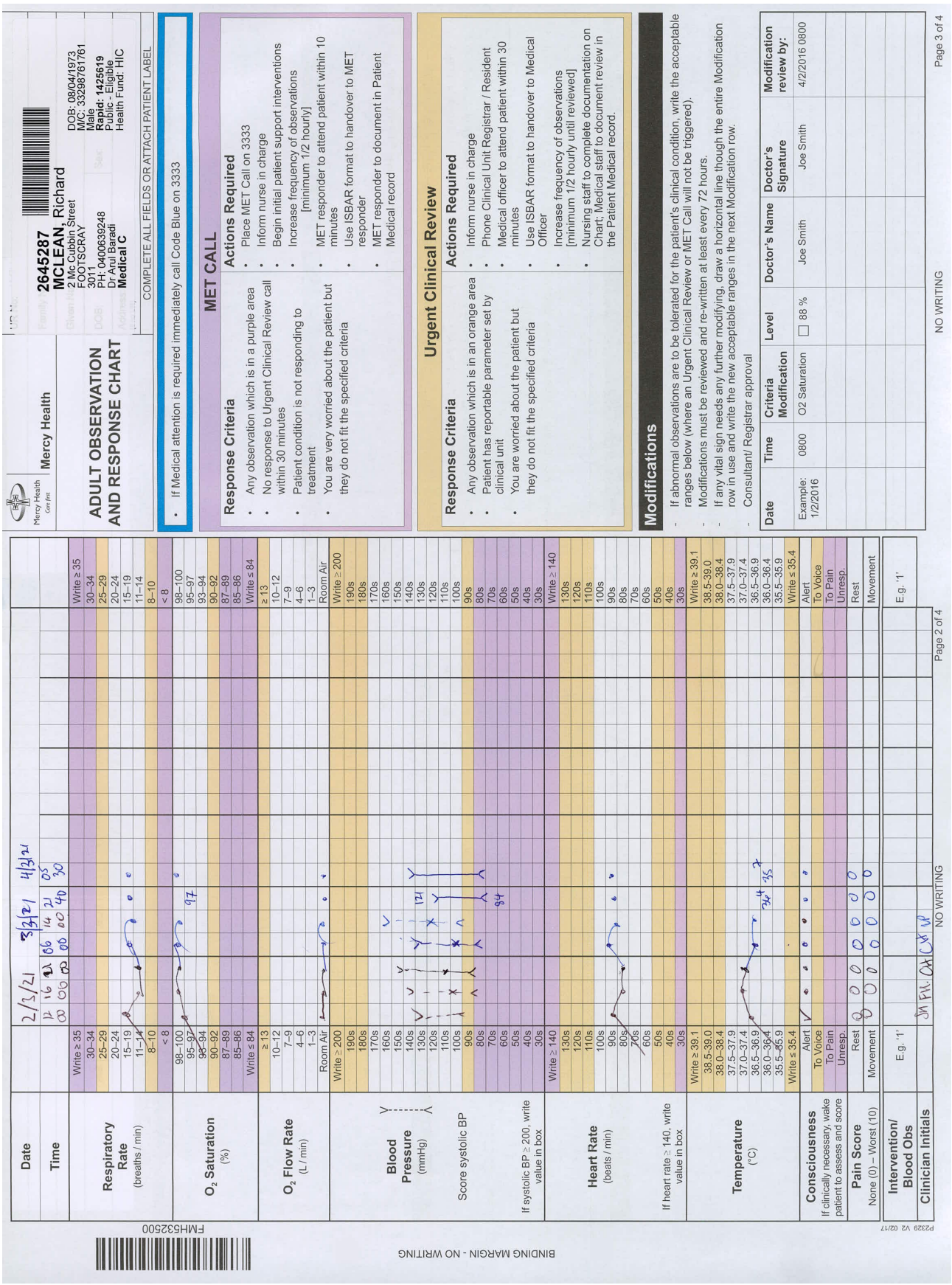
5.111. Adult Obs & Response Chart (FMH532500)



ADULT OBSERVATION AND RESPONSE CHART

Mercy Health

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COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

- If Medical attention is required immediately call Code Blue on 3333

MET CALL

Response Criteria	Actions Required
• Any observation which is in a purple area • No response to Urgent Clinical Review call within 30 minutes • Patient condition is not responding to treatment • You are very worried about the patient but they do not fit the specified criteria	• Place MET Call on 3333 • Inform nurse in charge • Begin initial patient support interventions • Increase frequency of observations [minimum 1/2 hourly] • MET responder to attend patient within 10 minutes • Use ISBAR format to handover to MET responder • MET responder to document in Patient Medical record

Urgent Clinical Review

Response Criteria	Actions Required
• Any observation which is in an orange area • Patient has reportable parameter set by clinical unit • You are worried about the patient but they do not fit the specified criteria	• Inform nurse in charge • Phone Clinical Unit Registrar / Resident • Medical officer to attend patient within 30 minutes • Use ISBAR format to handover to Medical Officer • Increase frequency of observations [minimum 1/2 hourly until reviewed] • Nursing staff to complete documentation on Chart; Medical staff to document review in the Patient Medical record.

Modifications

- If abnormal observations are to be tolerated for the patient's clinical condition, write the acceptable ranges below (where an Urgent Clinical Review or MET Call will not be triggered).
- Modifications must be reviewed and re-written at least every 72 hours.
- If any vital sign needs any further modifying, draw a horizontal line though the entire Modification row in use and write the new acceptable ranges in the next Modification row.
- Consultant/ Registrar approval

Date	Time	Criteria Modification	Level	Doctor's Name	Doctor's Signature	Modification review by:
Example: 1/2/2016	0800	O2 Saturation	☐ 88 %	Joe Smith	Joe Smith	4/2/2016 0800

NO WRITING

Page 3 of 4

Page 2 of 4

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FMH532500

P2229 V2 02/17

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DOB: 08-Apr-1973 URNO: 2645287 (MHS)
McLean, Richard

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ADULT OBSERVATION AND RESPONSE CHART

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Health Fund: HIC

(if no UR)

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Other Observation Charts In Use

- ☐ Alcohol
- ☐ Daily Fluid Balance
- ☐ Diabetes Record
- ☐ Neurological
- ☐ Neurovascular
- ☐ Pain/Analgesia Observation
- ☐ Peripheral Intravenous Record
- ☐ _____
- ☐ _____

General Instructions

» You must record a full set of observations:
  - On admission
  - At a frequency appropriate for the patient's clinical state
  - If the patient is deteriorating or an observation is in a shaded area
  - Whenever you are concerned about the patient.

» When graphing observations, place a dot (•) in the centre of the box which includes the current observation in its range of values and connect it to the previous dot with a straight line. For blood pressure, use the symbol indicated on the chart.

» Whenever an observation falls within a shaded area, you must initiate the actions required for that colour, unless a modification has been made (see overleaf).

» If observations fall within both purple and orange coloured areas for the same time period, the actions required for the purple area apply.

Blood Observations

B	Baseline
15	15 minutes post commencement
H	Hourly observations
C	On Completion

Frequency of Observations/ Monitoring Plan

Monitoring Plan must be reviewed daily

Refer to Adult Observation and Response Chart Procedure for Frequency of Observation instructions/minimum standard

If Rapid Response system is activated, Frequency of Observations will be as per Adult Escalation of Care Procedure

Date:	(e.g.) 6/4/14						
Frequency of Observations:	2/24 or QID						
Name / Designation:	~~Smith RN~~						

ADULT OBSERVATION AND RESPONSE CHART AD 4420

NO WRITING

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ADULT OBSERVATION AND RESPONSE CHART

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COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Interventions Associated With Abnormal Vital Signs

If you administer an intervention, record here and note number in Intervention row over page in appropriate time column.

Reference Number	Intervention (initial if required)
1	
2	
3	
4	
5	
6	
7	
8	

Urgent Clinical Review Requests

Review requested Date / / Time : Dr Name & Designation

Specify reason:

Review requested Date / / Time : Dr Name & Designation

Specify reason:

Review requested Date / / Time : Dr Name & Designation

Specify reason:

BINDING MARGIN - NO WRITING

Additional Observations

Date							
Time							
Weight (kg)							
Bowels							
Urinalysis	Specific gravity						
	pH						
	Leukocytes						
	Blood						
	Nitrite						
	Ketones						
	Bilirubin						
	Urobilinogen						
	Protein						
	Glucose						

NO WRITING

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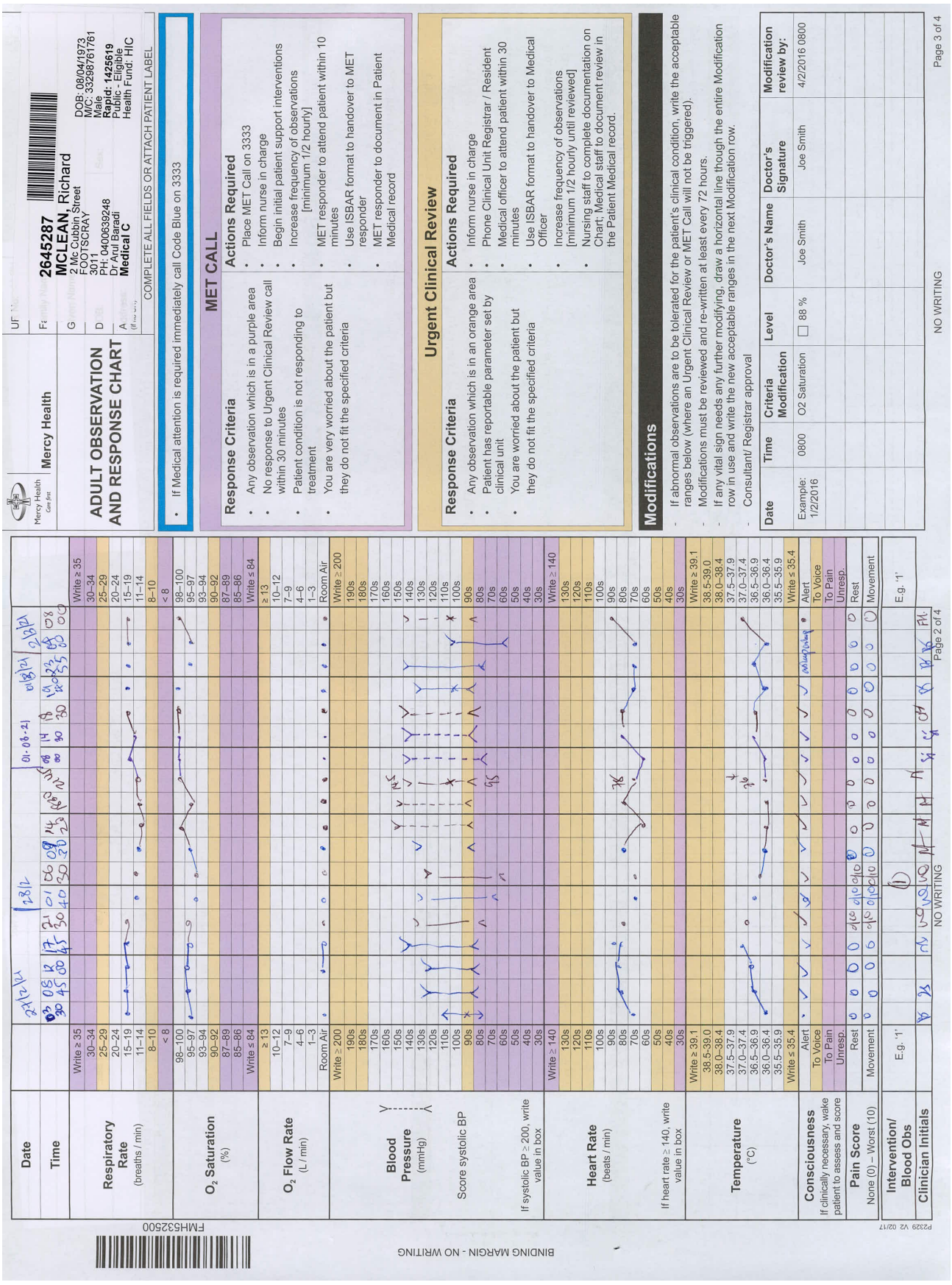
5.113. Adult Obs & Response Chart (FMH532500)



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ADULT OBSERVATION AND RESPONSE CHART

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Public - Eligible
Health Fund: HIC

UR No: (if no UR)

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Other Observation Charts In Use

☐ Alcohol	☐ Neurological	☑ Peripheral Intravenous Record
☑ Daily Fluid Balance	☑ Neurovascular	☑ Bowel chart
☐ Diabetes Record	☐ Pain/Analgesia Observation	☐

General Instructions

» You must record a full set of observations:
- On admission
- At a frequency appropriate for the patient's clinical state
- If the patient is deteriorating or an observation is in a shaded area
- Whenever you are concerned about the patient.

» When graphing observations, place a dot (•) in the centre of the box which includes the current observation in its range of values and connect it to the previous dot with a straight line. For blood pressure, use the symbol indicated on the chart.

» Whenever an observation falls within a shaded area, you must initiate the actions required for that colour, unless a modification has been made (see overleaf).

» If observations fall within both purple and orange coloured areas for the same time period, the actions required for the purple area apply.

Blood Observations

B	Baseline
15	15 minutes post commencement
H	Hourly observations
C	On Completion

Frequency of Observations/ Monitoring Plan

Monitoring Plan must be reviewed daily

Refer to Adult Observation and Response Chart Procedure for Frequency of Observation instructions/minimum standard

If Rapid Response system is activated, Frequency of Observations will be as per Adult Escalation of Care Procedure

Date:	(e.g.) 6/4/14	27/2/21	28/2/21				
Frequency of Observations:	2/24 or QID	4-6/24	4-6/24				
Name / Designation:	Smith RN	ml	ml				

ADULT OBSERVATION AND RESPONSE CHART AD 4420

Mercy Health
Care first

Mercy Health

ADULT OBSERVATION AND RESPONSE CHART

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Interventions Associated With Abnormal Vital Signs

If you administer an intervention, record here and note number in Intervention row over page in appropriate time column.

Reference Number	Intervention (initial if required)
1	Bloods sent, neurovase obs, wound done ml
2	
3	
4	
5	
6	
7	
8	

Urgent Clinical Review Requests

Review requested Date / / Time : Dr Name & Designation

Specify reason:

Review requested Date / / Time : Dr Name & Designation

Specify reason:

Review requested Date / / Time : Dr Name & Designation

Specify reason:

Additional Observations

Date								
Time								
Weight (kg)								
Bowels								
Urinalysis	Specific gravity							
	pH							
	Leukocytes							
	Blood							
	Nitrite							
	Ketones							
	Bilirubin							
	Urobilinogen							
	Protein							
	Glucose							

BINDING MARGIN - NO WRITING

Mercy Health
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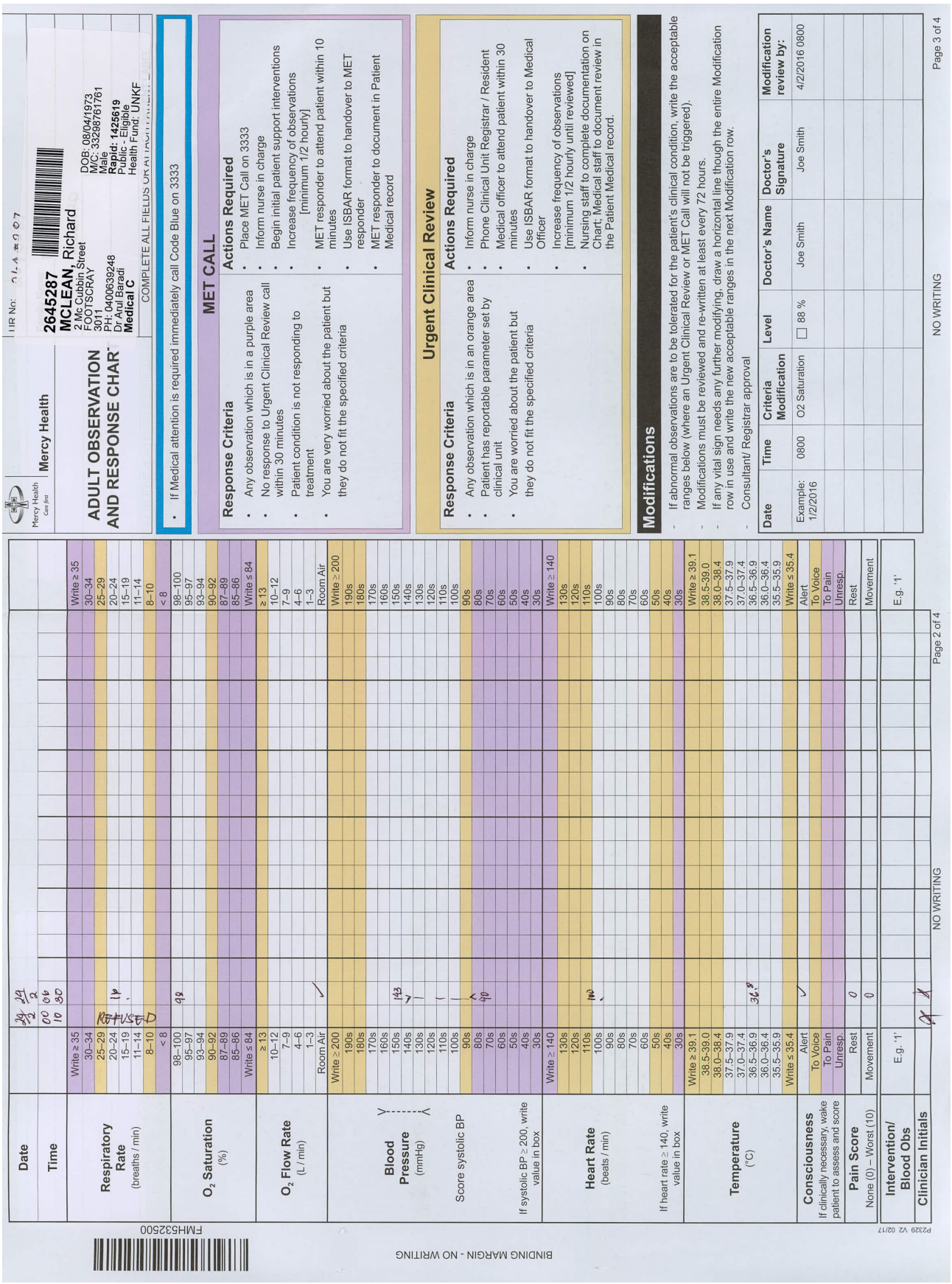
5.115. Adult Obs & Response Chart (FMH532500)



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Health Fund: UNKF

ADULT OBSERVATION AND RESPONSE CHART

COMPLETE ALL FIELDS OR AFFIX LABEL

- If Medical attention is required immediately call Code Blue on 3333

MET CALL

Response Criteria	Actions Required
• Any observation which is in a purple area • No response to Urgent Clinical Review call within 30 minutes • Patient condition is not responding to treatment • You are very worried about the patient but they do not fit the specified criteria	• Place MET Call on 3333 • Inform nurse in charge • Begin initial patient support interventions • Increase frequency of observations [minimum 1/2 hourly] • MET responder to attend patient within 10 minutes • Use ISBAR format to handover to MET responder • MET responder to document in Patient Medical record

Urgent Clinical Review

Response Criteria	Actions Required
• Any observation which is in an orange area • Patient has reportable parameter set by clinical unit • You are worried about the patient but they do not fit the specified criteria	• Inform nurse in charge • Phone Clinical Unit Registrar / Resident • Medical officer to attend patient within 30 minutes • Use ISBAR format to handover to Medical Officer • Increase frequency of observations [minimum 1/2 hourly until reviewed] • Nursing staff to complete documentation on Chart; Medical staff to document review in the Patient Medical record.

Modifications

- If abnormal observations are to be tolerated for the patient's clinical condition, write the acceptable ranges below (where an Urgent Clinical Review or MET Call will not be triggered).
- Modifications must be reviewed and re-written at least every 72 hours.
- If any vital sign needs any further modifying, draw a horizontal line though the entire Modification row in use and write the new acceptable ranges in the next Modification row.
- Consultant/ Registrar approval

Date	Time	Criteria Modification	Level	Doctor's Name	Doctor's Signature	Modification review by:
Example: 1/2/2016	0800	O2 Saturation	☐ 88 %	Joe Smith	Joe Smith	4/2/2016 0800

NO WRITING

Page 3 of 4

| Date | 24/2 | 24/2 | | |

5.116. Adult Obs & Response Chart (FMH532500)

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Dr Arul Baradi
Medical C
DOB: 08/04/1973
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Health Fund: UNKF

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

ADULT OBSERVATION AND RESPONSE CHART

Other Observation Charts In Use

- Alcohol
- Daily Fluid Balance
- Diabetes Record
- Neurological
- Neurovascular
- Pain/Analgesia Observation
- Peripheral Intravenous Record

General Instructions

» You must record a full set of observations:
- On admission
- At a frequency appropriate for the patient's clinical state
- If the patient is deteriorating or an observation is in a shaded area
- Whenever you are concerned about the patient.

» When graphing observations, place a dot (•) in the centre of the box which includes the current observation in its range of values and connect it to the previous dot with a straight line. For blood pressure, use the symbol indicated on the chart.

» Whenever an observation falls within a shaded area, you must initiate the actions required for that colour, unless a modification has been made (see overleaf).

» If observations fall within both purple and orange coloured areas for the same time period, the actions required for the purple area apply.

Blood Observations

B Baseline
15 15 minutes post commencement
H Hourly observations
C On Completion

Frequency of Observations/ Monitoring Plan

Monitoring Plan must be reviewed daily

Refer to Adult Observation and Response Chart Procedure for Frequency of Observation instructions/minimum standard

If Rapid Response system is activated, Frequency of Observations will be as per Adult Escalation of Care Procedure

Date:	(e.g.) 6/4/14						
Frequency of Observations:	2/24 or QID						
Name / Designation:	Smith RN						

NO WRITING

Page 1 of 4

ADULT OBSERVATION AND RESPONSE CHART *AD 4420*

BINDING MARGIN - NO WRITING

UR No: 2645287
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Health Fund: UNKF

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

ADULT OBSERVATION AND RESPONSE CHART

Interventions Associated With Abnormal Vital Signs

If you administer an intervention, record here and note number in intervention row over page in appropriate time column.

Reference Number	Intervention (initial if required)
1	
2	
3	
4	
5	
6	
7	
8	

Urgent Clinical Review Requests

Review requested Date / / Time : Dr Name & Designation
Specify reason:

Review requested Date / / Time : Dr Name & Designation
Specify reason:

Review requested Date / / Time : Dr Name & Designation
Specify reason:

Additional Observations

Date																								
Time																								
Weight (kg)																								
Bowels																								
Urinalysis	Specific gravity																							
	pH																							
	Leukocytes																							
	Blood																							
	Nitrite																							
	Ketones																							
	Bilirubin																							
	Urobilinogen																							
	Protein																							
	Glucose																							

NO WRITING

Page 4 of 4

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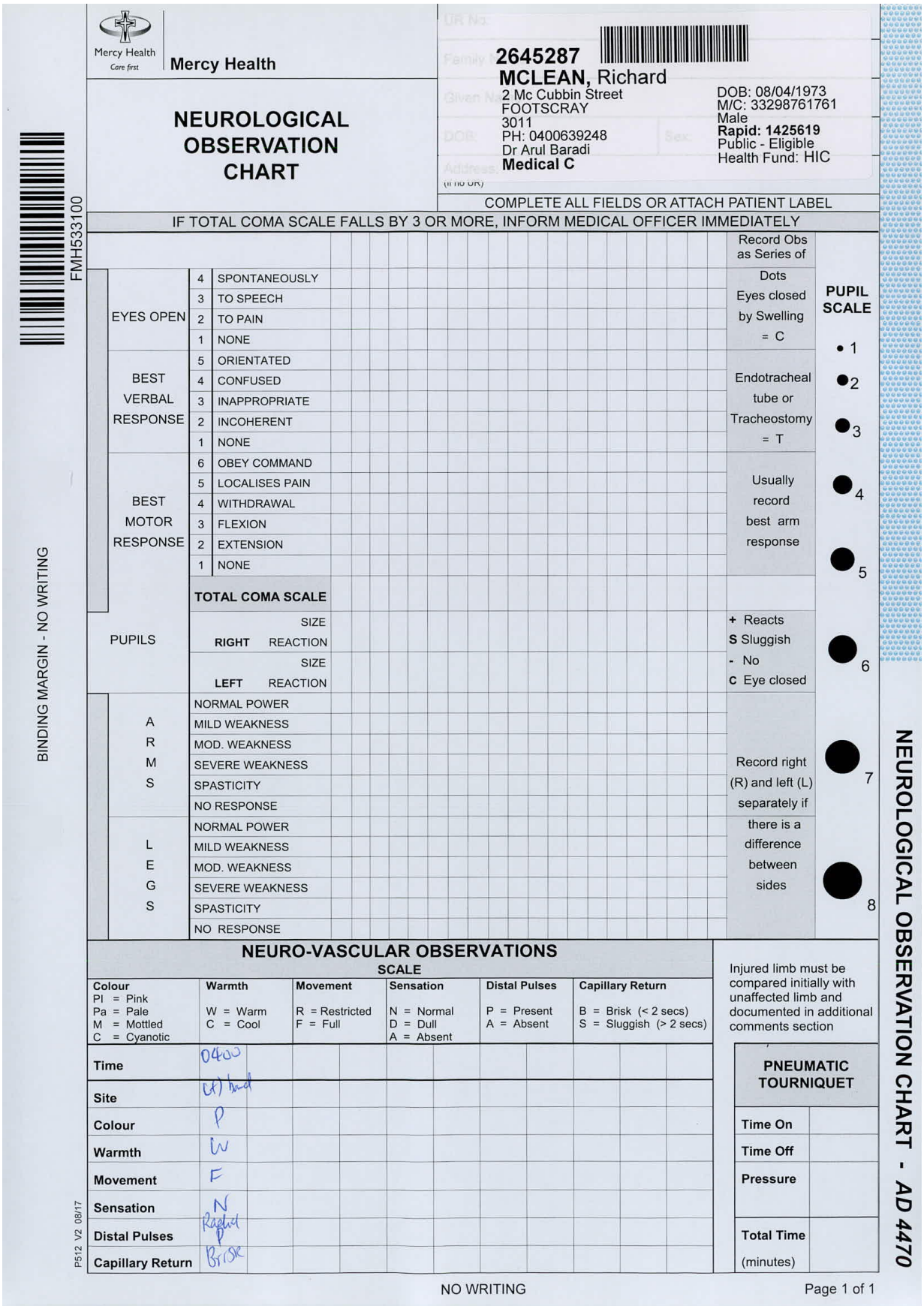
5.117. Neurological Obs Chart (FMH533100)



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NEUROLOGICAL OBSERVATION CHART

2645287
MCLEAN, Richard
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FOOTSCRAY
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PH: 0400639248
Dr Arul Baradi
Medical C

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Public - Eligible
Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

IF TOTAL COMA SCALE FALLS BY 3 OR MORE, INFORM MEDICAL OFFICER IMMEDIATELY

FMH533100

BINDING MARGIN - NO WRITING

			Record Obs as Series of
EYES OPEN	4	SPONTANEOUSLY	Dots
	3	TO SPEECH	Eyes closed by Swelling = C
	2	TO PAIN	
	1	NONE	
BEST VERBAL RESPONSE	5	ORIENTATED	Endotracheal tube or Tracheostomy = T
	4	CONFUSED	
	3	INAPPROPRIATE	
	2	INCOHERENT	
	1	NONE	
BEST MOTOR RESPONSE	6	OBEY COMMAND	Usually record best arm response
	5	LOCALISES PAIN	
	4	WITHDRAWAL	
	3	FLEXION	
	2	EXTENSION	
	1	NONE	
		TOTAL COMA SCALE	
PUPILS		SIZE / **RIGHT** REACTION	+ Reacts S Sluggish - No C Eye closed
		SIZE / **LEFT** REACTION	
ARMS		NORMAL POWER	Record right (R) and left (L) separately if there is a difference between sides
		MILD WEAKNESS	
		MOD. WEAKNESS	
		SEVERE WEAKNESS	
		SPASTICITY	
		NO RESPONSE	
LEGS		NORMAL POWER	
		MILD WEAKNESS	
		MOD. WEAKNESS	
		SEVERE WEAKNESS	
		SPASTICITY	
		NO RESPONSE	

PUPIL SCALE
1
2
3
4
5
6
7
8

NEURO-VASCULAR OBSERVATIONS
SCALE

Colour	Warmth	Movement	Sensation	Distal Pulses	Capillary Return
PI = Pink Pa = Pale M = Mottled C = Cyanotic	W = Warm C = Cool	R = Restricted F = Full	N = Normal D = Dull A = Absent	P = Present A = Absent	B = Brisk (< 2 secs) S = Sluggish (> 2 secs)

Time	0400											
Site	Lt hand											
Colour	P											
Warmth	W											
Movement	F											
Sensation	N											
Distal Pulses	Rapid P											
Capillary Return	Brisk											

Injured limb must be compared initially with unaffected limb and documented in additional comments section

PNEUMATIC TOURNIQUET	
Time On	
Time Off	
Pressure	
Total Time (minutes)	

P512 V2 08/17

NEUROLOGICAL OBSERVATION CHART - AD 4470

NO WRITING

Page 1 of 1

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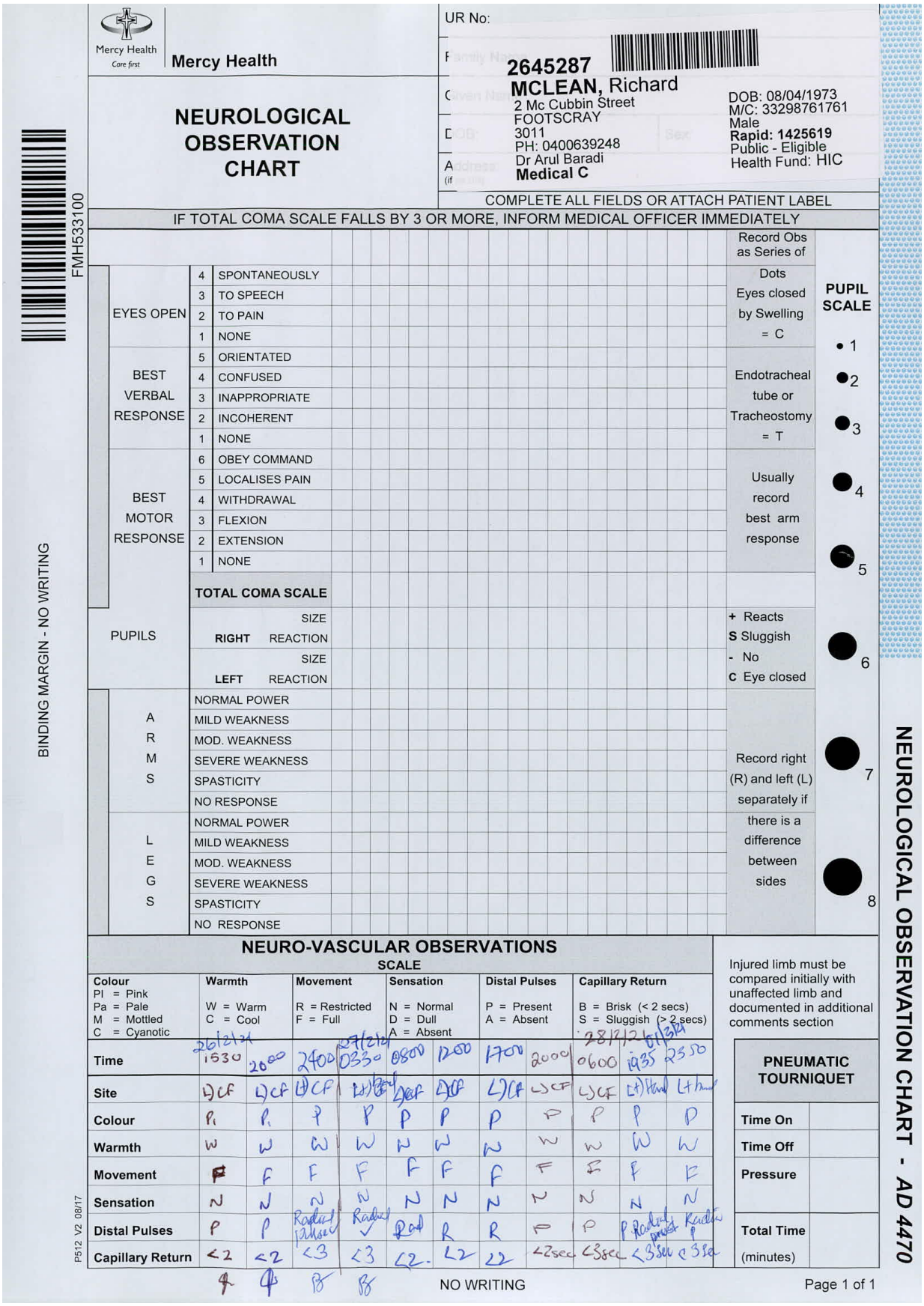
5.118. Neurological Obs Chart (FMH533100)



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Health Fund: HIC

NEUROLOGICAL OBSERVATION CHART

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

IF TOTAL COMA SCALE FALLS BY 3 OR MORE, INFORM MEDICAL OFFICER IMMEDIATELY

FMH533100

BINDING MARGIN - NO WRITING

EYES OPEN	4	SPONTANEOUSLY
	3	TO SPEECH
	2	TO PAIN
	1	NONE
BEST VERBAL RESPONSE	5	ORIENTATED
	4	CONFUSED
	3	INAPPROPRIATE
	2	INCOHERENT
	1	NONE
BEST MOTOR RESPONSE	6	OBEY COMMAND
	5	LOCALISES PAIN
	4	WITHDRAWAL
	3	FLEXION
	2	EXTENSION
	1	NONE
		TOTAL COMA SCALE
PUPILS		SIZE / **RIGHT** REACTION
		SIZE / **LEFT** REACTION
ARMS		NORMAL POWER
		MILD WEAKNESS
		MOD. WEAKNESS
		SEVERE WEAKNESS
		SPASTICITY
		NO RESPONSE
LEGS		NORMAL POWER
		MILD WEAKNESS
		MOD. WEAKNESS
		SEVERE WEAKNESS
		SPASTICITY
		NO RESPONSE

Record Obs as Series of Dots
Eyes closed by Swelling = C
Endotracheal tube or Tracheostomy = T
Usually record best arm response
+ Reacts
S Sluggish
- No
C Eye closed
Record right (R) and left (L) separately if there is a difference between sides

PUPIL SCALE
1 2 3 4 5 6 7 8

NEURO-VASCULAR OBSERVATIONS

SCALE

Colour	Warmth	Movement	Sensation	Distal Pulses	Capillary Return
Pl = Pink Pa = Pale M = Mottled C = Cyanotic	W = Warm C = Cool	R = Restricted F = Full	N = Normal D = Dull A = Absent	P = Present A = Absent	B = Brisk (< 2 secs) S = Sluggish (> 2 secs)

Injured limb must be compared initially with unaffected limb and documented in additional comments section

Time	26/2/21 1530	2000	2400	27/2/21 0330	0800	1200	1700	2000	28/2/21 0600	1935	2350
Site	L)CF	L)CF	L)CF	Lt)Hand	Hand	L)CF	L)CF	L)CF	L)CF	Lt)Hand	Lt hand
Colour	Pi	Pi	P	P	P	P	P	P	P	P	P
Warmth	W	W	W	W	W	W	W	W	W	W	W
Movement	F	F	F	F	F	F	F	F	F	F	F
Sensation	N	N	N	N	N	N	N	N	N	N	N
Distal Pulses	P	P	Radial palpable	Radial ✓	Rad	R	R	P	P	P Radial present	Radial ✓
Capillary Return	<2	<2	<3	<3	<2.	<2	<2	<2sec	<3sec	<3sec	<3sec
	4	4	BP	BP							

PNEUMATIC TOURNIQUET

Time On	
Time Off	
Pressure	
Total Time (minutes)	

NO WRITING

Page 1 of 1

P512 V2 08/17

NEUROLOGICAL OBSERVATION CHART - AD 4470

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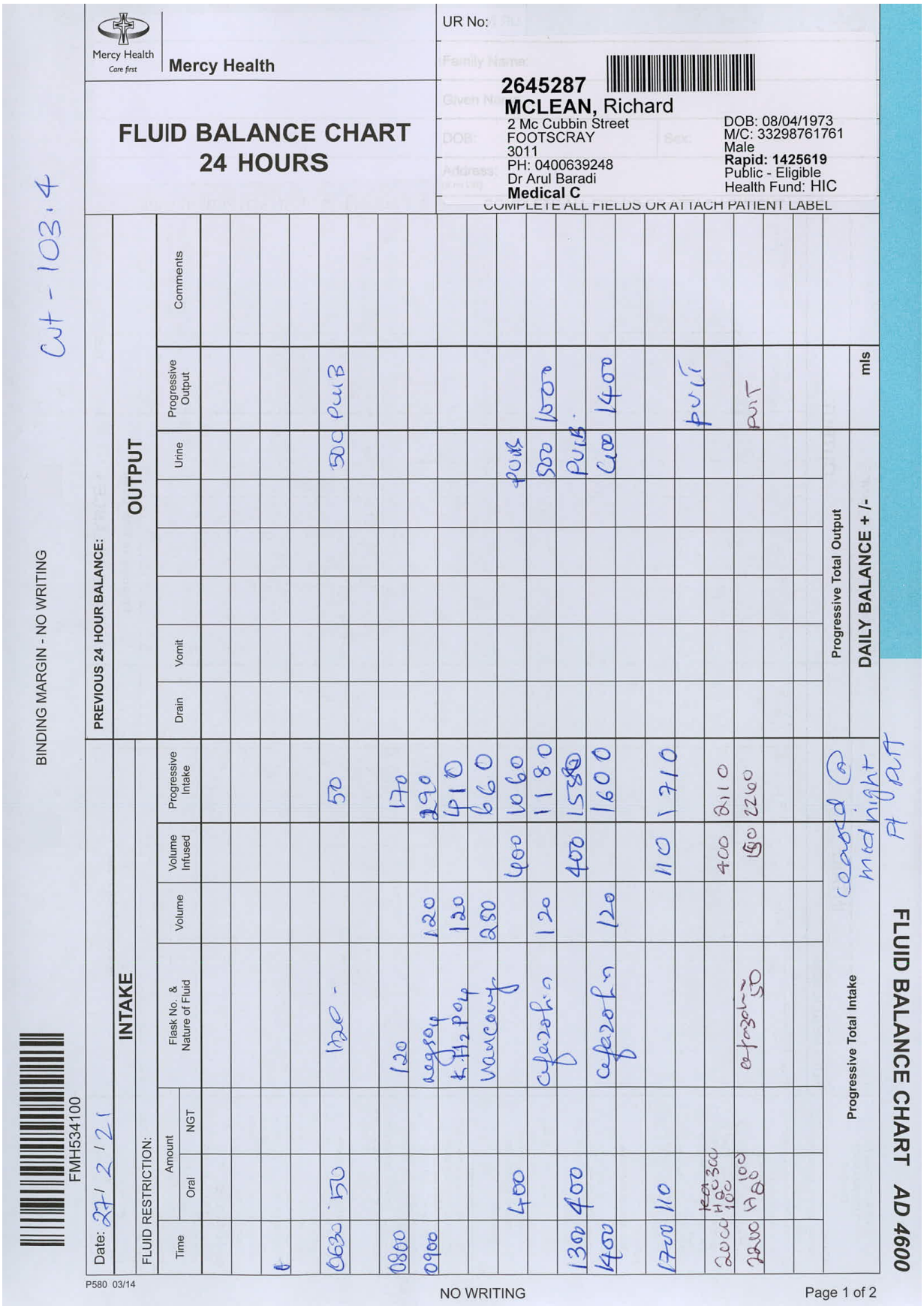
5.119. Fluid Balance Chart 24 Hours (FMH534100)



Mercy Health
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Public - Eligible
Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FLUID BALANCE CHART 24 HOURS

BINDING MARGIN - NO WRITING

FMH534100

Date: 27/2/21

PREVIOUS 24 HOUR BALANCE:

Cut - 103.4

	INTAKE						OUTPUT									
Time	FLUID RESTRICTION: Amount Oral	Amount NGT	Flask No. & Nature of Fluid	Volume	Volume Infused	Progressive Intake	Drain	Vomit				Urine	Progressive Output	Comments		
0630	150		120 -			50						500	500 PUB			
0800			120			170										
0900			0.9% NaCl	120		290										
			+ KCl 40													
			+ H_2PO_4	120		410										
			Vancomycin	250		660										
	400				400	1060						700	PUB			
			Cefazolin	120		1180						500	1500			
1300	400				400	1580						PUB				
1400			Cefazolin	120		1600						400	1400			
1700	110				110	1710							PUB			
2000	100	300			400	2110							PUB			
2200	100		Cefazolin	50	150	2260										

Progressive Total Intake

Progressive Total Output

DAILY BALANCE + /-

mls

cancelled @ mid night AT PAT

FLUID BALANCE CHART AD 4600

P580 03/14

NO WRITING

Page 1 of 2

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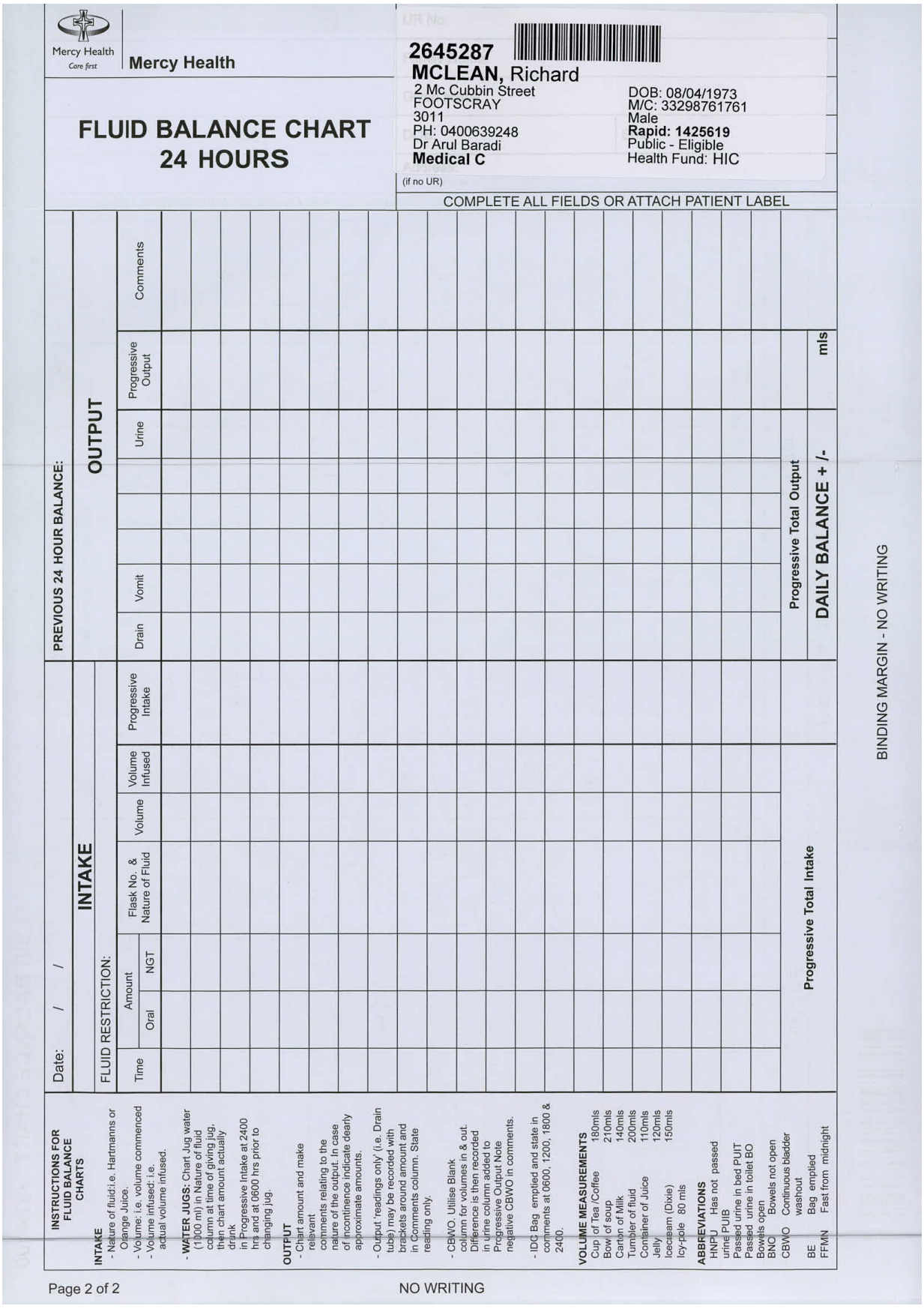
5.120. Fluid Balance Chart 24 Hours (FMH534100)



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FLUID BALANCE CHART 24 HOURS

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Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

(if no UR)

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

PREVIOUS 24 HOUR BALANCE:

Date: / /

INTAKE

FLUID RESTRICTION:

Time	Amount Oral	Amount NGT	Flask No. & Nature of Fluid	Volume	Volume Infused	Progressive Intake

OUTPUT

Drain	Vomit				Urine	Progressive Output	Comments

Progressive Total Intake

Progressive Total Output

DAILY BALANCE + /- mls

INSTRUCTIONS FOR FLUID BALANCE CHARTS

INTAKE
- Nature of fluid:i.e. Hartmanns or Orange Juice.
- Volume: i.e. volume commenced
- Volume infused: i.e. actual volume infused.

- **WATER JUGS:** Chart Jug water (1000 ml) in Nature of fluid column at time of giving jug, then chart amount actually drunk in Progressive Intake at 2400 hrs and at 0600 hrs prior to changing jug.

OUTPUT
- Chart amount and make relevant comments relating to the nature of the output. In case of incontinence indicate dearly approximate amounts.

- Output 'readings only' (i.e. Drain tube) may be recorded with brackets around amount and in Comments column. State reading only.

- CBWO. Utilise Blank column for volumes in & out. Difference is then recorded in urine column added to Progressive Output Note negative CBWO in comments.

- IDC Bag emptied and state in comments at 0600, 1200, 1800 & 2400.

VOLUME MEASUREMENTS

Cup of Tea /Coffee	180mls
Bowl of soup	210mls
Carton of Milk	140mls
Tumbler of fluid	200mls
Container of Juice	110mls
Jelly/ Icecream (Dixie)	120mls
Icy-pole 80 mls	150mls

ABBREVIATIONS

HNPU	Has not passed urine PUIB
Passed urine in bed PUIT	
Passed urine in toilet BO	
Bowels open	
BNO	Bowels not open
CBWO	Continuous bladder washout
BE	Bag emptied
FFMN	Fast from midnight

BINDING MARGIN - NO WRITING

Page 2 of 2

NO WRITING

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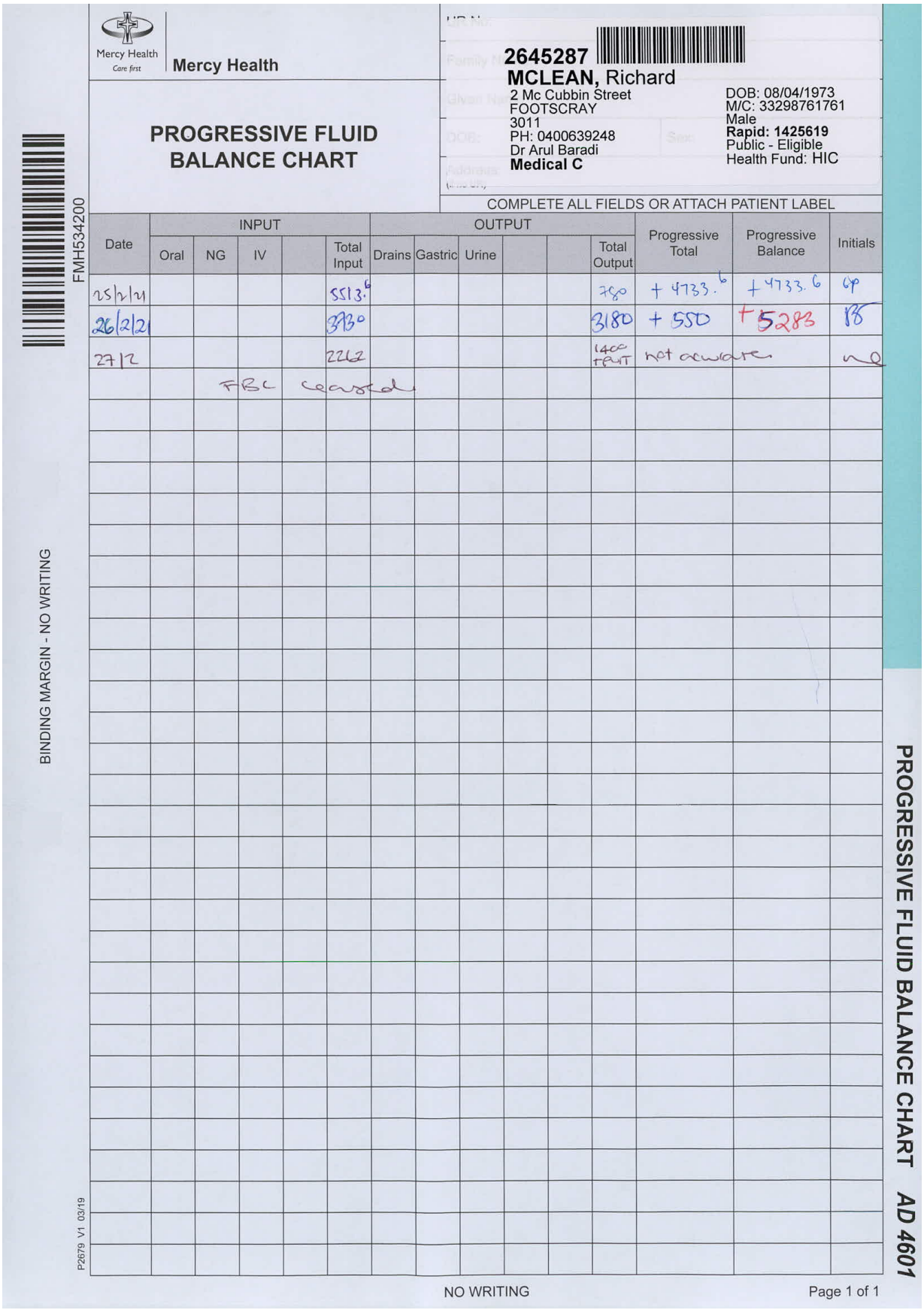
5.121. Fluid Balance Chart Progressive (FMH534200)



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PROGRESSIVE FLUID BALANCE CHART

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3011
PH: 0400639248
Dr Arul Baradi
Medical C

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Male
Rapid: 1425619
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Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Date	INPUT					OUTPUT						Progressive Total	Progressive Balance	Initials
	Oral	NG	IV		Total Input	Drains	Gastric	Urine			Total Output			
25/2/21					5513.6						780	+ 4733.6	+ 4733.6	GP
26/2/21					3730						3180	+ 550	+ 5283	BB
27/2					2262						1400 TPUT	not aware		ml
		FBC			cancelled									

FMH534200

BINDING MARGIN - NO WRITING

PROGRESSIVE FLUID BALANCE CHART AD 4601

P2679 V1 03/19

NO WRITING

Page 1 of 1

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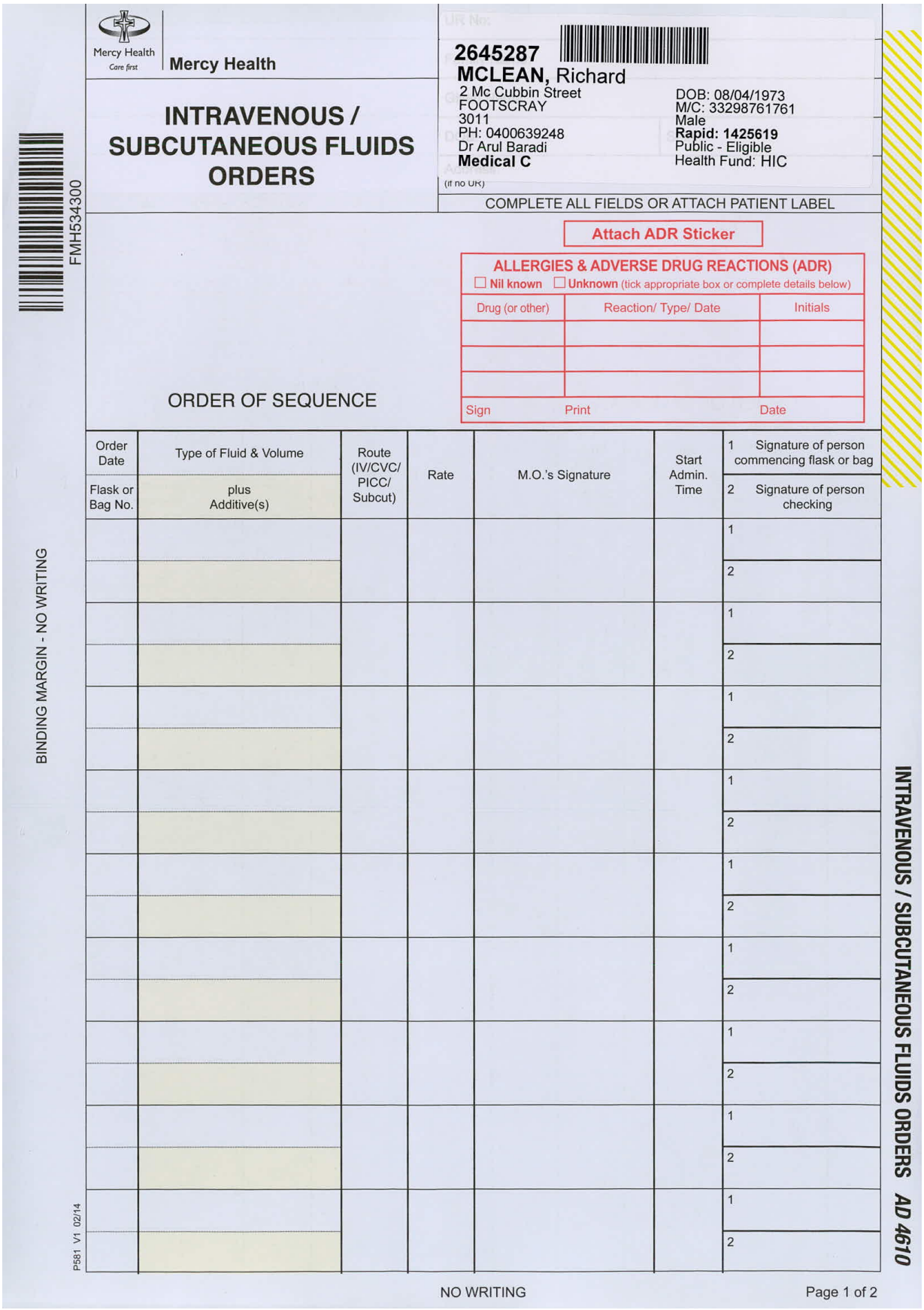
5.122. IV Subcutaneous Fluid Orders (FMH534300)



Mercy Health
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INTRAVENOUS / SUBCUTANEOUS FLUIDS ORDERS

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PH: 0400639248
Dr Arul Baradi
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DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Attach ADR Sticker

ALLERGIES & ADVERSE DRUG REACTIONS (ADR)
☐ **Nil known** ☐ **Unknown** (tick appropriate box or complete details below)

Drug (or other)	Reaction/ Type/ Date	Initials

Sign Print Date

ORDER OF SEQUENCE

Order Date / Flask or Bag No.	Type of Fluid & Volume plus Additive(s)	Route (IV/CVC/ PICC/ Subcut)	Rate	M.O.'s Signature	Start Admin. Time	1 Signature of person commencing flask or bag / 2 Signature of person checking
						1
						2
						1
						2
						1
						2
						1
						2
						1
						2
						1
						2
						1
						2
						1
						2
						1
						2

FMH534300

BINDING MARGIN - NO WRITING

INTRAVENOUS / SUBCUTANEOUS FLUIDS ORDERS AD 4610

P581 V1 02/14

NO WRITING

Page 1 of 2

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5.123. IV Subcutaneous Fluid Orders (FMH534300)

Mercy Health
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INTRAVENOUS / SUBCUTANEOUS FLUIDS ORDERS

UR No:
Family Name:
Given Name:
DOB:
Sex:
Address:

2645287
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2 Mc Cubbin Street
FOOTSCRAY
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PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Attach ADR Sticker

ALLERGIES & ADVERSE DRUG REACTIONS (ADR)
☐ **Nil known** ☐ **Unknown** (tick appropriate box or complete details below)

Drug (or other)	Reaction/ Type/ Date	Initials
Bee stings		
Augmentin	Vomiting	SS

Sign [signature] Print S.S Date 27/2

ORDER OF SEQUENCE

Order Date / Flask or Bag No.	Type of Fluid & Volume plus Additive(s)	Route (IV/CVC/ PICC/ Subcut)	Rate	M.O.'s Signature	Start Admin. Time	1 Signature of person commencing flask or bag / 2 Signature of person checking
27/2	10 mmol of $MgSO_4$ IN 100ml of NaCl (0.9%)	IV	1/24	[signature]	27/2 0900	1 LS / 2 CB
27/2	10mmol of Potassium Dihydrogen Phosphate in 100ml of NaCl (0.9%)	IV	1/24	[signature]	27/2 0900	1 LS / 2 CB
						1 / 2
						1 / 2
						1 / 2
						1 / 2
						1 / 2
						1 / 2
						1 / 2
						1 / 2
						1 / 2

BINDING MARGIN - NO WRITING

Page 2 of 2

NO WRITING

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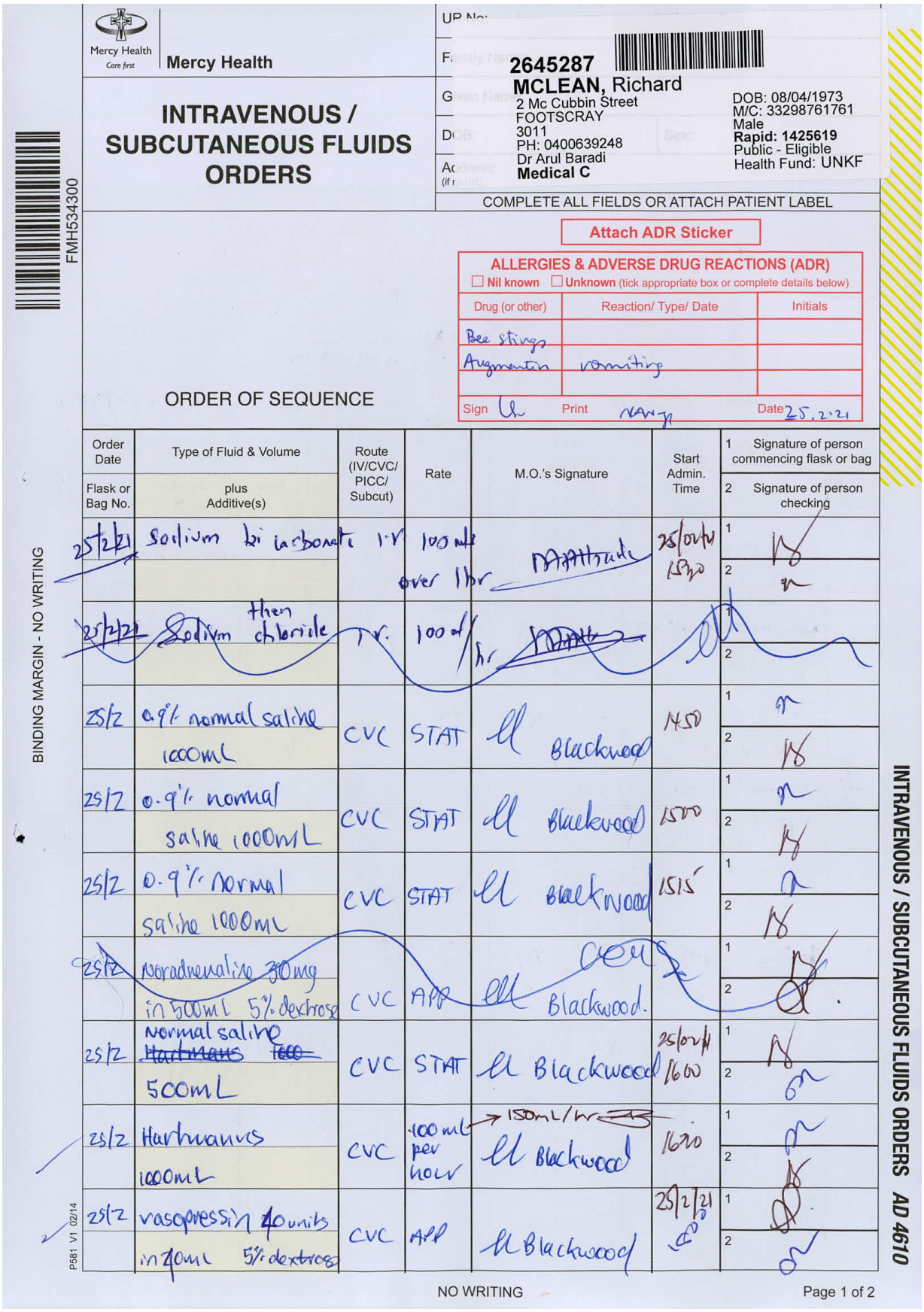
5.124. IV Subcutaneous Fluid Orders (FMH534300)



Mercy Health Care first
Mercy Health

INTRAVENOUS / SUBCUTANEOUS FLUIDS ORDERS

FMH534300

UR No:
Family Name:
Given Name:
DOB:
Sex:
Address:
(if r

2645287
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PH: 0400639248
Dr Arul Baradi
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Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Attach ADR Sticker

ALLERGIES & ADVERSE DRUG REACTIONS (ADR)
☐ Nil known ☐ Unknown (tick appropriate box or complete details below)

Drug (or other)	Reaction/ Type/ Date	Initials
Bee stings		
Augmentin	vomiting	

Sign [illegible] Print [illegible] Date 25.2.21

ORDER OF SEQUENCE

Order Date / Flask or Bag No.	Type of Fluid & Volume plus Additive(s)	Route (IV/CVC/ PICC/ Subcut)	Rate	M.O.'s Signature	Start Admin. Time	1 Signature of person commencing flask or bag / 2 Signature of person checking
25/2/21	Sodium bicarbonate	IV	100 mls over 1hr	[illegible]	25/2/21 1530	1 [illegible] / 2 [illegible]
25/2/21	Sodium then chloride	IV	100 ml/hr	[illegible]		1 [illegible] / 2 [illegible]
25/2	0.9% normal saline 1000mL	CVC	STAT	ll Blackwood	1450	1 [illegible] / 2 [illegible]
25/2	0.9% normal saline 1000mL	CVC	STAT	ll Blackwood	1500	1 [illegible] / 2 [illegible]
25/2	0.9% Normal saline 1000mL	CVC	STAT	ll Blackwood	1515	1 [illegible] / 2 [illegible]
25/2	Noradrenaline 30mg in 500mL 5% dextrose	CVC	APP	ll Blackwood.		1 [illegible] / 2 [illegible]
25/2	Normal saline ~~Hartmans~~ ~~1600~~ 500mL	CVC	STAT	ll Blackwood	25/2/21 1600	1 [illegible] / 2 [illegible]
25/2	Hartmanns 1000mL	CVC	100 ml per hour → 150mL/hr	ll Blackwood	1620	1 [illegible] / 2 [illegible]
25/2	vasopressin 40 units in 70mL 5% dextrose	CVC	APP	ll Blackwood	25/2/21 [illegible]	1 [illegible] / 2 [illegible]

BINDING MARGIN - NO WRITING

INTRAVENOUS / SUBCUTANEOUS FLUIDS ORDERS AD 4610

P581 V1 02/14

NO WRITING

Page 1 of 2

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5.125. IV Subcutaneous Fluid Orders (FMH534300)

Mercy Health
Care first

Mercy Health

INTRAVENOUS / SUBCUTANEOUS FLUIDS ORDERS

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Attach ADR Sticker

ALLERGIES & ADVERSE DRUG REACTIONS (ADR)
☐ Nil known ☐ Unknown (tick appropriate box or complete details below)

Drug (or other)	Reaction/ Type/ Date	Initials
Bee stings		
Augmentin	Vomiting	

Sign [signature] Print HTN Date 26/2/2021

ORDER OF SEQUENCE

Order Date / Flask or Bag No.	Type of Fluid & Volume / plus Additive(s)	Route (IV/CVC/ PICC/ Subcut)	Rate	M.O.'s Signature	Start Admin. Time	1 Signature of person commencing flask or bag / 2 Signature of person checking
25/2	Noradrenaline 32mg in 532mL 5% dextrose	CVC	APP	Blackwood	17 30 25/2	1 [signature] 2 Shanty
25/2	Noradrenaline 32mg in 500ml 5% Dex	CVC	APP	BRENDAN		1 2
25/2	50units actrapid in 50mL Nacl	CVC	APP	BRENDAN	25/2/21 1445	1 [signature] emm 2 given by Shanty
26/2	20mmol KCL in 100 mls normal saline	CVC	1/24	HTN	25/2/21 0610	1 Cep 2 [signature]
26/2	Normal Saline Drug & flush	CVC IV	APP	HTN	26/2/21 0000	1 [signature] 2 Cep
26/2	20mmol MgSO4 in 100mL Normal Saline	CVC/IV	1/24	HTN	26/2/21 0800	1 [signature] 2 [signature]
26.2	CSL 250 mL	CVC	Stat	[signature]	26/2/21 1000	1 [signature] 2 [signature]
26.2	CSL 750 mL	CVC	125 mL/hr	[signature]	26/2 1020	1 [signature] 2 [signature]
26/2	CSL 1000mL	CVC	125ml/ 1000mL/hr	[signature]	not given	1 2

BINDING MARGIN - NO WRITING

Page 2 of 2

NO WRITING

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5.126. IV Cannula Ass Tool (FMH534400)

Mercy Health
Care first

Mercy Health

I.V. CANNULA / PICC LINE ASSESSMENT TOOL

IR No:

2645287

MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH534400

BINDING MARGIN - NO WRITING

ASSESSMENT: **Remove the cannula if a score of 2 or more is assessed**

SCORE	DESCRIPTION
0	No pain, erythema, swelling, induration or palpable venous cord at or around venous site
1	Pain at I.V. site: no erythema, swelling, induration or palpable cord
2	Some erythema or swelling or both at I.V. site: no induration or palpable venous cord
3	Erythema and swelling at I.V. site: induration, palpable venous cord no more than 7 cms. above site
4	Erythema and swelling at I.V. site: induration, palpable venous cord more than 7 cms. above site

Date	Time	Date of Insertion	Site of Insertion	Gauge of Cannula	Score	Comments	Signature (Print Name & Designation)
25/2/21	2330	25/2/21	R fem	CVC	0	x 4 lumens	GP CERN
26/2/21	0800	25/2/21	R) fem	CVC	0	x 4 lumens - working	SD
26/2	2000	25/2	R/) fem	CVC	0	4 lumens	PJ CERN
27/2	0600	25/2	R) fem	CVC	0	4 lumens x4	SD CERN
27/2	2200	22/2	R) femoral	CVC	0	4 lumens 4FF	NE dressing CERN
28/2	1200	25/2	R/femoral	CVC	—	removed	AM Mulu
28/2	1230	28/2	R/ CF	IVC G. 20	0	inserted.	AM Mulu
01/2/21	1935	28/2	R) CF	PIVC G20	0	Patent.	BS CERN
2/3	0815	28/2	R) CF	G. 20	0	Flushed.	F. Mushtaq RN.
2/3	1430	28/2	R) CF	G.20	0	Leaking - removed.	F. Mushtaq RN.

I.V. CANNULA / PICC LINE ASSESSMENT TOOL *UR W3220*

P1862 03/13

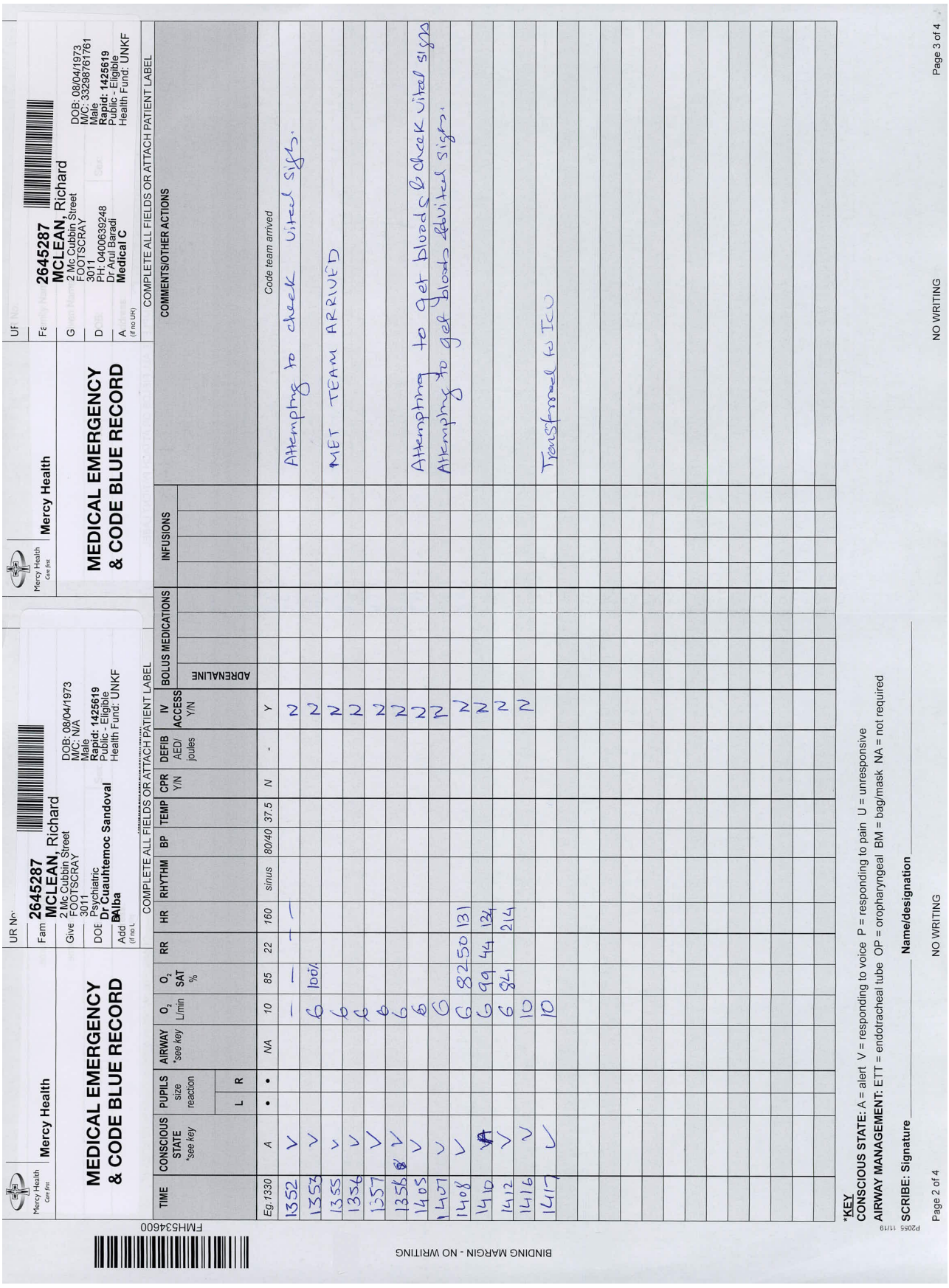
NO WRITING

Page 1 of 1

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

Mercy Health
Care first

5.127. Medical Emergency & Code Blue Record (FMH534600)



MEDICAL EMERGENCY & CODE BLUE RECORD

Mercy Health — UR No: 2645287 — Family Name: MCLEAN, Richard — Given: 2 Mc Cubbin Street FOOTSCRAY 3011 — DOB: 08/04/1973 M/C: N/A Male — DOB: Psychiatric — Dr Cuauhtemoc Sandoval — Rapid: 1425619 Public - Eligible Health Fund: UNKF — Address: BAlba (if no L)

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

TIME	CONSCIOUS STATE *see key	PUPILS size reaction L	PUPILS size reaction R	AIRWAY *see key	O_2 L/min	O_2 SAT %	RR	HR	RHYTHM	BP	TEMP	CPR Y/N	DEFIB AED/ joules	IV ACCESS Y/N	ADRENALINE	BOLUS MEDICATIONS	INFUSIONS	COMMENTS/OTHER ACTIONS
Eg.1330	A	•	•	NA	10	85	22	160	sinus	80/40	37.5	N	-	Y				Code team arrived
1352	V				1	1	1	1						N				Attempting to check vital signs.
1353	V				6	100								N				
1355	V				6									N				MET TEAM ARRIVED
1356	V				6									N				
1357	V				6									N				
1358	V				6									N				Attempting to get bloods & check vital sigs
1405	V				6									N				Attempting to get bloods & vital signs.
1407	V				6	82	50	131						N				
1408	V				6	99	44	124						N				
1410	V				6	84		214						N				
1412	V				10									N				
1416	V				10									N				Transferred to ICU
1417	V																	

*KEY
CONSCIOUS STATE: A = alert V = responding to voice P = responding to pain U = unresponsive
AIRWAY MANAGEMENT: ETT = endotracheal tube OP = oropharyngeal BM = bag/mask NA = not required

SCRIBE: Signature _____ Name/designation _____

P2055 11/19

Page 2 of 4 — NO WRITING — Page 3 of 4 — NO WRITING

FMH534600

BINDING MARGIN - NO WRITING

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Mercy Health
Care first

5.128. Medical Emergency & Code Blue Record (FMH534600)

Mercy Health Care first | UR No:

Mercy Health

MEDICAL EMERGENCY & CODE BLUE RECORD

Please tick appropriate code

☐ MET CALL ☑ CODE BLUE

Fam **2645287**
Give **MCLEAN**, Richard
2 Mc Cubbin Street
DOB FOOTSCRAY
3011
Psychiatric
Addi **Dr Cuauhtemoc Sandoval**
(if no l) **Baliba**

DOB: 08/04/1973
M/C: N/A
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

DATE: 25/02 | TEAM ARRIVAL TIME: 1355 | TIME CALLED: 1348

LOCATION: CMB POB 3 R | TIME STOOD DOWN:

RESUSCITATION TEAM

Medical Officers		Nurses / Midwives	
Print name	Initial	Print name	Initial

SUMMARY (to be completed by code team leader)

Inpatient ☐ Acute medical ☐
Outpatient ☐ Surgical ☐
Visitor ☐ Obstetric ☐

FOLLOW UP PLAN (including responsible medical officer)

Signature: Print Name:

NOTIFICATIONS

RESUSCITATION STATUS PRE CODE/MET CALL: Full Care ☐ Not For Resus ☐

RESUSCITATION STATUS POST CODE/MET CALL: Full Care ☐ Not For Resus ☐

Treating consultant/unit: Pastoral care: Yes ☐ No ☐ Relatives informed: Yes ☐ No ☐

Patient transferred to: VHIMS completed: Yes ☐ No ☐

Patient deceased: Yes ☐ No ☐ Time and date: Reported to coroner: Yes ☐ No ☐

Page 1 of 4 NO WRITING

MEDICAL EMERGENCY & CODE BLUE RECORD *AD 4620*

Mercy Health Care first | UR No:

Mercy Health

MEDICAL EMERGENCY & CODE BLUE RECORD

Fam **2645287**
Give **MCLEAN**, Richard
2 Mc Cubbin Street
DOB FOOTSCRAY
3011
Addi PH: 0400639248
(if no l) Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 3329876176
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

ADDITIONAL NOTES

BINDING MARGIN - NO WRITING

Page 4 of 4 NO WRITING

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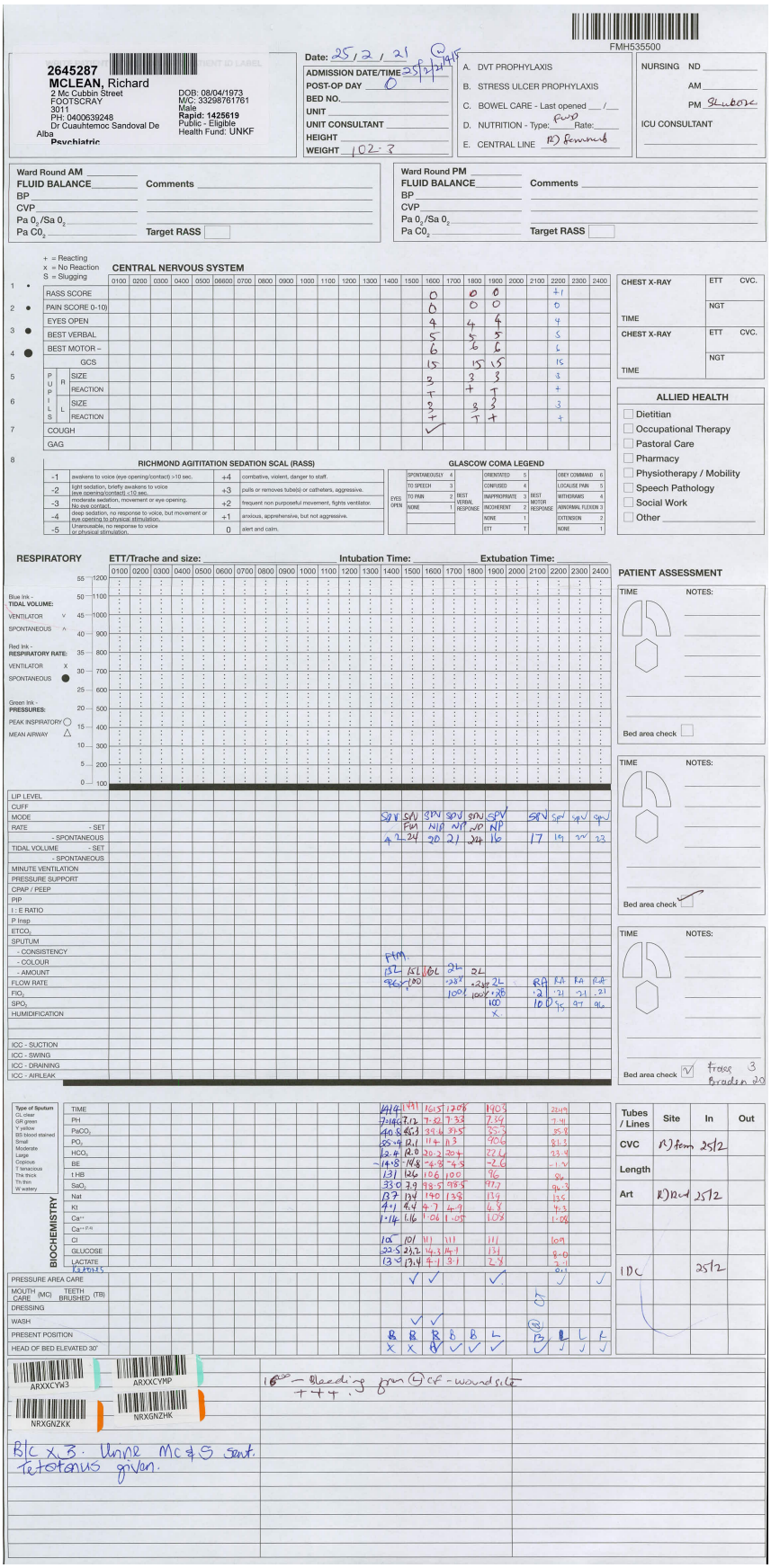
Mercy Health
Care first

5.129. Critical Care Treatment Chart (FMH535500)



FMH535500

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Cuauhtemoc Sandoval De Alba
Psychiatric
DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

Date: 25/2/21
ADMISSION DATE/TIME 25/2/21 1415
POST-OP DAY 0
BED NO.
UNIT
UNIT CONSULTANT
HEIGHT
WEIGHT 102.3

A. DVT PROPHYLAXIS
B. STRESS ULCER PROPHYLAXIS
C. BOWEL CARE - Last opened ___/___
D. NUTRITION - Type: PVP Rate:
E. CENTRAL LINE R) femmab

NURSING ND
AM
PM Shubose
ICU CONSULTANT

Ward Round AM
FLUID BALANCE
BP
CVP
Pa O_2/Sa O_2
Pa CO_2
Comments
Target RASS

Ward Round PM
FLUID BALANCE
BP
CVP
Pa O_2/Sa O_2
Pa CO_2
Comments
Target RASS

+ = Reacting
x = No Reaction
S = Slugging

CENTRAL NERVOUS SYSTEM

RASS SCORE
PAIN SCORE 0-10)
EYES OPEN
BEST VERBAL
BEST MOTOR –
GCS
PUPILS R SIZE
PUPILS R REACTION
PUPILS L SIZE
PUPILS L REACTION
COUGH
GAG

CHEST X-RAY
TIME
ETT CVC.
NGT
CHEST X-RAY
TIME
ETT CVC.
NGT

ALLIED HEALTH
- Dietitian
- Occupational Therapy
- Pastoral Care
- Pharmacy
- Physiotherapy / Mobility
- Speech Pathology
- Social Work
- Other

RICHMOND AGITITATION SEDATION SCAL (RASS)

-1	awakens to voice (eye opening/contact) >10 sec.	+4	combative, violent, danger to staff.
-2	light sedation, briefly awakens to voice (eye opening/contact) <10 sec.	+3	pulls or removes tube(s) or catheters, aggressive.
-3	moderate sedation, movement or eye opening. No eye contact.	+2	frequent non purposeful movement, fights ventilator.
-4	deep sedation, no response to voice, but movement or eye opening to physical stimulation.	+1	anxious, apprehensive, but not aggressive.
-5	Unarousable, no response to voice or physical stimulation.	0	alert and calm.

GLASGOW COMA LEGEND

EYES OPEN	SPONTANEOUSLY 4	BEST VERBAL RESPONSE	ORIENTATED 5	BEST MOTOR RESPONSE	OBEY COMMAND 6
	TO SPEECH 3		CONFUSED 4		LOCALISE PAIN 5
	TO PAIN 2		INAPPROPRIATE 3		WITHDRAWS 4
	NONE 1		INCOHERENT 2		ABNORMAL FLEXION 3
			NONE 1		EXTENSION 2
			ETT T		NONE 1

RESPIRATORY
ETT/Trache and size: _____ Intubation Time: _____ Extubation Time: _____

Blue Ink - TIDAL VOLUME:
VENTILATOR V
SPONTANEOUS ^
Red Ink - RESPIRATORY RATE:
VENTILATOR X
SPONTANEOUS ●
Green Ink - PRESSURES:
PEAK INSPIRATORY ○
MEAN AIRWAY △

LIP LEVEL
CUFF
MODE
RATE - SET
RATE - SPONTANEOUS
TIDAL VOLUME - SET
TIDAL VOLUME - SPONTANEOUS
MINUTE VENTILATION
PRESSURE SUPPORT
CPAP / PEEP
PIP
I : E RATIO
P Insp
$ETCO_2$
SPUTUM
- CONSISTENCY
- COLOUR
- AMOUNT
FLOW RATE
FIO_2
SPO_2
HUMIDIFICATION
ICC - SUCTION
ICC - SWING
ICC - DRAINING
ICC - AIRLEAK

PATIENT ASSESSMENT

TIME NOTES:
Bed area check

TIME NOTES:
Bed area check

TIME NOTES:
Bed area check ☑ Braden 20

Type of Sputum
CL clear
GR green
Y yellow
BS blood stained
Small
Moderate
Large
Copious
T tenacious
Thk thick
Thn thin
W watery

BIOCHEMISTRY
TIME
PH
$PaCO_2$
PO_2
HCO_3
BE
t HB
SaO_2
Nat
KI
Ca^{++}
Ca^{++} P/F
Cl
GLUCOSE
LACTATE

PRESSURE AREA CARE
MOUTH CARE (MC) TEETH BRUSHED (TB)
DRESSING
WASH
PRESENT POSITION
HEAD OF BED ELEVATED 30°

Tubes / Lines	Site	In	Out
CVC	R) fem	25/2	
Length			
Art	R) rad	25/2	
IDC		25/2	

ARXXCYM3
ARXXCYMP
NRXGNZKK
NRXGNZHK

16/52 – Bleeding from (L) CF – wound site
+++.

B/C x 3. Urine MC&S sent.
Tetanus given.

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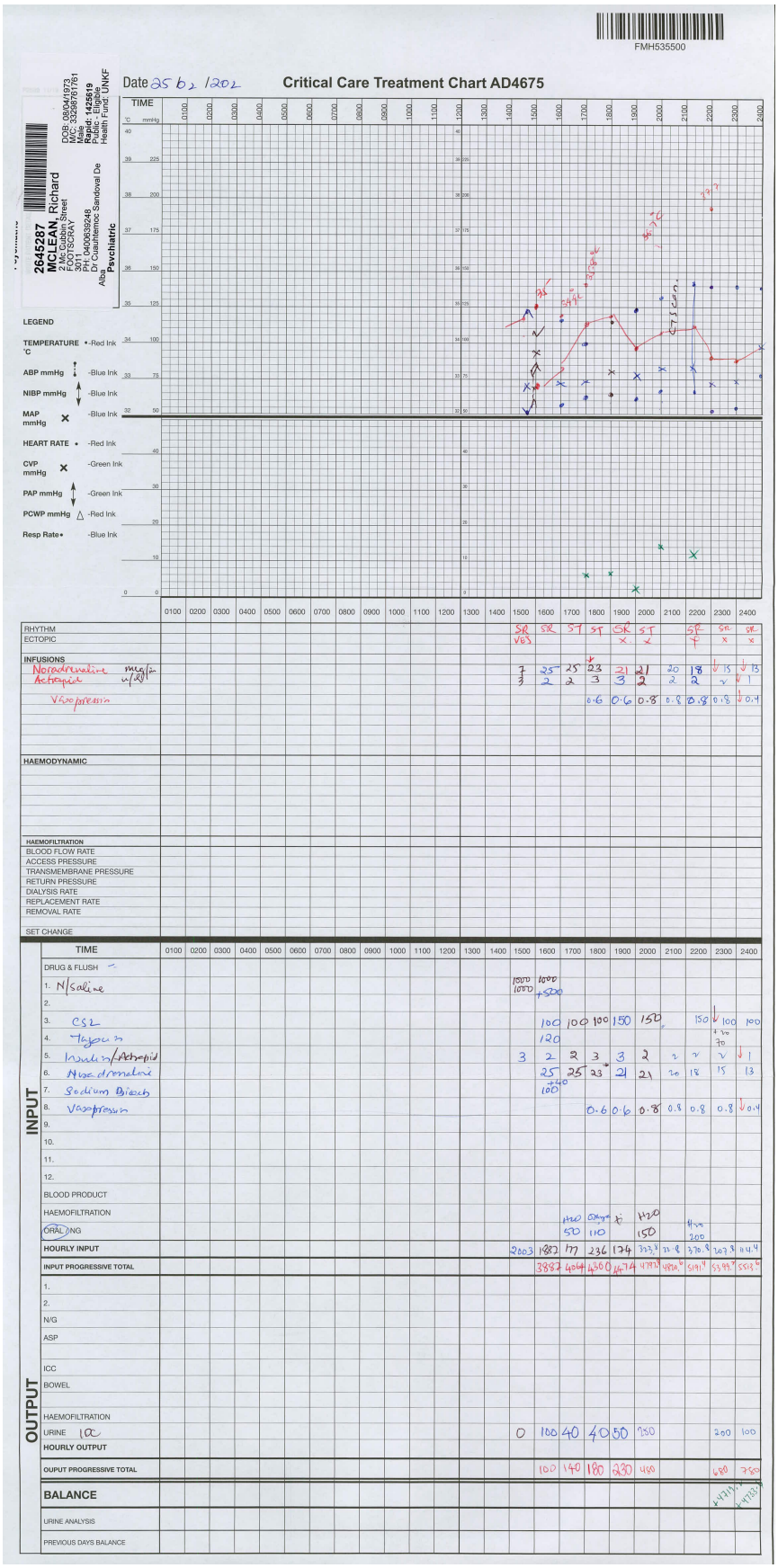
Mercy Health
Care first

5.130. Critical Care Treatment Chart (FMH535500)



FMH535500

Date 25 b2 1202 **Critical Care Treatment Chart AD4675**

2645287 MCLEAN, Richard
DOB: 08/04/1973
MRC: 33296767761
Age: 49
Rapid: 1425619
Public - Eligible
Health Fund: UNKF
2 Mc Cubbin Street
ROOTSCRAY
3011
PH: 0400639248
Alb Dr Cuauhtemoc Sandoval De
Psychiatric

TIME 0100 0200 0300 0400 0500 0600 0700 0800 0900 1000 1100 1200 1300 1400 1500 1600 1700 1800 1900 2000 2100 2200 2300 2400

LEGEND
TEMPERATURE °C •-Red Ink
ABP mmHg -Blue Ink
NIBP mmHg -Blue Ink
MAP mmHg ×-Blue Ink
HEART RATE •-Red Ink
CVP mmHg ×-Green Ink
PAP mmHg -Green Ink
PCWP mmHg △-Red Ink
Resp Rate• -Blue Ink

RHYTHM
ECTOPIC
INFUSIONS
Noradrenaline mcg/kg/min
Adrenalin
Vasopressin
HAEMODYNAMIC
HAEMOFILTRATION
BLOOD FLOW RATE
ACCESS PRESSURE
TRANSMEMBRANE PRESSURE
RETURN PRESSURE
DIALYSIS RATE
REPLACEMENT RATE
REMOVAL RATE
SET CHANGE

INPUT
DRUG & FLUSH
1. N/saline
2.
3. CSL
4. Hyperm
5. Insulin/Actrapid
6. Noradrenaline
7. Sodium Bicarb
8. Vasopressin
9.
10.
11.
12.
BLOOD PRODUCT
HAEMOFILTRATION
ORAL NG
HOURLY INPUT
INPUT PROGRESSIVE TOTAL

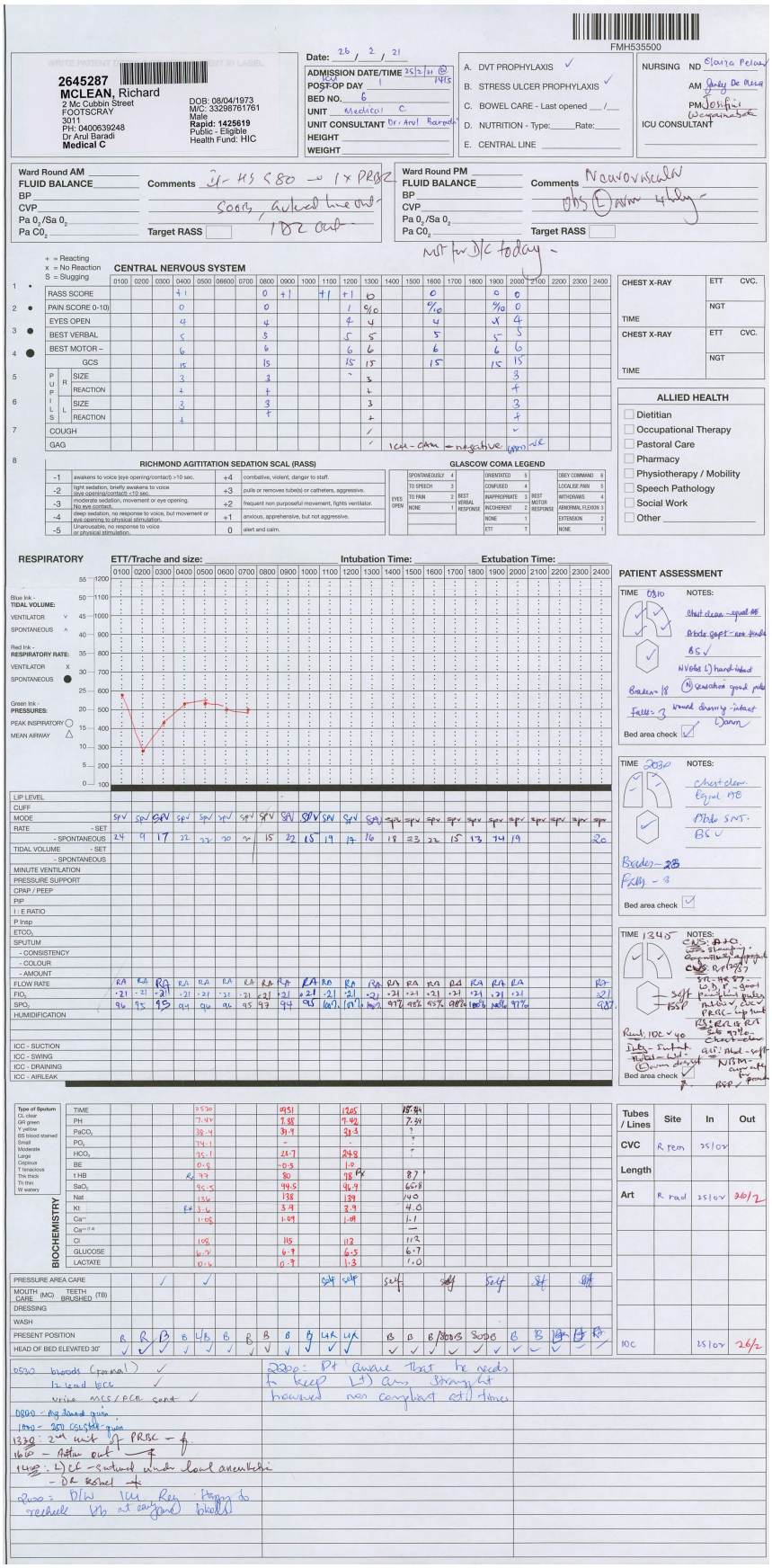
OUTPUT
1.
2.
N/G
ASP
ICC
BOWEL
HAEMOFILTRATION
URINE IDC
HOURLY OUTPUT
OUPUT PROGRESSIVE TOTAL

BALANCE
URINE ANALYSIS
PREVIOUS DAYS BALANCE

CPF COPY - DO NOT AMEND OR FILE IN THE MHVL RECORD

Mercy Health
Care first

5.131. Critical Care Treatment Chart (FMH535500)



FMH535500

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

Date: 26 / 2 / 21

ADMISSION DATE/TIME 25/2/21 @ 1915
POST-OP DAY 1
BED NO. 6
UNIT Medical C
UNIT CONSULTANT Dr. Arul Baradi
HEIGHT
WEIGHT

A. DVT PROPHYLAXIS ✓
B. STRESS ULCER PROPHYLAXIS ✓
C. BOWEL CARE - Last opened ___/___
D. NUTRITION - Type:_____Rate:_____
E. CENTRAL LINE

NURSING ND Slavica Petrov
AM Shirley De Asis
PM Josifi Kacanikovski
ICU CONSULTANT

Ward Round AM
FLUID BALANCE
BP
CVP
Pa 0_2/Sa 0_2
Pa CO_2
Comments U- HS <80 → 1x PRBC
500ml fluid bolus
182 cab
Target RASS

Ward Round PM
FLUID BALANCE
BP
CVP
Pa 0_2/Sa 0_2
Pa CO_2
Comments Neurovascular
obs (E) 4hrly
NOT for D/C today
Target RASS

+ = Reacting
x = No Reaction
S = Sluggish

CENTRAL NERVOUS SYSTEM

	0100	0200	0300	0400	0500	0600	0700	0800	0900	1000	1100	1200	1300	1400	1500	1600	1700	1800	1900	2000	2100	2200	2300	2400
RASS SCORE				+1				0	+1		+1	+1	0			0			0	0				
PAIN SCORE 0-10				0				0				1	0/10			0/10			0/10	0				
EYES OPEN				4				4				4	4			4			X	4				
BEST VERBAL				5				5				5	5			5			5	5				
BEST MOTOR – GCS				6				6				6	6			6			6	6				
				15				15				15	15			15			15	15				
PUPILS R SIZE				3				3				-	2							3				
PUPILS R REACTION				+				+					+							+				
PUPILS L SIZE				3				3					3							3				
PUPILS L REACTION				+				+					+							+				
COUGH													/											
GAG													✓											

1 CH - CAM - negative (pm) RE

RICHMOND AGITTATION SEDATION SCAL (RASS)

-1	awakens to voice (eye opening/contact) >10 sec.	+4	combative, violent, danger to staff.
-2	light sedation, briefly awakens to voice (eye opening/contact) <10 sec.	+3	pulls or removes tube(s) or catheters, aggressive.
-3	moderate sedation, movement or eye opening. No eye contact.	+2	frequent non purposeful movement, fights ventilator.
-4	deep sedation, no response to voice, but movement or eye opening to physical stimulation.	+1	anxious, apprehensive, but not aggressive.
-5	Unarousable, no response to voice or physical stimulation.	0	alert and calm.

GLASGOW COMA LEGEND

EYES OPEN	SPONTANEOUSLY 4	BEST VERBAL RESPONSE	ORIENTATED 5	BEST MOTOR RESPONSE	OBEY COMMAND 6
	TO SPEECH 3		CONFUSED 4		LOCALISE PAIN 5
	TO PAIN 2		INAPPROPRIATE 3		WITHDRAWS 4
	NONE 1		INCOHERENT 2		ABNORMAL FLEXION 3
			NONE 1		EXTENSION 2
			ETT T		NONE 1

CHEST X-RAY — ETT CVC. NGT — TIME
CHEST X-RAY — ETT CVC. NGT — TIME

ALLIED HEALTH
- ☐ Dietitian
- ☐ Occupational Therapy
- ☐ Pastoral Care
- ☐ Pharmacy
- ☐ Physiotherapy / Mobility
- ☐ Speech Pathology
- ☐ Social Work
- ☐ Other

RESPIRATORY

ETT/Trache and size: Intubation Time: Extubation Time:

Blue Ink - TIDAL VOLUME: VENTILATOR, SPONTANEOUS
Red Ink - RESPIRATORY RATE: VENTILATOR, SPONTANEOUS
Green Ink - PRESSURES: PEAK INSPIRATORY, MEAN AIRWAY

	0100	0200	0300	0400	0500	0600	0700	0800	0900	1000	1100	1200	1300	1400	1500	1600	1700	1800	1900	2000	2100	2200	2300	2400
LIP LEVEL																								
CUFF																								
MODE	SPV	SPV	SPV	SPV	SPV	SPV	SPV	SPV	SPV	SPV	SPV	SPV	SPV	SPV	SPV	SPV	SPV	SPV	SPV	SPV	SPV	SPV	SPV	SPV
RATE - SET	24	9	17	22	22	20	16	15	22	15	19	17	16	18	23	22	15	13	14	19				20
RATE - SPONTANEOUS																								
TIDAL VOLUME - SET																								
TIDAL VOLUME - SPONTANEOUS																								
MINUTE VENTILATION																								
PRESSURE SUPPORT																								
CPAP / PEEP																								
PIP																								
I : E RATIO																								
P Insp																								
$ETCO_2$																								
SPUTUM																								
- CONSISTENCY																								
- COLOUR																								
- AMOUNT																								
FLOW RATE	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA	RA				RA
FIO_2	.21	.21	.21	.21	.21	.21	.21	.21	.21	.21	.21	.21	.21	.21	.21	.21	.21	.21	.21	.21				.21
SPO_2	96	95	95	94	96	96	95	97	94	95	96%	97%	98%	99%	99%	98%	98%	98%	98%	97%				97%
HUMIDIFICATION																								
ICC - SUCTION																								
ICC - SWING																								
ICC - DRAINING																								
ICC - AIRLEAK																								

PATIENT ASSESSMENT

TIME 0810 NOTES:
Chest clear - equal AE
Pt rate apart - see trach
BS ✓
NVOBs L) hand indent
Braden 18 (N) sensation good pulse
Falls 3 wound draining intact
Drain
Bed area check ✓

TIME 2030 NOTES:
Chest clear
Equal AE
Mode SIMV
BS ✓
Bowels - 2B
Fully - B
Bed area check ✓

TIME 1345 NOTES:
CXR: A+C
Bed area check ✓

Type of Sputum: CL clear, GR green, Y yellow, BS blood stained, Small, Moderate, Large, Copious, T tenacious, Thk thick, Th thin, W watery

BIOCHEMISTRY

TIME				0530				1951			1205		15:54											
PH				7.49				7.38			7.42		7.34											
$PaCO_2$				38.4				34.9			33.2		?											
PO_2				74.1				-			-		?											
HCO_3				25.1				23.7			24.8		-											
BE				0.8				-0.5			1.9													
t HB			R	77				80			78 Hx		87											
SaO_2				96.5				94.5			96.9		66.8											
Nat				136				138			139		140											
Kl			F#	3.6				3.9			3.9		4.0											
Ca^{++}				1.05				1.09			1.09		1.1											
Ca^{++} (7.4)													—											
Cl				108				115			112		112											
GLUCOSE				6.9				6.9			6.5		6.7											
LACTATE				0.6				0.9			1.3		1.0											

PRESSURE AREA CARE			✓		✓					Self	self		Self		Self		Self		Self					
MOUTH CARE (MC) TEETH BRUSHED (TB)																								
DRESSING																								
WASH																								
PRESENT POSITION	R	R	B	B	LR	B	B	B	B	LR	LR		B	B	B/BOBS	SOBS	B	B	LR	B				
HEAD OF BED ELEVATED 30°	✓	✓	✓	✓	✓	✓	✓	✓	✓	✓	✓		✓	✓	✓	✓	✓	✓	✓	✓				

Tubes / Lines	Site	In	Out
CVC	R fem	25/2 0V	
Length			
Art	R rad	25/2 0V	26/2
IDC		25/2 0V	26/2

0530 bloods (Trauma) ✓
12 lead ECG ✓
urine MCS / PCR sent ✓
0800 - Ng blood gas
1000 - 250 CRYSTAL gram
1330 - 2nd unit of PRBC - ✓
1600 - Abbin out - ✓
1445: L) CC - sutured under local anaesthetic
- DR Restrict ✓
2100: D/W ICU Reg Happy to reshell NG at early bloods

2200: Pt aware that he needs to keep L↑ arm straight however non compliant at times

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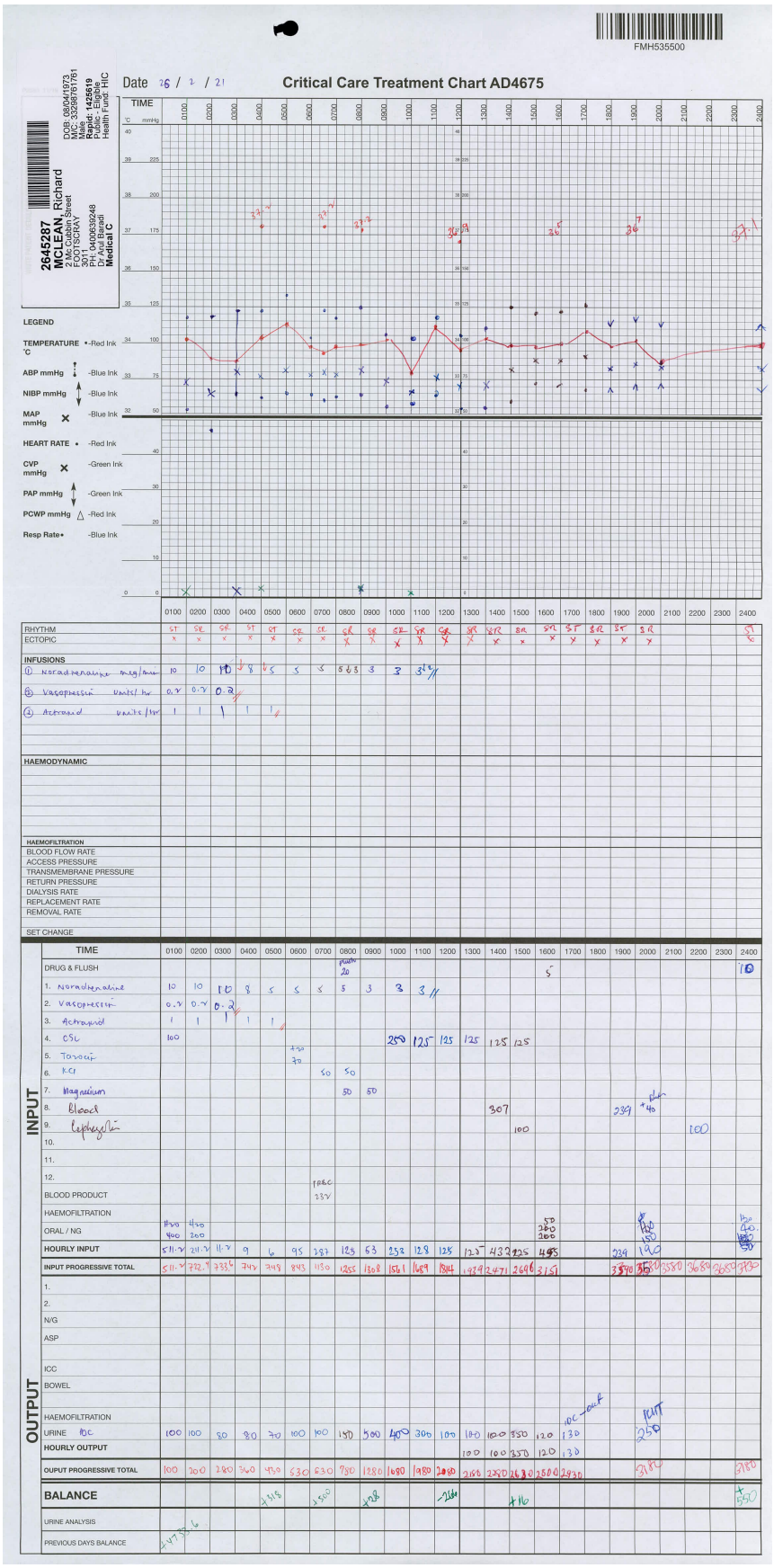
Mercy Health
Care first

5.132. Critical Care Treatment Chart (FMH535500)



FMH535500

Date 26 / 2 / 21

Critical Care Treatment Chart AD4675

2645287
MCLEAN, Richard
DOB: 08/04/1973
MC: 3298761761
Ward: 1425619
Public - Eligible
Health Fund: HC
Dr Atul Bansil
Medical C

LEGEND
TEMPERATURE °C • -Red Ink
ABP mmHg -Blue Ink
NIBP mmHg -Blue Ink
MAP mmHg × -Blue Ink
HEART RATE • -Red Ink
CVP mmHg × -Green Ink
PAP mmHg -Green Ink
PCWP mmHg △ -Red Ink
Resp Rate • -Blue Ink

TIME 0100 0200 0300 0400 0500 0600 0700 0800 0900 1000 1100 1200 1300 1400 1500 1600 1700 1800 1900 2000 2100 2200 2300 2400

RHYTHM
ECTOPIC
INFUSIONS
① Noradrenaline mcg/min
② Vasopressin units/hr
③ Actrapid units/hr
HAEMODYNAMIC
HAEMOFILTRATION
BLOOD FLOW RATE
ACCESS PRESSURE
TRANSMEMBRANE PRESSURE
RETURN PRESSURE
DIALYSIS RATE
REPLACEMENT RATE
REMOVAL RATE
SET CHANGE

INPUT
DRUG & FLUSH
1. Noradrenaline
2. Vasopressin
3. Actrapid
4. CSL
5. Tonocal
6. KCl
7. Magnesium
8. Blood
9. Cephazolin
10.
11.
12.
BLOOD PRODUCT
HAEMOFILTRATION
ORAL / NG
HOURLY INPUT
INPUT PROGRESSIVE TOTAL

OUTPUT
1.
2.
N/G
ASP
ICC
BOWEL
HAEMOFILTRATION
URINE IDC
HOURLY OUTPUT
OUPUT PROGRESSIVE TOTAL

BALANCE
URINE ANALYSIS
PREVIOUS DAYS BALANCE

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Mercy Health
Care first

Attach ADR Sticker

UR No:

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

ALLERGIES & ADVERSE REACTIONS (ADR)
☐ Nil Known ☐ Unknown (tick appropriate box or complete details below)

Drug (or other)	Reaction/Date	Initials
Augmentin	vomiting	
Bee Stings		

Sign [redacted] Print [redacted] Date 26/2/21

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH537400

Facility: ☐ MHW ☐ WMH ☐ MMH **Admitting ward:** **Date:**

HCC/Conc card holder: ☐ Yes ☐ No **Card Number:**

CONTACT DETAILS

General Practitioner: **Phone:** **Fax:**

Local Pharmacy: Footscray Plaza P'cy **Phone:** 0396871601 **Fax:**

Carer/Residential Care Facility: **Phone:** **Fax/Mobile:**

Interpreter Required: ☐ Yes (Language spoken _____) ☐ No

Presentation / Diagnosis: Laceration ? wrist.

Past Medical History:

Other Relevant Lab Data: Cor(adj(L). Hb(L). Trop TTT(T).

Clinical parameters on admission: Wt: N100kg. Ht: Cr: 108 CrCl:

Patient's own medications (POM) brought into hospital? ☑ Yes ☐ No

Location of POM: ☐ Bedside Drawer ☑ Drug Room ☐ Ward Fridge ☑ DD Safe ☐ Pharmacy ☐ Returned Home – Date: .....

Medication administration aid in current use: ☐ Dosette ☐ Webster / Sachet ☑ None

If Dosette/ Webster, filled by: ☐ Self ☐ Family ☐ Pharmacy ☐ Other:

Medications managed and administered by:
☑ Self ☐ Carer ☐ Bolton Clarke / RDNS ☐ Other: .....

Smoking History: ☐ Non-smoker ☐ Ex-smoker ☐ Smoker (Packs per day _____)

Patient's Immunisation Up to Date: ☐ Yes ☐ No

Medication Risk Identification	Yes	No		Yes	No
Lives alone			Falls risk		
Lives in residential care facility			Can open bottles/measure liquids		
Uses dose administration device (e..g inhaler, spacer)			Recent home medicine review		
Uses medication list			Suspected non-adherence		
Swallowing issues			Other information:		
Impaired hearing					
Impaired vision					
Can read/comprehend labels					

P1924 V3 01/20

KEEP WITH ACTIVE MEDICATION CHART – DO NOT REMOVE

Page 2 of 4 NO WRITING

BINDING MARGIN - NO WRITING

[redacted]

MEDICATIONS TAKEN PRIOR TO HOSPITAL PRESENTATION
(includes all prescription, non-prescription & complementary medications)

Generic name (Trade name) Strength / Form	Dose / Route / Frequency	Reconciled on Admission	Comments	On Discharge: Supply Y/N	On Discharge: Reconciled
- Paroxetine 20mg	(1) daily.	✓			
- Amisulpride 200mg	(2) nocte.	✓ (400mg in past week)			
- Dexamphetamine 5mg	(8) daily max.	✓			
- Tenofovir / Emtricitabine 300/200	(1) daily	✓	— last dispensed in April '20.		
- Diazepam 5mg	(2) nocte	✓	— dispensed in Feb '21		

✓ Pt states takes b/w 4–8 tablets of Dexamphetamine 5mg tablets
✓ Pt confirms still taking Tenofovir/Emtricitabine

Sources Used: Carer ☐ General Practitioner ☐ Nursing Home ☐
Patient ☑ Community Pharmacy ☑ Patient's Own medication ☑ Other Hospital ☐
MPHI record ☐ Patient's Medication List ☐ Safe Script ☑

Documented by: Signature: [redacted] Name: Someer Date: 26/2/21
Counter signature (if required): ..... Edited By: ..... Date: .....

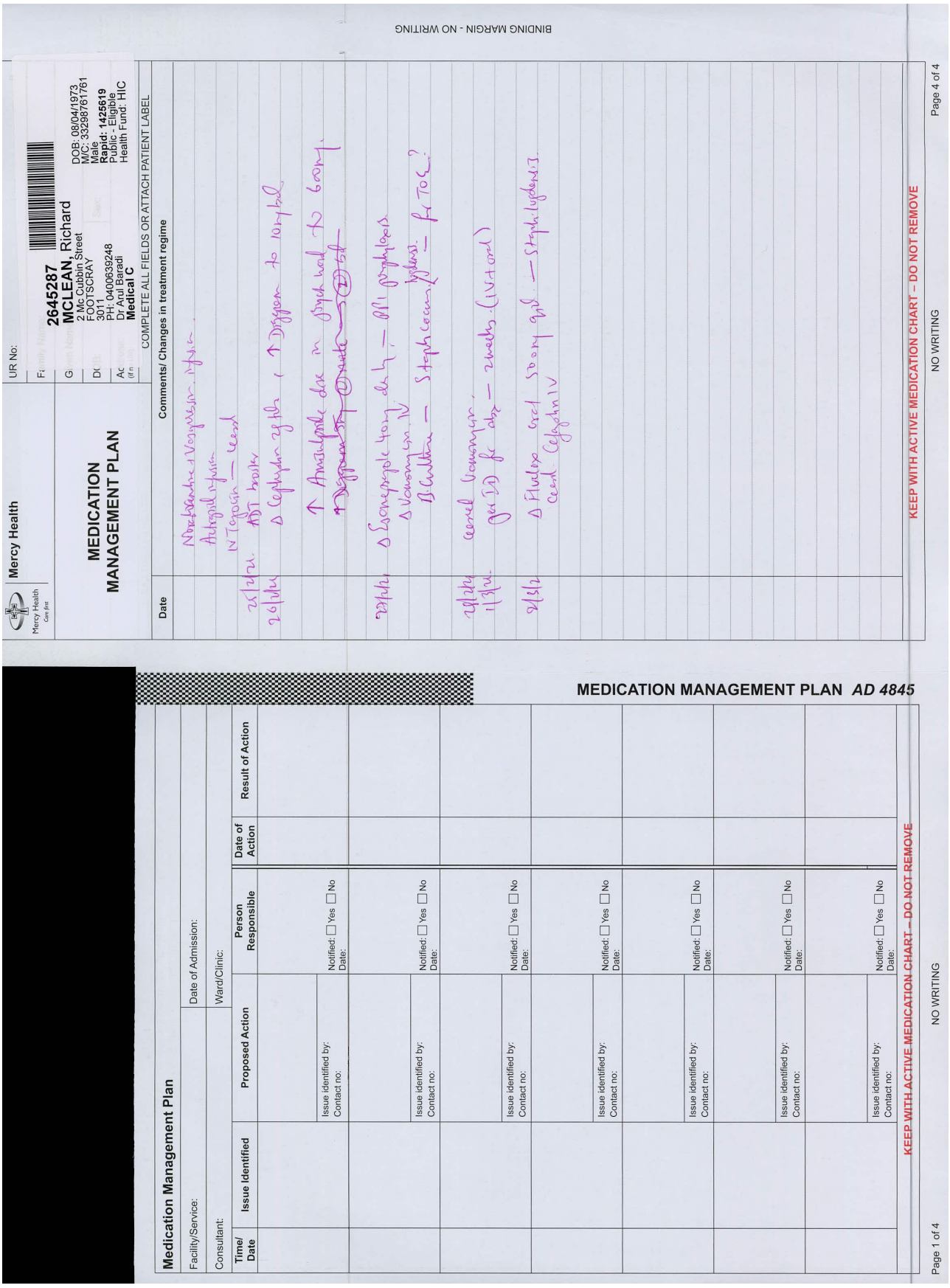
KEEP WITH ACTIVE MEDICATION CHART – DO NOT REMOVE

NO WRITING Page 3 of 4

Mercy Health
Care first

Mercy Health
Care first

5.134. Medication Management Plan (FMH537400)



Mercy Health | UR No:

MEDICATION MANAGEMENT PLAN

Fi
2645287
G **MCLEAN, Richard**
2 Mc Cubbin Street
D(FOOTSCRAY
3011
PH: 0400639248
Ac Dr Arul Baradi
Medical C

DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

Date | Comments/ Changes in treatment regime

Noteboarder Voguesan Infus.
Aduguil infus
IV Tegsan — ceased
26/4/24 ADT booky
26/4/24 Δ Cephalexin 2gm 6 hrly + Diflucan to 100mg
↑ Amiodarone dose in dysrhythmia to 600mg.
↑ Diazepam 5mg to 10mg BD
29/4/24 Δ Esomeprazole from daily — BD 40mg IV
Δ Vancomycin IV
Δ Cultures — Staphylococcus hominis — fr TOC?
29/4/24 ceased Vancomycin.
1/5/24 Δ Levofloxacin 750mg (IV+oral) 48hrly for 4/52 — 2 weeks (IV+oral)
9/5/24 Δ Flucloxacillin 2gm 6hrly — Staphylococcus hominis
ceased Cefalexin IV

KEEP WITH ACTIVE MEDICATION CHART – DO NOT REMOVE
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Page 4 of 4

BINDING MARGIN - NO WRITING

MEDICATION MANAGEMENT PLAN *AD 4845*

Medication Management Plan

Facility/Service: | Date of Admission:
Consultant: | Ward/Clinic:

Time/ Date	Issue Identified	Proposed Action	Person Responsible	Date of Action	Result of Action
		Issue identified by: Contact no:	Notified: ☐ Yes ☐ No Date:		
		Issue identified by: Contact no:	Notified: ☐ Yes ☐ No Date:		
		Issue identified by: Contact no:	Notified: ☐ Yes ☐ No Date:		
		Issue identified by: Contact no:	Notified: ☐ Yes ☐ No Date:		
		Issue identified by: Contact no:	Notified: ☐ Yes ☐ No Date:		
		Issue identified by: Contact no:	Notified: ☐ Yes ☐ No Date:		
		Issue identified by: Contact no:	Notified: ☐ Yes ☐ No Date:		

KEEP WITH ACTIVE MEDICATION CHART – DO NOT REMOVE
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Page 1 of 4

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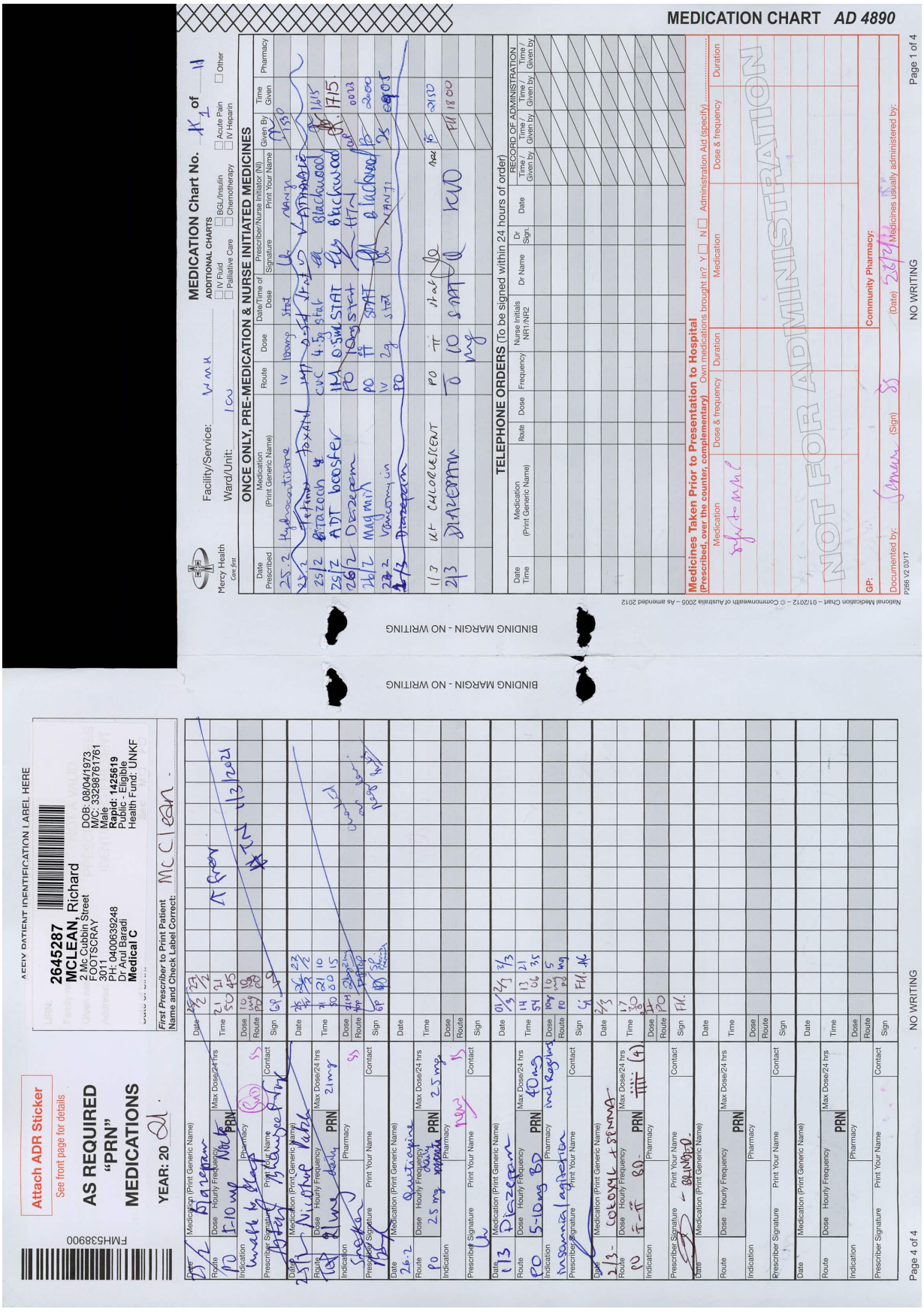
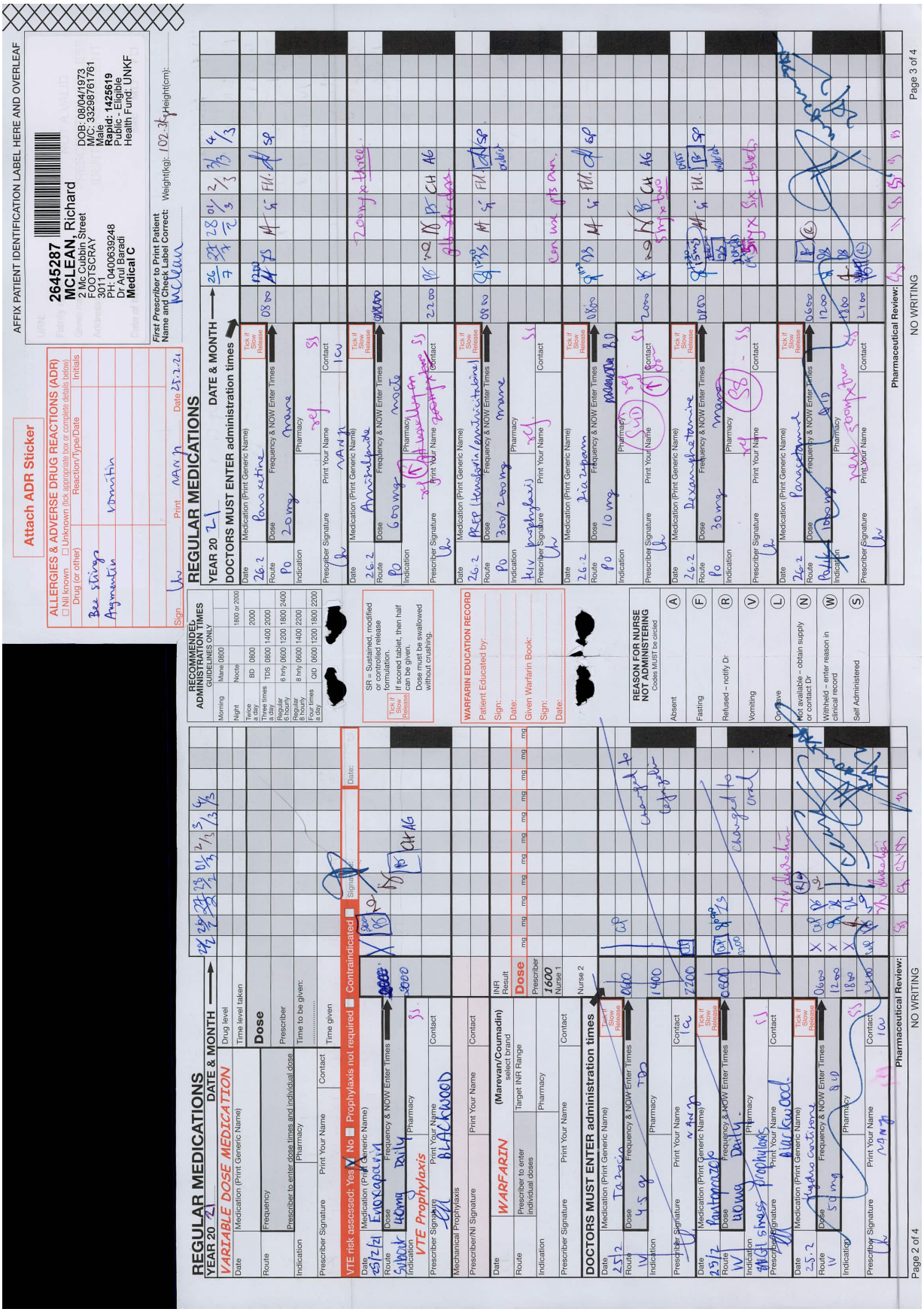
5.135. Medication Chart Adult General (FMH538900)



5.136. Medication Chart Adult General (FMH538900)



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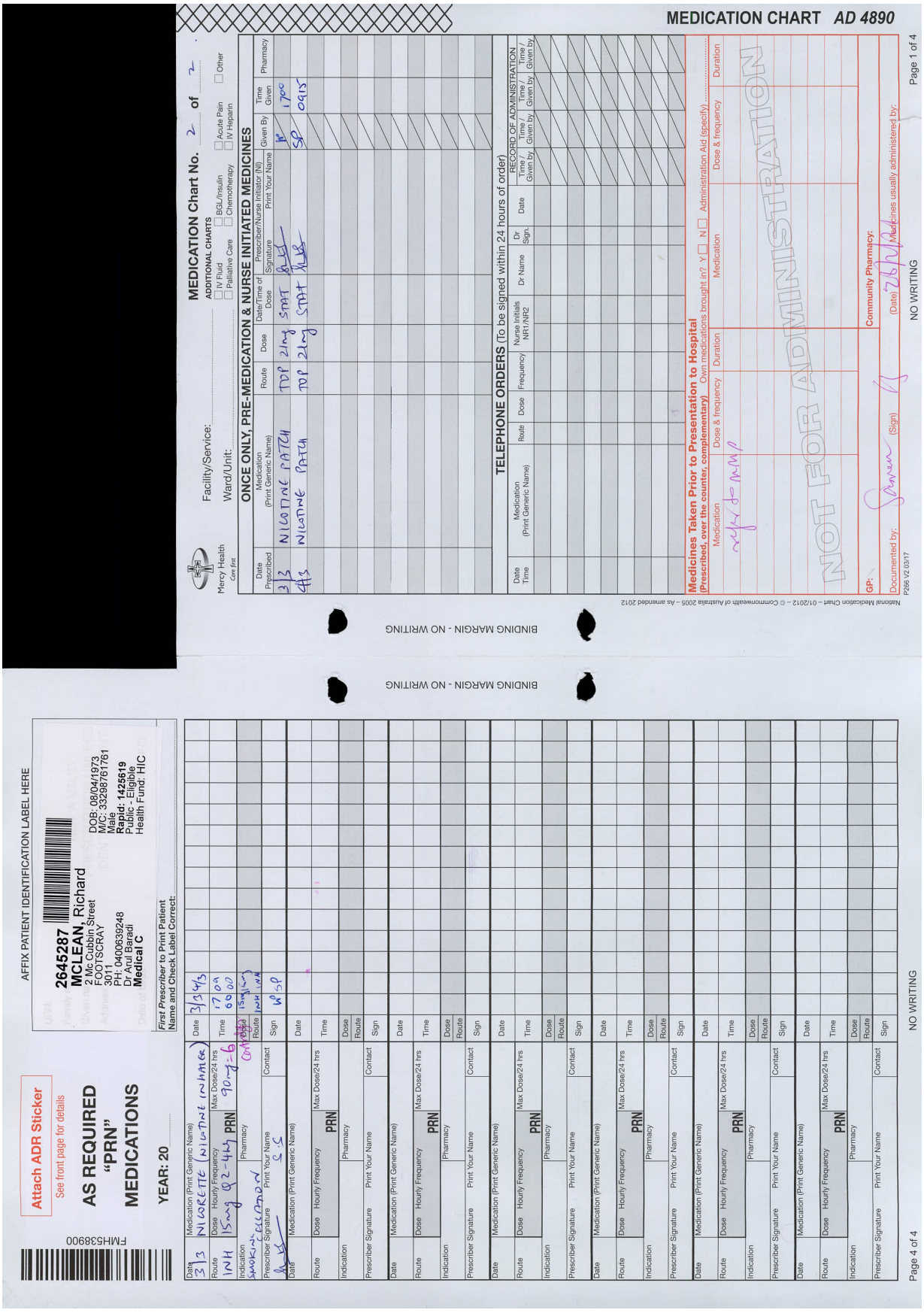
Care first

5.137. Medication Chart Adult General (FMH538900)



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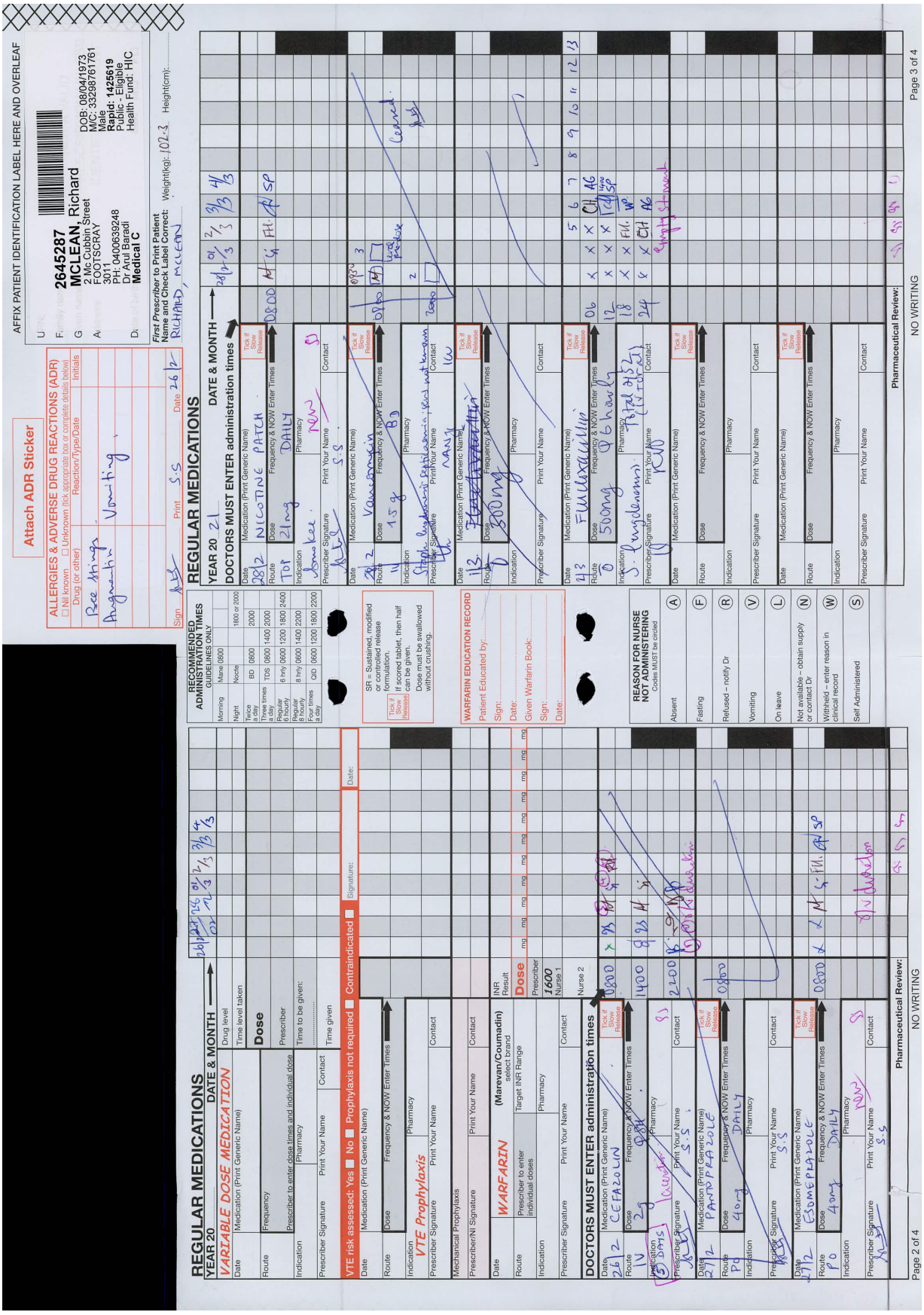
Mercy Health
Care first

5.138. Medication Chart Adult General (FMH538900)



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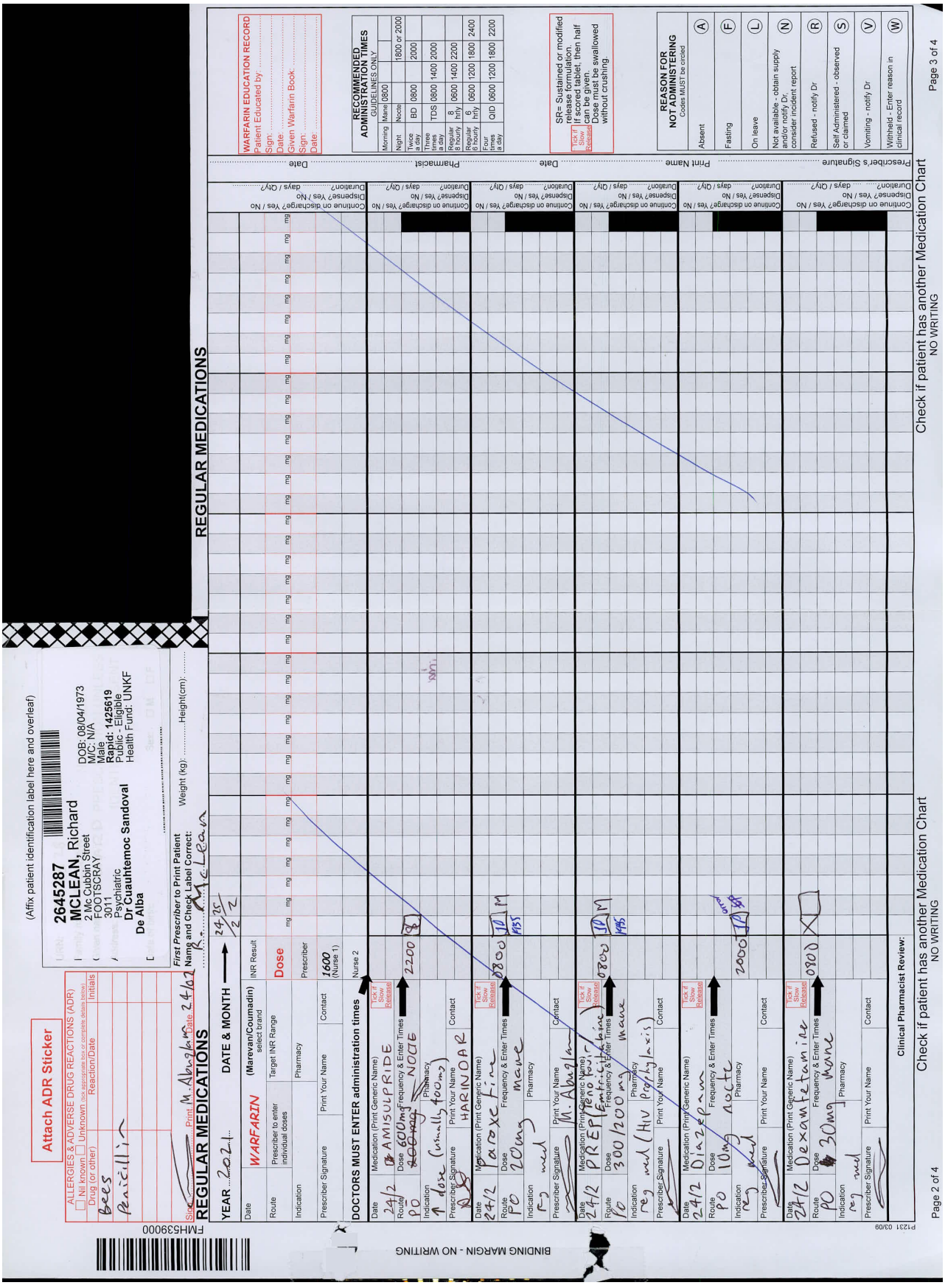
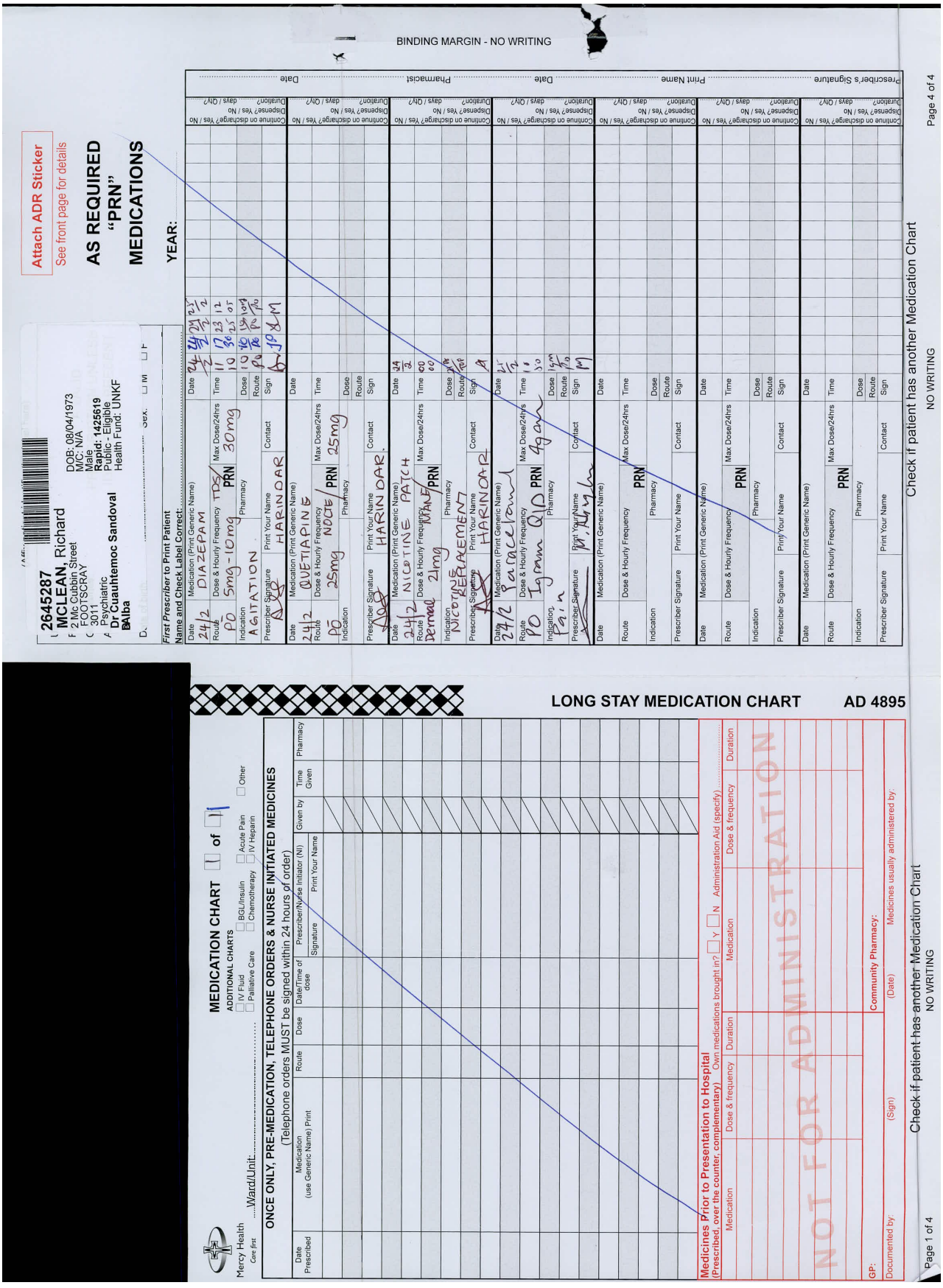
5.139. Medication Chart Adult Long Stay (FMH539000)



Mercy Health
Care first

5.140. Medication Chart Adult Long Stay (FMH539000)



LONG STAY MEDICATION CHART AD 4895

MEDICATION CHART

AS REQUIRED "PRN" MEDICATIONS

Attach ADR Sticker

See front page for details

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
Psychiatric
Dr Cuauhtemoc Sandoval
Alba

DOB: 08/04/1973
M/C: N/A
Male
Rapid: 1425619
Public - Eligible
Health Fund: UNKF

First Prescriber to Print Patient Name and Check Label Correct:

YEAR:

Medication (Print Generic Name): DIAZEPAM
Dose & Hourly Frequency: 5mg-10mg PRN
Max Dose/24hrs: 30mg
Indication: AGITATION
Prescriber Signature / Print Your Name: HARINDAR
Pharmacy / Contact

Medication (Print Generic Name): QUETIAPINE
Dose & Hourly Frequency: 25mg NOCTE / PRN
Max Dose/24hrs: 25mg
Indication:
Prescriber Signature / Print Your Name: HARINDAR
Pharmacy / Contact

Medication (Print Generic Name): NICOTINE PATCH
Dose & Hourly Frequency: Patch 21mg MANE / PRN
Max Dose/24hrs:
Indication: NICOTINE REPLACEMENT
Prescriber Signature / Print Your Name: HARINDAR
Pharmacy / Contact

Medication (Print Generic Name): Paracetamol
Dose & Hourly Frequency: 1gm QID PRN
Max Dose/24hrs: 4gm
Indication: Pain
Prescriber Signature / Print Your Name: M. Ng
Pharmacy / Contact

Date / Time / Dose / Route / Sign / Continue on discharge? Yes / No / Duration? days / Qty?

Prescriber's Signature / Print Name / Date / Pharmacist

Check if patient has another Medication Chart
NO WRITING

Page 4 of 4

BINDING MARGIN - NO WRITING

MEDICATION CHART [ ] of [ ]
ADDITIONAL CHARTS: IV Fluid, Palliative Care, BGL/Insulin, Chemotherapy, Acute Pain, IV Heparin, Other
Ward/Unit:

ONCE ONLY, PRE-MEDICATION, TELEPHONE ORDERS & NURSE INITIATED MEDICINES
(Telephone orders MUST be signed within 24 hours of order)

Date Prescribed	Medication (use Generic Name) Print	Route	Dose	Date/Time of dose	Prescriber/Nurse Initiator (NI) Signature	Prescriber/Nurse Initiator (NI) Print Your Name	Given by	Time Given	Pharmacy

Medicines Prior to Presentation to Hospital
(Prescribed, over the counter, complementary) Own medications brought in? Y N Administration Aid (specify)

Medication	Dose & frequency	Duration	Medication	Dose & frequency	Duration

GP: Community Pharmacy:
Documented by: (Sign) (Date) Medicines usually administered by:

Check if patient has another Medication Chart
NO WRITING

Page 1 of 4

NOT FOR ADMINISTRATION

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5.141. CPF Inpatient Checklist (FMH539600)

Mercy Health
Care first

Mercy Health

2645287
MCLEAN, Richard
2 Mc Cubbin Street
FOOTSCRAY
3011
PH: 0400639248
Dr Arul Baradi
Medical C
DOB: 08/04/1973
M/C: 33298761761
Male
Rapid: 1425619
Public - Eligible
Health Fund: HIC

CPF INPATIENT CHECKLIST

COMPLETE ALL FIELDS OR ATTACH PATIENT LABEL

FMH539600

BINDING MARGIN - NO WRITING

Discharge Information

Ward: ICU

Discharge Date: 4/3/21

Discharge Summary Completed: Yes ☑ No ☐

Statistical Discharge: Yes ☑ No ☐

Discharge Summary Faxed: Yes ☐ No ☑

To Ward: Clare Moore

No GP

Length of Stay > 10 days: Yes ☐ No ☑

Planned Follow-up

Planned Re-admission: Yes ☐ No ☑

Expected Date: _____

Outpatient Appointment:

Planned: Yes ☐ No ☑

OR

Expected: Yes ☐ No ☑

Date (if known): _____

48 Hours ☐ 1 week ☐ 2 weeks ☐

Compensable Status

Is the patient a private or ineligible patient: Yes ☐ No ☑

Record Compilation

All paperwork removed from folders and trays for this patient? Yes ☑ No ☐

Patient Labels on every page? Yes ☑ No ☐

Compiled by: Rebecca

Date Compiled: 4/3/21

HIS USE ONLY

Date Received: HIS Received 4 MAR 2021

Stamp here

Prepped by: Sarah

Date Prepped: 5-3-2021

CPF INPATIENT CHECKLIST AD 4925

P2801 02/20

NO WRITING

Page 1 of 1

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